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1. [Pulmonary Amyloidosis as a Differential Diagnosis in Interstitial Lung Diseases](#)

Cabrera-Cesar E., Garcia J.L., de Pedro I.P.

Open Respirat. Arc. 2026 8:1

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Abstract

Pulmonary amyloidosis is an uncommon condition that can mimic interstitial lung diseases (ILDs), leading to diagnostic delays. We describe two cases of diffuse alveoloseptal amyloidosis initially misdiagnosed as ILD. The first involved a 47-year-old woman with systemic AL amyloidosis, successfully treated with CyBorD (Cyclophosphamide, Bortezomib, Dexamethasone) and autologous stem cell transplantation. The second case was a 72-year-old man with localized pulmonary amyloidosis, managed empirically with azathioprine and prednisone, showing slow progression but clinical stability. Histological confirmation using Congo red staining and immunohistochemistry was essential for diagnosis, and systemic disease was excluded through a comprehensive workup. These cases highlight the importance of considering pulmonary amyloidosis in the differential diagnosis of ILDs, especially in patients with unexplained interstitial patterns and monoclonal gammopathy. A multidisciplinary approach is crucial to avoid inappropriate treatment and ensure timely management.

Original (Non-English) Title

Amiloidosis pulmonar como diagnóstico diferencial en las enfermedades pulmonares intersticiales

Drug Index Terms

(Based on full text)

azathioprine (drug therapy, special situation for pharmacovigilance), bortezomib (drug therapy), congo red, cyclophosphamide (drug therapy), dexamethasone (drug therapy), immunoglobulin lambda chain (endogenous compound), prednisone (drug therapy, special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

adult, aged, AL amyloidosis (drug therapy), alveolar septal amyloidosis (diagnosis), article, autologous stem cell transplantation, bone marrow biopsy, case report, clinical article, differential diagnosis, disease exacerbation, echocardiography, electrocardiogram, ex-smoker, female, histology, human, human tissue, immunohistochemistry, interstitial lung disease (diagnosis), low back pain (diagnosis), lung biopsy, lung function, lung lavage, lung parenchyma, male, mediastinoscopy, monoclonal immunoglobulinemia (diagnosis), multidisciplinary care, positron emission tomography-computed tomography, pulmonary amyloidosis (diagnosis, drug therapy, surgery), thorax radiography, video assisted thoracoscopic surgery

Comment: Invalid ICSR

Additional Information

Language of Article	English, Spanish
Language of Summary	English
Publisher Item Identifier	S2659663625001304
Publisher Item Identifier	S2659663625001304
Digital object identifier(DOI)	10.1016/j.opresp.2025.100506
Article Number	100506

Number of References

6

CAS Registry Numbers

azathioprine ([446-86-6](#), [55774-33-9](#))

bortezomib ([179324-69-7](#),
[197730-97-5](#),
[444576-08-3](#))

congo red ([573-58-0](#), [80701-77-5](#))

cyclophosphamide ([50-18-0](#),
[6055-19-2](#))

dexamethasone ([50-02-2](#))

prednisone ([53-03-2](#))

2. [A comparative study on the efficacy of dexketoprofen and methylprednisolone in the treatment of acute low back pain](#)

Meral M., Gur A.

BMC Emerg. Med. 2025 25:1

Embase, MEDLINE, NURSING

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Abstract

Background: Many methods have been used in to treat low back pain. In this study, we aimed to investigate the efficacy of dexketoprofen alone and in combination with methylprednisolone in the treatment of low back pain in the emergency department using the Visual Analog Scale (VAS) and the Oswestry Disability Index (ODI). Methods: This prospective, randomized, single-blind study included 150 patients admitted to the emergency department of a university hospital. Patients with low back pain were divided in to two groups: those receiving intravenous dexketoprofen treatment (Group D) and those receiving dexketoprofen + methylprednisolone treatment (Group DM). The efficacy of the

treatments received by the patients was evaluated with the VAS at minutes 0, 15, 30, and 60 and hour 48. Statistical evaluations were also undertaken on the ODI results evaluated at minute 0 and hour 48. Results: The VAS scores decreased in both groups at all follow-up evaluations performed throughout the treatment ($p = 0.000$). The mean VAS score evaluated 48 h after treatment was 1.69 ± 1.71 (Median:1) cm in Group DM and 4.13 ± 2.27 (Median:4) cm in Group D ($p = 0.000$). The decrease in the ODI score was greater in Group DM than in Group D ($p = 0.000$). Conclusion: Dexketoprofen showed analgesic efficacy in the treatment of non-traumatic low back pain and decreased disability. When combined with methylprednisolone, dexketoprofen treatment exhibits a greater analgesic effect and further reduced disability. Trial registration: Current Controlled Trials NCT06932367 (Retrospectively registered). Clinical trial number: Not applicable.

Drug Index Terms

(Based on full text)

arveles, **dexketoprofen** (clinical trial, drug combination, drug comparison, drug interaction, drug therapy, intravenous drug administration, pharmacology, special situation for pharmacovigilance), **methylprednisolone** (clinical trial, drug combination, drug comparison, drug interaction, drug therapy, pharmacology, special situation for pharmacovigilance), prednisone (clinical trial, drug combination, drug comparison, drug interaction, drug therapy, pharmacology, special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

adult, aged, analgesia, article, **comparative effectiveness**, comparative study, controlled study, disability, drug effect, **drug efficacy**, drug potentiation, emergency ward, female, follow up, hospital admission, human, **low back pain** (drug therapy), major clinical study, male, monotherapy, Oswestry Disability Index, outcome assessment, prospective study, randomized controlled trial, single blind procedure, statistical analysis, treatment response, university hospital, visual analog scale

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title BMC Emerg. Med.

ISSN	1471227X (electronic)
CODEN	BEMMC
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-21 (Full record), 2025-08-12 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Turkey
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1186/s12873-025-01276-y
MEDLINE PMID	40765043
Article Number	147
Number of References	33
Drug Tradenames	arvels, precort
CAS Registry Numbers	dexketoprofen (22161-81-5) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2)
Clinical Trial Numbers	ClinicalTrials.gov (NCT06932367)

3. [Focal Neurogenic Muscle Hypertrophy and Focal Neurogenic Myositis](#)

Felice K.J., Whitaker C.H., Pesce W.J., Wu Q.
Muscle Nerve 2025 72:6 (1208-1222)
Embase, MEDLINE

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Abstract

Focal neurogenic muscle hypertrophy (F-NMH) and focal myositis from neurogenic causes (F-NM) are terms used to describe the paradoxical changes in skeletal muscle following injury to peripheral motor axons. Since the first clinicopathologic description in 1978, over 100 patients with F-NMH and F-NM have been reported. We reviewed the medical records and clinicopathological data on our series of 6 patients from 2007 to 2025, and then expanded the review to include all patients reported in case studies or series with well-defined F-NMH or F-NM based electrodiagnostic studies, muscle histopathology, or both. Inclusive of our 6 patients, 94 patients were reviewed. Most presented with unilateral calf muscle hypertrophy followed in frequency by involvement of the trapezius, biceps brachii, tibialis anterior, and tensor fascia lata. Radiculopathy was the most common injury site. Electrodiagnostic studies showed a mixture of ongoing and chronic changes of denervation and reinnervation. Complex repetitive discharges were observed in 50%. Magnetic resonance imaging showed focal muscle hypertrophy with variable signal changes. Muscle biopsies showed mixed myopathic and neurogenic changes with 39% having inflammation. During the period of observation, 37 improved including 7 without treatment, and none worsened. Botulinum toxin and oral prednisone were the most efficacious treatments. F-NMH is a rare, non-progressive, and seemingly benign paradoxical reaction to peripheral nerve injury. The cause is unknown but likely multifactorial based on clinical observations and experimental studies. Muscle inflammation is not a consistent finding, does not appear to alter the clinical course or prognosis, and may be an epiphenomenon.

Drug Index Terms

(Based on full text)

analgesic agent (drug therapy), botulinum toxin (drug therapy), immunoglobulin (drug therapy, intravenous drug administration), prednisone (drug therapy, oral drug administration)

Non-drug Index Terms

(Based on full text)

adolescent, adult, aged, biceps brachii muscle, child, clinical feature, disease course, fascia lata, female, **focal myositis** (drug therapy), focal neurogenic muscle hypertrophy (drug therapy), focal neurogenic myositis (drug therapy), histopathology, human, human tissue, inflammation,

major clinical study, male, muscle biopsy, **muscle hypertrophy** (drug therapy), myopathy, nuclear magnetic resonance imaging, peripheral nerve injury, radiculopathy, reinnervation, review, skeletal muscle, tensor fascia lata muscle, tibialis anterior muscle, trapezius muscle, triceps surae muscle

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Muscle Nerve
ISSN	10974598 (electronic), 0148639X
CODEN	MUNED
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025 11-20 (Full record), 2025-09-04 (Article in Press/In process)
Publication Type	Review
Page Range	1208-1222
Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1002/mus.70008
MEDLINE PMID	40878394
Number of References	104
CAS Registry Numbers	
immunoglobulin	(9007-83-4)
prednisone	(53-03-2)

4. [Good response with nivolumab and cabozantinib combination therapy in patients with metastatic collecting duct carcinoma with high expression of PD-L1, c-MET, and AXL](#)

Umemoto T., Hasegawa M., Naruse J., Kano T., Nakajima N., Nitta M., Kawamura Y., Kajiwara H., Shoji S.

CEN Case Rep. 2025 14:6 (857-862)

Embase, MEDLINE, NURSING

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Abstract

Collecting duct carcinoma (CDC) is a rare subtype of renal cell carcinoma with a poor prognosis. Moreover, despite various chemotherapeutic strategies and administration of several tyrosine kinase inhibitors for metastatic CDC, the outcomes remain unfavorable, with no established treatment. Herein, we report the cases of two patients with CDC who exhibited a good response to nivolumab and cabozantinib combination therapy. Both patients were diagnosed with CDC via a needle biopsy of the renal tumor, revealing high expression levels of programmed death ligand 1 (PD-L1), c-MET, and AXL. After 10 and 12 courses of combination therapy for Cases 1 and 2, respectively, significant response was observed against the primary and metastatic lesions. Subsequently, the patients underwent laparoscopic nephrectomy. To the best of our knowledge, this is the first report documenting the favorable therapeutic response of nivolumab and cabozantinib combination therapy against metastatic CDC in patients with high expression of the corresponding molecular targets. These findings may have a strong implication in the selection of first-line systemic therapies for metastatic CDC.

Drug Index Terms

(Based on full text)

cabozantinib (adverse drug reaction, drug combination, drug therapy, oral drug administration, special situation for pharmacovigilance), heparin calcium (drug therapy, special situation for pharmacovigilance), **nivolumab** (adverse drug reaction, drug combination, drug therapy, special situation for pharmacovigilance), prednisone (drug therapy, special situation for pharmacovigilance), **programmed death 1 ligand 1**, uroplakin, vimentin

Non-drug Index Terms

(Based on full text)

abdominal mass, adult, aged, anticoagulant therapy, article, autoimmune thrombocytopenia (drug therapy, side effect), bone

metastasis, cancer immunotherapy, case report, cerebrovascular accident (drug therapy), clinical article, **collecting duct carcinoma** (diagnosis, drug therapy, surgery), computer assisted tomography, diffusion weighted imaging, female, fibrosis, Hematoxylin and eosin staining, hematuria, human, immunohistochemistry, kyphoplasty, kyphoplasty system, **laparoscopic nephrectomy**, laparoscopic radical nephrectomy, liver metastasis, low back pain, lung metastasis, lymph node metastasis, monotherapy, multiple cycle treatment, necrosis, needle biopsy, paraneoplastic syndrome (drug therapy), pedicle screw fixation device, protein expression, radical nephrectomy, spleen metastasis, staining, stereotactic body radiation therapy, systemic therapy, Trousseau's syndrome (drug therapy), tumor biopsy, tumor cell, tumor proportion score, whole body MRI

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	CEN Case Rep.
ISSN	21924449 (electronic)
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025 11-20 (Full record), 2025-09-16 (Article in Press/In process)
Publication Type	Article
Page Range	857-862
Country of Author	Japan
Country of Source	Japan
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s13730-025-01027-9
MEDLINE PMID	40921965
Number of References	10
Drug Manufacturers	Abcam
CAS Registry Numbers	cabozantinib (942407-59-2 , 1140909-48-3 , 849217-68-1)

heparin calcium ([37270-89-6](#))

nivolumab ([946414-94-4](#))

prednisone ([53-03-2](#))

5. [Rate and Predictors of Urine Culture Positivity Following Single-Dose Antibiotic Administration: a Prospective Single-Center Study](#)

Sror N., Bornstein G., Golan L., Rimbrot D., Barlev Y., Freund O., Tau L., Tchebiner J.Z., Zornitzki L.

Infect. Dis. Ther. 2025 14:12 (2679-2691)

Embase, NURSING

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Abstract

Introduction: International guidelines advocate obtaining urine cultures prior to antibiotic administration in hospitalized patients with suspected urinary tract infection (UTI). However, adherence remains suboptimal. The aim of this study was to evaluate the positivity rate of urine cultures following a single empiric antibiotic dose and to identify factors associated with post-antibiotic culture positivity. Methods: A prospective observational study was conducted in an internal medicine ward between December 2019 and June 2024. Patients diagnosed with UTI who had both pre- and post-antibiotic urine cultures were included. Positivity rates of post-antibiotic urine cultures were assessed. Factors associated with culture positivity were examined using logistic regression. Results: A total of 132 patients were included in the analysis (59% female; median age 79 years). Eighty-eight patients (67%) had a positive post-antibiotic urine culture. The positivity rate in patients with a sensitive and resistant uropathogen to the empirically administered antibiotic was 58% and 100%, respectively. Independent predictors for urine culture positivity included older age (adjusted odds ratio [aOR] 1.04, confidence interval [CI] 1.01-1.07, $p = 0.007$) and recent urinary catheter use in the last 30 days (aOR 14.7, CI 1.66-128, $p = 0.016$). Culture sampling more than 9 h after antibiotic administration was a negative predictor for culture positivity (aOR 0.41, CI 0.18-0.96, $p = 0.041$). Conclusion: Urine culture positivity remains high after a single antibiotic dose in hospitalized patients with UTI. In cases where pre-antibiotic urine cultures are missed, a timely post-antibiotic urine

sampling remains clinically relevant for pathogen identification and appropriate antibiotic selection.

Drug Index Terms

(Based on full text)

amikacin (drug therapy), aminoglycoside (drug therapy), **antibiotic agent** (drug therapy), C reactive protein (endogenous compound), ceftazidime (drug therapy), ceftriaxone (drug therapy), gentamicin (drug therapy), hemoglobin (endogenous compound), metronidazole (drug therapy), prednisone (drug therapy), steroid 5alpha reductase inhibitor (drug therapy), vancomycin (drug therapy)

Non-drug Index Terms

(Based on full text)

adult, aged, arterial pressure, article, bacteremia (diagnosis), clinical assessment, coronavirus disease 2019, cystitis, data analysis software, device material, Diaslide, **drug administration**, dysuria, electronic medical record, female, fever, flank pain, Foley balloon catheter, health care, hospitalization, human, immunosuppressive treatment, laboratory test, major clinical study, male, microbial identification system, nephrolithiasis, nephrostomy, neutrophil, observational study, prostate adenocarcinoma, prostate hypertrophy (drug therapy), pyelonephritis, septic shock (diagnosis), **single drug dose**, SPSS version 28.0, squamous cell carcinoma, Staphylococcus aureus, tertiary health care, urinary catheter, urinary tract infection (drug therapy), **urine culture**, urine culture device, uropathogen, very elderly

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Infect. Dis. Ther.
ISSN	21936382 (electronic), 21938229
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-20 (Full record), 2025-10-07 (Article in Press/In process)
Publication Type	Article
Page Range	2679-2691
Country of Author	Israel

Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s40121-025-01239-7
Number of References	16
Device Tradenames	Diaslide (Novamed, Israel), SPSS version 28.0, VITEK 2 (Merieux, France)
Device Manufacturers	Merieux (France), Novamed (Israel)
CAS Registry Numbers	amikacin (37517-28-5 , 39831-55-5 , 110660-83-8 , 1257517-67-1) C reactive protein (9007-41-4) ceftazidime (72558-82-8 , 73547-61-2 , 78439-06-2) ceftriaxone (73384-59-5 , 74578-69-1 , 104376-79-6) gentamicin (1392-48-9 , 1403-66-3 , 1405-41-0) hemoglobin (9008-02-0) metronidazole (39322-38-8 , 443-48-1 , 69198-10-3) prednisone (53-03-2) vancomycin (1404-90-6 , 1404-93-9)

6. [Summary of Japanese clinical practice guidelines for head and neck cancer - 2025 update edited by the Japan Society for Head and Neck Cancer](#)

Hanai N., Ando M., Ishiki H., Ueda N., Okano S., Okamoto I., Ozawa H., Kano S., Kanda T., Saito Y., Shinomiya H., Takanari K., Tomioka T., Fuseya H., Honma Y., Motegi A., Yasuda K., Kodaira T., Kurita H., Nibu K.-I., Yoshimoto S., Homma A.

Auris Nasus Larynx 2025 52:6 (709-724)

Embase, MEDLINE

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Abstract

The Japanese Clinical Practice Guidelines for Head and Neck Cancer - 2025 Edition were developed to provide clear evidence-based recommendations that reflect recent advancements and evolving clinical needs in head and neck cancer care. Advances in immunotherapy, targeted therapies, surgical techniques, and genomic medicine have expanded treatment options and added new complexities. This edition offers practical, up-to-date guidance to support consistent decision making across diverse clinical scenarios. Compared with the 2022 edition, the guidelines have been substantially expanded to 63 clinical questions (CQs) across 14 categories. A new feature is the inclusion of commentary columns that address clinically important topics not suited to the CQ format, such as special patient populations, novel therapies, and rare malignancies. To reflect the growing role of precision oncology, updated recommendations for molecular-targeted treatments have been added based on specific genomic alterations, in alignment with the current approvals in Japan. While maintaining the structure of previous editions, this edition addresses the changing clinical environment while also helping to inform future discussions and directions in head and neck cancer care.

Drug Index Terms

(Based on full text)

abiraterone (drug combination, drug therapy), apalutamide (drug combination, drug therapy), axitinib (drug therapy), bicalutamide (drug combination, drug therapy), binimetinib (drug combination, drug therapy), cabozantinib (drug combination, drug comparison, drug therapy), carboplatin (drug combination, drug therapy), cetuximab
sarotalocan (adverse drug reaction, drug therapy), cisplatin (drug combination, drug therapy), cyclophosphamide (drug combination, drug therapy), dabrafenib (drug combination, drug therapy), docetaxel (drug combination, drug therapy), doxorubicin (drug combination, drug therapy), encorafenib (drug combination, drug therapy), entrectinib (drug therapy), fluorodeoxyglucose f 18, fluorouracil (drug combination, drug therapy), goserelin (drug combination, drug therapy), imatinib (drug therapy), larotrectinib (drug therapy), lenvatinib (adverse drug reaction, drug combination, drug comparison, drug therapy, unexpected outcome of drug treatment), leuprorelin (drug combination, drug therapy), nivolumab (drug therapy), paclitaxel (drug combination, drug therapy), pembrolizumab (drug combination, drug therapy), placebo, prednisone

(drug combination, drug therapy), selpercatinib (drug combination, drug therapy), sorafenib (adverse drug reaction, drug combination, drug therapy), trametinib (drug combination, drug therapy), trastuzumab (drug combination, drug therapy), vandetanib (adverse drug reaction, drug combination, drug comparison, drug therapy), vemurafenib (drug therapy), vinorelbine tartrate (drug combination, drug therapy), vitamin D (adverse drug reaction, drug therapy)

Non-drug Index Terms

(Based on full text)

acute kidney failure (side effect), adenoid cystic carcinoma (drug therapy), adjuvant chemotherapy, anaplastic thyroid carcinoma (drug therapy), application site pain (side effect), article, bleeding (side effect), brachytherapy, cancer of unknown primary site, cancer prognostic test kit, cancer recurrence, cancer staging, chemoradiotherapy, **clinical practice guideline**, consciousness disorder (side effect), diarrhea (side effect), differentiated thyroid cancer (drug therapy), drug withdrawal, dysphagia (side effect), facial nerve paralysis (complication), fatigue (side effect), fine needle aspiration biopsy, gene mutation, hand foot syndrome (side effect), **head and neck cancer** (drug therapy, radiotherapy, therapy), head and neck squamous cell carcinoma (drug resistance, drug therapy), hemithyroidectomy, human, hypercalcemia (side effect), hypertension (side effect), hypoparathyroidism (complication, drug therapy), hypopharynx cancer (surgery), induction chemotherapy, intensity modulated radiation therapy, larynx cancer (radiotherapy, surgery), larynx edema (side effect), lymph node dissection, maxillary sinus carcinoma, molecularly targeted therapy, multiple cycle treatment, nasopharynx cancer (radiotherapy, surgery), nausea (side effect), near infrared therapy, neck dissection, nuclear magnetic resonance imaging, oral fistula (side effect), oropharyngeal fistula (side effect), oropharynx cancer (drug therapy, radiotherapy, surgery), paranasal sinus cancer, partial glossectomy, particle therapy, pharyngeal edema (side effect), pharyngolaryngectomy, positron emission tomography, preoperative chemotherapy, proteinuria (side effect), QT prolongation (side effect), rash (side effect), salivary gland cancer (diagnosis, radiotherapy), salivary gland carcinoma (drug therapy), sentinel lymph node biopsy, side effect (side effect), skin disease (side effect), skin fistula (side effect), solid tumor (drug therapy), thyroid cancer (radiotherapy, surgery), thyroid medullary carcinoma (drug therapy), thyroid papillary carcinoma (drug therapy), thyroidectomy,

tongue cancer (radiotherapy, surgery), tongue reconstruction, trachea stenosis (side effect)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Auris Nasus Larynx
ISSN	18791476 (electronic), 03858146
CODEN	ANLAD
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-18 (Full record), 2025-11-11 (Article in Press/In process)
Publication Type	Article
Page Range	709-724
Country of Author	Japan
Country of Source	Ireland
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S0385814625001403
Publisher Item Identifier	S0385814625001403
Digital object identifier(DOI)	10.1016/j.anl.2025.09.004
MEDLINE PMID	41124841
Number of References	94
Device Tradenames	Oncomine Dx
Drug Tradenames	herceptin
CAS Registry Numbers	abiraterone (154229-19-3) apalutamide (956104-40-8) axitinib (319460-85-0) bicalutamide (90357-06-5) binimetinib (120168-50-5 , 606143-89-9 , 1073666-70-2) cabozantinib (942407-59-2 , 1140909-48-3 ,

[849217-68-1](#))
carboplatin ([41575-94-4](#))
cetuximab sarotalocan ([2166339-33-7](#))
cisplatin ([15663-27-1](#), [26035-31-4](#), [96081-74-2](#))
cyclophosphamide ([50-18-0](#), [6055-19-2](#))
dabrafenib ([1195765-45-7](#), [1195768-06-9](#))
docetaxel ([114977-28-5](#))
doxorubicin ([23214-92-8](#), [25316-40-9](#))
encorafenib ([1269440-17-6](#))
entrectinib ([1108743-60-7](#))
fluorodeoxyglucose f 18 ([63503-12-8](#))
fluorouracil ([51-21-8](#), [1004-03-1](#))
goserelin ([65807-02-5](#), [145781-92-6](#))
imatinib ([152459-95-5](#), [220127-57-1](#))
larotrectinib ([1223403-58-4](#), [1223405-08-0](#))
lenvatinib ([417716-92-8](#), [857890-39-2](#))
leuprorelin ([53714-56-0](#), [74381-53-6](#),
[944347-41-5](#))
nivolumab ([946414-94-4](#))
paclitaxel ([33069-62-4](#))
pembrolizumab ([1374853-91-4](#))
prednisone ([53-03-2](#))
selpercatinib ([2152628-33-4](#))
sorafenib ([284461-73-0](#), [475207-59-1](#))
trametinib ([1187431-43-1](#), [871700-17-3](#))
trastuzumab ([180288-69-1](#), [1446410-98-5](#))
vandetanib ([338992-00-0](#), [338992-48-6](#),
[443913-73-3](#))
vemurafenib ([918504-65-1](#))
vinorelbine tartrate ([125317-39-7](#), [71486-22-1](#))

7. [Nutritional Management of Acute and Chronic Pancreatitis](#)

Templeton K., Grover A.S.

Gastroenterol. Clin. North Am. 2025 54:4 (823-839)

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Drug Index Terms

(Based on full text)

asparaginase, azathioprine, cotrimoxazole, enalapril, furosemide, mercaptopurine, mesalazine, pancrelipase (drug therapy), prednisone, relizorb, valproic acid

Non-drug Index Terms

(Based on full text)

acute pancreatitis (diagnosis, drug therapy, epidemiology, etiology, therapy), bone disease, **chronic pancreatitis** (diagnosis, drug therapy, epidemiology, etiology, therapy), clinical outcome, clinical research, diabetes mellitus, **diet therapy**, digestion, disease severity, endocrine pancreatic insufficiency (diagnosis, epidemiology, etiology, therapy), enteric feeding, exocrine pancreatic insufficiency (diagnosis, epidemiology, etiology, therapy), human, hypertriglyceridemia, malnutrition, metabolism, nutritional health, pancreatogenic diabetes (diagnosis, epidemiology, etiology, therapy), pathogenesis, pediatric patient, pediatrics, review, risk factor, vitamin deficiency

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Gastroenterol. Clin. North Am.

ISSN 15581942 (electronic), 08898553

CODEN GCNAE

Source Type Journal

Source Publication Date 2025-12-01

Entry Date 2025 11-21 (Full record), 2025-10-09
(Article in Press/In process)

Publication Type Review

Page Range 823-839

Country of Author United States

Country of Source United States

Language of Article English

Publisher Item Identifier	S0889855325000561
Publisher Item Identifier	S0889855325000561
Digital object identifier(DOI)	10.1016/j.gtc.2025.08.005
MEDLINE PMID	41238278
Number of References	90
Drug Tradenames	creon, pancreaze, pertzye, relizorb, viokace, zenpep
CAS Registry Numbers	asparaginase (9015-68-3 , 1349719-22-7) azathioprine (446-86-6 , 55774-33-9) cotrimoxazole (8064-90-2) enalapril (75847-73-3) furosemide (54-31-9) mercaptopurine (31441-78-8 , 50-44-2 , 6112-76-1) mesalazine (89-57-6) pancrelipase (71060-52-1 , 83869-36-7) prednisone (53-03-2) valproic acid (1069-66-5 , 99-66-1)

8. [Pulmonary alveolar proteinosis that improved after a COVID-1G infection: a case report](#)

Xu C., Gao L., Wang X., Hu Z., Que C.

BMC Pulm. Med. 2025 25:1

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Pulmonary alveolar proteinosis (PAP) is a rare disease caused by the accumulation of abnormal surfactant in the alveoli, leading to impaired gas exchange. A 54-year-old woman presented with intermittent wheezing who developed increasing infiltrates gradually on chest CT, was diagnosed with PAP in our department in December 2017. She then underwent granulocyte-macrophage colony-stimulating factor (GM-CSF) inhalation therapy for three months without improvement. Regular

follow-up imaging and pulmonary function tests suggested disease progression. However, the patient declined whole-lung lavage (WLL) due to her unrestricted daily activities. Two months after a coronavirus disease 2019 (COVID-19) infection in February 2023, the patient's chest CT became clear, accompanied by improved pulmonary function. Repeated HRCT of the chest in March 2024 confirmed no signs of recurrence, nor in pulmonary function tests. In this case, COVID-19 infection may have contributed to the improvement of this patient's PAP, raising the hypothesis of an immunomodulatory effect.

Drug Index Terms

(Based on full text)

antinuclear antibody (endogenous compound), granulocyte macrophage colony stimulating factor (drug therapy), immunoglobulin E (endogenous compound), prednisolone, prednisone acetate (oral drug administration)

Non-drug Index Terms

(Based on full text)

adult, allergic rhinitis, article, asthma, blood cell count, blood clot, breathing rate, bronchiectasis, bronchoalveolar lavage fluid, bronchoscopy, case report, clinical article, computer assisted tomography, **coronavirus disease 201G**, coughing, disease exacerbation, disease severity, dyspnea, erythrocyte sedimentation rate, female, fever, follow up, forced expiratory flow, forced expiratory volume, forced vital capacity, gas exchange, ground glass opacity, hospitalization, human, immunoglobulin blood level, inhalational drug administration, interstitial lung disease, interstitial pneumonia, liver and kidney function test, **lung alveolus proteinosis**, lung biopsy, lung function, lung function test, lung infiltrate (drug therapy), lung lavage, macrophage, middle aged, nasopharyngeal swab, oxygen saturation, pneumonia, polymerase chain reaction, Severe acute respiratory syndrome coronavirus 2, thorax radiography, wheezing

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	BMC Pulm. Med.
ISSN	14712466 (electronic)
CODEN	BPMMB
Source Type	Journal

Source Publication Date	2025-12-01
Entry Date	2025-11-18 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	China
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1186/s12890-025-03988-0
MEDLINE PMID	41204279
Article Number	513
Number of References	12
CAS Registry Numbers	immunoglobulin E (37341-29-0) prednisolone (50-24-8) prednisone acetate (125-10-0)

9. [Hydroxychloroquine in the treatment of lichen planus-like cutaneous immune-related adverse events](#)

Mueller K., Bogner P., Kuraitis D.

JAAD Case Rep. 2025 66: (144-151)

Embase, NURSING

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Embase Open URL: [redirect to full text](#)

Drug Index Terms

(Based on full text)

amoxicillin (adverse drug reaction, special situation for pharmacovigilance), diphenhydramine (drug therapy, special situation for pharmacovigilance), hydrocortisone (drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment),

hydroxychloroquine (drug therapy, special situation for

pharmacovigilance), pembrolizumab (adverse drug reaction, drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), prednisone (drug therapy, special situation for pharmacovigilance), triamcinolone (drug therapy, special situation for pharmacovigilance, topical drug administration)

Non-drug Index Terms

(Based on full text)

aged, article, cancer palliative therapy, case report, checkpoint inhibitor therapy, clinical article, colon adenocarcinoma (drug therapy), corticosteroid therapy, cream, drug dose reduction, **drug induced disease** (diagnosis, drug therapy, side effect), epidermis hyperplasia, erythema, female, histopathology, human, human cell, human tissue, **lichen planus like cutaneous immune related adverse event** (diagnosis, drug therapy, side effect), **lichenoid eruption** (diagnosis, drug therapy, side effect), male, measles like rash (drug therapy, side effect), multiple cycle treatment, papule, physical examination, postinflammatory hyperpigmentation, prostate adenocarcinoma (drug therapy), prurigo (drug therapy, side effect), pruritus (drug therapy), psoriasis vulgaris, punch biopsy, skin biopsy

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	JAAD Case Rep.
ISSN	23525126 (electronic)
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-18 (Full record), 2025-11-11 (Article in Press/In process)
Publication Type	Article
Page Range	144-151
Country of Author	United States
Country of Source	United States
Language of Article	English
Publisher Item Identifier	S2352512625005168
Publisher Item Identifier	S2352512625005168

Digital object identifier (DOI) [10.1016/j.jdc.2025.10.002](https://doi.org/10.1016/j.jdc.2025.10.002)

Number of References 12

CAS Registry Numbers amoxicillin ([26787-78-0](#), [34642-77-8](#), [61336-70-7](#))
diphenhydramine ([147-24-0](#), [58-73-1](#))
hydrocortisone ([50-23-7](#))
hydroxychloroquine ([118-42-3](#), [525-31-5](#), [137433-23-9](#), [137433-24-0](#))
pembrolizumab ([1374853-91-4](#))
prednisone ([53-03-2](#))
triamcinolone ([124-94-7](#))

10. [Invasive fungal sinusitis by *Scedosporium boydii* in an immunocompromised patient. A case report](#)

Rey-Suarez C., Caramazana-Lopez P., Fernández-Castro I., Esteban-Cartelle H., Navarro-de la Cruz D., Losada-Arias E.

Med. Mycol. Case Rep. 2025 50:

Embase, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

Invasive fungal sinusitis by *Scedosporium* spp. is a rare but severe infection in immunocompromised patients. We report an 80-year-old woman with poorly controlled diabetes and chronic corticosteroid use who presented with headache, hearing loss, and facial paralysis. She was ultimately diagnosed with sphenoid sinusitis due to *Scedosporium boydii*. Management required surgical debridement and prolonged antifungal therapy complicated by drug-induced liver injury. Early diagnosis and multidisciplinary treatment are crucial to improve prognosis in these complex infections.

Drug Index Terms

(Based on full text)

alanine aminotransferase (endogenous compound), allopurinol (drug therapy, special situation for pharmacovigilance), amphotericin B (adverse drug reaction, drug combination, drug interaction, drug therapy,

drug toxicity, special situation for pharmacovigilance), analgesic agent (special situation for pharmacovigilance), beta glucan (endogenous compound), C reactive protein (endogenous compound), chloramphenicol, corticosteroid (drug therapy, special situation for pharmacovigilance), dapagliflozin plus metformin (drug therapy, special situation for pharmacovigilance), ezetimibe plus simvastatin (adverse drug reaction, drug therapy, special situation for pharmacovigilance), hemoglobin A1c (endogenous compound), insulin (drug therapy, special situation for pharmacovigilance), internal transcribed spacer (endogenous compound), internal transcribed spacer 1 (endogenous compound), internal transcribed spacer 4 (endogenous compound), omeprazole (special situation for pharmacovigilance), prednisone (drug therapy, special situation for pharmacovigilance), procalcitonin (endogenous compound), ribosome DNA (endogenous compound), terbinafine (drug combination, drug interaction, drug therapy, oral drug administration, special situation for pharmacovigilance), unclassified drug, voriconazole (adverse drug reaction, drug combination, drug concentration, drug interaction, drug therapy, drug toxicity, oral drug administration, special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

aged, antifungal susceptibility, antifungal therapy, article, aspergillosis (diagnosis, drug therapy), blood vessel biopsy, case report, clinical article, combination drug therapy, computer assisted tomography, corticosteroid therapy, diabetes mellitus, differential diagnosis, drug blood level, drug dose reduction, drug withdrawal, drug-induced liver injury (side effect), dyslipidemia (drug therapy), ear edema, enzyme activity, facial nerve paralysis, female, filamentous fungus, follow up, **fungal sinusitis** (diagnosis, drug therapy, surgery), genetic analyzer, GenomeLab GeXP, giant cell arteritis (diagnosis, drug therapy), headache, hearing impairment, hospital admission, hospital discharge, human, hypertension, hypertransaminasemia, hyperuricemia (drug therapy), **immunocompromised patient**, insulin treatment, laboratory test, liver toxicity (side effect), loading drug dose, long term care, maintenance drug dose, medical history, migraine, minimum inhibitory concentration, non insulin dependent diabetes mellitus (drug therapy), nonhuman, otorrhea, otoscopy, peripheral paralysis, physical examination, positron emission tomography, **pseudallescheriasis** (diagnosis, drug therapy, surgery), rheumatoid arthritis (drug therapy),

sabouraud dextrose agar, Sanger sequencing, **Scedosporium boydii**, surgical debridement, **systemic mycosis** (diagnosis, drug therapy, surgery), very elderly

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	Med. Mycol. Case Rep.
ISSN	22117539
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Spain
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2211753925000582
Publisher Item Identifier	S2211753925000582
Digital object identifier(DOI)	10.1016/j.mmcr.2025.100749
Article Number	100749
Number of References	8
Device Tradenames	GenomeLab GeXP (Beckman Coulter, United States)
Device Manufacturers	Beckman Coulter, Beckman Coulter (United States)
CAS Registry Numbers	alanine aminotransferase (9000-86-6 , 9014-30-6) allopurinol (315-30-0 , 17795-21-0) amphotericin B (1397-89-3 , 30652-87-0) C reactive protein (9007-41-4) chloramphenicol (134-90-7 , 2787-09-9 , 56-

[75-7\)](#)

ezetimibe plus simvastatin ([444313-53-5](#))

hemoglobin A1c ([62572-11-6](#))

insulin ([9004-10-8](#))

omeprazole ([73590-58-6](#), [95510-70-6](#))

prednisone ([53-03-2](#))

procalcitonin ([56645-65-9](#))

terbinafine ([91161-71-6](#), [78628-80-5](#),

[176168-78-8](#), [78628-81-6](#))

voriconazole ([137234-62-9](#), [188416-29-7](#))

11. [A case of bullous pemphigoid in an ulcerative colitis patient treated with vedolizumab](#)

Odom M.B., Chen J.N., Echavarria J.F., Gilson R.T.

JAAD Case Rep. 2025 66: (176-179)

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Embase Open URL: [redirect to full text](#)

Drug Index Terms

(Based on full text)

autoantibody (endogenous compound), azathioprine (drug therapy, unexpected outcome of drug treatment), bp180 antibody (endogenous compound), budesonide (drug therapy, unexpected outcome of drug treatment), clobetasol (drug therapy, topical drug administration, unexpected outcome of drug treatment), complement component C3 (endogenous compound), doxycycline (drug therapy, unexpected outcome of drug treatment), dupilumab (drug therapy), infliximab (drug therapy), mesalazine (drug therapy, oral drug administration, unexpected outcome of drug treatment), prednisone (drug therapy, unexpected outcome of drug treatment), rituximab (drug therapy, unexpected outcome of drug treatment), unclassified drug, **vedolizumab** (adverse drug reaction, drug therapy)

Non-drug Index Terms

(Based on full text)

adult, article, blister, **bullous pemphigoid** (diagnosis, drug therapy, side effect), bullous skin disease, case report, clinical article, corticosteroid

therapy, drug dose increase, drug dose reduction, drug withdrawal, human, immunofluorescence assay, impetigo (drug therapy), inflammatory infiltrate, leg, limb, male, middle aged, papule, pruritus, rash, skin biopsy, suppository, trunk, **ulcerative colitis** (drug therapy), urticaria

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	JAAD Case Rep.
ISSN	23525126 (electronic)
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-20 (Full record), 2025-11-14 (Article in Press/In process)
Publication Type	Article
Page Range	176-179
Country of Author	United States
Country of Source	United States
Language of Article	English
Publisher Item Identifier	S2352512625005132
Publisher Item Identifier	S2352512625005132
Digital object identifier(DOI)	10.1016/j.jdcr.2025.09.033
Number of References	9
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) budesonide (51333-22-3 , 51372-29-3) clobetasol (25122-41-2) complement component C3 (80295-41-6) doxycycline (10592-13-9 , 17086-28-1 , 564-25-0 , 94088-85-4) dupilumab (1190264-60-8) infliximab (170277-31-3) mesalazine (89-57-6) prednisone (53-03-2)

rituximab ([174722-31-7](#))
vedolizumab ([943609-66-3](#))

12. [Refractory livedoid vasculopathy successfully treated with immunosuppressive therapy](#)

Chaponan-Lavalle A., Cortegana C.Q., Estrada-Grossmann J.M.,
Mena E.Q., Diaz N., Robinson G.R.

JAAD Case Rep. 2025 66: (180-183)

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Drug Index Terms

(Based on full text)

antibiotic agent (drug therapy, unexpected outcome of drug treatment),
hydroxychloroquine (drug combination, drug therapy), prednisone (drug
combination, drug therapy, unexpected outcome of drug treatment),
rivaroxaban (unexpected outcome of drug treatment)

Non-drug Index Terms

(Based on full text)

adult, analgesia, article, burning sensation, case report, cellulitis (drug
therapy), clinical article, drug withdrawal, female, foot ulcer,
histopathology, human, human cell, human tissue, **immunosuppressive
treatment**, leg edema, **livedoid vasculopathy** (drug therapy), low drug
dose, medial malleolus, physical examination, skin biopsy,
thrombocytosis (diagnosis), weakness

Comment: valid ICSR

Abbreviated Journal Title	JAAD Case Rep.
ISSN	23525126 (electronic)
Source Type	Journal
Source Publication Date	2025-12-01
Entry Date	2025-11-21 (Full record), 2025-11-17 (Article in Press/In process)

Publication Type	Article
Page Range	180-183
Country of Author	Peru
Country of Source	United States
Language of Article	English
Publisher Item Identifier	S2352512625005399
Publisher Item Identifier	S2352512625005399
Digital object identifier (DOI)	10.1016/j.jdc.2025.10.012
Number of References	7
CAS Registry Numbers	hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) prednisone (53-03-2) rivaroxaban (366789-02-8)

13. [Molecular Characterization of Pericardial Mesothelioma With Constrictive and Recurrent Pericarditis: TAZ Amplification and Hippo Pathway](#)

Tudino R., Smith S., Cheng L., French K., Azzoli C.

JACC: Case Reports 2025 30:38

Embase

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Abstract

Background: Malignant pericardial mesothelioma (MPM) is a rare, aggressive neoplasm with nonspecific clinical presentations. Although BAP1 loss and CDKN2A deletion are common in pleural mesothelioma, they are not universally present in MPM, complicating diagnosis. Case Summary: A 73-year-old man presented with recurrent fever and chest pain from pericarditis. Positron emission tomography-computed tomography and transthoracic echocardiography revealed pericardial thickening and aortitis, concerning for inflammatory or infectious etiology. Initial histopathology was inconclusive. Diagnosis of MPM was

confirmed via lymph node biopsy and molecular profiling, which revealed TAZ amplification. The patient was treated with IL-1 blockage and chemotherapy, with symptom improvement. Discussion: In BAP1/CDKN2A-negative mesothelial proliferations, molecular testing for additional mutations, including TAZ amplification, may guide diagnostic certainty and novel therapeutic treatment. Take-Home Messages: MPM can closely mimic chronic inflammatory disease, and accurate diagnosis with comprehensive profiling is essential to guide targeted therapy. TAZ amplification suggests Hippo pathway as a driver of BAP-intact tumors and may serve as a predictive biomarker of mesothelial immunogenicity.

Drug Index Terms

(Based on full text)

apixaban (drug therapy, special situation for pharmacovigilance), C reactive protein (endogenous compound), calretinin (endogenous compound), colchicine (endogenous compound), creatinine (endogenous compound), cryoglobulin (endogenous compound), D dimer (endogenous compound), ethambutol (drug therapy, special situation for pharmacovigilance), gamma interferon (endogenous compound), hemoglobin (endogenous compound), HLA B51 antigen (endogenous compound), immunoglobulin G4 (endogenous compound), isoniazid (drug therapy, special situation for pharmacovigilance), potassium (endogenous compound), prednisone (drug therapy, special situation for pharmacovigilance), prostate specific antigen (endogenous compound), pyrazinamide (drug therapy, special situation for pharmacovigilance), pyridoxine (drug therapy, special situation for pharmacovigilance), rifabutin (drug therapy, special situation for pharmacovigilance), sodium (endogenous compound), tocilizumab (drug therapy, special situation for pharmacovigilance), **transcriptional coactivator with PDZ binding motif protein** (endogenous compound)

Non-drug Index Terms

(Based on full text)

aged, aortitis (drug therapy), article, case report, cell proliferation, chill, clinical article, **constrictive pericarditis**, differential diagnosis, erythrocyte sedimentation rate, fatigue, fever, fluorescence in situ hybridization, follow up, gene mutation, **hippo signaling**, human, human cell, human tissue, hypertension, immunohistochemistry, large vessel vasculitis (drug therapy), leukocyte count, liver function test, lung tuberculosis (drug therapy), lymph node biopsy, male, molecular

fingerprinting, night sweat, paroxysmal atrial fibrillation (drug therapy), pericardial effusion, **pericardial mesothelioma**, platelet count, positron emission tomography-computed tomography, prostate hypertrophy, thorax pain, tobacco dependence, transthoracic echocardiography

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	JACC: Case Reports
ISSN	26660849 (electronic)
Source Type	Journal
Source Publication Date	2025-11-26
Entry Date	2025-11-21 (Full record), 2025-10-23 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2666084925025422
Publisher Item Identifier	S2666084925025422
Digital object identifier(DOI)	10.1016/j.jaccas.2025.105759
Article Number	105759
Number of References	10
CAS Registry Numbers	apixaban (503612-47-3) C reactive protein (9007-41-4) colchicine (64-86-8) creatinine (19230-81-0 , 60-27-5) ethambutol (10054-05-4 , 1070-11-7 , 3577-94-4 , 74-55-5) gamma interferon (82115-62-6) hemoglobin (9008-02-0) isoniazid (54-85-3 , 62229-51-0 , 6597 9-32-0)

potassium ([7440-09-7](#))
prednisone ([53-03-2](#))
pyrazinamide ([98-96-4](#))
pyridoxine ([12001-77-3](#), [58-56-0](#), [65-23-6](#),
[8059-24-3](#))
rifabutin ([72559-06-9](#))
sodium ([7440-23-5](#))
tocilizumab ([375823-41-9](#))

14. [Smoldering Myocarditis Unmasked by Pembrolizumab-Induced Myositis: Delayed-Onset Double M Syndrome](#)

Mark J.D., Tapia-Orihuela R.K.A., Llanos A., Charnick B.L., Stone J.R., Muniz J., Collado E.

JACC: Case Reports 2025 30:38

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Abstract

Background: Immune checkpoint inhibitors (ICIs) have revolutionized cancer therapy but can trigger immune-related adverse events, including rare, variable double M syndromes combining myocarditis and myositis. Case Presentation: A 67-year-old woman with stage IIb triple-negative breast cancer treated per KEYNOTE-522 protocol presented 4 months after ICI use with worsening myalgias, weakness, low-grade fever, and transaminitis. Laboratory results showed creatine kinase 7,760 U/L, aspartate aminotransferase 369 U/L, alanine transaminase 626 U/L, high-sensitivity cardiac troponin I >15,000 ng/L, and creatine kinase-MB 185 ng/mL. Electrocardiogram revealed sinus tachycardia with diffuse ST-segment depressions. Transthoracic echocardiography showed preserved left ventricular ejection fraction. Coronary angiography ruled out obstructive disease. Endomyocardial biopsy confirmed ICI-associated myocarditis with CD8+ T-cell infiltrates and PD-L1 overexpression. She was treated with pulse-dose steroids and a prednisone taper with multidisciplinary follow-up. Advanced imaging 2 months later showed complete resolution. Discussion: ICI-related double M syndromes are rare, high-risk, and typically present acutely. This delayed, smoldering case highlights the need for recognizing

subacute presentations. Take-Home Message: Timely recognition and use of multimodal imaging in ICI-related double M syndromes, even in delayed or asymptomatic cases, are essential for improving outcomes.

Drug Index Terms

(Based on full text)

acetylsalicylic acid (drug therapy, special situation for pharmacovigilance), alanine aminotransferase (endogenous compound), alkaline phosphatase (endogenous compound), aspartate aminotransferase (endogenous compound), bilirubin (endogenous compound), biologic factors and agents acting on the immune system (endogenous compound), brain natriuretic peptide (endogenous compound), C reactive protein (endogenous compound), carboplatin (drug combination, drug therapy, special situation for pharmacovigilance), creatine kinase (endogenous compound), cyclophosphamide (drug combination, drug therapy, special situation for pharmacovigilance), doxorubicin (drug combination, drug therapy, special situation for pharmacovigilance), epidermal growth factor receptor 2 (endogenous compound), glucocorticoid (drug therapy, special situation for pharmacovigilance), heparin (drug therapy, special situation for pharmacovigilance), immune checkpoint inhibitor (drug therapy, special situation for pharmacovigilance), lactate dehydrogenase (endogenous compound), methylprednisolone (drug dose, drug therapy, special situation for pharmacovigilance), paclitaxel (drug combination, drug therapy, special situation for pharmacovigilance), **pembrolizumab** (drug therapy, special situation for pharmacovigilance), prednisone (drug dose, drug therapy, oral drug administration, special situation for pharmacovigilance), programmed death 1 ligand 1 (endogenous compound), steroid (drug therapy, special situation for pharmacovigilance), troponin I (endogenous compound)

Non-drug Index Terms

(Based on full text)

acute coronary syndrome, aged, article, blood cell count, cancer therapy, cardiac muscle cell, case report, CD8+ T lymphocyte, cholecystectomy, clinical article, computer assisted tomography, coronary angiography, coronary artery dissection, coughing, drug withdrawal, electrocardiogram, emergency ward, erythrocyte sedimentation rate, female, fever, headache, heart catheterization, heart left ventricle ejection fraction, heart muscle biopsy, hemodynamics, human,

hypertransaminasemia, liver function test, lymphocytopenia, muscle weakness, myasthenia gravis, **myocarditis** (drug therapy), myoglobinuria, **myositis**, neoadjuvant chemoimmunotherapy, non-obstructive coronary artery disease, normochromic normocytic anemia, **pembrolizumab induced myositis**, physical examination, polymerase chain reaction, positron emission tomography-computed tomography, radical mastectomy, sinus tachycardia, smoldering myocarditis (drug therapy), ST segment depression, symmetric proximal muscle weakness, Takotsubo cardiomyopathy, transthoracic echocardiography, triple negative breast cancer

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	JACC: Case Reports
ISSN	26660849 (electronic)
Source Type	Journal
Source Publication Date	2025-11-26
Entry Date	2025-11-21 (Full record), 2025-11-10 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2666084925018492
Publisher Item Identifier	S2666084925018492
Digital object identifier(DOI)	10.1016/j.jaccas.2025.105069
Article Number	105069
Number of References	10
CAS Registry Numbers	acetylsalicylic acid (493-53-8 , 50-78-2 , 53663-74-4 , 53664-49-6 , 63781-77-1) alanine aminotransferase (9000-86-6 , 9014-30-6)

alkaline phosphatase ([9001-78-9](#))
aspartate aminotransferase ([9000-97-9](#))
bilirubin ([18422-02-1](#), [635-65-4](#))
brain natriuretic peptide ([114471-18-0](#))
C reactive protein ([9007-41-4](#))
carboplatin ([41575-94-4](#))
creatine kinase ([9001-15-4](#))
cyclophosphamide ([50-18-0](#), [6055-19-2](#))
doxorubicin ([23214-92-8](#), [25316-40-9](#))
epidermal growth factor receptor 2
([137632-09-8](#))
heparin ([37187-54-5](#), [8057-48-5](#), [8065-01-8](#), [9005-48-5](#), [9041-08-1](#))
lactate dehydrogenase ([9001-60-9](#))
lactate dehydrogenase A
methylprednisolone ([6923-42-8](#), [83-43-2](#))
paclitaxel ([33069-62-4](#))
pembrolizumab ([1374853-91-4](#))
prednisone ([53-03-2](#))
troponin I ([77108-40-8](#))

15. [Lactitol-Induced Acute Kidney Injury with Oxalate Nephropathy: A Case Report](#)

Guan M., Hu H., Song H., Wan Q.
[Article in Press] Nephron 2025 : (1-13)
MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

INTRODUCTION: Oxalate nephropathy, characterized by calcium oxalate crystal deposition in renal tissue, represents an underrecognized etiology of acute and chronic kidney injury. Secondary hyperoxaluria can emerge from diverse pathogenetic mechanisms, including excessive oxalate precursor intake, augmented intestinal absorption, or iatrogenic interventions. The therapeutic potential of glucocorticoids in managing this condition remains incompletely elucidated. CASE PRESENTATION: A 66-year-old man with no significant prior medical history developed

acute kidney injury (AKI), manifesting as a profound serum creatinine elevation from 81 $\mu\text{mol/L}$ to 1140.4 $\mu\text{mol/L}$. The patient had consumed a "corn germ powder" supplement containing lactitol for five consecutive months, concurrently experiencing persistent diarrhea. Initial laboratory investigations revealed severe renal dysfunction without notable proteinuria or hematuria. Renal ultrasonography demonstrated normal kidney morphology and dimensions. Definitive kidney biopsy revealed extensive calcium oxalate crystal deposition within renal tubular structures, conclusively diagnosing oxalate nephropathy. Therapeutic intervention comprised prednisone (60 mg daily) and comprehensive supportive management. Following a three-month treatment protocol with gradual corticosteroid dose reduction, the patient's renal function demonstrated substantial improvement, with serum creatinine declining to 118.2 $\mu\text{mol/L}$. **CONCLUSION:** This case underscores lactitol-induced secondary oxalate nephropathy as a rare yet clinically significant contributor to acute kidney injury. Prompt diagnostic recognition and targeted therapeutic intervention, potentially incorporating glucocorticoid therapy, may substantially facilitate renal functional recovery. Clinicians should maintain heightened awareness of nephrotoxic risks associated with over-the-counter laxative supplements and consider oxalate nephropathy in cryptogenic renal dysfunction scenarios.

Drug Index Terms

calcium oxalate, corticosteroid, glucocorticoid, **lactitol**, laxative, **oxalic acid**, prednisone

Non-drug Index Terms

acute kidney failure, adult, article, case report, clinical article, creatinine blood level, diagnosis, diarrhea, drug dose reduction, drug therapy, echography, hematuria, human, hyperoxaluria, intestine absorption, kidney biopsy, kidney function, kidney structure, kidney tissue, male, middle aged, nephrotoxicity, **oxalate nephropathy**, proteinuria, severe renal impairment, special situation for pharmacovigilance, therapy, treatment protocol

Comment: valid ICSR

ADDITIONAL INFORMATION

Abbreviated Journal title Nephron

ISSN 22353186 (electronic)

Source Type	Journal
Source Publication Date	2025-11-13
Entry Date	2025-11-21 (Article in Press)
Publication Type	Article in Press
Page Range	1-13
Country of Source	Switzerland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1159/000548254
MEDLINE PMID	41231736
CAS Registry Numbers	calcium oxalate (563-72-4) lactitol (585-86-4) oxalic acid (144-62-7) prednisone (53-03-2)

16. [Pediatric CHOP Chemotherapy Acutely Disrupts Satellite-Cell Dynamics and Blunts Muscle Mass in a Sex-Specific Manner](#)

Daredia J., Magaña M.A., Nascimento C.M.C., Wells J.M., Thomas N.T., Wen Y., Rauschendorfer S., Dungan C.M., Wiggs M.P.

[Article in Press] Am JPhysiol Cell Physiol 2025 :

MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Pediatric cancer survival now exceeds 85 percent owing, in part, advances and use of combination chemotherapy treatments such as CHOP (cyclophosphamide, doxorubicin, vincristine, and prednisone). Despite its efficacy, CHOP may cause off-target effects during critical pediatric development periods such as impairments of skeletal muscle. We evaluated the acute effects of a CHOP administered to C57Bl/6J mice from postnatal day 28 to 48. CHOP slowed body-weight gain and led to smaller gastrocnemius fiber cross-sectional area by approximately

25% in both sexes (n=11-12 males; n=8-9 females). RNA sequencing detected 214 differentially expressed genes in males and 217 in females relative to controls, yet only 29 transcripts overlapped. Males exhibited downregulation of myogenic regulators, indicating impaired progenitor maintenance, whereas females showed an upregulation of extracellular-matrix and translational machinery genes plus cell-cycle regulators. Using immunohistochemistry to assess satellite cell abundance, there were 60% fewer satellite cells in males and a 40% fewer in females, which supported our transcriptional findings. These results demonstrate that pediatric CHOP acutely disrupts muscle stem-cell dynamics via sex-specific molecular programs and identify satellite cells as a potential target for preserving muscle health in pediatric cancer survivors.

Drug Index Terms

cyclophosphamide, doxorubicin, prednisone, vincristine

Non-drug Index Terms

animal experiment, animal model, animal tissue, article, body weight gain, C57BL 6 mouse, **cachexia**, cancer survival, cancer survivor, cell cycle, child, **CHOP cell line**, combinationchemotherapy, controlled study, differential gene expression, down regulation, extracellular matrix, female, gastrocnemius muscle, gene expression, human, immunohistochemistry, male, mouse, **muscle mass**, muscle stem cell, nonhuman, off-target effect, RNA sequencing, **satellite cell**, **sex difference**, skeletal muscle, **transcriptomics**, upregulation

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Am JPhysiol Cell Physiol
ISSN	15221563 (electronic)
Source Type	Journal
Source Publication Date	2025-11-12
Entry Date	2025-11-20 (Article in Press)
Publication Type	Article in Press
Page Range	
Country of Source	United States
Language of Article	English

Language of Summary	English
Digital object identifier(DOI)	10.1152/ajpcell.00758.2025
MEDLINE PMID	41223066
CAS Registry Numbers	cyclophosphamide (50-18-0 , 6055-19-2) doxorubicin (23214-92-8 , 25316-40-9) prednisone (53-03-2) vincristine (57-22-7)

17. [A multicenter phase 1/2 trial of lenalidomide and dose-adjusted EPOCH-R in MYC-associated DLBCL](#)

Cahill K., Godfrey J., Nabhan C., Kline J., Riedell P.A., Cohen K.S., Narula S., Karrison T.G., Robertson J., Karmali R., Venugopal P., Kim S.-H., Rapoport A.P., Lee S.T., Law J., Fishkin P.A.S., Isufi I., Velasco M., Kaminer L., Smith S.M.

[In Process] Blood Adv. 2025 9:21 (5665-5675)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

In patients with diffuse large B-cell lymphoma (DLBCL), concurrent deregulation of MYC and BCL2 confers inferior outcomes following R-CHOP (rituximab, cyclophosphamide, doxorubicin, vincristine, and prednisone). DA-EPOCH-R (dose-adjusted etoposide, prednisone, vincristine, cyclophosphamide, doxorubicin, and rituximab) produces favorable results in patients with dual MYC and BCL2 rearrangement (double-hit lymphoma [DHL]), but there are limited prospective data in both DHL and DLBCL with dual protein overexpression of Myc and Bcl2 (double-expressor lymphoma [DEL]). Lenalidomide (LEN) may enhance the response in MYC-driven lymphomas, prompting this investigator-initiated multicenter phase 1/2 study evaluating LEN with DA-EPOCH-R in adults with newly diagnosed DHL and DEL. Fifty-five patients (DHL, 23; DEL, 32) were enrolled and treated. Patients had a median age of 65 years (range, 25-82), International Prognostic Index ≥ 3 in 69% (38/55), and stage III/IV in 91% (50/55). The overall response rate was 90.9%, with a complete response rate of 83.6%. With a median follow-up of 3.4 years,

the primary end point efficacy criterion was met with 1- and 2-year progression-free survival (PFS) rates of 85.5% and 78.2%, respectively. The 2-year overall survival was 83.6%. The most common adverse events (grade ≥ 3) were neutropenia (67%), anemia (67%), thrombocytopenia (49%), and neutropenic fever (35%). There were no grade 5 events. Second primary malignancy occurred in 6 patients (11%). LEN with DA-EPOCH-R for patients with DEL and DHL has a high response rate, encouraging survival, and met the primary PFS efficacy criterion. A randomized trial of DA-EPOCH-R with and without LEN would be needed to determine the specific benefit of LEN. This trial was registered at www.ClinicalTrials.gov as #NCT02213913.

Drug Index Terms

cyclophosphamide, cyclophosphamide plus doxorubicin plus etoposide plus prednisolone plus rituximab plus vincristine, cyclophosphamide plus doxorubicin plus etoposide plus prednisolone plus vincristine, cyclophosphamide plus doxorubicin plus prednisolone plus rituximab plus vincristine, doxorubicin, etoposide, lenalidomide, prednisone, rituximab, vincristine

Non-drug Index Terms

adult, adverse drug reaction, aged, anemia, article, controlled study, diffuse large B cell lymphoma, double-expressor lymphoma, double-hit lymphoma, drug combination, drug therapy, febrile neutropenia, female, follow up, human, International Prognostic Index, major clinical study, male, multicenter study, multiple cycle treatment, neutropenia, overall response rate, overall survival, phase 2 clinical trial, progression free survival, randomized controlled trial, second cancer, side effect, thrombocytopenia

Comment: Invalid ICSR

ADDITIONAL INFORMATION

Abbreviated Journal Title	Blood Adv.
ISSN	24739537 (electronic), 24739529
Source Type	Journal
Source Publication Date	2025-11-11
Entry Date	2025-11-18 (In process)
Publication Type	Article

Page Range	5665-5675
Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2473952925004410
Publisher Item Identifier	S2473952925004410
Digital object identifier (DOI)	10.1182/bloodadvances.2025017092
MEDLINE PMID	40720747
Number of References	63

18. [Systemic mRNA vaccines elicit rapid immune activation in canine brain tumors](#)

Carrera-Justiz S., Aghaee M., Qdaisat S., Weidert F., Garcia G.A., Fagman L., Zhang D., Stover B., Moor R.S.F., Xie C., Goldenberg E., Von Roemeling C., Doonan B., Chardon-Robles J., Elliott L., Sawyer W.G., Deleyrolle L.P., Sahay B., Foster T.P., Seligson N.D., Rahman M., Ghiaseddin A., Castillo P., Lee J.-H., Silver N.L., Doty A., Ligon J.A., Milner R.J., Mitchell D., Mendez-Gomez H., Moore H., Sayour E.J.
J.Immunother. Cancer 2025 13:11
Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Malignant glioma is considered invariably recalcitrant, thus necessitating development of new therapeutic approaches. Spontaneously arising canine glioma exhibits important molecular and pathological similarities to malignant pediatric glioma, making it an invaluable model for testing novel therapies. We evaluated safety and activity of RNA lipid particle aggregates (LPAs) designed to provoke cancer immunogenicity in 10 dogs with histologically confirmed glioma. Vaccine manufacturing was feasible in all subjects and generally well tolerated. Median survival time

was 123 and 155 days in dogs receiving three (Group A) or four (Group B) vaccines, respectively; this was increased from historical reporting of subjects receiving palliative care. Responding animals had a robust cytokine response following infusions. In animals receiving non-specific RNA-LPAs, there was localization of messenger RNA (mRNA) payloads to the brain tumor microenvironment and a rapid shift in immunologic gene signatures consistent with a viral response. These data demonstrate the ability to induce rapid viremic response in the tumor microenvironment following a single peripherally administered mRNA vaccine. Since RNA-LPAs are amenable to repeat boosts, these formulations may overcome challenges of tumor immunosuppression and therapeutic access and are currently under evaluation in first-in-human trials (NCT04573140, NCT06389591).

Drug Index Terms

(Based on full text)

gamma interferon (endogenous compound), interleukin 2 (endogenous compound), levetiracetam (drug combination), lomustine, maropitant (subcutaneous drug administration), phenobarbital (drug combination), prednisone, **RNA vaccine**, STAT1 protein (endogenous compound), STAT2 protein (endogenous compound), tumor necrosis factor (endogenous compound), zonisamide

Non-drug Index Terms

(Based on full text)

algorithm, animal experiment, animal tissue, article, astrocytoma, blood cell count, **brain tumor**, cancer survival, cell infiltration, cytokine response, Cytomegalovirus, differential gene expression, dog, enzyme linked immunospot assay, flow cytometry, gel electrophoresis, gene expression, glioma, histopathology, **immune activation**, immune response, immunogenicity, immunohistochemistry, immunosuppressive treatment, immunotherapy, median survival time, nonhuman, nuclear magnetic resonance imaging, oligodendroglioma, overall survival, palliative therapy, principal component analysis, RNA extraction, RNA lipid particle aggregate, RNA quality, **seizure**, survival time, thrombocytopenia, tumor biopsy, tumor growth, tumor microenvironment

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title J.Immunother. Cancer

ISSN	20511426 (electronic)
Source Type	Journal
Source Publication Date	2025-11-11
Entry Date	2025-11-19 (Full record), 2025-11-17 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	United States, United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1136/jitc-2025-011817
MEDLINE PMID	41218853
Article Number	e011817
Number of References	18
Drug Tradenames	cerenia (Zoetis, United States)
Drug Manufacturers	Zoetis (United States)
CAS Registry Numbers	gamma interferon (82115-62-6) interleukin 2 (85898-30-2) levetiracetam (102767-28-2) lomustine (13010-47-4) maropitant (147116-67-4 , 359875-09-5) phenobarbital (50-06-6 , 57-30-7 , 8028-68-0) prednisone (53-03-2) zonisamide (68291-97-4)
Clinical Trial Numbers	ClinicalTrials.gov (NCT04573140 , NCT06389591)

19. [ATF6 and pS6K as prognostic biomarkers in newly diagnosed diffuse large B-cell lymphoma](#)

Su S., Wu L., Huang C., Wang L., Li W., Liu W., Liu L.

Sci Rep 2025 15:1 (39326)

MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

This study aimed to investigate the clinical and prognostic significance of activating transcription factor 6 (ATF6) and phosphorylated S6 kinase (pS6K) expression in newly diagnosed diffuse large B-cell lymphoma (DLBCL). We conducted a retrospective analysis of 107 DLBCL patients treated with the R-CHOP regimen at the Department of Hematology, Fourth Hospital of Hebei Medical University, between January 2019 and December 2022. Immunohistochemical staining was performed to evaluate ATF6 and pS6K expression levels in tumor tissues, followed by correlation analyses with clinical features and survival outcomes. Elevated ATF6 and pS6K expression levels were significantly correlated with advanced Ann Arbor stage (III-IV), presence of B symptoms, high International Prognostic Index (IPI) scores (≥ 2), extranodal involvement (> 1 site), peripheral blood lymphopenia ($< 1.1 \times 10^9/L$), and increased BCL-6 positivity. Survival curve analysis revealed that the overall survival (OS) and progression-free survival (PFS) rates in the high ATF6 (OS, 56.20% vs. 84.40%, $P < 0.001$; PFS, 41.10% vs. 77.00%, $P < 0.001$) and pS6K (OS, 52.30% vs. 79.60%, $P < 0.001$; PFS, 51.80% vs. 64.00%, $P = 0.003$) expression groups were significantly lower than those in the low expression groups. Multivariate analysis additionally confirmed that B symptoms (HR = 2.819, 95% CI: 1.131-7.027, $P = 0.026$), IPI scores of 2-3 (HR = 4.906, 95% CI: 1.501-16.042, $P = 0.009$), and high ATF6 expression (HR = 5.699, 95% CI: 1.223-26.552, $P = 0.027$) were risk factors for overall survival (OS), whereas the non-germinal center B cell (non-GCB) subtype (HR = 3.245, 95% CI: 1.080-9.748, $P = 0.036$), IPI scores of 2-3 (HR = 2.776, 95% CI: 1.156-6.662, $P = 0.022$), and high ATF6 expression (HR = 4.085, 95% CI: 1.418-11.768, $P = 0.009$) were identified as risk factors for progression-free survival (PFS). ATF6 and pS6K overexpression were strongly associated with aggressive clinicopathological features and closely associated with poor OS and PFS. These findings suggest that ATF6 and pS6K may serve as prognostic biomarkers, highlighting a

potential avenue for risk stratification in newly diagnosed DLBCL patients.

Drug Index Terms

(Based on full text)

activating transcription factor 6, antineoplastic agent (drug therapy), ATF6 protein, human, cyclophosphamide (drug therapy), doxorubicin (drug therapy), prednisone (drug therapy), R-CHOP protocol, rituximab (drug therapy), **tumor marker**, vincristine (drug therapy)

Non-drug Index Terms

(Based on full text)

adult, aged, diagnosis, **diffuse large B cell lymphoma**, drug therapy, female, human, male, metabolism, middle aged, mortality, pathology, phosphorylation, prognosis, retrospective study, very elderly

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Sci Rep
ISSN	20452322 (electronic)
Source Type	Journal
Source Publication Date	2025-11-10
Entry Date	2025-11-17 (Full record)
Publication Type	Article
Page Range	39326
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1038/s41598-025-22920-4
MEDLINE PMID	41214097
CAS Registry Numbers	cyclophosphamide (50-18-0 , 6055-19-2) doxorubicin (23214-92-8 , 25316-40-9) prednisone (53-03-2) rituximab (174722-31-7) vincristine (57-22-7)

20. [Tocilizumab monotherapy versus combined in aortitis associated with giant cell arteritis: Factors associated with imaging remission in a multicenter open-label study of 166 patients](#)

Martín-Gutiérrez A., Loricera J., Prieto-Peña D., Aldasoro V., Maiz O., de Miguel E., Galíndez-Agirregoikoa E., Ferraz-Amaro I., Castañeda S., Blanco R.

[Article in Press] Semin Arthritis Rheum 2025 75: (152869)

MEDLINE

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Abstract

BACKGROUND: Aortitis is a frequent and potentially serious complication of giant cell arteritis (GCA). The phenotype associated with aortitis (GCA-aortitis) can present greater therapeutic challenges than the cranial-dominant phenotype. Tocilizumab (TCZ) is approved for GCA, although its efficacy in GCA-aortitis has not been specifically assessed in randomized controlled trials. **OBJECTIVE:** To assess the effectiveness and safety of TCZ monotherapy (TCZmono) compared with TCZ plus conventional synthetic disease-modifying antirheumatic drugs (csDMARDs) (TCZcombo) in patients with GCA-aortitis in real-world practice, and to identify variables associated with imaging remission. **METHODS:** A multicenter observational study comparing TCZcombo versus TCZmono. Outcomes at 12 and 24 months included clinical remission, EULAR-defined remission, imaging remission, prednisone-sparing effect, and safety. Multivariable logistic regression identified factors linked to imaging remission at 24 months. **RESULTS:** A total of 196 patients (148 female, 48 male) with GCA-aortitis received TCZ (136 TCZmono, 60 TCZcombo). At 24 months, imaging remission was higher in the TCZcombo group (50.0% vs. 15.8%; $p=0.026$). No significant differences were found in clinical or EULAR-defined remission. TCZcombo demonstrated a prednisone-sparing effect. The serious adverse event (SAE) rate was numerically lower with TCZcombo (8.7 vs. 13.2 per 100 patient-years; $p=0.21$). TCZcombo increased the odds of imaging remission 5.26-fold (odds ratio [OR] 5.26; 95% confidence interval [CI]: 1.03-26.85; $p=0.046$). **CONCLUSION:** In patients with GCA-aortitis, TCZcombo may be more effective than TCZmono in achieving

imaging remission and promoting glucocorticoid tapering, but without definitive evidence regarding a reduction of serious adverse events.

Drug Index Terms

disease modifying antirheumatic drug, glucocorticoid, immunosuppressive agent, prednisone, tocilizumab

Non-drug Index Terms

adult, aortitis, biological therapy, complication, drug therapy, female, giant cell arteritis, human, major clinical study, male, monotherapy, multicenter study, observational study, open study, phenotype, randomized controlled trial, remission, review, side effect

Comment: Invalid ICSR

Abbreviated Journal Title	Semin Arthritis Rheum
ISSN	1532866X (electronic)
Source Type	Journal
Source Publication Date	2025-11-06
Entry Date	2025-11-18 (Article in Press)
Publication Type	Article in Press
Page Range	152869
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S0049017225002409
Publisher Item Identifier	S0049017225002409
Digital object identifier(DOI)	10.1016/j.semarthrit.2025.152869
MEDLINE PMID	41218409 52869
CAS Registry Numbers	prednisone (53-03-2) tocilizumab (375823-41-9)

21. [Symmetrical drug-related intertriginous and flexural exanthema \(SDRIFE\) following first administration of immunoglobulin replacement](#)

Legakis L., Liao J., Zidan E., Rodenas M.

BMJ Case Rep. 2025 18:11

Embase, MEDLINE, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

This case report describes a rare instance of symmetrical drug-related intertriginous and flexural exanthema (SDRIFE) following the first administration of intravenous immunoglobulin (Ig) in a woman in her 30s with secondary immunodeficiency from rituximab treatment for multiple sclerosis. 3 days after receiving her first dose of intravenous Ig, she developed a pruritic rash in her intertriginous regions of the body. Laboratory and dermatologic evaluation confirmed the SDRIFE diagnosis. Treatment with systemic and topical steroids led to the resolution of her symptoms and rash. This case represents one of the few reported cases of SDRIFE resulting from intravenous Ig treatment and the first to demonstrate successful continuation of immunoglobulin therapy with a change in intravenous Ig formulation. The patient has tolerated multiple subsequent infusions well, with no further rash, and with improvement in her frequency and severity of infections. This case highlights a rare complication of intravenous Ig therapy and describes a successful management to continue this critical therapy.

Drug Index Terms

(Based on full text)

C reactive protein, human immunoglobulin, **immunoglobulin** (endogenous compound), immunoglobulin G (endogenous compound), immunoglobulin M (endogenous compound), prednisolone, prednisone, rituximab, steroid (endogenous compound), triamcinolone

Non-drug Index Terms

(Based on full text)

article, **baboon syndrome**, blood cell count, blood pressure, case report, clinical article, drug dose reduction, emergency ward, erythema, female, human, immunoglobulin deficiency, multiple sclerosis, oxygen

saturation, physical examination, **pruritus**, rash, **substitution therapy**, upper respiratory tract infection

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	BMJ Case Rep.
ISSN	1757790X (electronic)
Source Type	Journal
Source Publication Date	2025-11-05
Entry Date	2025-11-19 (Full record), 2025-11-17 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1136/bcr-2025-268228
MEDLINE PMID	41198151
Article Number	e268228
Number of References	9
CAS Registry Numbers	C reactive protein (9007-41-4) immunoglobulin (9007-83-4) immunoglobulin G (97794-27-9 , 308067-58-5) immunoglobulin M (9007-85-6) prednisolone (50-24-8) prednisone (53-03-2) rituximab (174722-31-7) triamcinolone (124-94-7)

22. [Diagnosis and Management of Hypersensitivity to Antiplatelet Drugs: EAACI Position Paper](#)

Cortellini G., Nakonechna A., Barbaud A., Bavbek S., Calogiuri G., Doña I., Kvedariene V., Laguna Martinez J.J., Liberati S., Pagani M., Romano A.

Allergy Eur. J. Allergy Clin. Immunol. 2025 80:11 (2973-2987)
Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Antiplatelet drug (APD) therapy is the cornerstone for the prevention of atherosclerotic cardiovascular disease. The main APDs are aspirin and thienopyridines, particularly clopidogrel. These drugs may induce hypersensitivity reactions (HSRs). The most common reported reactions to these drugs are cutaneous, such as exanthemas associated with thienopyridine and urticaria/angioedema by aspirin, which can also induce respiratory symptoms. APDs other than aspirin, particularly ticlopidine, can also cause hematologic reactions consisting mainly of isolated thrombocytopenia, agranulocytosis, and leukopenia. Immune-mediated reactions to aspirin are very rare. Few data suggest the usefulness of skin testing in patients with cutaneous reactions to APDs other than aspirin, particularly clopidogrel. Therefore, the drug provocation test is the gold standard for diagnosing hypersensitivity to APDs. Low-dose aspirin challenge (i.e., up to 150-180 mg) and aspirin desensitization have emerged as effective and safe approaches in patients with suspected or confirmed aspirin hypersensitivity who require aspirin therapy. Both, a short course of oral glucocorticoids without interruption of clopidogrel treatment and desensitization, appears to be effective and safe options in patients with cutaneous HSRs to clopidogrel. This position paper provides data and recommendations regarding the characteristics of HSRs to APDs and related diagnostic procedures in order to make them as safe and effective as possible. Management and treatment options, including desensitization protocols, are also provided.

Drug Index Terms

(Based on full text)

abciximab (adverse drug reaction), acetylsalicylic acid (adverse drug

reaction), **antithrombocytic agent** (adverse drug reaction), clopidogrel (adverse drug reaction), dipyridamole (adverse drug reaction), nonsteroid antiinflammatory agent (adverse drug reaction), prasugrel (adverse drug reaction), prednisone, thienopyridine derivative, ticagrelor (adverse drug reaction), ticlopidine (adverse drug reaction), triflusal (adverse drug reaction)

Non-drug Index Terms

(Based on full text)

acute generalized exanthematous pustulosis (side effect), anemia (side effect), angioneurotic edema (side effect), article, aspirin hypersensitivity (side effect), bronchospasm (side effect), cholestatic hepatitis (side effect), clinical feature, cross reaction, desensitization, diagnostic procedure, disease classification, DRESS syndrome (side effect), **drug hypersensitivity** (diagnosis, disease management, side effect), drug withdrawal, eczema (side effect), eosinophilia (side effect), fever (side effect), gastrointestinal discomfort (side effect), gastrointestinal hemorrhage (side effect), human, leukocytoclastic vasculitis (side effect), lymphocyte transformation, photosensitivity (side effect), provocation test, prurigo (side effect), rash (side effect), serum sickness (side effect), Stevens Johnson/toxic epidermal necrolysis overlap syndrome (side effect), systematic review (topic), thrombocytopenia (side effect), thrombotic thrombocytopenic purpura (side effect), treatment indication, urticaria (side effect)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Allergy Eur. J.Allergy Clin. Immunol.
ISSN	13989995 (electronic), 01054538
CODEN	LLRGD
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-18 (Full record), 2025-08-01 (Article in Press/In process)
Publication Type	Article
Page Range	2973-2987
Country of Author	Italy, Spain, Italy

Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1111/all.16663
MEDLINE PMID	40704552
Number of References	108
CAS Registry Numbers	abciximab (143653-53-6) acetylsalicylic acid (493-53-8 , 50-78-2 , 53663-74-4 , 53664-49-6 , 63781-77-1) clopidogrel (113665-84-2 , 120202-66-6 , 90055-48-4 , 94188-84-8 , 120202-65-5 , 120202-67-7 , 894353-16-3 , 744256-69-7) dipyridamole (58-32-2) prasugrel (389574-19-0 , 150322-43-3) prednisone (53-03-2) ticagrelor (274693-27-5) ticlopidine (53885-35-1 , 55142-85-3) triflusal (322-79-2)

23. [Linear IgA Bullous Dermatitis in a 15-Month-Old Child: Classic 'String-of-Pearls' Presentation With Auricular Involvement](#)

Khan B., Verheyden M., Scurry J.

J.Paediatr. Child Health 2025 61:11 (1812-1814)

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Drug Index Terms

(Based on full text)

azathioprine, betamethasone dipropionate (drug therapy, special situation for pharmacovigilance), cefalexin (drug dose, drug therapy, special situation for pharmacovigilance), dapsone (drug dose, drug therapy, special situation for pharmacovigilance), **immunoglobulin A**, immunoglobulin G, mycophenolate mofetil, prednisone (drug

administration, drug dose, drug therapy, oral drug administration, special situation for pharmacovigilance), rituximab

Non-drug Index Terms

(Based on full text)

article, bullous pemphigoid, **bullous skin disease** (drug therapy, therapy), **carotid artery**, case report, child, clinical article, epidermolysis bullosa acquisita (diagnosis), histopathology, human, human tissue, male, preschool child, Streptococcus pyogenes

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Paediatr.Child Health
ISSN	14401754 (electronic), 10344810
CODEN	JPCHE
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-17 (Full record), 2025-09-16 (Article in Press/In process)
Publication Type	Article
Page Range	1812-1814
Country of Author	Australia
Country of Source	Australia
Language of Article	English
Digital object identifier(DOI)	10.1111/jpc.70189
MEDLINE PMID	40920023
Number of References	5

CAS Registry Numbers

azathioprine ([446-86-6](#), [55774-33-9](#))
betamethasone dipropionate ([5593-20-4](#))
cefalexin ([15686-71-2](#), [23325-78-2](#))
dapsone ([80-08-0](#))
immunoglobulin G ([97794-27-9](#), [308067-58-5](#))
mycophenolate mofetil ([116680-01-4](#),
[128794-94-5](#), [115007-34-6](#))
prednisone ([53-03-2](#))
rituximab ([174722-31-7](#))

24. [Prevalence of major malformations and small for gestational age in newborns of female transplant recipients on tacrolimus-containing regimens during pregnancy](#)

Constantinescu S., Robinson N.J., Marcus M.W., Blogg M., Zhu F., Coscia L.A., Kliniewski D., Moritz M.J.

Am. J. Transplant. 2025 25:11 (2434-2450)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

In female transplant recipients, immunosuppressant use during pregnancy raises potential concerns about adverse maternal and fetal outcomes. This noninterventional study used prospectively and retrospectively reported cases from the Transplant Pregnancy Registry International database from 1991 until 2020. The study used 2 separate cohorts to evaluate the prevalence of major malformations and small for gestational age (SGA), respectively, among live-born infants of female kidney or liver transplant recipients receiving tacrolimus-containing regimens (Tac) or non-tacrolimus-containing regimens (non-Tac). There were 1988 pregnancies (2056 livebirths) in the cohort used to assess the prevalence of malformations and 2172 pregnancies (2248 livebirths) in the cohort used to assess the prevalence of SGA. Prevalence of major malformations was similar in Tac and non-Tac groups: 2.0% and 1.9% in prospectively reported cases, and 4.7% and 3.1% in retrospectively reported cases, respectively (overall adjusted odds ratio, 1.10; 95% confidence interval, 0.68, 1.81). There was a lower prevalence of SGA in the Tac group than in the non-Tac group: 17.9% and 32.2% in prospectively reported cases and 18.1% and 23.5% in retrospectively reported cases, respectively (overall odds ratio, 0.70; 95% confidence interval, 0.57, 0.87). The use of tacrolimus in female transplant recipients was not associated with a high risk of major malformations or SGA in live-born infants.

Drug Index Terms

(Based on full text)

azathioprine, belatacept, cyclosporine, everolimus, immunosuppressive agent, mycophenolic acid, prednisone, sirolimus, **tacrolimus** (special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

adult, advanced maternal age, article, cohort analysis, conception, controlled study, female, fertility, fetus outcome, follow up, **graft recipient**, human, immunosuppressive treatment, infant, kidney graft, kidney transplantation, live birth, liver graft, liver transplantation, major clinical study, medical history, newborn, **pregnancy**, pregnancy outcome, **prevalence**, retrospective study, sensitivity analysis, **small for gestational age**, therapeutic abortion, treatment switching

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Am. J. Transplant.
ISSN	16006143 (electronic), 16006135
CODEN	AJTMB
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-19 (Full record), 2025-08-28 (Article in Press/In process)
Publication Type	Article
Page Range	2434-2450
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S1600613525028515
Publisher Item Identifier	S1600613525028515
Digital object identifier(DOI)	10.1016/j.ajt.2025.07.2472
MEDLINE PMID	40706680

Number of References	37
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) belatacept (706808-37-9) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) everolimus (159351-69-6) mycophenolic acid (23047-11-2 , 24280-93-1 , 37415-62-6) prednisone (53-03-2) sirolimus (53123-88-9) tacrolimus (104987-11-3 , 109581-93-3)

25. [Chronic Inflammatory Demyelinating Polyneuropathy and Variants](#)

Stino A.M., Barohn R.J., Katz J.S., Dimachkie M.M.

Neurol. Clin. 2025 43:4 (761-780)

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Abstract

Chronic inflammatory demyelinating polyradiculoneuropathy (CIDP) is the prototypic chronic inflammatory polyneuropathy, characterized by well-defined clinical and electrodiagnostic criteria, despite the absence of biomarkers or full understanding of disease pathogenesis. It has both a typical form and 5 variants. IVIG remains the treatment of choice for induction or maintenance of remission, although corticosteroids, plasma exchange, and most recently, efgartigimod, demonstrate therapeutic efficacy. Subcutaneous immunoglobulin is indicated only as maintenance therapy. Validated outcome measures exist to track disability, strength, and quality of life and should be routinely employed in clinical practice.

Drug Index Terms

(Based on full text)

acetylsalicylic acid, azathioprine (drug therapy), corticosteroid (drug combination, drug therapy, intravenous drug administration, oral drug administration, unexpected outcome of drug treatment),

cyclophosphamide (drug therapy), cyclosporine (drug therapy), dexamethasone (adverse drug reaction, drug therapy, oral drug administration), efgartigimod alfa (drug therapy, intravenous drug administration), efgartigimod alfa plus hyaluronidase (drug comparison, drug therapy, subcutaneous drug administration), fingolimod (drug therapy), immunoglobulin (adverse drug reaction, drug administration, drug combination, drug comparison, drug dose, drug therapy, intravenous drug administration, subcutaneous drug administration, unexpected outcome of drug treatment), methotrexate (drug therapy), methylprednisolone (drug therapy, intravenous drug administration), mycophenolate mofetil (drug therapy), placebo, prednisone (adverse drug reaction, drug therapy, oral drug administration), riliprubart (drug comparison, drug therapy, subcutaneous drug administration), rituximab (drug therapy)

Non-drug Index Terms

(Based on full text)

article, cerebrospinal fluid analysis, cerebrovascular disease (side effect), **chronic inflammatory demyelinating polyneuropathy** (drug resistance, drug therapy, therapy), coronary artery disease (side effect), deep vein thrombosis (side effect), diagnostic error, disability, distal acquired demyelinating symmetric neuropathy (diagnosis, drug therapy), drug megadose, drug withdrawal, echography, electrodiagnosis, focal chronic inflammatory demyelinating polyradiculoneuropathy (diagnosis), grip strength, human, immunotherapy, insomnia (side effect), kidney dysfunction (side effect), Lewis-Sumner syndrome (drug therapy), limb weakness, low drug dose, lung embolism (side effect), maintenance therapy, moon face (side effect), motor chronic inflammatory demyelinating polyradiculoneuropathy (diagnosis, drug therapy), motor nerve conduction, motor neuropathy (drug therapy), multiple cycle treatment, myasthenia gravis (drug therapy), nerve biopsy, nerve conduction velocity test, nuclear magnetic resonance imaging, paresthesia, plasma exchange, prognosis, relapse, remission, sensory chronic inflammatory demyelinating polyradiculoneuropathy (diagnosis), treatment failure, venous thromboembolism (side effect)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Neurol. Clin.

ISSN 15579875 (electronic), 07338619

CODEN	NECLE
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-19 (Full record), 2025-09-05 (Article in Press/In process)
Publication Type	Article
Page Range	761-780
Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S0733861925000349
Publisher Item Identifier	S0733861925000349
Digital object identifier (DOI)	10.1016/j.ncl.2025.05.015
MEDLINE PMID	41161995
Number of References	43
CAS Registry Numbers	acetylsalicylic acid (493-53-8 , 50-78-2 , 53663-74-4 , 53664-49-6 , 63781-77-1) azathioprine (446-86-6 , 55774-33-9) cyclophosphamide (50-18-0 , 6055-19-2) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) dexamethasone (50-02-2) efgartigimod alfa (1821402-21-4) fingolimod (162359-56-0 , 162359-55-9 , 1967800-35-6) immunoglobulin (9007-83-4) methotrexate(15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6)

prednisone ([53-03-2](#))
riliprubart ([2756228-76-7](#))
rituximab ([174722-31-7](#))

26. [Dengue Infection as a Trigger for Collapsing Glomerulopathy in Patients With APOL1 High-Risk Genotype](#)

Neves P.D.M.M., Silva L.C.S., Pereira M.E.V.C., Oliveira F.A.M., Araújo S.A., Wanderley D.C., Ledesma F.L., Guedes A.L., Loos B.H., Ribeiro A.T., Watanabe A., Watanabe E.H., Murata G.M., Silva G.E.B., Vieira-Júnior J.M., Melo N.C.V., Malheiros D.M.A.C., Noronha I.L., Onuchic L.F.

Kidney Intl. Rep. 2025 10:11 (4101-4105)

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Embase Open URL: [redirect to full text](#)

Drug Index Terms

(Based on full text)

angiotensin receptor antagonist, **apolipoprotein L1** (endogenous compound), complement component C3 (endogenous compound), immunoglobulin M (endogenous compound), methylprednisolone, mycophenolate mofetil, prednisone

Non-drug Index Terms

(Based on full text)

acute kidney tubule necrosis, adult, article, case report, chronic kidney failure, clinical article, **collapsing glomerulopathy**, dehydration, **dengue**, edema, facial nerve paralysis, fatigue, female, **genotype**, genotyping, hematuria, hemiplegia, hemodialysis, high risk patient, hospital admission, human, human tissue, hyperplasia, hypertrophy, hypotension, immunofluorescence assay, kidney biopsy, kidney fibrosis, light microscopy, male, middle aged, nuclear magnetic resonance imaging, physical examination, podocyte, proteinuria, rash, renal replacement therapy, somnolence, tissue adhesion

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Kidney Intl. Rep.

ISSN	24680249
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-17 (Full record), 2025-09-08 (Article in Press/In process)
Publication Type	Article
Page Range	4101-4105
Country of Author	Brazil
Country of Source	United States
Language of Article	English
Publisher Item Identifier	S2468024925004978
Publisher Item Identifier	S2468024925004978
Digital object identifier(DOI)	10.1016/j.ekir.2025.08.005
Number of References	9
CAS Registry Numbers	complement component C3 (80295-41-6) immunoglobulin M (9007-85-6) methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2)

27. [Clinical, therapeutic, and stigma-related challenges in leprosy and the emerging role of apremilast in managing leprosy reactions](#)

Gayoso Cantero D., Fernández-González P., Chamorro-Tojeiro S., Quereda C., Tato M., Berna-Rico E., Naharro-Rodríguez J., Perez-Molina J.A.

Travel Med. Infect. Dis. 2025 68:
Embase, MEDLINE, NURSING

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Leprosy is a chronic granulomatous infection caused by *Mycobacterium leprae* (or *M. lepromatosis*) that targets macrophages and Schwann cells. The resulting neuropathy and skin lesions can lead to lifelong deformities and disabilities. Despite being poorly transmissible and readily treatable, leprosy remains a neglected disease. Management is hampered by delayed diagnosis, multidrug resistance, recalcitrant leprosy reactions, and entrenched social stigma. We report a case of imported multidrug-resistant borderline lepromatous leprosy that progressed with recurrent, steroid-dependent erythema nodosum leprosum (ENL). The patient experienced multiple toxicities secondary to the medications used for treating leprosy and managing ENL. Disease was controlled through a clofazimine-clarithromycin-minocycline regimen combined with the phosphodiesterase-4 inhibitor apremilast, enabling complete withdrawal of corticosteroids and thalidomide while maintaining sustained clinical and microbiological remission. We examine the key clinical challenges in managing this patient and summarise current evidence, including a literature review on apremilast as treatment for refractory leprosy reactions.

Drug Index Terms

(Based on full text)

amoxicillin plus clavulanic acid, **apremilast** (drug therapy, special situation for pharmacovigilance), clarithromycin (drug therapy), clofazimine (adverse drug reaction, drug therapy), corticosteroid, dapsone (drug therapy), lactate dehydrogenase, methotrexate (drug therapy), minocycline (adverse drug reaction, drug therapy), prednisone (drug therapy), rifampicin (drug therapy), steroid, thalidomide, urea

Non-drug Index Terms

(Based on full text)

acute febrile neutrophilic dermatosis (diagnosis), adult, article, case report, clinical article, cushingoid habitus, cytomegalovirus infection, delayed diagnosis, drug withdrawal, ear lobe, Epstein Barr virus, erythema nodosum leprosum (diagnosis, drug resistance, drug therapy), fever, foam cell, Hepatitis B virus, Hepatitis C virus, human, ichthyosis (side effect), knee pain (drug therapy), leg pain (drug therapy), leprosy (drug therapy), leukocytosis, lymphadenopathy, malaise, male, **multibacillary leprosy** (diagnosis, drug therapy), multidrug resistance, *Mycobacterium leprae*, neglected disease, null result, photosensitivity

(side effect), polymerase chain reaction, serology, skin biopsy, skin defect, skin discoloration (side effect), **social stigma**

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Travel Med. Infect. Dis.
ISSN	18730442 (electronic), 14778939
CODEN	TMIDA
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-17 (Full record), 2025-11-11 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Spain
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S1477893925001243
Publisher Item Identifier	S1477893925001243
Digital object identifier(DOI)	10.1016/j.tmaid.2025.102918
MEDLINE PMID	41093045
Article Number	102918
Number of References	87
CAS Registry Numbers	amoxicillin plus clavulanic acid (74469-00-4 , 79198-29-1) apremilast (608141-41-9) clarithromycin (81103-11-9) clofazimine (2030-63-9) dapsone (80-08-0) lactate dehydrogenase (9001-60-9) lactate dehydrogenase A methotrexate (15475-56-6 , 59-05-2 , 7413-

[34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#),
[60388-53-6](#))
minocycline ([10118-90-8](#), [11006-27-2](#),
[13614-98-7](#))
prednisone ([53-03-2](#))
rifampicin ([13292-46-1](#))
thalidomide ([50-35-1](#))
urea ([57-13-6](#))

28. [Robotic Extended Thymectomy in Late-Onset Myasthenia Gravis: A 21-Year Retrospective Cohort Study of 172 Patients](#)

Huang L., Li F., Li Z., Zhang H., Elsner A., Strauchmann J., Andreas M.N., Dziodzio T., Lask A., Neudecker J., Xie D., Yao L., Wu S., Zhou H., Meisel A., Rueckert J.-C.

Eur. J. Neurol. 2025 32:11

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Background: The safety and feasibility of robotic-assisted (RATS) thymectomy for myasthenia gravis (MG) with onset age ≥ 50 years remain unverified, particularly in very late-onset MG (V-LOMG). Methods: Patients were classified into late-onset MG (LOMG, 50-64) and very late-onset MG (V-LOMG ≥ 65) based on age of onset. Composite neurological remission (CNR) included complete stable remission (CSR), pharmacologic remission (PR), and minimal manifestations-0 (MM-0), while favorable outcomes comprised CNR and MM1-3. Results: Among 1041 patients, 172 with MG onset at ≥ 50 years who underwent RATS extended thymectomy were included in the final analysis. The LOMG group comprised 104 patients (45.2% male), while the V-LOMG group included 68 patients (60.3% male). V-LOMG patients had more preoperative MG crises, shorter onset-to-thymectomy intervals, heavier thymic specimens, and less hyperplasia. In ocular-onset MG, generalization was more frequent in LOMG than in V-LOMG. No significant differences were found in other baseline characteristics, perioperative parameters, postoperative complications, and adverse composite outcomes. At a 5.1-year mean follow-up, the V-LOMG group

had slightly higher CSR (7.4% vs. 6.7%), CNR (16.2% vs. 11.5%), and favorable outcome rates (52.9% vs. 45.2%) than the LOMG group, with no statistical significance. Both groups, especially V-LOMG (16.0 mg vs. 2.1 mg, $p < 0.001$), showed a significant corticosteroid dose reduction at the last follow-up, confirming the steroid-sparing effect of thymectomy. Conclusions: RATS extended thymectomy appears to be a safe and feasible treatment for patients with MG of onset at age ≥ 50 years, including those with V-LOMG, demonstrating a significant steroid-sparing effect while maintaining favorable neurological outcomes.

Drug Index Terms

(Based on full text)

azathioprine (drug therapy), cholinesterase inhibitor (drug combination, drug therapy), corticosteroid (drug therapy), daratumumab (drug therapy), eculizumab (drug therapy), efgartigimod alfa (drug therapy), immunoglobulin (drug therapy, intravenous drug administration), immunosuppressive agent (drug combination, drug therapy), methotrexate (drug therapy), mycophenolate mofetil (drug therapy), prednisone (drug combination, drug therapy), ravulizumab (drug therapy), rituximab (drug therapy)

Non-drug Index Terms

(Based on full text)

adult, adverse outcome, aged, article, clinical outcome, cohort analysis, controlled study, drug dose reduction, ectopic thyroid gland (surgery), female, follow up, human, human tissue, immunosuppressive treatment, **late onset disorder**, major clinical study, male, **myasthenia gravis** (drug therapy, surgery, therapy), observational study, onset age, patient safety, perioperative period, plasma exchange, postoperative complication, remission, retrospective study, **robot assisted surgery**, **robotic assisted thymectomy**, **thymectomy**, thymic neoplasm (surgery), thymoma type A (surgery), thymoma type AB (surgery), thymoma type B1 (surgery), thymoma type B2 (surgery), thymoma type B3 (surgery), thymus atrophy (surgery), thymus carcinoma (surgery), thymus hyperplasia (surgery)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Eur. J. Neurol.

ISSN 14681331 (electronic), 13515101

CODEN EJNEF

Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-19 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Germany
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1111/ene.70388
MEDLINE PMID	41190556
Article Number	e70388
Number of References	38
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) daratumumab (945721-28-8) eculizumab (219685-50-4) efgartigimod alfa (1821402-21-4) immunoglobulin (9007-83-4) methotrexate(15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2) ravulizumab (1803171-55-2) rituximab (174722-31-7)

29. [Infectious Complications and Prognostic Factors of Mortality in Patients with Lupus Nephritis Admitted to Intensive Care Units](#)

Aldana-Pérez S.E., García-Bañol D.F., Arias-Choles A.M., Aroca-Martínez G.J., Musso C.G., Dominguez-Vargas A., González-Torres H.J.

J.Clin. Med. 2025 14:21

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Abstract

Objective: To determine infectious complications and explore potential prognostic factors associated with mortality in patients with lupus nephritis (LN) admitted to the intensive care unit (ICU). **Methods:** We conducted a retrospective analytical study of 20 patients with biopsy-proven LN admitted to a tertiary ICU between 2022 and 2023. Clinical, histopathological, microbiological, and paraclinical data were collected. Associations with mortality were explored using Firth's penalized logistic regression. **Results:** The mean age was 37 ± 14 years; 85% were female. Hypertension (50%) was the most frequent comorbidity. Mean ICU stay was 13 ± 27 days; in-hospital mortality was 15%, and 60% required hospital readmission. Sepsis was the leading reason for ICU admission (55%), predominantly respiratory and gastrointestinal. In the exploratory analysis, respiratory tract infection (OR 1.43; 95% CI: 1.19-9.90; $p = 0.04$), proliferative LN (OR 2.12; 95% CI: 1.32-17.34; $p = 0.03$), and hypocomplementemia (C3) (OR 1.72; 95% CI: 1.25-10.40; $p = 0.02$) showed point estimates suggestive of higher odds of mortality. **Conclusions:** In this cohort of critically ill patients with LN, respiratory tract infection, proliferative histological class, and hypocomplementemia were associated with higher mortality. These findings require validation in larger prospective studies to determine their utility in risk stratification and ICU management.

Drug Index Terms

(Based on full text)

azathioprine (drug therapy), complement component C3 (endogenous compound), complement component C4 (endogenous compound), corticosteroid (drug therapy), cyclophosphamide (drug therapy), hemoglobin (endogenous compound), hydroxychloroquine (drug therapy), methylprednisolone (intravenous drug administration), mycophenolate mofetil (drug therapy), prednisone (oral drug administration)

Non-drug Index Terms

(Based on full text)

adult, age, antibiotic sensitivity, article, central nervous system infection, clinical article, clinical decision making, clinical feature, cohort analysis, comorbidity, comparative study, disease duration, drug pulse therapy, Escherichia coli infection, exploratory research, female, hematological parameters, **hospital admission**, hospital readmission, human, human tissue, hypertension, hypocomplementemia, immunological parameters, immunosuppressive treatment, **in-hospital mortality**, **infectious complication** (complication), **intensive care unit**, kidney biopsy, Klebsiella pneumoniae infection, leukocyte count, logistic regression analysis, **lupus erythematosus nephritis** (drug therapy), male, mean corpuscular hemoglobin, mean corpuscular hemoglobin concentration, mean corpuscular volume, methicillin resistant Staphylococcus aureus infection, observational study, platelet count, **prognosis**, proliferative lupus nephritis, prospective study, respiratory tract infection, retrospective study, sepsis, skin infection, SLEDAI, soft tissue infection, Staphylococcus aureus infection, Staphylococcus epidermidis infection, tuberculosis, urinary tract infection

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Clin. Med.
ISSN	20770383 (electronic)
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-21 (Full record), 2025-11-18 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Colombia
Country of Source	Switzerland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.3390/jcm14217561
Article Number	7561

Number of References	42
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) complement component C3 (80295-41-6) complement component C4 (80295-48-3 , 80295-71-2) cyclophosphamide (50-18-0 , 6055-19-2) hemoglobin (9008-02-0) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2)

30. [Evaluating the therapeutic potential of tofacitinib in Sjögren's disease: A comprehensive clinical and immunological assessment](#)

Liu Q., Zeng Y., Xing X., Huang B., Feng R., Wang Y., Wang N., Zhang X., Li Y., Su L., Jacob A., Ambrus J.L., Shen L., Suresh L., Yu D., Lin X., He J.

Rheumatology 2025 64:11 (5899-5910)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Objective To evaluate the efficacy, safety and immunological effects of tofacitinib in patients with Sjögren's disease (SjD), focusing on its impact on disease activity and immune cell modulation. **Methods** Two independent cohort studies, one retrospective (Cohort I) and one prospective (Cohort II), were conducted to investigate the efficacy of oral tofacitinib treatment in patients diagnosed with SjD. All participants were evaluated for changes in disease activity and lab parameters. Circulating Tcells were analysed, focusing on follicular helper T(Tfh) cells and peripheral helper T(Tph) cells. **Results** In cohort I, 112 patients treated with tofacitinib showed a significant improvement in the ESSDAI score [median (IQR), 8.00 (4.25, 15.75) vs 6.50 (2.25, 12.75), $P < 0.001$]. In cohort II, ten patients completed the 12-month treatment period. There

was a significant reduction in ESSDAI scores at the sixth month compared with baseline ($P = 0.001$). In total, 80% (8/10) of patients achieved a decrease of at least one point or 15% in ESSPRI scores. A significant reduction in the proportion of Th17 cells was observed (mean $\pm$ SD, 14.84 ± 7.70 vs 7.74 ± 4.24 , $P = 0.008$). A decrease in Tfh and Tph cells was also observed, along with decreased pSTAT-3 levels in CD4+ T cells and disease activity scores. No serious adverse events were observed in the two cohorts. Conclusions Tofacitinib effectively improves disease activity and immune regulation in SjD, and it is associated with suppressing Tfh and Tph cells, suggesting its potential as a treatment option. Trial registration ClinicalTrials.gov Identifier: NCT05087589.

Drug Index Terms

(Based on full text)

alanine aminotransferase, aspartate aminotransferase, azathioprine (drug therapy), C reactive protein, complement component C3, complement component C4, creatinine, hydroxychloroquine (drug therapy), immunoglobulin, immunoglobulin A, immunoglobulin G, immunoglobulin M, leflunomide (drug therapy), methotrexate (drug therapy), mycophenolic acid (drug therapy), prednisone, salazosulfapyridine (drug therapy), **STAT3 protein**, **tofacitinib** (adverse drug reaction, drug therapy, oral drug administration)

Non-drug Index Terms

(Based on full text)

adult, aged, arthritis, article, CD4+ Tlymphocyte, cohort analysis, controlled study, disease activity, disease activity score, drug efficacy, erythema multiforme, erythrocyte sedimentation rate, EULAR Sjogren syndrome disease activity index, follow up, human, immunocompetent cell, logistic regression analysis, maculopapular rash, major clinical study, observational study, retrospective study, **Sjogren syndrome** (drug therapy), skin manifestation, Tlymphocyte, **Tfh cell**, Th1 cell, Th17 cell, univariate analysis, upper respiratory tract infection (side effect), urticarial rash

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Rheumatology

ISSN 14620332 (electronic), 14620324

CODEN RUMAF

Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	5899-5910
Country of Author	China
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1093/rheumatology/keaf173
MEDLINE PMID	40139690
Number of References	43
CAS Registry Numbers	alanine aminotransferase (9000-86-6 , 9014-30-6) aspartate aminotransferase (9000-97-9) azathioprine (446-86-6 , 55774-33-9) C reactive protein (9007-41-4) complement component C3 (80295-41-6) complement component C4 (80295-48-3 , 80295-71-2) creatinine (19230-81-0 , 60-27-5) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) immunoglobulin (9007-83-4) immunoglobulin G (97794-27-9 , 308067-58-5) immunoglobulin M (9007-85-6) leflunomide (75706-12-6) methotrexate(15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) mycophenolic acid (23047-11-2 , 24280-93-1 , 37415-62-6) prednisone (53-03-2)

salazosulfapyridine ([599-79-1](#))
tofacitinib ([477600-75-2](#), [540737-29-9](#))

Clinical Trial Numbers

ClinicalTrials.gov ([NCT05087589](#))

31. [Efficacy and safety of rhGH in juvenile dermatomyositis children with glucocorticoid-related growth failure](#)

Wang X., Wang X., Huang B., Jiang Q., Ren G., Liu Y., Li J.

Rheumatology 2025 64:11 (5611-5615)

Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Objective To evaluate the efficacy and safety of recombinant human growth hormone (rhGH) in children with JDM experiencing glucocorticoid (GC)-induced growth failure. **Methods** A retrospective cohort study included 16 refractory JDM patients treated with rhGH (0.10-0.15 IU/kg/day) for 6-24 months. Outcomes including height Z-score, growth velocity (GV), biochemical markers, disease activity and adverse events were recorded and analysed. **Results** After 6 months of rhGH treatment, the height Z-score improved from -2.37 ± 1.15 at baseline to -1.73 ± 1.06 ($P < 0.001$), and GV increased from 2.74 ± 1.53 cm/year to 13.05 ± 3.64 cm/year ($P < 0.001$). Serum insulin-like growth factor 1 and alkaline phosphatase levels rose significantly (from 264.13 ± 81.24 - 351.59 ± 123.41 ng/ml and 242.40 ± 42.25 - 318.70 ± 86.12 U/l, respectively; $P < 0.05$). Creatine kinase levels increased but remained within the normal range (from 79.33 ± 27.56 - 111.60 ± 42.84 U/l; $P < 0.01$). Only one patient (1/16, 6.25%) experienced JDM relapse, a rate lower than the reported recurrence rate (31.6%) in JDM patients not receiving rhGH treatment. No adverse metabolic or endocrine effects were observed. **Conclusion** In JDM patients with growth failure, rhGH therapy may be considered as a potential treatment option, supported by preliminary evidence. Our study suggests an acceptable safety profile and potential efficacy in improving growth parameters, even in refractory cases. Further larger-scale multicentre studies will be necessary to confirm its application value in JDM.

Drug Index Terms

(Based on full text)

2 hydroxybutyrate dehydrogenase (endogenous compound), alkaline phosphatase (endogenous compound), aspartate aminotransferase (endogenous compound), autoantibody (endogenous compound), biochemical marker (endogenous compound), C reactive protein (endogenous compound), calcium (drug therapy, special situation for pharmacovigilance), creatine kinase (endogenous compound), cyclosporine (drug therapy, special situation for pharmacovigilance), disease modifying antirheumatic drug (drug therapy, special situation for pharmacovigilance), **glucocorticoid** (adverse drug reaction, drug therapy, special situation for pharmacovigilance), glucose (endogenous compound), hemoglobin A1c (endogenous compound), **human growth hormone** (adverse drug reaction, drug therapy, special situation for pharmacovigilance), lactate dehydrogenase (endogenous compound), liothyronine (endogenous compound), mda5 antibody (endogenous compound), methotrexate (drug therapy, special situation for pharmacovigilance), methylprednisolone (drug therapy, special situation for pharmacovigilance), mycophenolate mofetil (drug therapy, special situation for pharmacovigilance), myositis specific autoantibody (endogenous compound), npx2 antibody (endogenous compound), pl 7 antibody (endogenous compound), prednisone (drug therapy, special situation for pharmacovigilance), somatomedin C (endogenous compound), tacrolimus (drug therapy, special situation for pharmacovigilance), thalidomide (drug therapy, special situation for pharmacovigilance), thyrotropin (endogenous compound), thyroxine (endogenous compound), tif1 gamma antibody (endogenous compound), tofacitinib (drug therapy, special situation for pharmacovigilance), unclassified drug

Non-drug Index Terms

(Based on full text)

adolescent, age, alkaline phosphatase level, article, aspartate aminotransferase blood level, birth length, bone age, child, Childhood Myositis Assessment Scale score, clinical article, clinical outcome, cohort analysis, controlled study, **corticosteroid induced disease** (side effect), corticosteroid therapy, creatine kinase blood level, disease activity, disease duration, drug dose reduction, **drug efficacy**, **drug safety**, drug withdrawal, erythrocyte sedimentation rate, fatigue (side effect), female, follow up, free liothyronine index, free thyroxine index,

glucose blood level, **growth disorder** (side effect), growth rate, **hormonal therapy**, human, human tissue, interstitial lung disease, **juvenile dermatomyositis** (drug therapy), male, maximum permissible dose, multicenter study, muscle strength, nuclear magnetic resonance imaging, recurrence risk, refractory disease (drug therapy), relapse, retrospective study, school child, scoring system, thigh muscle, thyroid function test, treatment duration, treatment protocol

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Rheumatology
ISSN	14620332 (electronic), 14620324
CODEN	RUMAF
Source Type	Journal
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Publication Type	Article
Page Range	5611-5615
Country of Author	China
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1093/rheumatology/keaf236
MEDLINE PMID	40577072
Number of References	18
CAS Registry Numbers	2 hydroxybutyrate dehydrogenase (9067-92-9) alkaline phosphatase (9001-78-9) aspartate aminotransferase (9000-97-9) C reactive protein (9007-41-4) calcium (7440-70-2 , 14092-94-5) creatine kinase (9001-15-4) cyclosporine (59865-13-3 , 63798-73-2 ,

[79217-60-0](#))

glucose ([50-99-7](#), [84778-64-3](#), [8027-56-3](#))

hemoglobin A1c ([62572-11-6](#))

human growth hormone ([12629-01-5](#))

lactate dehydrogenase ([9001-60-9](#))

lactate dehydrogenase A

liothyronine ([6138-47-2](#), [6893-02-3](#))

methotrexate ([15475-56-6](#), [59-05-2](#), [7413-34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#), [60388-53-6](#))

methylprednisolone ([6923-42-8](#), [83-43-2](#))

mycophenolate mofetil ([116680-01-4](#), [128794-94-5](#), [115007-34-6](#))

prednisone ([53-03-2](#))

somatomedin C ([67763-96-6](#))

tacrolimus ([104987-11-3](#), [109581-93-3](#))

thalidomide ([50-35-1](#))

thyrotropin ([9002-71-5](#))

thyroxine ([7488-70-2](#))

tofacitinib ([477600-75-2](#), [540737-29-9](#))

32. [A case-control study on autoimmune polyendocrine syndromes in patients with systemic lupus erythematosus](#)

Piovani E., Gozzoli G.I., Della Pina S.E.L., Barison C., Orlandi C., Gatta E., Tomasi C., Fredi M., Cappelli C., Franceschini F.

Rheumatology 2025 64:11 (5656-5664)

Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Objectives This study aimed to investigate the prevalence of autoimmune polyendocrine syndromes in patients with SLE and to assess whether autoimmune polyendocrine syndromes predict higher disease activity or worse outcomes. **Methods** The clinical charts of 417 SLE patients (meeting SLICC 2012 and/or EULAR/ACR 2019 criteria) (referred to our centre between 2021 and 2023) were analysed.

Autoimmune polyendocrine syndrome cases were identified using

ORPHA definitions; 185 autoimmune polyendocrine syndrome-free SLE patients, randomly enrolled, served as controls. Demographic, clinical and serological data were collected. Results Forty-seven of 417 (11%) SLE patients had another autoimmune disease affecting the glands that allowed the diagnosis of autoimmune polyendocrine syndrome: 39 were diagnosed with Hashimoto thyroiditis, 6 with Graves' disease, and 3 with type 1 diabetes mellitus. Forty-five were affected by autoimmune polyendocrine syndrome type 3, and 2 by autoimmune polyendocrine syndrome type 4; no patients were diagnosed with autoimmune polyendocrine syndrome types 1 or 2. A comparison between the autoimmune polyendocrine syndrome-positive and -negative patients revealed no significant differences in clinical or serological features. At the last evaluation, ~80% of both groups of patients were in clinical remission, and approximately half of the patients remained on steroid therapy. Autoimmune polyendocrine syndrome-positive patients had a slightly higher median SLICC Damage Index (SDI), although this was not associated with increased disease activity. Conclusion The prevalence of autoimmune polyendocrine syndrome among SLE patients is significantly higher than in the general population (11% vs 0.005%), confirming the association between autoimmune thyroiditis and SLE. However, autoimmune polyendocrine syndrome-positive patients do not appear to have a more aggressive disease or develop more complications.

Drug Index Terms

(Based on full text)

anifrolumab (drug therapy), antimalarial agent (drug therapy), belimumab (drug therapy), beta2 glycoprotein 1 antibody (endogenous compound), biological product (drug therapy), corticosteroid (drug therapy), double stranded DNA antibody (endogenous compound), hydroxychloroquine (drug therapy), immunoglobulin G (endogenous compound), immunoglobulin M (endogenous compound), prednisone (drug therapy), rituximab (drug therapy), steroid (drug therapy), thyroid antibody (endogenous compound), thyroid peroxidase antibody (endogenous compound)

Non-drug Index Terms

(Based on full text)

acute lupus pneumonitis, Addison disease, adult, alopecia, antibody blood level, antiphospholipid syndrome, article, asthenia, atrophic

gastritis, **autoimmune disease** (drug therapy), autoimmune endocrine disease, autoimmune polyendocrinopathy candidiasis ectodermal dystrophy, autoimmune urticaria, case control study, Caucasian, chronic mucocutaneous candidiasis, clinical feature, cognitive defect, cohort analysis, controlled study, demographics, depression, disease activity, disease duration, endocarditis, enzyme linked immunosorbent assay, female, gene mutation, glomerulonephritis, Graves disease, Hashimoto disease, heart arrhythmia, heart infarction, hemolytic anemia, human, hypoparathyroidism, insulin dependent diabetes mellitus, interstitial lung disease, interstitial nephritis, kidney failure, leukopenia, livedo reticularis, lymphocytopenia, major clinical study, malar rash, male, microangiopathy, mouth ulcer, **multiple endocrine neoplasia** (drug therapy), multiple sclerosis, myocarditis, neutropenia, pericarditis, photosensitivity, pleurisy, premature ovarian failure, pulmonary hypertension, remission, rheumatoid arthritis, serology, serositis, skin manifestation, SLEDAI, steroid therapy, **systemic lupus erythematosus** (drug therapy), thrombocytopenia, treatment outcome

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Rheumatology
ISSN	14620332 (electronic), 14620324
CODEN	RUMAF
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	5656-5664
Country of Author	Italy
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1093/rheumatology/keaf320
MEDLINE PMID	40495344

Number of References	49
CAS Registry Numbers	anifrolumab (1326232-46-5) belimumab (356547-88-1) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) immunoglobulin G (97794-27-9 , 308067-58-5) immunoglobulin M (9007-85-6) prednisone (53-03-2) rituximab (174722-31-7)

33. [Association of IgG4-related disease with human inborn errors of immunity](#)

Batani V., Benetti E., Minici C., Mahajne J., Segala F., Lanzillotta M., Colavito D., Della-Torre E.

Rheumatology 2025 64:11 (5932-5947)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Objectives IgG4-related disease (IgG4-RD) is considered a complex multifactorial condition because immunological, environmental and genetic factors contribute to its pathogenesis and protean clinical manifestations. While immunological and environmental factors have been extensively investigated, the contribution of genetic factors remains poorly understood. We sought to investigate a predisposing genetic background associated with IgG4-RD. Methods Eighteen patients with IgG4-RD were Whole Exome Sequenced (WES) to assess germline variants in genes associated with human inborn errors of immunity (HIEI). Rare and ultra-rare variants with potential impact on protein structure were called and analysed. Fifty age-sex matched healthy individuals were used as controls. Results Germline ultra-rare variants in HIEI genes were enriched in IgG4-RD patients compared with the controls ($P = 0.0025$). Up to 10 rare gene variants were observed in a single patient. The most frequently mutated genes were PRKDC and LRBA. All variants were heterozygous, of uncertain significance and not

per se causative of a reported monogenic disorder. Incomplete phenotype of monogenic diseases associated with HIEI was observed in some patients. These variants were unique to IgG4-RD patients and were not observed in the controls. Conclusions Patients with IgG4-RD carry an increased frequency of ultra-rare germline variants in HIEI genes. This genetic background might predispose to IgG4-RD and to its variable manifestations while additional genetic or environmental triggers may be required to establish the complete clinical phenotype.

Drug Index Terms

(Based on full text)
cytotoxic T lymphocyte antigen 4 (endogenous compound), DNA methyltransferase 3B (endogenous compound), immunoglobulin E (endogenous compound), **immunoglobulin G** (endogenous compound), prednisone, rituximab

Non-drug Index Terms

(Based on full text)
adult, aged, amino acid substitution, aortitis, article, autoimmune lymphoproliferative syndrome, biliary tract disease, bone marrow depression, bronchiectasis, clinical article, clinical feature, combined immunodeficiency, comparative study, computer model, controlled study, disease association, disease duration, environmental factor, enzyme active site, female, gene frequency, genetic background, genetic variation, heredity, human, **human inborn errors of immunity**, immune dysregulation, immunoglobulin blood level, immunoglobulin deficiency, **immunoglobulin G4 related disease**, **inborn error of metabolism**, inheritance, innate immunity, lymphocytic infiltration, male, monogenic disorder, phagocyte dysfunction, pneumonia, protein stability, protein structure, retroperitoneal fibrosis, systemic disease, whole exome Sequencing

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Rheumatology
ISSN	14620332 (electronic), 14620324
CODEN	RUMAF
Source Type	Journal
Source Publication Date	2025-11-01

Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	5932-5947
Country of Author	Italy
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1093/rheumatology/keaf353
MEDLINE PMID	40569058
Number of References	43
CAS Registry Numbers	DNA (cytosine 5) methyltransferase DNA (cytosine 5) methyltransferase 1 DNA methyltransferase 3A DNA methyltransferase 3B immunoglobulin E (37341-29-0) immunoglobulin G (97794-27-9 , 308067-58-5) prednisone (53-03-2) rituximab (174722-31-7)

34. [Mepolizumab treatment for eosinophilic granulomatosis with polyangiitis \(EGPA\) with cardiac involvement](#)

Miller-Barmak A., Butbul Aviel Y.

Rheumatology 2025 64:11 (5993-5994)

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Drug Index Terms

(Based on full text)

amino terminal pro brain natriuretic peptide (endogenous compound),

antibiotic agent (drug therapy, special situation for pharmacovigilance),

low molecular weight heparin (drug therapy, special situation for pharmacovigilance), **mepolizumab** (drug combination, drug therapy, special situation for pharmacovigilance), methylprednisolone (drug therapy, intravenous drug administration, special situation for pharmacovigilance), prednisone (drug combination, drug therapy, oral drug administration, special situation for pharmacovigilance), ramipril (drug combination, drug therapy, special situation for pharmacovigilance), spironolactone (drug combination, drug therapy, special situation for pharmacovigilance), troponin (endogenous compound)

Non-drug Index Terms

(Based on full text)

adolescent, allergy, asthma, case report, child, **Churg Strauss syndrome** (diagnosis, drug therapy), clinical article, coughing, deep vein thrombosis (diagnosis, drug therapy), drug megadose, dyspnea, echocardiography, eosinophilia, female, femoral vein, fever, first-line treatment, heart atrium septum defect (diagnosis), **heart disease** (diagnosis, drug therapy), heart left ventricle ejection fraction, human, human tissue, intracardiac thrombosis (diagnosis), letter, leukocytoclastic vasculitis (diagnosis), low drug dose, lymphadenopathy, male, mediastinum lymphadenopathy (diagnosis), mitral valve regurgitation, myalgia, pansinusitis, pericardial effusion (diagnosis), pneumonia (diagnosis, drug therapy), pollen allergy, popliteal vein, prurigo, school child, skin biopsy, vernal conjunctivitis, x-ray computed tomography

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Rheumatology
ISSN	14620332 (electronic), 14620324
CODEN	RUMAF
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Letter
Page Range	5993-5994

Country of Author	Israel
Country of Source	United Kingdom
Language of Article	English
Digital object identifier(DOI)	10.1093/rheumatology/keaf412
MEDLINE PMID	40758478
Number of References	8
CAS Registry Numbers	mepolizumab (196078-29-2) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) ramipril (87333-19-5) spironolactone (52-01-7)

35. [Tolicizumab: a new option for refractory idiopathic recurrent pericarditis: a case report](#)

Collini V., Venturelli F., Quinzani F., Sappa R., Imazio M.

Eur. Heart J. - Case Rep. 2025 9:11

Embase

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Abstract

Background Recurrent pericarditis is the most common and troublesome complication of pericarditis and can be particularly challenging when it becomes refractory to conventional therapies, often resulting in a prolonged disease course. Interleukin-1 (IL-1) inhibitors have transformed the management of such cases; however, not all patients tolerate or respond adequately to these agents. Case summary We present a 37-year-old man suffering from idiopathic recurrent pericarditis since the age of 25. After failure of standard therapies, he became corticosteroid-dependent, so treatment with anakinra (an IL-1 inhibitor) was attempted twice, but discontinued early due to allergic reactions. As the recurrences were always characterised by an overt inflammatory phenotype (characterised by fever, increased C-reactive protein, and pericardial effusion), we administered off-label tocilizumab (an IL-6 receptor antagonist) as a last treatment option. The patient gained rapid

benefit and was able to discontinue steroid therapy during the 6-month follow-up, achieving complete recovery from pericarditis. Discussion Interleukin-6 is a key cytokine in the pathogenesis of pericarditis, acting synergistically with IL-1 to drive inflammation and acute phase responses. In our patient, blockade of IL-6 with tocilizumab led to the complete resolution of inflammation, suggesting a possible new therapeutic option for refractory cases. This result highlights the potential of IL-6 inhibition as an alternative strategy, when IL-1 blockers are contraindicated or ineffective.

Drug Index Terms

(Based on full text)

anakinra (adverse drug reaction, drug therapy), C reactive protein (endogenous compound), colchicine (drug therapy), corticosteroid (drug therapy), immunoglobulin (intravenous drug administration), indometacin, methylprednisolone (intravenous drug administration), nonsteroid antiinflammatory agent (drug therapy), prednisone (drug therapy), rilonacept, **tocilizumab** (drug therapy, special situation for pharmacovigilance, subcutaneous drug administration)

Non-drug Index Terms

(Based on full text)

adult, allergy (side effect), article, case report, clinical article, drug withdrawal, electrocardiography, fever, heart muscle necrosis, human, **idiopathic disease** (drug therapy), injection site, low drug dose, male, pericardial effusion, **pericarditis** (drug therapy), phenotype, **recurrent disease** (drug therapy), ST segment, steroid therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Eur. Heart J. - Case Rep.
ISSN	25142119 (electronic)
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-19 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	

Country of Author	Italy
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1093/ehjcr/ytaf535
Article Number	ytaf535
Number of References	18
CAS Registry Numbers	anakinra (143090-92-0) C reactive protein (9007-41-4) colchicine (64-86-8) immunoglobulin (9007-83-4) indometacin (53-86-1 , 74252-25-8 , 7681-54-1) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) rilonacept (501081-76-1) tocilizumab (375823-41-9)

36. [Donor-Derived Dengue Virus Transmission in Kidney Transplant—A Report of Three Cases](#)

Aceves-Sánchez B., Espinosa-González R., Castro-Almanza C.A., Ponce-De-León A., Ruiz-Palacios G.M., Ramos-Cervantes P., Román-Montes C.M., Morales-Buenrostro L.E., Vilatobá M., González-Lara M.F.

Am. J.Trop. Med. Hyg. 2025 113:5 (1079-1082)

Embase, MEDLINE, NURSING

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Abstract

Donor-derived dengue virus transmission is a rare but serious concern in hyperendemic regions, such as Mexico. Despite the risk, no standardized guidelines exist for dengue screening or organ acceptance criteria for donors. This report includes three kidney transplant recipients who

developed dengue virus infection post-transplant. Case 1: Recipient 1 was a 41-year-old man with chronic kidney disease (CKD) who received a second kidney transplant from a donor with a history of dengue. He developed severe dengue and recovered with supportive care. Recipient 2 was a 41-year-old woman with rheumatoid arthritis and diabetic nephropathy who received the other kidney from the same donor. She died due to severe dengue, which caused respiratory failure and coagulopathy. Case 2: A 40-year-old woman with CKD secondary to polycystic kidney disease developed mild dengue after a 4-week surgical delay due to the donor's dengue infection and recovered with supportive care. These cases highlight the urgent need for guidelines that address donor-derived dengue transmission in transplant patients. The current algorithm used for evaluating at-risk donors and recipients of solid organ transplantation at the study center is presented. Evidence of donor-derived dengue virus transmission in the literature is limited. Decisions regarding organ procurement from potential donors should be evidence-based; however, there are no international guidelines designed to assist clinicians in this context. In the current study, three cases of donor-derived dengue virus transmission after kidney transplantation in an endemic region of Mexico are described.

Drug Index Terms

(Based on full text)

immunoglobulin G (endogenous compound), mycophenolic acid, prednisone (drug therapy), tacrolimus, thymocyte antibody (endogenous compound)

Non-drug Index Terms

(Based on full text)

abdominal ecchymosis, abdominal hematoma, abnormally high substrate concentration in blood, adult, algorithm, anemia, arthralgia, article, bleeding, blood cell count, blood clotting disorder, brain hemorrhage, case report, chronic kidney failure, clinical article, clinician, cold ischemia time, computer assisted tomography, conservative treatment, dengue (diagnosis), **Dengue virus**, **dengue virus transmission**, diabetic nephropathy, Doppler flowmetry, drug withdrawal, ecchymosis, endemic disease, evidence based medicine, eye pain, female, fever, glycemic dysregulation, headache, hematoma, hematuria, human, hyperthyroidism, hypoxemia, immunoassay, immunosuppressive treatment, induction therapy, **kidney donor**, **kidney**

graft, kidney polycystic disease, kidney transplantation, maintenance therapy, male, middle aged, petechia, platelet count, pleura effusion, respiratory failure, retro orbital pain, reverse transcription polymerase chain reaction, rheumatoid arthritis (drug therapy), serotyping, somnolence, supportive care need, thrombocytopenia, thromboelastometry, virus transmission

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Am. J.Trop. Med. Hyg.
ISSN	14761645 (electronic), 00029637
CODEN	AJTHA
Source Type	Journal
Source Publication Date	2025-11-01
Entry Date	2025-11-17 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	1079-1082
Country of Author	Mexico
Country of Source	United States
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.4269/ajtmh.25-0060
MEDLINE PMID	40829580
Number of References	12
CAS Registry Numbers	immunoglobulin G (97794-27-9 , 308067-58-5) mycophenolic acid (23047-11-2 , 24280-93-1 , 37415-62-6) prednisone (53-03-2) tacrolimus (104987-11-3 , 109581-93-3)

37. [83. Anxiety and Vexas Syndrome: A Case Series of Six Patients Diagnosed With an Emerging Autoinflammatory Disorder](#)

Powell D., Raza H., Colon I.P., Patel B., Pao M.

J. Acad. Consul. Liaison. Psychiatry 2025 66: (S87)

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Abstract

Background: VEXAS (vacuoles, E1 ubiquitin mutation, X-linked, autoinflammation, somatic) syndrome is a newly emerging autoinflammatory syndrome first identified in 2020 (Beck, 2020). It impacts multiple organ systems, with hematologic and inflammatory disorders as especially common sequelae (Grayson, 2021). A recent study described that neurologic manifestations can be common in VEXAS (Bert-Marcaz, 2025). However, there is no known research detailing psychiatric manifestations. Prior SF36 survey data from patients with VEXAS syndrome have suggested these patients do not self-report significant mental health symptoms (Patel, 2024). The purpose of this case series is to describe psychiatric symptoms in six patients with VEXAS syndrome referred for consultation. Methods: Clinical information from six patients evaluated in consultation was obtained from psychiatric interview, ongoing clinical assessment, and detailed chart review. Results: All patients were male, ages 42-70 years old. The time between symptom onset and VEXAS genomic diagnosis was an average of 4.7 years (range: 1-8 years). All patients were steroid-dependent on admission (average prednisone dose: 25.8mg, range: 20-35mg). Five of six patients were referred for psychiatric consultation due to anxiety, four of whom were referred during pre-transplant evaluation. No patients had previously seen a psychiatrist, two were prescribed psychotropics by non-psychiatric physicians, and one had previously engaged in psychotherapy. Psychiatric evaluations revealed clinically significant anxiety in five of six cases. Discussion: Despite most patients in this case series self-reporting no prior psychiatric history, psychiatric consultations identified a cluster of patients with anxiety symptoms. This suggests the importance of prospective evaluation for patients with new or rare medical illnesses, as there is less known about associated psychiatric manifestations. Prospective evaluation can help identify potential relationships between a rare diagnosis, medical treatments

(e.g., chronic steroids) and psychiatric symptoms. Given the known connection between other autoinflammatory illness such as systemic lupus erythematosus, further research regarding psychiatric manifestations in VEXAS syndrome may identify specific psychiatric symptoms that are associated with the illness. As VEXAS is better characterized, it will be important to evaluate if anxiety is an early presenting symptom, perhaps due to a neuroinflammatory process. Alternatively, anxiety could be a psychological response to unexplained symptoms given the long period of time between medical symptom onset and VEXAS diagnosis. Conclusions: Patients with VEXAS syndrome, who have highly morbid disease with 10-year survival of 50%, may be at elevated risk for anxiety symptoms. However, additional prospective research is needed in order to better determine the true prevalence in this new patient population. Routine mental health screening and evaluation may help more accurately identify patients with psychiatric comorbidities in this patient population. References: - Beck, D.B., et al. (2020). Somatic mutations in UBA1 and severe adult-onset autoinflammatory disease. *New England Journal of Medicine*, 383(27), 2628-2638. - Bert-Marcz, et al. (2025). Neurological manifestations in patients with VEXAS syndrome. *Journal of Neurology*, 272(2), 181. - Grayson, P.C., Patel, B.A., and Young, N.S. (2021). VEXAS syndrome. *Blood, The Journal of the American Society of Hematology*, 137(26), 3591-3594. - Patel, B.A., et al. (2024). Patient Reported Outcomes in VEXAS Syndrome. *Blood*, 144(Supplement 1), 3711-3711.

Drug Index Terms

(Based on full text)

prednisone, psychotropic agent, steroid

Non-drug Index Terms

(Based on full text)

adolescent, adult, aged, **anxiety**, **autoinflammatory disease**, **case study**, child, clinical article, clinical assessment, comorbidity, conference abstract, controlled study, diagnosis, drug therapy, female, human, interview, male, medical history, medical record review, mental deficiency, mental disease, middle aged, neurologic disease, patient-reported outcome, prevalence, psychiatrist, psychotherapy, self report, Short Form 36, somatic mutation, special situation for pharmacovigilance, systemic lupus erythematosus

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J. Acad. Consul. Liaison. Psychiatry
ISSN	26672960 (electronic), 26672960
Source Type	Journal
Conference Name	Academy of Consultation-Liaison Psychiatry
Conference Location	United States, San Antonio
Conference Date	2025-11-19 to 2025-11-22
Source Publication Date	2025-11-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	S87
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2667296025006470
Publisher Item Identifier	S2667296025006470
Digital object identifier(DOI)	10.1016/j.jaclp.2025.10.085
CAS Registry Numbers	prednisone (53-03-2)

38. [236. Tacrolimus-Induced Parasomnia Post-Transplantation: A Case Report](#)

Nathu R., Jiang S., Nolasco D., Shah R., House C., Czuma R., Janssen B., Gunther M.

J. Acad. Consul. Liaison. Psychiatry 2025 66: (S167-S168)

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Abstract

Background: Tacrolimus, a calcineurin inhibitor, is widely used to prevent organ rejection post-transplantation. Despite its efficacy, it carries significant neuropsychiatric side effects, including psychosis and delirium, even at therapeutic doses. However, tacrolimus-induced parasomnias have not been previously reported. Parasomnias are abnormal motor or verbal behaviors during sleep, which can disrupt recovery and patient safety. This case highlights a rare instance of tacrolimus-induced parasomnia following bilateral lung transplantation, adding to the growing body of evidence on the drug's neurotoxic effects.

Case: Mrs. D, a 58-year-old woman with bronchiectasis and COPD, underwent a bilateral lung transplant due to hypoxemia. Her immunosuppressive regimen included tacrolimus, mycophenolate mofetil, and prednisone. Three days post-transplant, she developed auditory and visual hallucinations, sleep-talking, and bradyphrenia. Hallucinations consistently occurred one to two hours after her morning and evening tacrolimus doses, and sleep-talking followed nighttime doses. Neuroimaging revealed no abnormalities, and tapering or discontinuing other neuropsychiatric medications failed to improve her symptoms. The consult-liaison psychiatry team diagnosed delirium with mixed features and initiated haloperidol, guanfacine, and melatonin, resulting in partial improvement. However, her parasomnias and hallucinations persisted. The temporal association between tacrolimus administration and symptom onset prompted a switch to cyclosporine. Within two days of discontinuing tacrolimus, her parasomnias and hallucinations fully resolved.

Discussion: Tacrolimus-induced parasomnias are likely driven by the drug's neurotoxic effects on sleep-wake regulation. Tacrolimus disrupts gamma-aminobutyric acid (GABA) and glutamate pathways, which regulate cortical arousal and sleep stability. By increasing glutamate and reducing GABA levels, tacrolimus promotes wakefulness and sleep fragmentation, creating an unstable sleep-wake boundary that can trigger parasomnias. Additionally, postoperative delirium and increased blood-brain barrier permeability may have amplified tacrolimus's neurotoxic effects in this patient.

Management strategies include dose reduction or switching to alternative immunosuppressants, such as cyclosporine or mTOR inhibitors. However, mTOR inhibitors may impair wound healing, making them less suitable in early postoperative periods. In this case, switching to cyclosporine effectively resolved the symptoms without compromising immunosuppression.

Conclusion: This case underscores a novel neuropsychiatric side effect of tacrolimus: parasomnia.

Clinicians should be aware of this rare but significant complication, particularly in transplant patients with delirium. Early recognition, close monitoring, and immunosuppressant modification are essential to optimize both psychiatric and medical outcomes. This case also highlights the value of consultation-liaison psychiatry in identifying and managing complex neuropsychiatric presentations in transplant patients.

Drug Index Terms

(Based on full text)

4 aminobutyric acid, calcineurin inhibitor, cyclosporine, glutamic acid, guanfacine, haloperidol, melatonin, mycophenolate mofetil, prednisone, tacrolimus

Non-drug Index Terms

(Based on full text)

adverse drug reaction, arousal, bilateral lung transplantation, blood brain barrier, bradyphrenia, bronchiectasis, case report, chronic obstructive lung disease, clinical article, conference abstract, delirium, drug dose reduction, drug therapy, drug withdrawal, female, graft rejection, hallucination, human, human tissue, hypoxemia, immune deficiency, immunosuppressive treatment, neuroimaging, neurotoxicity, oral drug administration, **parasomnia**, patient safety, postoperative delirium, postoperative period, psychosis, side effect, sleep deprivation, sleep talking, sleep waking cycle, therapeutic dose, verbal behavior, visual hallucination, wakefulness, wound healing

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	J. Acad. Consul. Liaison. Psychiatry
ISSN	26672960 (electronic), 26672960
Source Type	Journal
Conference Name	Academy of Consultation-Liaison Psychiatry
Conference Location	United States, San Antonio
Conference Date	2025-11-19 to 2025-11-22
Source Publication Date	2025-11-01
Entry Date	2025-11-17 (Full record)

Publication Type	Conference Abstract
Page Range	S167-S168
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2667296025008006
Publisher Item Identifier	S2667296025008006
Digital object identifier (DOI)	10.1016/j.jaclp.2025.10.238
CAS Registry Numbers	4 aminobutyric acid (28805-76-7 , 56-12-2) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) glutamic acid (11070-68-1 , 138-15-8 , 56-86-0 , 6899-05-4) guanfacine (29110-47-2 , 29110-48-3) haloperidol (52-86-8 , 1511-16-6) melatonin (73-31-4) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2) tacrolimus (104987-11-3 , 109581-93-3)

39. [Clopidogrel-induced acquired haemophilia A in a post-TAVR patient: A case report and literature review](#)

Zaman M., Pan Z., Kohli R., Amir A., Zhang Y.
[In Process] Eur. Heart J.- Case Rep. 2025 9:11
Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Background Acquired haemophilia A is a rare bleeding disorder characterized by spontaneous bleeding. Apart from diagnosing and treating it promptly, finding the causative factor for acquired haemophilia A, if any, is equally important. Case summary In this case report, we

discuss a 63-year-old female who underwent a recent transcatheter aortic valve replacement and presented to the emergency department with posterior thigh pain, where she was found to have a haematoma. She was ultimately diagnosed with acquired haemophilia A secondary to clopidogrel. In addition to discontinuing clopidogrel, the patient received immunosuppressive therapy, including cyclosporine, prednisone, intravenous immunoglobulins, rituximab, and recombinant factor VIIa. Discussion Clopidogrel, one of the most commonly used medications in cardiology, is recognized as one of the leading drugs associated with acquired haemophilia A. In this case report, a review of the literature on ticagrelor and clopidogrel-induced acquired haemophilia A is also included, emphasizing the need for extreme vigilance in patients on antiplatelet therapy who present with unexplained bleeding.

Drug Index Terms

clopidogrel, cyclosporine, prednisone, recombinant blood clotting factor 7a, rituximab, **ticagrelor**

Non-drug Index Terms

adult, case report, clinical article, diagnosis, drug administration, drug dose, drug therapy, emergency ward, female, hematoma, **hemophilia A**, human, immunosuppressive treatment, intravenous drug administration, middle aged, review, surgery, thigh, **transcatheter aortic valveImplantation**

Comment: valid ICSR

ADDITIONAL INFORMATION

Abbreviated Journal Title Eur. Heart J.- Case Rep.

ISSN 25142119 (electronic)

Source Type Journal

Source Publication Date 2025-11-01

Entry Date 2025-11-19 (In process)

Publication Type Review

Page Range

Country of Author United States

Country of Source United Kingdom

Language of Article English

Language of Summary	English
Digital object identifier(DOI)	10.1093/ehjcr/ytaf459
Article Number	ytaf459
Number of References	17

40. [Granuloma faciale and Takayasu arteritis in a child: a case report](#)

Liao W., Long J., Tang J.-P., Wo D.-N., Shu Y., Wei Z.

[In Process] Chin. J. Contemp. Pediatr. 2025 27:10 (1266-1270)

Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

An 11-year-old boy presented with erythematous plaques over the bilateral mandibular and mental regions for 2 years, accompanied by cough and dyspnea for more than 2 months. Chest computed tomography angiography revealed marked stenosis of the right pulmonary artery, irregular aortic caliber, and aortic wall thickening. Histopathological examination of the skin lesion, including immunohistochemistry and special stains, confirmed a chronic suppurative inflammation. Whole-exome sequencing was negative. A final diagnosis of granuloma faciale and Takayasu arteritis was established. Combination therapy with systemic tocilizumab, prednisone, and methotrexate, along with topical 0.1% tacrolimus ointment, resulted in a favorable clinical response. This report summarizes the clinical features of a pediatric case of granuloma faciale and Takayasu arteritis and reviews the etiology, diagnostic approach, and current treatment strategies for the disorders, aiming to enhance clinicians' understanding of these conditions.

Drug Index Terms

methotrexate, prednisone, tacrolimus, tocilizumab

Non-drug Index Terms

adverse drug reaction, **aortic arch syndrome**, arterial wall thickening, article, case report, child, clinical article, clinical feature, computed

tomographic angiography, coughing, diagnosis, drug dose reduction, drug therapy, dyspnea, erythema, histopathology, human, human tissue, immunohistochemistry, male, mandible, ointment, oral drug administration, right pulmonary artery, school child, skin defect, **skin granuloma**, special situation for pharmacovigilance, suppuration, unexpected outcome of drug treatment, whole exome sequencing

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Chin. J.Contemp. Pediatr.
ISSN	20969228 (electronic), 10088830
Source Type	Journal
Source Publication Date	2025-10-15
Entry Date	2025-11-21 (In process)
Publication Type	Article
Page Range	1266-1270
Country of Author	China
Country of Source	China
Language of Article	Chinese
Language of Summary	English, Chinese
Digital object identifier (DOI)	10.7499/j.issn.1008-8830.250418 5
MEDLINE PMID	41121705
Number of References	21

41. [Single Center Experiences on Chimeric Antigen Receptor T-cell Therapy](#)

Sun K.-J., Chang Y.-C., Chang Y.-F., Chen Y.-J.

Int. J.Gerontol. 2025 19:4 (289-293)

Embase, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

Chimeric antigen receptor T-cell therapy (CAR T-cell therapy) has emerged as a groundbreaking treatment for relapsed or refractory B-cell lymphomas and leukemia. This personalized immunotherapy has demonstrated high efficacy, with complete remission (CR) rates ranging from 40% to over 80% in aggressive forms of lymphoma. We presents three cases from MacKay Memorial Hospital, illustrating the benefits and challenges associated with CAR T-cell therapy. In the first case, a 53-year-old female with B-acute lymphoblastic leukemia achieved CR after CAR T-cell therapy, with CAR T-cells detectable for at least six months. The second case involved a 49-year-old female with diffuse large B-cell lymphoma who experienced severe cytokine release syndrome (CRS) and ultimately passed away due to complications. The third case featured a 62-year-old male with mantle cell lymphoma, who initially responded to CAR T-cell therapy but later relapsed and succumbed to disease complications. The article emphasizes the importance of effective management of side effects such as CRS and neurotoxicity, which can significantly impact patient outcomes. Individualized management strategies are crucial. Future directions for CAR T-cell therapy include exploring next-generation constructs, alternative targets beyond CD19, and combining other treatment modalities. In conclusion, while CAR T-cell therapy represents a significant advancement, ongoing research is essential to optimize patient selection and management strategies.

Drug Index Terms

(Based on full text)

antibiotic agent (drug therapy), axicabtagene ciloleucel (adverse drug reaction, drug comparison, drug therapy, drug toxicity), BCR ABL protein (endogenous compound), BCR ABL1 protein (endogenous compound), bortezomib (drug combination, drug therapy, unexpected outcome of drug treatment), carmustine (drug combination, drug therapy), cefepime (drug therapy, unexpected outcome of drug treatment), cisplatin (drug combination, drug therapy), cyclophosphamide (drug combination, drug therapy, intrathecal drug administration, unexpected outcome of drug treatment), cyclophosphamide plus doxorubicin plus prednisolone plus rituximab plus vincristine (drug therapy), cytarabine (drug combination, drug therapy), dasatinib (adverse drug reaction, drug therapy), dexamethasone (drug combination, drug therapy, intrathecal drug administration, unexpected outcome of drug treatment), doxorubicin

(drug combination, drug therapy, intrathecal drug administration, unexpected outcome of drug treatment), etoposide (drug combination, drug therapy), fludarabine (drug therapy), glofitamab (drug combination, drug therapy), ibrutinib (drug therapy), imatinib (drug therapy, unexpected outcome of drug treatment), inotuzumab ozogamicin (drug therapy), lactate dehydrogenase (endogenous compound), melphalan (drug combination, drug therapy), meropenem (drug therapy), methotrexate (drug combination, drug therapy), methylprednisolone (drug combination, drug therapy), micafungin (drug therapy), obinutuzumab (drug combination, drug therapy), ponatinib (drug therapy), prednisolone (drug therapy), prednisone (drug combination, drug therapy, unexpected outcome of drug treatment), rituximab (drug combination, drug therapy, unexpected outcome of drug treatment), ruxolitinib (drug therapy, unexpected outcome of drug treatment), teicoplanin (drug therapy), tirabrutinib (drug therapy), tisagenlecleucel T (adverse drug reaction, drug comparison, drug therapy, drug toxicity), tocilizumab (drug therapy, unexpected outcome of drug treatment), unclassified drug, vincristine (drug combination, drug therapy, intrathecal drug administration)

Non-drug Index Terms

(Based on full text)

adult, antibiotic therapy, article, ascites, autologous hematopoietic stem cell transplantation, autologous stem cell transplantation, **B cell acute lymphoblastic leukemia** (drug therapy, therapy), bladder, **brain lymphoma** (drug therapy, therapy), cancer combination chemotherapy, cancer radiotherapy, cancer recurrence, cancer survival, capillary leak syndrome (diagnosis), case report, chimeric antigen receptor T-cell (adverse device effect, device comparison), **chimeric antigen receptor T-cell immunotherapy**, clinical article, compassionate use, computer assisted tomography, **cytokine release syndrome** (complication, drug therapy, side effect, therapy), **diffuse large B cell lymphoma** (drug therapy, therapy), disease severity, drug dose reduction, drug megadose, drug substitution, drug withdrawal, ECOG Performance Status, female, fever (drug therapy, complication, therapy), follicular lymphoma (drug therapy, therapy), follow up, human, human cell, hypotension (complication, drug therapy), hypoxemic respiratory failure (complication), hypoxia (drug therapy, complication, therapy), **immune effector cell associated neurotoxicity syndrome** (complication, drug therapy, side effect), intracranial hypertension, leukemia relapse,

leukemia remission, lung consolidation, lymphadenopathy, lymphocytosis, maintenance chemotherapy, male, **mantle cell lymphoma** (drug therapy, radiotherapy, therapy), middle aged, multiple cycle treatment, **neurotoxicity** (complication, drug therapy, side effect), neutropenia, omentum, overall survival, oxygen therapy, Philadelphia 1 chromosome (drug therapy, therapy), progression free survival, QT prolongation (side effect), refractory cancer (therapy), seizure, sepsis, single drug dose, thorax radiography

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Int. J. Gerontol.
ISSN	1873958X (electronic), 18739598
Source Type	Journal
Source Publication Date	2025-10-01
Entry Date	2025-11-17 (Full record), 2025-11-10 (Article in Press/In process)
Publication Type	Article
Page Range	289-293
Country of Author	Taiwan
Country of Source	Singapore
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.6890/IJGE.202510_19(4).0017
Number of References	20
Device Manufacturers	pell bmt (Taiwan)
Drug Manufacturers	Hoffmann La Roche (Switzerland), Ono (Japan), TTY (Taiwan), AbbVie (United States), Bristol Myers Squibb (United States), Pfizer (United States), Takeda (United States)
CAS Registry Numbers	BCR ABL protein bortezomib (179324-69-7 , 197730-97-5 , 444576-08-3)

Bruton tyrosine kinase
carmustine ([154-93-8](#))
cefepime ([88040-23-7](#))
cisplatin ([15663-27-1](#), [26035-31-4](#), [96081-74-2](#))
cyclophosphamide ([50-18-0](#), [6055-19-2](#))
cytarabine ([147-94-4](#), [69-74-9](#))
dasatinib ([302962-49-8](#), [863127-77-9](#), [2398469-56-0](#))
dexamethasone ([50-02-2](#))
doxorubicin ([23214-92-8](#), [25316-40-9](#))
etoposide ([33419-42-0](#), [433304-61-1](#))
fludarabine ([21679-14-1](#))
glofitamab ([2229047-91-8](#))
ibrutinib ([936563-96-1](#), [936563-97-2](#))
imatinib ([152459-95-5](#), [220127-57-1](#))
inotuzumab ozogamicin ([635715-01-4](#))
lactate dehydrogenase ([9001-60-9](#))
lactate dehydrogenase A
melphalan ([148-82-3](#), [3223-07-2](#))
meropenem ([96036-03-2](#))
methotrexate ([15475-56-6](#), [59-05-2](#), [7413-34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#), [60388-53-6](#))
methylprednisolone ([6923-42-8](#), [83-43-2](#))
micafungin ([208538-73-2](#), [235114-32-6](#))
obinutuzumab ([949142-50-1](#))
ponatinib ([943319-70-8](#), [1114544-31-8](#))
prednisolone ([50-24-8](#))
prednisone ([53-03-2](#))
proteintyrosine kinase Csk
rituximab ([174722-31-7](#))
ruxolitinib ([1092939-17-7](#), [941678-49-5](#))
teicoplanin ([61036-62-2](#), [61036-64-4](#))
tirabrutinib ([1351636-18-4](#), [1439901-97-9](#))
tisagenlecleucel T([1823078-37-0](#))
tocilizumab ([375823-41-9](#))
vincristine ([57-22-7](#))

42. [BCMA-targeted CAR Tcell therapy can effectively induce disease remission in refractory lupus nephritis](#)

Hu Z., Cai S., Yu Y., Chen Y., Wang B., Wang D., Zeng R., He X., Shen G., Yu F., Zeng Z., Chen Y., Luo X., Zhang Z., Zhang P., Xiong H., Bai L., Ye P., Lin S., Zhang J., Ye C., Li C., Dong L.

Ann. Rheum. Dis. 2025 84:10 (1675-1683)

Embase, MEDLINE

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Abstract

Objectives: This study aims to evaluate the safety and efficacy of anti-B cell maturation antigen (BCMA)-targeting chimeric antigen receptor (CAR) Tcells in treating refractory lupus nephritis (LN). **Methods:** This is an open-label, single-arm clinical trial assessing anti-BCMA CAR T cells in treating refractory LN patients. CAR Tcells were infused following lymphodepletion therapy with cyclophosphamide and fludarabine. Patients were regularly followed up, in which clinical assessments were performed and laboratory indices were collected. Repeated renal biopsies and single cell RNA sequencing in certain patients were performed to help evaluate the therapeutic efficacy. The primary endpoints were the safe dosage of single infusion and the occurrence of adverse events, while the secondary endpoints focused on the therapeutic efficacy (eg, changes of Systemic Lupus Erythematosus Disease Activity Index 2000 [SLEDAI-2000]). **Results:** Seven biopsy-confirmed LN patients were enrolled in this study with a median follow-up of 9 months, whose peripheral B cells were effectively eliminated within the first month postinfusions and restored within 3 months except one patient. Hypogammaglobulinemia was frequently observed in this study. Only one case of grade 1 cytokine release syndrome was noted, and no immune effector cell-associated neurotoxicity syndrome or severe infection was reported. SLEDAI-2K scores decreased from a median of 18 (range 10-22) at baseline to 0 (range 0-4) at the last follow-up, and 5 out of 7 patients achieved definition of remission in systemic lupus erythematosus (DORIS) complete remission. Decreased deposition of immune complex, reduced clonal abundance, and alleviated proinflammatory status of peripheral lymphocytes were also observed in repeated renal biopsy and transcriptomic analyses,. **Conclusions:** Anti-BCMA CAR T therapy can help LN patients inducing

and maintaining disease remission status safely and effectively, which indicated the potential feasibility of the BCMA-only-targeting strategy in treating autoimmune diseases with abnormal humoral immune responses through CAR products.

Drug Index Terms

(Based on full text)

B cell maturation antigen (endogenous compound), belimumab (drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), cyclophosphamide (special situation for pharmacovigilance), cyclosporine (drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), fludarabine (special situation for pharmacovigilance), hydroxychloroquine (drug therapy, special situation for pharmacovigilance), immunoglobulin (intravenous drug administration, special situation for pharmacovigilance), mycophenolate mofetil (drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), prednisone (special situation for pharmacovigilance), rituximab (drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), tacrolimus (drug therapy, special situation for pharmacovigilance), telitacicept (drug therapy, special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

adult, aged, article, bioinformatics software, CD4+ Tlymphocyte, CellRanger Software, **chimeric antigen receptor T-cell immunotherapy**, clinical article, continuous infusion, creatinine blood level, cytokine release syndrome, data analysis software, disease activity, drug dose reduction, drug withdrawal, enzyme linked immunosorbent assay, estimated glomerular filtration rate, female, follow up, GSVA package, human, human cell, human tissue, immunoglobulin deficiency, immunosuppressive treatment, kidney biopsy, leukocyte count, leukopenia, **lupus erythematosus nephritis** (drug therapy), lymphocyte count, male, neutropenia, outcome assessment, package Seurat, peripheral lymphocyte, platelet count, proteinuria, **remission**, scRepertoire, single cell RNA seq, systemic lupus erythematosus, transcriptomics

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Ann. Rheum. Dis.
ISSN	14682060 (electronic), 00034967
CODEN	ARDIA
Source Type	Journal
Source Publication Date	2025-10-01
Entry Date	2025-11-17 (Full record), 2025-07-24 (Article in Press/In process)
Publication Type	Article
Page Range	1675-1683
Country of Author	China, China, China, China
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S0003-4967(25)04186-X
Publisher Item Identifier	S0003-4967(25)04186-X
Digital object identifier(DOI)	10.1016/j.ard.2025.06.2128
MEDLINE PMID	40681435
Number of References	36
Device Tradenames	CellRanger Software, GSVA package, package Seurat, scRepertoire
CAS Registry Numbers	belimumab (356547-88-1) cyclophosphamide (50-18-0 , 6055-19-2) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) fludarabine (21679-14-1) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) immunoglobulin (9007-83-4) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2) rituximab (174722-31-7)

tacrolimus ([104987-11-3](#), [109581-93-3](#))
telitacicept ([2136630-26-5](#))

43. [56-Year-Old Woman With Lower Extremity Pain and Edema, Nausea, and Chest Pain](#)

Hill K.A., Grassi A.R., Melduni R.M.

Mayo Clin. Proc. 2025 100:10 (1830-1835)

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Drug Index Terms

(Based on full text)

acetylsalicylic acid (drug therapy), adalimumab (drug therapy), clopidogrel (drug therapy), creatine kinase (endogenous compound), creatinine (endogenous compound), cystatin C (endogenous compound), electrolyte (endogenous compound), heparin (drug therapy), myoglobin (endogenous compound), pantoprazole (drug therapy), paracetamol (drug therapy), prednisone (drug therapy, unexpected outcome of drug treatment), rituximab (drug therapy), steroid (drug therapy), troponin T (endogenous compound)

Non-drug Index Terms

(Based on full text)

adult, alcohol consumption, blood pressure, case report, clinical article, coronary atherosclerosis, drug dose increase, electrocardiogram, electromyography, estimated glomerular filtration rate, female, heart rate, human, **lower limb**, middle aged, note, nuclear magnetic resonance imaging, osteoarthritis, oxygen saturation, **pain** (drug therapy), percutaneous coronary intervention, positron emission tomography, rhabdomyolysis (drug therapy), rheumatoid arthritis (drug therapy), ST segment elevation myocardial infarction (drug therapy), **thorax pain**

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Mayo Clin. Proc.

ISSN 14682060 (electronic), 00034967

CODEN ARDIA

Source Type	Journal
Source Publication Date	2025-10-01
Entry Date	2025-11-17 (Full record), 2025-09-05 (Article in Press/In process)
Publication Type	Note
Page Range	1830-1835
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Publisher Item Identifier	S0025-6196(24)00661-X
Publisher Item Identifier	S0025-6196(24)00661-X
Digital object identifier(DOI)	10.1016/j.mayocp.2024.10.028
MEDLINE PMID	40888784
Number of References	12
CAS Registry Numbers	acetylsalicylic acid (493-53-8 , 50-78-2 , 53663-74-4 , 53664-49-6 , 63781-77-1) adalimumab (331731-18-1 , 1446410-95-2) clopidogrel (113665-84-2 , 120202-66-6 , 90055-48-4 , 94188-84-8 , 120202-65-5 , 120202-67-7 , 894353-16-3 , 744256-69-7) creatine kinase (9001-15-4) creatinine (19230-81-0 , 60-27-5) heparin (37187-54-5 , 8057-48-5 , 8065-01-8 , 9005-48-5 , 9041-08-1) pantoprazole (102625-70-7 , 164579-32-2 , 138786-67-1) paracetamol (103-90-2) prednisone (53-03-2) rituximab (174722-31-7) troponin T (60304-72-5)

44. [Systemic Lupus Erythematosus Presenting as Chorea](#)

Chovatiya H.

Mov. Disord. 2025 40: (S303)

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Abstract

Objective: To study the clinical course, imaging, diagnosis and management of acute onset of generalized chorea with suspected Systemic lupus Erythematosus (SLE). **Background:** Neuropsychiatric symptoms of SLE (NPSLE) comprise of 19 syndromes, including movement disorders¹. Chorea, a hyperkinetic movement disorder characterised by involuntary, dance-like, and unpredictable movements, is rare in SLE, with a cumulative incidence of 0.6%.² **Methods:** A 17 year old boy, first-born child of healthy and nonconsanguineous parents, presented with generalized chorea, difficulty in speaking, writing and walking for last 1 months, without history of dystonia, tremor, feeding difficulty and psychiatric or cognitive impairment. There is no relevant past or family history, including neuroleptic ingestion. Further, his mother informed of intermittent fever, dry cough, back pain, joint pain and leg pain with pedal oedema since last 3 years, with multiple admissions and symptomatic management. On physical examination, he was conscious, oriented and following commands, with normal cranial nerve, motor, sensory and cerebellar examination. He has generalized chorea involving all four limbs, face and tongue. **Results:** Renal and Liver function tests were normal. ASO Titer [47 IU/ml (normal<200IU/ml)] was normal.

Significant derangements were Hypochromic Microcytic Anemia [Hb 9.5g/dl, MCV 73.9fl, MCH 23.2pg] with proteinuria (176 mg/dl) and positive APLA, Direct Coomb's Test and ANA. Further evaluation showed positive dsDNA antibody (187.85 IU/ml), low C3 level [51 mg/dl] (Normal 88-165) and low C4 Level [<8 mg/dl] (Normal 14-44)]. 2D Echo and MRI Brain were normal. He was started on immunosuppressive therapy - methylprednisolone 1g/day for three days, followed by prednisone 1mg/kg/day and mycophenolate mofetil, up to a dose of 2g/day and Tab. Tetrabenazine 75mg/day and Tab. Risperidone 2mg/day without any response. Hence, Inj. Rituximab 1gm was administered with favourable response with completely resolution of chorea. **Conclusions:** Our case highlights the crucial importance of a wellconducted interview and

clinical observation, and the richness of a thorough differential diagnosis, underlining the importance of maintaining a high index of suspicion. This case illustrates the relevance of SLE in the differential diagnosis of acute-onset movement disorders in young patients.

Drug Index Terms

(Based on full text)
methylprednisolone, mycophenolate mofetil, neuroleptic agent, prednisone, risperidone, rituximab, tetrabenazine

Non-drug Index Terms

(Based on full text)
arthralgia, backache, case report, cerebellum, child, **chorea**, clinical article, cognitive defect, conference abstract, Coombs test, cranial nerve, dancing, diagnosis, differential diagnosis, disease course, drug therapy, dry cough, dystonia, family history, feeding difficulty, fever, foot edema, human, hyperkinesia, immunosuppressive treatment, interview, iron deficiency anemia, iron refractory iron deficiency anemia, leg pain, liver function test, male, mean corpuscular volume, mental deficiency, motor dysfunction, MRI scanner, nuclear magnetic resonance imaging, physical examination, proteinuria, streptolysin titer, **systemic lupus erythematosus**, tremor, walking

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Mov. Disord.
ISSN	08853185
Source Type	Journal
Conference Name	2025 International Congress of Parkinson's Disease and Movement Disorders
Conference Location	United States, Honolulu, HI
Conference Date	2025-10-05 to 2025-10-09
Source Publication Date	2025-10-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	S303

Country of Author	India
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1002/mds.30317
CAS Registry Numbers	methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2) risperidone (106266-06-2) rituximab (174722-31-7) tetrabenazine (58-46-8)

45. [Adult-Onset Chorea and Psychosis in a 5G-Year-Old Man with Elevated Anti-Streptolysin O \(ASO\) Titer](#)

Soriano Ballivian A., Singh R., Reshi R., Verma U., Khan Z., Los O.
Mov. Disord. 2025 40: (S307)
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Abstract

Objective: To review the pathophysiology of neuroimmune disease caused by streptococcal infection using as an example a case of a 59-year-old man with choreiform movements and psychosis having elevated antistreptolysin O (ASO) titer. **Background:** Excessive activation of immune cells after a group A β -hemolytic streptococcal infection can lead to antibodies attacking the basal ganglia, disrupting movement, cognition, and behavior. This can result in syndromes such as pediatric autoimmune neuropsychiatric disorders associated with streptococcal infection (PANDAS) and Sydenham chorea. **Methods:** Case report and review of literature **Results:** 59-year-old man with a history of recurrent pharyngitis during childhood, presented with abnormal movements manifesting as brief elevation of the right shoulder, orofacial dyskinesias, followed by dystonic rightward head turn, and non-rhythmic choreiform

shoulder shrugging and abduction movements associated with insomnia and visual hallucinations for 7 days. Due to the concern for status epilepticus, he was intubated and sedated with propofol, with continuous electroencephalogram capturing involuntary movements without epileptiform correlation. He was given a 5-day course of intravenous immunoglobulins for possible autoimmune encephalitis (steroids were avoided since infectious etiology could not be ruled out), without any clinical response, with re-emerging choreiform movements off sedation. Diagnostic workup showed the following: elevated serum ASO titer at 297 (normal <200) and anti-DNAase antibody titer at 148 (normal <85), mild elevation of ESR and CRP and normal brain imaging. Mayo autoimmune encephalitis panel was negative. As a phenomenology similar to PANDAS was suspected, the patient was treated with a 7-day course of ceftriaxone, methylprednisolone 1000 mg daily, and risperidone 1 mg every 12 hours for 5 days leading to complete resolution of the abnormal movements and normalization of mental status. He was discharged home with 6 weeks of prednisone taper. Conclusions: The autoantibodies generated due to the cross-reactivity of the group A carbohydrate of the streptococci impairs the activation of the calcium-calmodulin-dependent protein kinase II (CaM kinase II) in the basal ganglia cells. The understanding of the pathophysiology of autoimmune neuropsychiatric disorders associated with streptococcal infection can aid to an early diagnosis and treatment.

Drug Index Terms

(Based on full text)

antistreptolysin, autoantibody, C reactive protein, calcium calmodulin dependent protein kinase II, carbohydrate, ceftriaxone, methylprednisolone, prednisone, propofol, risperidone, steroid

Non-drug Index Terms

(Based on full text)

abduction, adult, antibody titer, autoimmune encephalitis, basal ganglion, case report, **chorea**, chorea minor, clinical article, cognition, conference abstract, cross reaction, diagnosis, drug therapy, dyskinesia, dystonia, electroencephalogram, electroencephalography, epilepsy, epileptic state, group A streptococcal infection, human, immunocompetent cell, insomnia, intravenous drug administration, involuntary movement, male, mental disease, neuroimaging, orofacial dyskinesia, pharyngitis, phenomenology, **psychosis**, rheumatic fever,

sedation, Streptococcus infection, **streptolysin titer**, visual hallucination

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Mov. Disord.
ISSN	08853185
Source Type	Journal
Conference Name	2025 International Congress of Parkinson's Disease and Movement Disorders
Conference Location	United States, Honolulu, HI
Conference Date	2025-10-05 to 2025-10-09
Source Publication Date	2025-10-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	S307
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1002/mds.30317
CAS Registry Numbers	antistreptolysin (9006-92-2) C reactive protein (9007-41-4) calcium calmodulin dependent protein kinase II (141467-21-2) ceftriaxone (73384-59-5 , 74578-69-1 , 104376-79-6) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) propofol (2078-54-8 , 113981-41-2) risperidone (106266-06-2)

46. [OCRELIZUMAB INDUCED COLITIS](#)

Potts K.

BMJ Neurol. Open 2025 7: (A36)

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Abstract

Background Ocrelizumab is a humanised anti CD20 monoclonal antibody. Thirty nine cases of ocrelizumab associated colitis are reported in literature and adverse event databases. The majority describe onset within months of ocrelizumab initiation. We herein report a case of severe ocrelizumab induced colitis following five years of therapy. Case presentation A 68-year-old male presented with four weeks of non-bloody diarrhoea and generalised abdominal pain. Pertinent medical history included primary progressive multiple sclerosis, on 6 monthly ocrelizumab for the proceeding five years. Stool testing identified Blastocystic species but was otherwise negative. Colonoscopy demonstrated deep ulcers in the left colon and near complete denudation of the right colon. Biopsy revealed extensive ulceration with focal ischaemic changes and an inflammatory infiltrate characterised by an absence of B cells. CMV was excluded by serology and biopsy. No improvement occurred following 48 hours of empirical intravenous antibiotics. Budesonide and intravenous methylprednisone were commenced for severe drug induced colitis. Prednisone was initiated at 60mg daily. On day 17 of prednisone therapy the patient experienced worsening abdominal pain. Inflammatory markers were newly up trending. They received a three-day course of methylprednisone and tofacitinib was commenced. Clinical and biochemical improvement was achieved. Three months later the patient remained well, with normalisation of biochemical parameters. Budesonide and prednisone were weaned. Tofacitinib continues. Conclusions CD20+ cells play a regulatory and protective role in the gastrointestinal system. Variable responses to treatment of ocrelizumab induced colitis are evidenced in case reports. This is the first case employing tofacitinib to achieve disease control.

Drug Index Terms

(Based on full text)

antibiotic agent, budesonide, meprednisone, **ocrelizumab**, prednisone, tofacitinib

Non-drug Index Terms

(Based on full text)

abdominal pain, adult, bloody diarrhea, case report, clinical article, **colitis**, colonoscopy, conference abstract, drug therapy, drug withdrawal, human, inflammatory infiltrate, intravenous drug administration, male, medical history, progressive multiple sclerosis, serology, therapy

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	BMJ Neurol. Open
ISSN	26326140
Source Type	Journal
Conference Name	ANZAN Annual Scientific Meeting 2025
Conference Location	Australia, Brisbane
Conference Date	2025-06-10 to 2025-06-13
Source Publication Date	2025-10-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	A36
Country of Author	Australia
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1136/bmjno-2025-ANZAN.97
CAS Registry Numbers	budesonide (51333-22-3 , 51372-29-3) meprednisone (1247-42-3) ocrelizumab (637334-45-3) prednisone (53-03-2) tofacitinib (477600-75-2 , 540737-29-9)

47. [Thyroid Tempest and Heartbreak: A Rare Cause of Cardiogenic Shock](#)

Diaz E.R., Owczarzak L., Harhxi R., Porter T.

J.Endocr. Soc. 2025 9: (A1246)

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Abstract

Background: Thyroid storm is a rare but lifethreatening condition, often triggered by infection, surgery, or untreated hyperthyroidism. This case presents an atypical presentation, mimicking rapidly progressing cardiogenic shock. Body: A 50-year-old female with a history of CAD, hyperlipidemia, SJS-TEN, end stage renal disease, and atrial fibrillation presented with weight loss, alopecia, diarrhea, and URI symptoms. On exam she was alert and oriented, warm, diaphoretic, tachypneic, and hypotensive, with tachycardia (172 bpm) and febrile (101.1 °F). Lab studies showed HCO₃ 18 mmol/L (22 - 29 mmol/L), AG 21 (8 - 16), high sensitivity troponin 34309 ng/L (0 - 17 ng/L), lactic acid 5.9 mmol/L (0.4 - 1.3 mmol/L), WBC 16.3 K/uL (4 - 10 K/uL). Blood cultures grew pan susceptible MSSA. EKG showed T-wave inversions in I and aVL, CXR revealed bilateral pleural effusions, interstitial edema, and severe coronary artery calcifications. Transthoracic echocardiogram showed EF of 25% with global hypokinesis. The patient was started on treatment for cardiogenic and septic shock and was subsequently transferred to the cardiac ICU. Further work up revealed TSH < 0.008 uIU/mL (0.35 - 4.94 uIU/mL), free T₄ > 5 ng/dL (0.7 - 1.5 ng/dL), free T₃ 7.42 pg/mL (1.58 - 3.91 pg/mL), TSI 0.23 IU/L (0 - 0.55 IU/L), BNP 4951 pg/mL (0 - 100 pg/mL), MvO₂ 55% (60-85%), CI 1.7 L/min/m² (2.5 to 3.5 L/min/ m²), CVP 18 cmH₂O (5 to 12 cm H₂O) indicating poor perfusion. Thyroid ultrasound showed thyromegaly without increased vascularity. Dobutamine was initiated but LHC was deferred due to her history of SJS/TEN with iodine contrast. The patient's Burch-Wartofsky was calculated to be 85 so treatment for thyroid storm was initiated with propylthiouracil 500 mg loading dose, then 200 mg every 4 hours, and hydrocortisone 50 mg every 6 hours; potassium iodide avoided due to iodine allergy. Within 12 hours, the patient's lab studies improved, vasopressors were discontinued and EF improved to 50-55% without motion abnormalities. She was

transitioned to methimazole 40 mg daily and prednisone taper with 40 mg daily for 5 days. On outpatient follow up; methimazole was reduced to 20 mg daily. Conclusion: This case is noteworthy for its atypical presentation, with reversible cardiogenic shock in the context of thyrotoxicosis which occurs in 1-3% in patients experiencing thyroid storm. The patient's rapid deterioration and subsequent improvement, reflected by rapidly weaned vasopressors and improved cardiac output, emphasizes the alterations in thyroid hormone metabolism driven by thyrotoxicosis. These observations shed light on the complex interplay between thyroid hormone regulation and cardiovascular function.

Drug Index Terms

(Based on full text)

antihypertensive agent, dobutamine, hydrocortisone, iodine, lactic acid, potassium iodide, prednisone, propylthiouracil, thiamazole, thyroid hormone, thyrotropin, troponin

Non-drug Index Terms

(Based on full text)

adult, alopecia, atrial fibrillation, blood culture, body weight loss, **cardiogenic shock**, cardiovascular function, case report, clinical article, conference abstract, controlled study, coronary artery calcification, coronary care unit, diagnosis, diarrhea, drug therapy, drug withdrawal, echocardiograph, echocardiography, edema, electrocardiography, end stage renal disease, female, follow up, free liothyronine index, free thyroxine index, goiter, heart output, hormonal regulation, human, hyperlipidemia, hyperthyroidism, hypokinesia, hypotension, leukocyte count, loading drug dose, methicillin susceptible Staphylococcus aureus, pleura effusion, septic shock, side effect, special situation for pharmacovigilance, Stevens Johnson syndrome, Stevens Johnson/toxic epidermal necrolysis overlap syndrome, Twave inversion, tachycardia, thorax radiography, thyroid crisis, **thyroid gland**, thyroid hormone metabolism, thyrotoxicosis

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
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Source Type	Journal
Conference Name	ENDO 2025

Conference Location	United States, San Francisco, CA
Conference Date	2025-07-12 to 2025-07-15
Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A1246
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.2366
CAS Registry Numbers	dobutamine (34368-04-2 , 52663-81-7 , 49745-95-1 , 61661-06-1) hydrocortisone (50-23-7) iodine (7553-56-2) lactic acid (113-21-3 , 50-21-5) potassium iodide (7681-11-0) prednisone (53-03-2) propylthiouracil (51-52-5) thiamazole (60-56-0) thyrotropin (9002-71-5)

48. [Amiodarone induced thyroiditis Harshita Marasandra Ramesh, MD, Anubhuti Sharma, MD](#)

Kothari V.A., Lee F.J.

J.Endocr. Soc. 2025 9: (A1169-A1170)

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Abstract

Introduction: Amiodarone, an iodine-rich antiarrhythmic drug, is known to cause thyroid dysfunction in 15-20% of patients, with amiodarone-induced thyroiditis (AIT) presenting a complex diagnostic and therapeutic challenge. AIT manifests in two forms: type 1, iodine-induced hyperthyroidism, typically occurring in preexisting thyroid abnormalities like Grave's, and type 2, a drug-induced destructive thyroiditis in an otherwise normal thyroid. Amiodarone-induced hypothyroidism occurs if there is a failure to escape from Wolff Chaikoff effect, more commonly in patients with Hashimoto's. Case presentation: A 74-year-old male with a past medical history of atrial fibrillation, ischemic cardiomyopathy, hypertension, hyperlipidemia, chronic kidney disease stage 3, was seen at the endocrinology clinic for abnormal thyroid labs. He presented with symptoms of fatigue, drowsiness, weight loss of 18 lbs in 2 months, dry skin, diarrhea and cold intolerance. Labs were pertinent for elevated thyroid stimulating hormone of 7.11 uIU/ml (normal: 0.27-4.2 uIU/ml), free T4 of 2.1 ng/dl (normal: 0.9-1.7 ng/dl), total T3 of 78 ng/dl (normal: 80-200 ng/dl). He was initiated on amiodarone about 4 months prior to presentation. His previous thyroid function tests were normal. He was initially diagnosed with suspected type 2 amiodarone induced thyroiditis and treated with a course of prednisone for 10 days. The following month, he was hospitalized for the treatment of rhabdomyolysis and myopathy when he presented with symptoms of extreme fatigue and myalgia. Labs during hospitalization showed elevated creatine kinase of 5200 U/L and deranged liver function tests of aspartate transaminase of 65 U/L, alanine transaminase of 274 U/L and alkaline phosphatase of 172 U/L. Atorvastatin and amiodarone were stopped at this time. The rhabdomyolysis could be precipitated by the prolonged usage of amiodarone and atorvastatin which could have been worsened by prolonged amiodarone induced thyroiditis. Thyroid ultrasound revealed no hypervascularity or no thyroid gland enlargement. He closely followed up in the clinic after 6 months. The patient was doing well and repeat labs showed normalization of thyroid function tests: TSH of 2.67 uIU/ml, fT4 of 1.3 ng/dl, total T3 of 103 thereby confirming the diagnosis of type 2 amiodarone induced thyroiditis. Conclusion: Type 1 AIT is usually treated with thioamides, enhanced by potassium perchlorate to block iodine uptake, while type 2 AIT is best treated with oral glucocorticoids. Mixed forms may require a combination of these therapies. In refractory cases, thyroidectomy can be considered. Effective management depends on precise diagnosis and a tailored treatment approach. Amiodarone, even

if stopped, remains in the body for about 142 days due to its lipophilic nature which may cause persistent symptoms.

Drug Index Terms

(Based on full text)

alanine aminotransferase, alkaline phosphatase, **amiodarone**, antiarrhythmic agent, aspartate aminotransferase, atorvastatin, creatine kinase, glucocorticoid, iodine, potassium perchlorate, prednisone, thioamide, thyrotropin

Non-drug Index Terms

(Based on full text)

atrial fibrillation, body weight loss, case report, chronic kidney failure, conference abstract, diagnosis, diarrhea, drug therapy, drug withdrawal, dry skin, fatigue, free thyroxine index, goiter, Graves disease, Hashimoto disease, hospitalization, human, hyperlipidemia, hypertension, hypothyroidism, iodine-induced hyperthyroidism, ischemic cardiomyopathy, liver function test, male, myalgia, myopathy, oral drug administration, rhabdomyolysis, somnolence, special situation for pharmacovigilance, therapy, thyroid disease, thyroid function test, thyroidectomy, **thyroiditis**, Wolff Chaikoff effect

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
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Source Type	Journal
Conference Name	ENDO 2025
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Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A1169-A1170
Country of Author	United States
Country of Source	Netherlands

Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.2218
CAS Registry Numbers	alanine aminotransferase (9000-86-6 , 9014-30-6) alkaline phosphatase (9001-78-9) amiodarone (1951-25-3 , 19774-82-4 , 62067-87-2) aspartate aminotransferase (9000-97-9) atorvastatin (134523-00-5 , 134523-03-8 , 110862-48-1) creatine kinase (9001-15-4) iodine (7553-56-2) potassium perchlorate (7778-74-7) prednisone (53-03-2) thyrotropin (9002-71-5)

49. [Iodine-Induced Thyrotoxicosis Leading to Severe Osteoporosis: A Case Report](#)

Vallejo S., Shakil J., Dowlatshahi S.

J.Endocr. Soc. 2025 9: (A1172)

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Abstract

Introduction: Maintaining euthyroid status is crucial for skeletal health, as elevated thyroid hormones accelerate bone loss, leading to decreased bone mineral density (BMD) and increased fracture risk. While most individuals tolerate high iodine intake, excessive iodine can induce thyrotoxicosis in susceptible individuals. Common sources include iodized salt, amiodarone, and seaweed-based supplements such as kelp. We report a case of iodine-induced thyrotoxicosis secondary to kelp consumption, resulting in severe osteoporosis, emphasizing the importance of dietary awareness in thyroid management. Case: A 58-year-old woman with osteoporosis, low BMI, and premature ovarian

insufficiency (POI) was evaluated for worsening bone density despite prior treatment with alendronate. Initial BMD revealed the lowest T-score of -2.6. Laboratory evaluation showed: TSH <0.005 mIU/L, FT4: 7.14 ng/dL, FT3: 6.1 pg/dL, Calcium: 10.5 mg/dL, Albumin: 4.1 g/dL, PTH: 21 pg/mL. She had no thyroid disease history or relevant medication use. Physical exam was unremarkable. A whole-body I-123 scan showed decreased thyroid uptake, suggesting subacute thyroiditis. She was advised to avoid excessive iodine and was treated with prednisone. She also started denosumab but was lost to follow-up. Two years later, she presented with tremors, palpitations, and persistent thyrotoxicosis: FT4: 5.34 ng/dL, FT3: 7.0 pg/dL, TSH: <0.005 mIU/L, TSI: <0.10 IU/L, TRAb: <1.10 IU/L, Thyroglobulin: 5.4 ng/mL. Ultrasound showed an atrophic thyroid gland. A Tc-99m pertechnetate scan ruled out increased uptake or ectopic thyroid tissue. A repeat BMD showed worsening osteoporosis (T-score -3.0). Dietary history revealed substantial kelp supplement use, indicating chronic iodine excess. While POI contributes to osteoporosis, her worsening BMD corresponded with persistent thyrotoxicosis, suggesting iodine-induced thyrotoxicosis as the primary factor. Nutritional counseling and psychiatric assessment were recommended to evaluate behavioral factors contributing to excessive supplement use. Discussion: Excess iodine can overwhelm thyroid adaptation (Jod-Basedow effect), causing thyrotoxicosis, particularly in susceptible individuals. Beyond cardiovascular effects, prolonged thyrotoxicosis accelerates bone loss, exacerbating osteoporosis. Kelp and similar supplements can exceed the recommended iodine intake of 1,100 mcg/day, necessitating dietary education. This case highlights the difficulty of managing iodine-induced thyrotoxicosis, especially when unrecognized supplement intake is involved. Comprehensive care should include dietary counseling and, when needed, psychiatric evaluation to address behavioral patterns contributing to excessive supplement use and related health risks.

Drug Index Terms

(Based on full text)

albumin, alendronic acid, amiodarone, calcium, denosumab, **iodine**, iodine 123, liotrix, prednisone, sodium chloride, technetium 99m, thyroglobulin, thyroid hormone, thyrotropin

Non-drug Index Terms

(Based on full text)

adult, adverse drug reaction, bone density, cardiovascular effect, case report, clinical article, conference abstract, drug therapy, early menopause, ectopic thyroid gland, female, follow up, heart palpitation, human, human tissue, iodine intake, **iodine-induced hyperthyroidism**, iodism, middle aged, nutritional counseling, osteolysis, **osteoporosis**, physical examination, premature ovarian failure, seaweed, side effect, subacute thyroiditis, therapy, thyroid disease, thyrotoxicosis, tremor

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
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Source Type	Journal
Conference Name	ENDO 2025
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A1172
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.2223
CAS Registry Numbers	alendronic acid (66376-36-1) amiodarone (1951-25-3 , 19774-82-4 , 62067-87-2) calcium (7440-70-2 , 14092-94-5) denosumab (615258-40-7) iodine (7553-56-2) iodine 123 (15715-08-9) liotrix (8065-29-0 , 8061-58-3)

prednisone ([53-03-2](#))
sodium chloride ([7647-14-5](#), [23724-87-0](#),
[49658-21-1](#))
technetium 99m ([14133-76-7](#))
thyroglobulin ([9010-34-8](#))
thyrotropin ([9002-71-5](#))

50. [Effect of Androgen Deprivation Therapy on Asthma Exacerbations](#)

Nayak Y., Deol E., Liebich J., Yu K., Ayoub Y., Amini M., Ghimire A.,
Portillo-Canales S., Patel P., Kamel G., Dhindsa S.

J.Endocr. Soc. 2025 9: (A1072)

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Abstract

Background: A gender-related disparity exists in asthma morbidity and mortality. Asthma tends to be more common and severe in women compared to men. Epidemiological studies have suggested that serum testosterone (T) concentrations are inversely related to asthma prevalence as well as exacerbations in men. It is not known if male hypogonadism precipitated by androgen deprivation therapy (ADT) has any effect on asthma exacerbations. Methods: This is a retrospective chart review study conducted in the electronic health records of a tertiary care academic hospital. We included adult men with asthma who had at least one interaction with the health system in the last 7 years and had received ADT for prostate cancer. The primary endpoint of the study was to compare the incidence of asthma exacerbations before and after the use of ADT. Asthma exacerbation was defined as any hospitalization due to asthma or usage of oral prednisone if exacerbation was treated in the outpatient setting. Averages are presented as mean $\pm$ SD. Comparisons among groups were made by t-test, ANOVA, or non-parametric tests as appropriate. Results: Data were available on 93 men (mean age 75 ± 8 years, BMI 31 ± 7 kg/m²). 89% were White and 11% were Black. The total duration of follow up was 8.8 ± 5.2 years. Asthma was classified as intermittent, mild, moderate, and severe in 46%, 23%, 31% and 0% of patients respectively. Inhaled steroids were used continuously by 51% of the patients. 6% were current smokers and 48% were former

smokers. Emphysema or chronic bronchitis were present in 33% of patients. The mean follow-up before starting ADT was 6.1 ± 5.1 years. Patients took ADT for 3.2 ± 3.0 years. GnRH agonist treatment was used in 45% of the patients, while 55% had received more than one type of ADT. 72% of patients had documentation of undetectable serum testosterone concentrations during ADT. The annualized incidence of asthma exacerbations (number of exacerbations/duration of follow up in years) before ADT and during ADT was similar (0.21 ± 1.3 versus 0.06 ± 0.17 per year respectively, $p=0.13$). Mean follow up after stopping ADT was 1.9 ± 1.8 years. The rate of asthma exacerbations following ADT (0.08 ± 0.34 per year) was similar to the rate during ADT ($p=0.31$). 67% of patients did not have any exacerbation during the study period. After restricting the analysis to only those with at least one exacerbation ($N=31$), the rates of exacerbations were not different before, during and after ADT (0.61 ± 2.1 , 0.18 ± 0.27 and 0.24 ± 0.57 respectively, $p=0.11$). Conclusion: Our study shows that ADT in men with asthma did not result in a reduction in asthma exacerbations. These data are informative since prostate cancer and asthma are commonly encountered comorbidities in the ambulatory setting.

Drug Index Terms

(Based on full text)

gonadorelin agonist, prednisone, steroid

Non-drug Index Terms

(Based on full text)

adult, androgen deprivation therapy, asthma, Caucasian, comorbidity, conference abstract, current smoker, drug therapy, electronic health record, emphysema, ex-smoker, follow up, hospitalization, human, incidence, inhalation, male, male hypogonadism, medical record review, morbidity, mortality, nonparametric test, oral drug administration, prevalence, prostate cancer, side effect, special situation for pharmacovigilance, tertiary health care, testosterone blood level

Comment: Invalid ICSR

Additional Information

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Page Range	A1072
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.2027
CAS Registry Numbers	prednisone (53-03-2)

51. [The Double-Edged Sword of Methimazole: Managing Graves' Disease with a Twist](#)

Patel V.H.

J.Endocr. Soc. 2025 9: (A1151)

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Abstract

Background: Graves' disease (GD) is a common autoimmune disorder leading to hyperthyroidism, which can occasionally impact hematological parameters, including thrombocytopenia. The mechanisms contributing to thrombocytopenia in GD are multifactorial, involving thyrotoxicosis, autoimmune responses, and drug-induced effects. This case study explores a 40-year-old female with GD who developed symptomatic thrombocytopenia following initiation of methimazole therapy, presenting as an exacerbation of existing thrombocytopenia. Case: A 40-year-old female with no significant medical history presented with a 2-week history of difficulty swallowing,

palpitations, weight loss, anxiety, hair loss, and insomnia. Physical examination revealed a moderately enlarged, nontender thyroid gland with no bruit. Initial laboratory investigations revealed a platelet count of 97,000/ μ L, TSH of 0.10 μ U/mL, free T4 of 6.01 ng/dL, and total T3 of 4.54 ng/mL. Thyroid ultrasound showed bilaterally enlarged thyroid glands with increased vascularity and no discrete nodules. Positive thyroid peroxidase (109 IU/mL) and TSH receptor antibodies (36.60 IU/L) confirmed the diagnosis of Graves' disease (GD). The patient was started on methimazole 10 mg BID and propranolol for symptom control. Two weeks later, she presented to the emergency department with epistaxis and a new bruise on her left anterior thigh. Laboratory findings revealed severe thrombocytopenia with a platelet count of 9,000/ μ L, alongside signs of improving hyperthyroidism. Additional workups, including vitamin B12, folate, coagulation profile, immunological tests, platelet function tests, and abdominal ultrasound, were unremarkable, ruling out other causes of thrombocytopenia. The patient received a platelet transfusion and was started on IV methylprednisolone, later tapered to oral prednisone. Given the temporal association with methimazole initiation and exclusion of other causes, methimazole-exacerbated immune thrombocytopenia was diagnosed. Endocrinology recommended transitioning to radioactive iodine therapy or surgery for definitive management of GD. Conclusion: This case underscores the rare but serious risk of methimazole-induced thrombocytopenia. While achieving an euthyroid state using medical therapy is generally expected to improve hematologic parameters in hyperthyroid patients, clinicians should remain vigilant for potential drug-induced thrombocytopenia, especially in the presence of new bleeding symptoms.

Drug Index Terms

(Based on full text)

cobalamin, cyanocobalamin, folic acid, methylprednisolone, prednisone, propranolol, radioactive iodine, **thiamazole**, thyroid peroxidase, thyrotropin, thyrotropin receptor antibody

Non-drug Index Terms

(Based on full text)

adult, adverse drug reaction, anxiety, autoimmune thrombocytopenia, autoimmunity, bleeding, blood clotting, blood clotting parameters, body weight loss, case report, clinical article, conference abstract, contusion, diagnosis, drug combination, drug therapy, dysphagia, emergency ward,

epistaxis, euthyroidism, female, free thyroxine index, goiter, **Graves disease**, hair loss, heart palpitation, hematological parameters, human, hyperthyroidism, insomnia, oral drug administration, physical examination, platelet count, special situation for pharmacovigilance, surgery, thigh, thrombocyte transfusion, thrombocytopenia, thyrotoxicosis

Comment: valid ICSR

Additional Information

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Page Range	A1151
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1210/jendso/bvaf149.2181
CAS Registry Numbers	cobalamin (13408-78-1) cyanocobalamin (53570-76-6 , 68-19-9 , 8064-09-3) folic acid (59-30-3 , 6484-89-5) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) propranolol (13013-17-7 , 318-98-9 , 3506-09-0 , 4199-09-1 , 525-66-6) thiamazole (60-56-0) thyrotropin (9002-71-5)

52. [Refractory Painful Hashimoto's Thyroiditis](#)

Villalobos J.L.A., Lane C., Ereifej L., Dorin R.I., Aguirre L.E.

J.Endocr. Soc. 2025 9: (A1204)

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Abstract

Background Painful recurrent Hashimoto's thyroiditis (pHT) is an uncommon form of chronic lymphocytic thyroiditis, marked by thyroid pain and tenderness. The differential diagnosis for pHT includes infection, subacute granulomatous thyroiditis (SAT) and malignancy. Unlike SAT, pHT usually shows only slight or temporary relief with steroids. Treatment options include l-thyroxine, NSAIDs, or aspirin; however, if symptoms persist, surgical intervention may be necessary. Clinical Case A 23-year-old female G1P0 with PTSD was referred to endocrinology at 14 weeks gestational age for subclinical hypothyroidism and mild neck pain. Initial labs showed elevated TSH at 5.4 uIU/mL (n 0.465-4.680 uIU/mL), TgAb at 3 IU/mL (n ≤ 1 IU/mL), and TPOAb at 1310 IU/mL (n 0-5.61 IU/mL), with normal FT4 at 1.01 ng/dL (n 0.8-2.2 ng/dL). Levothyroxine 50 mcg daily was initiated, and follow-up labs showed improvement. At 1 month postpartum, the patient reported fatigue, worsening anterior neck pain accompanied by pruritus and rash, neck fullness, voice changes, odynophagia, and compression symptoms when lying supine. Physical examination revealed an enlarged, tender thyroid and a sandpaper-like rash over the anterior neck. Labs showed suppressed TSH < 0.015 uIU/mL and FT4 1.74 ng/dL, and Levothyroxine was suspended. TSH rose to 28.4 uIU/mL within two weeks and peaked to 45 uIU/mL by four months postpartum. Levothyroxine was restarted and gradually increased to 112 mcg daily, TSH was normalized. Further evaluation, including serial thyroid ultrasounds and a neck CT, revealed an enlarged thyroid with persistent heterogeneous echogenicity consistent with Hashimoto thyroiditis and no suspicious lymphoid nodes. Labs showed positive TgAb at 6 IU/mL and TPOAb > 2000 IU/mL. Inflammatory markers were not elevated, CRP at 0.8 mg/L (n ≤ 10 mg/L) and ESR at 11 mm/hr (n ≤ 20 mm/hr). Initial treatment with prednisone 40 mg daily for one week provided temporary relief, while NSAIDs were

ineffective. A second, longer trial of prednisone at 40 mg daily for one week followed by a three-week taper provided lasting neck pain reduction but failed to resolve symptoms completely. At 18 months after symptom onset, she was referred to an ENT for consideration of thyroidectomy due to recurrent pHT refractory to treatment with NSAIDs and prednisone. Conclusion Recurrent painful Hashimoto's thyroiditis is a rare cause of recurrent thyroiditis and can be difficult to diagnose and treat. In this case, the lack of response to glucocorticoids, along with normal CRP and ESR levels, led to the diagnosis. Surgery may be an option if other treatments do not adequately manage symptoms.

Drug Index Terms

(Based on full text)

acetylsalicylic acid, C reactive protein, glucocorticoid, levothyroxine, nonsteroid antiinflammatory agent, prednisone, steroid, thyrotropin

Non-drug Index Terms

(Based on full text)

adult, case report, clinical article, conference abstract, diagnosis, differential diagnosis, drug therapy, fatigue, female, follow up, gestational age, goiter, **Hashimoto disease**, human, neck pain, odynophagia, physical examination, posttraumatic stress disorder, pruritus, rash, soreness, subacute thyroiditis, subclinical hypothyroidism, surgery, thyroidectomy, thyroiditis, voice change

Comment: valid ICSR

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Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.2282
CAS Registry Numbers	acetylsalicylic acid (493-53-8 , 50-78-2 , 53663-74-4 , 53664-49-6 , 63781-77-1) C reactive protein (9007-41-4) levothyroxine (51-48-9) prednisone (53-03-2) thyrotropin (9002-71-5)

53. [Steroid-Responsive Encephalopathy Associated With Autoimmune Thyroiditis in a Patient With Graves' Disease](#)

Dominic M.R., Itersky P., Pereira M.C., Saad P., Englert D.

J.Endocr. Soc. 2025 9: (A1206)

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Abstract

Background: Steroid-Responsive Encephalopathy associated with Autoimmune Thyroiditis (SREAT), also known as Hashimoto's encephalopathy, is a rare autoimmune disorder with an estimated prevalence of 2/100,000. It is characterized by acute encephalopathy, presence of anti-thyroid antibodies, and a robust response to corticosteroids. SREAT is most commonly associated with Hashimoto's disease, although cases associated with Graves' disease have been previously reported. Prompt recognition is critical for initiating timely treatment with steroids or immunomodulators. Clinical Case: A 45-year-old male with no significant past medical history presented with acute altered mental status, fever, and headache. He was admitted for suspected meningitis after initial evaluation and was started on empiric antimicrobials. He later developed seizures, confirmed on EEG, and was started on levetiracetam. MRI brain revealed edema in the right parietal

and posterior temporal lobe cortex. Blood cultures, urinalysis, hepatitis panel, HIV, RPR and West Nile PCR returned negative. CSF studies showed a WBC count of 11 (n 0-5), with 100% lymphocytes, and a negative meningoencephalitis panel, including HSV, HHV-6, and VDRL. Endocrine workup was significant for undetectable TSH ($<0.01 \mu\text{IU/mL}$, n 0.4-4), elevated T3 (342 ng/dL, n 60-180) and free T4 (3.51 ng/dL, n 0.75-1.5), high thyroid peroxidase (TPO) antibody titers (200 IU/mL, n <6), and positive thyrotropin receptor antibodies consistent with Graves' disease. The patient was started on methimazole, SSKI and cholestyramine. Despite normalization in free T4 and T3, seizures and altered mentation persisted. Further workup, including a paraneoplastic panel and CSF movement disorder panel, were negative. A diagnosis of antibody-associated autoimmune encephalitis was made at this point, and patient was started on IV methylprednisolone 1 g daily x 5 days, followed by oral prednisone 60 mg daily. Mental status and seizures gradually improved. Repeat brain MRI showed resolution in the cortical edema. The patient was discharged on methimazole and prednisone. Clinical lessons: SREAT is characterized by new encephalopathy of unclear etiology, in the context of elevated anti-thyroid (anti-TPO or anti-thyroglobulin) antibodies. Presentation can vary from mild cognitive dysfunction or psychological abnormalities to status epilepticus and coma. Although it is typically linked to hypothyroidism or euthyroidism, it can rarely occur in hyperthyroid conditions like Graves' disease. Around 20 cases have been reported globally in association with Graves' disease, as in this case. Corticosteroids are the cornerstone of treatment, with immunosuppressants or plasmapheresis as alternatives. Routine testing for thyroid antibodies in autoimmune encephalitis workups is essential to identify SREAT and prevent delayed treatment.

Drug Index Terms

(Based on full text)

antithyroid agent, colestyramine, corticosteroid, levetiracetam, methylprednisolone, prednisone, **steroid**, thyroglobulin antibody, thyroid antibody, thyroid peroxidase, thyrotropin, thyrotropin receptor antibody

Non-drug Index Terms

(Based on full text)

acute brain disease, adult, adverse drug reaction, aged, altered state of consciousness, antibody titer, autoimmune encephalitis, blood culture, case report, cerebrospinal fluid flow, clinical article, cognitive defect,

conference abstract, diagnosis, drug therapy, edema, electroencephalogram, electroencephalography, epileptic state, euthyroidism, fever, free liothyronine index, free thyroxine index, **Graves disease**, Hashimoto disease, **Hashimoto encephalitis**, **Hashimoto encephalopathy**, headache, human, hyperthyroidism, hypothyroidism, immunosuppressive treatment, leukocyte count, male, meningitis, meningoencephalitis, motor dysfunction, MRI scanner, nuclear magnetic resonance imaging, oral drug administration, plasmapheresis, prevalence, seizure, side effect, temporal cortex, therapy, therapy delay, urinalysis, venereal disease reaction test

Comment: invalid ICSR

Additional Information

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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A1206
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.2286
CAS Registry Numbers	colestyramine (11041-12-6 , 58391-37-0) levetiracetam (102767-28-2) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) thyrotropin (9002-71-5)

54. [Therapeutic Plasma Exchange as Bridging Modality in Thyroid Storm With Contraindications to Anti-thyroid Medications: A Case Report](#)

Bondoc M.K.V., Chua C.C.

J.Endocr. Soc. 2025 9: (A1208)

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Introduction: Thyroid storm is a life-threatening complication of thyrotoxicosis, characterized by acute, severe manifestations and a high mortality rate of 20-30%. Prompt intervention is essential, with treatment strategies focused on reducing thyroid hormone production, inhibiting the conversion of T4 to T3, and managing the symptoms with anti-thyroid medications. In cases where these medications are contraindicated, therapeutic plasma exchange (TPE) can be used as a bridging modality to definitive intervention. Presentation of the Case This case report describes a 33-year-old female with Graves' disease who initially presented with symptoms of hyperthyroidism, including easy fatigability, exophthalmos, heat intolerance, and palpitations that was treated with methimazole. She developed agranulocytosis leading to the discontinuation of methimazole. Symptoms improved on discontinuation of methimazole and was discharged. However, she was readmitted and work-up were consistent with sepsis from pneumonia and thyrotoxic heart disease. IV meropenem, sacubitril+valsartan 50 mg/tab twice daily, and carvedilol 6.25 mg/tab twice daily were started. Since methimazole was contraindicated, lithium 450mg 1 tab twice daily was started. Symptoms improved and was discharged. She was again readmitted due to recurrence of symptoms of severe hyperthyroidism and treated as a case of thyroid storm and lithium toxicity with TSH 0.01 ulu/mL, FT4 25.14 pmol/L, creatinine 388 mg/dL, lithium elevated at 1.59 mmol/L, that required to hold lithium. She underwent hemodialysis and repeat lithium level dropped to 0.3 mmol/L however repeat FT4 increased to 41.22 pmol/L. Due to contraindication to methimazole and lithium, therapeutic plasma exchange (TPE) was done. After the first session of TPE, FT4 decreased from 41.22 pmol/L to 25.02 pmol/L. Patient then underwent radioactive iodine therapy at 14 MCI. Post RAI, repeat FT4 was

21.11 pmol/L. Patient was then started on prednisone 60 mg per day and underwent another cycle of TPE. Repeat 2D echo was done which showed improvement of ejection fraction to 65% with resolution of global hypokinesia and diastolic dysfunction. Thyroid function test showed further decrease in FT4 level to 16.95 pmol/L. The symptoms resolved and was discharged stable. Continued follow-ups showed that patient was maintained on levothyroxine 100mcg once daily. Her latest TSH is 9 ulu/mL and her levothyroxine is being adjusted accordingly. Conclusion: Thyroid storm remains a medical emergency requiring rapid and comprehensive management. TPE offers a viable bridging modality to definitive intervention when anti-thyroid medications are contraindicated.

Drug Index Terms

(Based on full text)

antithyroid agent, carvedilol, creatinine, levothyroxine, lithium, meropenem, prednisone, radioactive iodine, sacubitril plus valsartan, thiamazole, thyroid hormone, thyrotropin

Non-drug Index Terms

(Based on full text)

adult, agranulocytosis, case report, clinical article, conference abstract, **contraindication**, diastolic dysfunction, drug therapy, drug withdrawal, exophthalmos, female, follow up, Graves disease, heart ejection fraction, heart palpitation, heat intolerance, hemodialysis, human, hyperthyroidism, hypokinesia, lithium intoxication, mortality rate, **plasma exchange**, pneumonia, sepsis, **thyroid crisis**, thyroid function test, thyrotoxicosis

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A1208
Country of Author	Philippines
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1210/jendso/bvaf149.2290
CAS Registry Numbers	carvedilol (72956-09-3) creatinine (19230-81-0 , 60-27-5) levothyroxine (51-48-9) lithium (7439-93-2) meropenem (96036-03-2) prednisone (53-03-2) sacubitril plus valsartan (936623-90-4) thiamazole (60-56-0) thyrotropin (9002-71-5)

55. [The Long Shadow of Immune Checkpoint inhibitors: Adrenal Insufficiency After Pembrolizumab](#)

Zulqadar K., Iftikhar S., Jain T., Khan A., Wazin F.

J.Endocr. Soc. 2025 9: (A164-A165)

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Embase Open URL: [redirect to full text](#)

Abstract

Introduction: Pembrolizumab is an immune checkpoint inhibitor targeting programmed death receptor-1 (PD-1), widely used for advanced renal cell carcinoma. While generally well tolerated, it can cause immune-related adverse events (irAEs) affecting the thyroid, pituitary, adrenal glands, and pancreatic islet cells. Adrenal insufficiency (AI) is a rare but serious complication, often resulting from hypophysitis or direct adrenal dysfunction. We present a case of late-onset AI following

pembrolizumab therapy. Case description: A 57-year-old male with a history of stage III right renal cell carcinoma was treated with pembrolizumab as adjuvant immunotherapy. His treatment was discontinued after he developed by cytokine release syndrome as an irAEs, requiring high-dose prednisone, which was gradually tapered over two months. Shortly after completing his steroid taper, he presented to the emergency department with profound fatigue, nausea, and generalized weakness. He denied fever, headache, chest pain, or shortness of breath. Initial laboratory evaluation revealed severe hyponatremia (serum sodium 108 mEq/ L). Endocrine workup during admission demonstrated adrenal insufficiency, with early morning cortisol of 0.8 µg/ dL. An ACTH stimulation test showed an inadequate cortisol response, and ACTH was 5 pg/mL, confirming secondary adrenal insufficiency as an irAE secondary to pembrolizumab. He was started on stress-dose steroids, which were gradually tapered to hydrocortisone 20 mg twice daily, leading to resolution of symptoms. Discussion: Pembrolizumab, a PD-1 receptor inhibitor, is widely used to treat various malignancies but can cause immune-related adverse events (irAEs), including adrenal insufficiency (AI). AI commonly presents with fatigue, nausea, hypotension, and electrolyte abnormalities, diagnosed with abnormal morning cortisol levels and ACTH stimulation tests. In cases of low ACTH levels, secondary AI due to hypophysitis should be suspected, while elevated ACTH indicates primary AI due to direct adrenal dysfunction. Hypophysitis is a rare but serious irAE that may require lifelong glucocorticoid therapy. While AI typically develops during treatment, delayed presentation post therapy is uncommon. Given the potential for delayed AI onset, routine post-treatment cortisol monitoring should be considered. Early recognition and prompt glucocorticoid replacement are crucial to preventing adrenal crisis, a life-threatening complication.

Drug Index Terms

(Based on full text)

corticotropin, glucocorticoid, hydrocortisone, **immune checkpoint inhibitor**, **pembrolizumab**, prednisone

Non-drug Index Terms

(Based on full text)

adrenal cortex insufficiency, **adrenal insufficiency**, adverse drug reaction, case report, clinical article, complication, conference abstract,

corticotropin test, cytokine release syndrome, drug therapy, drug withdrawal, dyspnea, emergency ward, fatigue, fever, headache, human, hyponatremia, hypophysitis, hypotension, male, muscle weakness, nausea, pancreas islet cell, renal cell carcinoma, side effect, sodium blood level, special situation for pharmacovigilance, thorax pain

Comment: valid ICSR

Additional Information

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Conference Name	ENDO 2025
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A164-A165
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.304
CAS Registry Numbers	corticotropin (11136-52-0 , 9002-60-2 , 9061-27-2) hydrocortisone (50-23-7) pembrolizumab (1374853-91-4) prednisone (53-03-2)

56. [Hydrocortisone Absorption While on Tirzepatide](#)

Reyes S., Gupta G., Aftab H.

J.Endocr. Soc. 2025 9: (A224-A225)

Embase

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Abstract

Introduction: Glucagon-like peptide-1 receptor agonists (GLP-1 RAs) have evolved into a staple in the management of diabetes. One of its mechanisms of action is a delayed gastric emptying and small bowel transit (2). As a result, the use of GLP-1 RAs has been noted to slow the absorption of oral drugs which can be challenging to recognize. We present a case of adrenal insufficiency (AI) on hydrocortisone (HC) and fludrocortisone while on GLP-1 RA. Case: A 63 yo female with DM II, ACTH-dependent Cushing's disease s/p hypophysectomy with recurrent cyclic Cushing's disease s/p bilateral adrenalectomy on chronic HC was admitted for an adrenal crisis. She had doubled her usual HC dose for the past 2 weeks and administered IM HC prior to admission but was hypotensive on arrival. Serum sodium wnl, potassium 3.2 mEq/L, TSH 7.52 uIU/ mL, fT4 1.35 ng/dL. She was treated with IV HC and IV antibiotics. She was discharged on her usual home PO HC and fludrocortisone. She continued to have BP on the lower end of normal, weight gain, worsening glucose control, hyperpigmentation, and nausea. Her HC dose was appropriate for her weight and her symptoms raised the possibility of HC malabsorption. She tried prednisone but symptoms were unchanged. Labs sodium 132 mmol/L, potassium 5.4 mmol/L, cortisol 6.2 ug/dL, ACTH 74 pg/mL, and renin activity 2.1 ng/mL/hr. She had been on tirzepatide for 2 years and decision was made to discontinue tirzepatide for 1 month. Repeat sodium 136 mmol/L, potassium 4.4 mmol/L, cortisol 11.1 ug/dL, ACTH 60.9 pg/mL and renin activity of 0.1 ng/ mL/hr. She was restarted on tirzepatide but at a lower dose. Discussion: The results were suggestive of AI despite maintaining consistent intake of HC and fludrocortisone. There is limited data on drug-drug interactions between GLP-1 RA and other oral medications but we considered the possibility of the use of high-dose tirzepatide impacting absorption of HC. This has previously been observed however the GLP-1 RA used was exenatide (1). It also has been reported that GLP-1 RA can delay the absorption of dexamethasone (2). Our patient had improvement of all of her labs after discontinuation. This case highlights

the utility of discontinuing a GLP-1 RA when there is a clinical suspicion of AI. (1)Fujita Y, Kitamura T, Otsuki M, Tamada D, Tabuchi Y, Kozawa J, Yasuda T, Okita K, Imagawa A, Kaneto H, Funahashi T, Shimomura I. Exenatide alters absorption of hydrocortisone in a diabetic patient with panhypopituitarism: iatrogenic adrenal insufficiency.

Drug Index Terms

(Based on full text)

antibiotic agent, antihypertensive agent, dexamethasone, exendin 4, fludrocortisone, glucagon like peptide 1, glucose, **hydrocortisone**, potassium, prednisone, sodium, thyrotropin, **tirzepatide**

Non-drug Index Terms

(Based on full text)

ACTH-dependent Cushing syndrome, adrenal cortex insufficiency, adrenal insufficiency, bilateral adrenalectomy, body weight gain, case report, clinical article, conference abstract, Cushing disease, diabetic patient, drug interaction, drug megadose, drug therapy, female, gastric stasis, human, hyperpigmentation, hypophysectomy, hypopituitarism, hypotension, low drug dose, nausea, non insulin dependent diabetes mellitus, small intestine, sodium blood level, stomach paresis

Comment: valid ICSR

Additional Information

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Conference Location	United States, San Francisco, CA
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A224-A225
Country of Author	United States
Country of Source	Netherlands

Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.414
CAS Registry Numbers	dexamethasone (50-02-2) exendin 4 (141732-76-5 , 141758-74-9 , 286014-72-0 , 335149-21-8) fludrocortisone (127-31-1) glucagon like peptide 1 (89750-14-1) glucose (50-99-7 , 84778-64-3 , 8027-56-3) hydrocortisone (50-23-7) potassium (7440-09-7) prednisone (53-03-2) sodium (7440-23-5) thyrotropin (9002-71-5) tirzepatide (2023788-19-2)

57. [Trulicity Troubles: When Diabetes Medications Go Kidney-ward](#)

Pendyala R., Basaleli C., Jamal Y.

J.Endocr. Soc. 2025 9: (A566-A567)

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Abstract

Background: Glucagon-like peptide-1 (GLP-1) receptor agonists, like Trulicity (dulaglutide) are popular for managing type 2 diabetes mellitus (T2DM) due to their glucose control and cardiovascular benefits. However, emerging reports link these medications to acute kidney injury (AKI), including acute interstitial nephritis (AIN). To date, only two AIN cases associated with dulaglutide have been documented, one without biopsy confirmation. We present a third case of biopsy-confirmed AIN in a 65-year-old female with no prior renal disease, developing within a month of starting Trulicity, underscoring the need for awareness of this potential complication. Case Description: A 65-year-old female with T2DM and hypertension presented with decreased oral intake, chills, and weakness. One month after starting Trulicity, she experienced decreased

appetite, persistent nausea/vomiting, and low blood glucose readings. Laboratory results showed severe renal impairments (creatinine 11.3 mg/dL (normal {nl} <1.25 mg/dL), Blood urea nitrogen 81 mg/dL (nl 7-18), sodium 129 mEq/L (nl 135-145), HCO₃ 17 mEq/L (nl 22-28) lipase 657 U/L (nl 0-160), with normal kidney ultrasound, glycated hemoglobin/A1c 6.6%, and hematuria on urinalysis. Emergent hemodialysis was initiated, and a renal biopsy confirmed AIN with calcium oxalate crystals. Patient denied use of non-steroidal antiinflammatory medications, antibiotics, proton pump inhibitors, diuretics, and had no recent illnesses, contrast imaging, or blood transfusions. Autoimmune workup was negative. With no other cause to clarify the etiology of the acute kidney injury (AKI), Trulicity was suspected and promptly discontinued. Prednisone 50 mg daily was initiated, leading to creatinine improvement to 3.5 mg/dL after two dialysis sessions, with ongoing AIN recovery. Discussion: AIN is a rare cause of AKI, often triggered by medications like GLP-1 receptor agonists, including Trulicity. It typically presents with renal dysfunction, eosinophiluria, and interstitial inflammation on renal biopsy. The exact mechanism of Trulicity-induced AIN is unclear but thought to be immune-mediated, triggering a hypersensitivity reaction in susceptible individuals. The primary treatment is discontinuing the offending agent. In this case, Trulicity was stopped, and prednisone 50 mg daily was started. Supportive care and dialysis were also required. The patient's creatinine improved from 11.3 mg/dL to 3.5 mg/dL, confirming recovery potential with early intervention. Though generally safe, emerging case reports associate GLP-1 receptor agonists with AKI, particularly in patients with dehydration or pre-existing kidney disease. In this case, given the timing of symptoms and lack of other causes, Trulicity-induced AIN was suspected. Clinicians should be cognizant of this rare but serious complication for timely management of AIN.

Drug Index Terms

(Based on full text)

antibiotic agent, calcium oxalate, creatinine, diuretic agent, **dulaglutide**, glucagon like peptide 1, glucagon like peptide 1 receptor agonist, glucose, hemoglobin A1c, prednisone, proton pump inhibitor, sodium, triacylglycerol lipase

Non-drug Index Terms

(Based on full text)

acute kidney failure, adverse drug reaction, aged, blood transfusion,

case report, chill, clinical article, conference abstract, decreased appetite, dehydration, **diabetes mellitus**, drug therapy, female, hematuria, hemodialysis, human, hypertension, hypoglycemia, interstitial nephritis, kidney biopsy, nausea and vomiting, non insulin dependent diabetes mellitus, severe renal impairment, special situation for pharmacovigilance, therapy, urea nitrogen blood level, urinalysis, **ward**, weakness

Comment: valid ICSR

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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A566-A567
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1066
CAS Registry Numbers	calcium oxalate (563-72-4) creatinine (19230-81-0 , 60-27-5) dulaglutide (923950-08-7) glucagon like peptide 1 (89750-14-1) glucose (50-99-7 , 84778-64-3 , 8027-56-3) hemoglobin A1c (62572-11-6) prednisone (53-03-2) sodium (7440-23-5) triacylglycerol lipase (9001-62-1)

58. [A Case of Central Diabetes Insipidus Following CAR-T Cell Therapy](#)

Lung J., Bang N., Pandey M., Punni E., Goyal I.

J.Endocr. Soc. 2025 9: (A883)

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Abstract

Introduction: Chimeric antigen receptor T-cell therapy (CAR-T) cell therapy is used for treatment for several lymphomas and leukemias. However, endocrine-related toxicities are reported rarely. We present an adult male with central diabetes insipidus (DI) related to CAR-T cell therapy. Case presentation: A 33-year-old male presented to the hospital to start CAR-T therapy. He had a notable past medical history of hypothyroidism, testicular seminoma treated with orchiectomy and chemotherapy 8 years ago, and stage IV high-grade B-cell lymphoma diagnosed 8 months ago with bone marrow and bone involvement. He started chemotherapy R-CHOP (rituximab plus cyclophosphamide, doxorubicin, vincristine, and prednisone) for 1 cycle, dose-adjusted EPOCH-R (etoposide, prednisone, vincristine, cyclophosphamide, doxorubicin, and rituximab) for 5 cycles completed 3 months ago. He had relapse with CNS involvement within weeks of chemotherapy completion. He then started salvage chemotherapy with cytarabine and methotrexate 2 months ago. He received whole brain radiotherapy and jaw mass radiotherapy 1 month ago. Six days after starting CAR-T therapy, the patient developed hypernatremia with a sodium of 162 mmol/L (136-145) and increased 24-hour urine output of 5.8 L. The patient never had hypernatremia prior to admission. He was treated with IV fluids and 1 mcg desmopressin (DDAVP) with improvement in sodium to 146 mmol/L and urine osmolality of 577 mOsm/kg (300-900). An MRI pituitary showed a decrease in the CNS lymphoma lesions involving the infundibulum and hypothalamus compared to 2 months ago with the posterior pituitary bright spot present. The patient matched oral intake with urine output. After IV fluids were stopped, he developed hypernatremia with increased urine output. Endocrinology ordered a water deprivation test overnight. Post water deprivation test, the patient had subsequent urine osmolality of 148 mOsm/kg with serum sodium of

153 mmol/L. A morning 8am cortisol level was adequate at 14.28 µg/dL (2.5-16.6). He had suppressed TSH 0.01 uIU/mL (0.35-4.89) and normal free T4 1.1 ng/dL (1.0-2.1), thus his home levothyroxine dose was adjusted. The patient was started on DDAVP 0.1 mg daily with improvement in urine osmolality and sodium levels. The DDAVP dose was increased to 0.1 mg twice daily prior to discharge, with stable sodium levels in the following months. Discussion: Our case demonstrates a rare cause of central DI related to CAR-Tcell therapy. With a normal cortisol level and known previous hypothyroidism, the patient did not have panhypopituitarism, normally seen in CNS lymphoma involvement. Moreover, there was a decrease in CNS lymphoma lesions following CAR-Ttherapy. Instead, the patient developed acute hypernatremia following CAR-Tcell therapy initiation. The pathophysiology is uncertain but may be related to an inflammatory reaction by immunological attack of CAR-Tcells.

Drug Index Terms

(Based on full text)

cyclophosphamide, cyclophosphamide plus doxorubicin plus etoposide plus prednisolone plus rituximab plus vincristine, cyclophosphamide plus doxorubicin plus etoposide plus prednisolone plus vincristine, cyclophosphamide plus doxorubicin plus prednisolone plus rituximab plus vincristine, cytarabine, desmopressin, doxorubicin, etoposide, hydrocortisone, levothyroxine, methotrexate, prednisone, rituximab, sodium, thyrotropin, vincristine

Non-drug Index Terms

(Based on full text)

adult, adverse drug reaction, B cell lymphoma, case report, **chimeric antigen receptor T-cell**, **chimeric antigen receptor T-cell immunotherapy**, clinical article, conference abstract, diagnosis, drug combination, drug dose, drug therapy, free thyroxine index, human, hypernatremia, hypophysis, hypopituitarism, hypothalamus, hypothyroidism, male, MRI scanner, multiple cycle treatment, **neurogenic diabetes insipidus**, neurohypophysis, nuclear magnetic resonance imaging, orchiectomy, radiotherapy, relapse, seminoma, side effect, sodium blood level, special situation for pharmacovigilance, therapy, urine osmolality, urine volume, water deprivation, whole brain radiotherapy

Comment: valid ICSR

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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A883
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1669
CAS Registry Numbers	cyclophosphamide (50-18-0 , 6055-19-2) cytarabine (147-94-4 , 69-74-9) desmopressin (16679-58-6 , 62288-83-9 , 62357-86-2 , 16789-98-3) doxorubicin (23214-92-8 , 25316-40-9) etoposide (33419-42-0 , 433304-61-1) hydrocortisone (50-23-7) levothyroxine (51-48-9) methotrexate (15475-56-6 , 59-05-2 , 7413 - 34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) prednisone (53-03-2) rituximab (174722-31-7) sodium (7440-23-5) thyrotropin (9002-71-5) vincristine (57-22-7)

59. [Adult Vanishing Bile Duct Syndrome with Hyperlipidemia, Secondary to Pembrolizumab](#)

Martone J., Douglas S., Kristan M.

J.Endocr. Soc. 2025 9: (A439-A440)

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Abstract

Introduction: Vanishing bile duct syndrome (VBDS) is a rare progressive condition defined by loss of intrahepatic bile ducts with complications of cholestasis and liver dysfunction. VBDS can be idiopathic, autoimmune, viral, due to malignancy, or secondary as a drug-induced side effect. Hyperlipidemia, though rare, is a severe side effect of VBDS in adults and is thought to be due to disrupted bile acid metabolism and lipid homeostasis. Management of drug-induced VBDS with hyperlipidemia has been underreported in the literature with one previous report. Ursodiol was initiated with subsequent improvement in the patient's condition. Further documentation of ursodiol's effect on drug-induced VBDS with hyperlipidemia is critical to patient outcomes. Case Report: A 61-year-old female with a history of metastatic sarcomatoid carcinoma of the breast presented with immune-mediated liver injury thought to be secondary to pembrolizumab, an immune checkpoint inhibitor. The patient had been on a stable dose of pembrolizumab for four months when routine labs revealed alarming liver function tests (LFTs), including elevated aspartate aminotransferase (AST), Alanine aminotransferase (ALT), alkaline phosphatase (ALP) and total bilirubin. A diagnosis of immunotherapy-induced autoimmune hepatitis was reached, prompting the discontinuation of pembrolizumab and initiation of prednisone (80 mg), CellCept (2000 mg), and adriamycin cytoxan. Three months later, LFTs showed a continued elevation in total bilirubin, leading to a diagnosis of choledocholithiasis. Cholecystectomy and liver biopsy resulted in immunemediated cholangiopathy with VBDS, likely due to pembrolizumab. The case was further complicated by hospital admission for sepsis days before the cholecystectomy, and steroid-induced hyperglycemia, hyponatremia, and Cushing syndrome. Prednisone was tapered to 40 mg daily. Additionally, repeat labs resulted

in high total cholesterol (1168 mg/dL) and triglycerides (593 mg/dL). Ursodiol (500 mg BID) was initiated for VBDS with hyperlipidemia. One month post ursodiol, AST has declined from 350 to 127, ALT declined from 350 to 149, ALP declined from 1020 to 502, while total bilirubin has decreased from 10.1 to 7.5. Discussion: This case highlights VBDS with hyperlipidemia as a significant complication of pembrolizumab therapy. Physicians should consider VBDS secondary to pembrolizumab in the setting of prolonged elevated LFTs beginning 1-6 months after drug initiation. VBDS with hyperlipidemia is poorly researched in adults, with only one other case that resulted in LFT and lipid abnormalities being improved by ursodiol treatment. Further research regarding ursodiol treatment in VBDS is critical for adult VBDS cases complicated by hyperlipidemia.

Drug Index Terms

(Based on full text)

alanine aminotransferase, alkaline phosphatase, aspartate aminotransferase, bilirubin, cyclophosphamide, doxorubicin, immune checkpoint inhibitor, mycophenolate mofetil, **pembrolizumab**, prednisone, triacylglycerol, ursodeoxycholic acid

Non-drug Index Terms

(Based on full text)

adult, adverse drug reaction, aged, autoimmune hepatitis, bile acid metabolism, **bile duct disease**, breast carcinoma, case report, cholecystectomy, cholestasis, cholesterol blood level, clinical article, common bile duct stone, conference abstract, Cushing syndrome, diagnosis, drug therapy, drug withdrawal, female, hospital admission, human, human tissue, hyperglycemia, **hyperlipidemia**, hyponatremia, intrahepatic bile duct, lipid homeostasis, liver biopsy, liver dysfunction, liver function test, liver injury, male, sarcomatoid carcinoma, sepsis, side effect

Comment: valid ICSR

Additional Information

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Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.822
CAS Registry Numbers	alanine aminotransferase (9000-86-6 , 9014-30-6) alkaline phosphatase (9001-78-9) aspartate aminotransferase (9000-97-9) bilirubin (18422-02-1 , 635-65-4) cyclophosphamide (50-18-0 , 6055-19-2) doxorubicin (23214-92-8 , 25316-40-9) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) pembrolizumab (1374853-91-4) prednisone (53-03-2) ursodeoxycholic acid (128-13-2 , 2898-95-5)

60. [Optimal Timing of Denosumab with Parathyroidectomy in the Setting of Primary Hyperparathyroidism](#)

Leod R.S.M., Majumdar S.K.

J.Endocr. Soc. 2025 9: (A279)

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Abstract

Introduction: Primary hyperparathyroidism should be considered in patients with elevated PTH once secondary causes like vitamin D deficiency and calcium malabsorption are corrected. Optimal timing of denosumab therapy surrounding parathyroid surgery to avoid hypocalcemia and rebound resorption is not well described. **Case:** A 62-year-old woman with history of psoriasis, Grave's disease, SLE, and angioedema was evaluated for steroid-induced osteoporosis. She had difficulty controlling her symptoms with steroid-sparing agents, needing 20-60mg of daily prednisone for 2 years. She started alendronate, calcium citrate, and high dose ergocalciferol prior to presenting to endocrinology. A follow-up DXA scan 4 years from baseline (prior to initiating steroids) showed stable osteopenia in the lumbar spine and total hip with T-scores of -1.1 and -2.2, respectively. However, the femoral neck T-score was -3.0 indicating osteoporosis and significant worsening of BMD by 15.4% since baseline. Due to worsening of her DXA while on alendronate and continued glucocorticoid therapy, denosumab was initiated for antiresorptive therapy. Poor absorption of calcium, vitamin D, and alendronate were thought to contribute to osteoporosis and supposed secondary hyperparathyroidism-PTH 118.7 (15-65) pg/mL, calcium 9.8 (8.8-10.2) mg/dL. She remained on prednisone for 3 additional years while on denosumab. A follow up DXA scan after this period showed lumbar spine T-score of -0.9, total hip T-score of -2.0, and femoral neck T-score of -2.8 with all sites showing non-significant increases in BMD, indicating stabilization. Routine labs showed normalization of serum vitamin D 25-OH levels at 40.7 (30-50) ng/mL with continued PTH elevations (between 69-178 pg/mL) and intermittent hypercalcemia (range 10.0-11.4 mg/dL). It was determined that what was thought to be secondary hyperparathyroidism was now consistent with primary hyperparathyroidism. A 4D CT scan of the neck identified a parathyroid adenoma, and she was referred for parathyroidectomy. **Outcome and Conclusion:** Parathyroidectomy was scheduled in proximity to the next denosumab dose since timing is important due to rebound resorption phenomenon and increased risk of vertebral fractures after delayed or missed denosumab doses. However, peri-procedural hypoparathyroidism and severe hypocalcemia can result if potent antiresorptive therapy prevents liberation of calcium from bone. Considering this, denosumab was given 8 weeks after parathyroidectomy. Initial post-operative PTH was low at 14.3 pg/mL with calcium remaining elevated at 10.7 mg/dL. One month after surgery,

calcium was in normal range at 9.8 mg/dL and PTH had also normalized to 58.5 pg/mL. She experienced no hypocalcemia symptoms or vertebral fractures since her surgery or follow up denosumab injections.

Drug Index Terms

(Based on full text)
alendronic acid, calcium, citrate calcium, **denosumab**, ergocalciferol, glucocorticoid, parathyroid hormone, prednisone, steroid, vitamin D

Non-drug Index Terms

(Based on full text)
adult, adverse drug reaction, angioneurotic edema, case report, clinical article, conference abstract, corticosteroid induced osteoporosis, drug administration, drug therapy, dual energy X ray absorptiometry, female, femoral neck, follow up, four dimensional computed tomography, Graves disease, human, hypercalcemia, hypocalcemia, hypoparathyroidism, lumbar spine, malabsorption, osteopenia, osteoporosis, parathyroid adenoma, **parathyroidectomy**, **primary hyperparathyroidism**, psoriasis, secondary hyperparathyroidism, side effect, spine fracture, surgery, vitamin blood level, vitamin D deficiency

Comment: Invalid ICSR

Additional Information

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Country of Author	United States
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Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.520
CAS Registry Numbers	alendronic acid (66376-36-1) calcium (7440-70-2 , 14092-94-5) citrate calcium (7693-13-2) denosumab (615258-40-7) ergocalciferol (50-14-6 , 50809-47-7 , 8042-78-2) parathyroid hormone (12584-96-2 , 68893-82-3 , 9002-64-6) prednisone (53-03-2)

61. [Once Daily Low Dose Prednisolone/Prednisone Improves Cardiometabolic Profiles in Adrenal Insufficiency: A Double Blind Randomised Crossover Trial](#)

Choudhury S., Lazarus K., Sharma A., Narula K., Go C., Cro S., Thaventhiran T., Khoo B., Tan T., Meeran K.

J.Endocr. Soc. 2025 9: (A147)

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Embase Open URL: [redirect to full text](#)

Abstract

Background: Prednisolone is the active and biochemically equivalent form of prednisone after conversion by the liver in first pass metabolism. It is a separate drug to methyl-prednisolone. Once daily low dose prednisolone is an alternative treatment to standard thrice daily hydrocortisone, for the treatment of adrenal insufficiency. We present findings from the first ever clinical trial comparing once daily low dose prednisolone with thrice daily hydrocortisone. Methods: This double-blind randomised crossover study treated patients with adrenal insufficiency using either 4 months of low dose prednisolone (2-4 mg) with matching placebo at noon and in the afternoon, or 4 months of standard regimen hydrocortisone (10 / 5 / 2.5 mg) in the first study period, followed by the alternative medication in the second study

period. Anthropometrics, biochemical data for cardiometabolic and bone health, and subjective health survey data were collected at Day 1, 30 and 120 of each study period. Results: There was a significant reduction in body weight at 4 months, with a treatment difference of -1.87 Kg ($p=0.002$) favouring prednisolone treatment. Concordant treatment differences in waist circumference and HbA1c of -2.26 cm ($p=0.010$) and -1.23 mmol/mol ($p=0.001$) were also detected. Short term and medium-term bone formation markers were suppressed on prednisolone with a treatment difference of -1.22 $\mu\text{g/L}$ ($p=0.035$) in osteocalcin levels and -13.8 ng/L ($p<0.001$) in Procollagen 1 N-Terminal Propeptide. Urinary N-telopeptide levels, a bone resorption marker was also suppressed by -9.34 nmol/mmol ($p=0.002$) associated with prednisolone. Other cardiovascular markers such as blood pressure, high-sensitivity troponin and CRP, did not show significant differences between prednisolone and hydrocortisone. Subjective health outcomes were measured by the SF-36 survey and Addisons Quality of Life (Addi-QoL) questionnaire. There was no significant difference in any of the SF-36 domains including physical functioning, emotional wellbeing or general health. Addi-QoL scores were comparable between both treatments. Discussion: Once daily low dose prednisolone or prednisone is a safe alternative to thrice daily hydrocortisone to treat adrenal insufficiency, without compromising wellbeing. There is evidence of improved cardiometabolic and glycaemic outcomes with prednisolone. This may be because hydrocortisone is associated with uncoupling from the normal physiological profile of cortisol, or multiple daily doses causing increased steroid exposure. Conclusion: This study supports the routine use of low dose prednisolone, which would further permit longer term studies using real world mortality and morbidity outcomes.

Drug Index Terms

(Based on full text)

amino terminal telopeptide, C reactive protein, hemoglobin A1c, hydrocortisone, methylprednisolone, osteocalcin, placebo, **prednisolone**, **prednisone**, procollagen, steroid, troponin

Non-drug Index Terms

(Based on full text)

Addison disease, **adrenal insufficiency**, adult, anthropometry, body weight, clinical article, clinical trial, conference abstract, controlled study, crossover procedure, double blind procedure, drug therapy,

female, glucose blood level, health survey, human, hypertension, low drug dose, morbidity, mortality, oral drug administration, ossification, osteolysis, physical functioning, psychological well-being, quality of life, questionnaire, randomized controlled trial, self evaluation, Short Form 36, waist circumference

Comment: Invalid ICSR

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Country of Author	United Kingdom
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.273
CAS Registry Numbers	C reactive protein (9007-41-4) hemoglobin A1c (62572-11-6) hydrocortisone (50-23-7) methylprednisolone (6923-42-8 , 83-43-2) osteocalcin (136461-80-8) prednisolone (50-24-8) prednisone (53-03-2)

62. [Multifactorial Diabetic Ketoacidosis in an Older Patient With Myasthenia Gravis Exacerbation: A Case for Enhanced Endocrinology Education and Co-management Strategies](#)

Marrero-Torres E.L., Casal-Hidalgo J.

J.Endocr. Soc. 2025 9: (A699)

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Abstract

Diabetic Ketoacidosis (DKA) is a serious complication of diabetes that can be precipitated by multiple factors, including infections and steroid therapy. This case underscores the complexities of managing DKA in patients with multiple comorbidities, particularly in the context of Myasthenia Gravis (MG) exacerbation and highlights the importance of educating endocrinologists about integrated care for such high-risk patients. We present the case of an 81-year-old male with a medical history of Diabetes Mellitus Type 2, Myasthenia Gravis (MG), Hypertension, and Oropharyngeal dysphagia, who was admitted to our institution ICU for Diabetic Ketoacidosis (DKA), suspected to be secondary to either an infectious process or steroid-induced etiology. The patient had recently been discharged after a MG exacerbation, for which he was discharge with prednisone 25 mg daily. Upon readmission, he exhibited symptoms of lethargy and hypoactivity. Laboratory results revealed a significant hyperglycemic episode, anion gap metabolic acidosis with pH<7.3, and an elevated beta-hydroxybutyrate level at (83.3). The patient was diagnosed with multifactorial DKA, likely induced by recent steroid therapy and a *Pseudomonas aeruginosa* bacteremia, for which he received appropriate antibiotics. The patient's management included insulin infusion, IV fluids, and a gradual transition to subcutaneous insulin after 24 hours. However, as his prednisone tapering proceeded to aid in further glucose control, signs of MG exacerbation (e.g., generalized weakness, bilateral ptosis) emerged, requiring careful adjustment of both his insulin regimen and prednisone dose. This case illustrates the challenges of managing DKA in patients with multiple comorbidities, especially when steroids are involved in the treatment of conditions like Myasthenia Gravis. The importance of comprehensive patient education and coordinated care is highlighted, particularly in the context of steroid use and its impact on glycemic

control. Previous admissions revealed episodes of hyperglycemia likely due to steroid therapy, which could have been mitigated with better follow-up care and closer monitoring. Endocrinologists must be equipped with a broad understanding of managing DKA in the context of complex comorbid conditions such as Myasthenia Gravis. This case underscores the critical role of interdisciplinary care and the need for tailored educational interventions aimed at improving patient outcomes, reducing readmissions, and preventing complications such as DKA. Enhanced training in managing multifactorial DKA, especially in older patients with complex medical histories, is crucial for optimizing care in endocrinology practice.

Drug Index Terms

(Based on full text)

3 hydroxybutyric acid, antibiotic agent, glucose, insulin, prednisone, steroid

Non-drug Index Terms

(Based on full text)

acidosis, adverse drug reaction, aged, bacteremia, case report, clinical article, comorbidity, complication, conference abstract, diabetes mellitus, **diabetic ketoacidosis**, drug combination, drug dose, drug dose reduction, drug megadose, drug therapy, dysphagia, follow up, glycemic control, high risk patient, hospital readmission, human, hyperglycemia, hypertension, hypoactivity, insulin infusion, intensive care unit, intravenous drug administration, lethargy, male, medical history, muscle weakness, **myasthenia gravis**, non insulin dependent diabetes mellitus, oral drug administration, patient education, pH, *Pseudomonas aeruginosa*, ptosis (eyelid), special situation for pharmacovigilance, steroid therapy, subcutaneous tissue, therapy, treatment outcome, very elderly

Comment: Invalid ICSR

Additional Information

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Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1323
CAS Registry Numbers	3 hydroxybutyric acid (300-85-6) glucose (50-99-7 , 84778-64-3 , 8027-56-3) insulin (9004-10-8) prednisone (53-03-2)

63. [Immune Checkpoint Inhibitor Hypophysitis: Delayed Resolution on Low Dose Glucocorticoid Therapy](#)

Kavuri S., Athimulam S.

J.Endocr. Soc. 2025 9: (A858-A859)

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Abstract

Background: Hypophysitis is a known adverse effect of immune checkpoint inhibitors (ICIs) that can present with a variable clinical course. Here we present a case of metastatic melanoma on immunotherapy who presented with acute pituitary enlargement and delayed but complete resolution on systemic glucocorticoid (GC) therapy. Case: A 48-year-old female with past medical history of stage IV melanoma with metastasis to the breast, treated with ipilimumab and nivolumab, presented to the emergency department with intractable headache after receiving two cycles of immunotherapy. Magnetic

resonance imaging (MRI) showed pituitary enlargement with suprasellar extension abutting optic chiasm. Visual field testing was normal. Initial biochemical testing was unremarkable. High dose GCs were avoided initially as it may interfere with efficacy of ICI therapy and worsen outcome. However, due to progressive headache despite analgesics, the patient was started on prednisone 40mg daily, tapered over 1 month, and ICIs were held. MRI pituitary 1 month following initiation of GC revealed interval increase in pituitary size. The lack of response was concerning for potential metastatic lesion but based on available literature, longer period of GC therapy may be required for resolution of hypophysitis. The patient continued prednisone 5mg daily for a further 2 months and ICIs was resumed. Repeat MRI pituitary 3 months later showed resolution of pituitary enlargement. Baseline biochemical testing after GCs were held for 24 hours revealed secondary hypothyroidism and adrenal insufficiency. She transitioned from prednisone to maintenance dose hydrocortisone daily. Serial follow up imaging 18 months following initial diagnosis continue to demonstrate normal pituitary gland, but persistent central hormone deficiencies. Discussion: ICIs are associated with various immune-related adverse effects, including hypophysitis, which occurs in 1-10% of patients on ICIs. Both symptomology and management of hypophysitis are highly variable due to the array of effects, from mass effect due to pituitary enlargement to multiple endocrinopathies. High dose GCs are not typically needed to manage hypophysitis and have been associated with worse clinical outcomes. There are no established guidelines on indication and optimal duration of systemic GC therapy in severe ICI-related hypophysitis. A short course (1 week) of high dose systemic GC therapy can be considered for a patient who presents with compressive symptoms (headache, double vision). In our case, given persistent pituitary enlargement, longer duration of low dose of prednisone was administered alongside immunotherapy, which led to resolution of symptoms and clinical presentation. Management of hypophysitis should be tailored to each patient with close monitoring of hormone levels and pituitary imaging.

Drug Index Terms

(Based on full text)

glucocorticoid, hydrocortisone, **immune checkpoint inhibitor**, ipilimumab, nivolumab, prednisone

Non-drug Index Terms

(Based on full text)

adrenal insufficiency, adult, adverse drug reaction, case report, clinical article, clinical outcome, conference abstract, diagnosis, diplopia, disease course, drug combination, drug dose, drug dose reduction, drug megadose, drug therapy, drug withdrawal, emergency ward, female, follow up, headache, human, human tissue, **hypophysitis**, low drug dose, maintenance drug dose, metastasis in breast, metastatic melanoma, MRI scanner, nuclear magnetic resonance imaging, optic chiasm, polyendocrinopathy, secondary hypothyroidism, side effect, therapy, unexpected outcome of drug treatment, visual field

Comment: valid ICSR

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Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1627
CAS Registry Numbers	hydrocortisone (50-23-7) ipilimumab (477202-00-9) nivolumab (946414-94-4) prednisone (53-03-2)

64. [Treatment Patterns and Changes in Health States in Patients with Classic Congenital Adrenal Hyperplasia: An Analysis of Data from the CAHtalog™ Registry](#)

Jeha G.S., White P.C., Lin-Su K., Matos D., Tang F., Friedler H.S., Roberts E., Cheng H.K., Sen G.P.

J.Endocr. Soc. 2025 9: (A120-A121)

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Abstract

Background: Classic congenital adrenal hyperplasia (CAH) often requires supraphysiologic glucocorticoid (GC) doses to reduce excess androgens. CAHtalog™ is an active US-based registry of patients with classic CAH developed in partnership with CARES Foundation on the PicnicHealth platform. Objective: To analyze treatment patterns and health states in patients in the CAHtalog registry. Methods: Medical records from pediatric (<18 yrs) and adult (≥18 yrs) patients in CAHtalog were abstracted into a deidentified database. GCs were summarized by type (hydrocortisone [HC], dexamethasone [DEX], prednisone [PRED]) and total daily GC dose in hydrocortisone equivalents. Due to the longitudinal nature of the registry, patients could be included in both age groups and multiple GC-treatment groups. Patients who took different GC types and doses over the observation period were counted in multiple categories. Health states for each patient were determined by matching GC records with the nearest androstenedione (A4) record (within the next 365 days). Based on the 95th percentile of cortisol production in healthy persons, GC doses were categorized as higher or lower (>11 or ≤11 mg/m² /dHCE) for the health state analysis. Based on the upper limit of normal (ULN) for age and sex, A4 was categorized as ≥ULN or <ULN. Transitions through health states based on GC dose (higher to lower, lower to higher) and A4 (≥ULN to <ULN, <ULN to ≥ULN) were analyzed in patients with ≥3 matched GC-A4 records. Results: Among 74 patients (37 adult; 51 pediatric; 14 both) with ≥1 medication record during the observation period, and without a standardized threshold or qualification of androgen control across individuals, the most common GC treatment (77.8% [adult]; 90.2% [pediatric]) at one point in their treatment journey was HC alone (mean±SD daily dose, 22.9±7.4 mg/d [adult]; 11.8±4.1 mg/m² /d [pediatric]), followed by DEX alone (47.2%; 35.1 ±18.4 mg/d) for adults.

For pediatric patients, the next most common GC treatments after HC alone were DEX +HC (7.8%; 23.4±7.7 mg/m² /d) and PRED alone (7.8%, 12.6±9.8 mg/m² /d). Among 51 patients (20 adult; 33 pediatric; 2 both) with ≥3 matched GC-A4 records, health state transitions were observed across an age range of 0-67 yrs (median observation=6.1 yrs). Most (88.2%) patients had ≥1 health state transition and 64.7% had ≥3 health state transitions. Nearly all (98.0%) patients (with 76.5% of matched records) were prescribed higher GC doses and/or had A4 ≥ ULN at least once during their treatment journey. Conclusions: Longitudinal data from patients in the CAHtalog registry show various GC regimens and multiple transitions through health states characterized by higher GC doses and A4 ≥ULN, illustrating the lifelong balancing act between A4 management and GC dosing. These data also suggest that achievement of disease control today (A4 <ULN) does not predict maintenance of control tomorrow.

Drug Index Terms

(Based on full text)

androgen, androstenedione, dexamethasone, glucocorticoid, hydrocortisone, prednisone

Non-drug Index Terms

(Based on full text)

adolescent, adult, cohort analysis, conference abstract, **congenital adrenal hyperplasia**, controlled study, disease control, drug combination, drug therapy, female, groups by age, **health status**, human, longitudinal study, major clinical study, male, pediatric patient, special situation for pharmacovigilance

Comment: invalid ICSR

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Country of Author	United States
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Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1210/jendso/bvaf149.225
CAS Registry Numbers	androstenedione (26264-53-9 , 63-05-8) dexamethasone (50-02-2) hydrocortisone (50-23-7) prednisone (53-03-2)

65. [Atypical Presentation of a Rare Bone Disease in Adolescence: A Rare Case Report](#)

Rehm L.R., Drake L.T., Dahir S.S., Lichrenfeld C.R., Dahir K.M.

J.Endocr. Soc. 2025 9: (A365)

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Abstract

Background: Chronic recurrent multifocal osteomyelitis (CRMO) is an autoinflammatory disorder that causes ostealgia due to bone inflammation without any underlying infection. Typically, this is a diagnosis of exclusion. CRMO often appears in children at age 9 or 10 and can lead to bone deformities or irregular bone growth patterns.

Hypophosphatasia (HPP) is an inherited disorder caused by variants in the gene ALPL that affects bone and tooth development. HPP disrupts the body's mineralization process, resulting in abnormal mineral deposits throughout the skeletal structure and impacting bone growth.

Case Description: A 22-year-old female suffered pain in her femur, hips, pelvis, humerus, hand, foot, and wrist. At the age of 12 she developed debilitating jaw pain requiring a biopsy and extraction of two teeth.

Malignancy and an infectious etiology were excluded, she was diagnosed with CRMO and referred to rheumatology. NSAIDs proved ineffective, leading to treatment with prednisone and linezolid from ages 12 to 18, with periods requiring high doses of steroids for pain relief. Furthermore, the patient suffered from both acute pain and generalized bone/muscle pain. At 16, the patient had bilateral metatarsal stress fractures, arthralgias, fatigue, back pain, migraines, and POTS. She transferred to an adult clinic at age 18, laboratory tests revealed low AlkP of 19 U/L (rr 40-150 U/L) and elevated B6 of 287.2 nmol/L (rr 20-125 nmol/L). A review of the EHR showed a persistently low AlkP since the age of 19 days, which was below the age-adjusted reference range at 65U/L (rr 150-400 U/L) but had not been clinically recognized. As a young adult, the patient reported progressive, widespread bone and muscle pain that significantly limited her daily activities. Genetic testing identified a heterozygous pathogenic variant in ALPL (c.1171C>T). Since her initial visit in the adult endocrinology clinic, she has had additional bilateral metatarsal stress fractures. Discussions are underway to initiate ERT. Conclusions: A deficiency in tissue nonspecific alkaline phosphatase (TNAP) causes HPP. TNAP plays a critical role in mineralization by dephosphorylating inorganic pyrophosphate (PPi), a mineralization inhibitor produced from adenosine triphosphate (ATP) through the action of ectonucleotide pyrophosphatase phosphodiesterase 1 (NPP1). CRMO has been reported in association with HPP. Given the variable phenotypes and atypical presentations of HPP, increased awareness of these variations is essential. Accurate diagnosis of HPP and CRMO is important to avoid unnecessary glucocorticoid exposure. Further research is needed to better understand the relationship between HPP and CRMO.

Drug Index Terms

(Based on full text)

alkaline phosphatase, glucocorticoid, inorganic pyrophosphatase, linezolid, nonsteroid antiinflammatory agent, phosphodiesterase I, prednisone, pyrophosphate

Non-drug Index Terms

(Based on full text)

adolescence, adult, analgesia, arthralgia, backache, **bone disease**, bone growth, bone malformation, case report, child, clinical article, conference abstract, congenital disorder, daily life activity, diagnosis,

drug megadose, drug therapy, electronic health record, fatigue, female, genetic screening, human, hypophosphatasia, jaw pain, laboratory test, migraine, myalgia, osteitis, osteomyelitis, phenotype, therapy, tooth development, young adult

Comment: Invalid ICSR

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Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.683
CAS Registry Numbers	alkaline phosphatase (9001-78-9) inorganic pyrophosphatase (9024-82-2 , 9033-44-7) linezolid (165800-03-3) phosphodiesterase I (9025-82-5) prednisone (53-03-2) pyrophosphate (14000-31-8 , 7722-88-5 , 7758-16-9)

66. [Autoimmune Polyglandular Syndrome Type 2 Presenting as Hashimoto's Thyroiditis and Primary Adrenal Insufficiency](#)

Dela Cruz Balina M.M., Uy-Ho J., Yao H., Reburiano S.B., David E., Sorsona F.

J.Endocr. Soc. 2025 9: (A124-A125)

Embase

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Abstract

Background: Autoimmune Polyglandular Syndromes (APS) are groups of 2 or more autoimmune endocrine and non-endocrine disorders that occur in discreet patterns. APS type 2, considered to be the most common form, is composed of primary adrenal insufficiency, type 1 diabetes mellitus, and autoimmune thyroid disease. Recognition of signs and symptoms of adrenal insufficiency before management of a diagnosed thyroid disease is of utmost importance. Case: This is a case of a 58-year-old female who was referred to endocrine service due to nodular goiter. Additional physical examination showed areas of hyperpigmentation, particularly sun-exposed areas, gums, and ears, which developed gradually over 3 years. On further probing, the patient reported brief episodes of lightheadedness, easy fatigability, and weight loss. She was initially diagnosed with hypothyroidism and tested positive for anti-thyroid peroxidase antibodies. She had mild hyponatremia with normal potassium and blood sugar levels. Further endocrine work-up showed low fasting 8 am serum cortisol levels at 1.99 mcg/dL associated with elevated ACTH of 1488 pg/mL. A tuberculosis infection was also ruled out. Ideally, 21-hydroxylase antibody testing should be done to diagnose autoimmune adrenal insufficiency. However, it was not available in our country. Anti-glutamic acid decarboxylase was negative. The adrenal CT scan result was also unremarkable. She was started on a physiologic dose of prednisone 5mg tab once daily and eventually given levothyroxine replacement for Hashimoto's thyroiditis. On her latest follow-up, she reported significant improvement in daily activities and showed no signs of steroid excess. Conclusion: This case underscores the importance of the ability to diagnose clinical syndromes. Starting a patient on a thyroid hormone replacement for autoimmune thyroiditis without suspecting the presence of possible adrenal insufficiency will lead to worsening of the patient's symptoms. Early recognition and

intervention are crucial in preventing complications and improving the quality of life for affected individuals while also being able to anticipate the future development of associated disorders.

Drug Index Terms

(Based on full text)

antithyroid agent, **epinephrine**, glutamate decarboxylase, levothyroxine, potassium, prednisone, steroid 21 monooxygenase, thyroid hormone, thyroid peroxidase antibody

Non-drug Index Terms

(Based on full text)

Addison disease, **adrenal gland**, adrenal insufficiency, adult, **autoimmune polyendocrinopathy candidiasis ectodermal dystrophy**, autoimmune thyroiditis, body weight loss, case report, clinical article, computer assisted tomography, conference abstract, daily life activity, diagnosis, dizziness, drug therapy, fasting, female, follow up, glucose blood level, **Hashimoto disease**, hormone substitution, human, hydrocortisone blood level, hyperpigmentation, hyponatremia, hypothyroidism, insulin dependent diabetes mellitus, nodular goiter, physical examination, quality of life, thyroid disease, tuberculosis

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
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Conference Name	ENDO 2025
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Publication Type	Conference Abstract
Page Range	A124-A125
Country of Author	Philippines
Country of Source	Netherlands

Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.232
CAS Registry Numbers	epinephrine (51-43-4 , 55-31-2 , 6912-68-1) glutamate decarboxylase (9024-58-2) levothyroxine (51-48-9) potassium (7440-09-7) prednisone (53-03-2) steroid 21 monooxygenase (9029-68-9)

67. [Unexpected Low: A Case of Disulfiram Induced Hypoglycemia](#)

Anderson S., Ahmed Z., Bayrakdar K., Hor T.K.

J.Endocr. Soc. 2025 9: (A678)

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Introduction: Disulfiram treats alcohol use disorder by inhibiting aldehyde dehydrogenase. Recently discovered, disulfiram and its metabolites also bind to and deactivate fructose-1,6-bisphosphatase leading to decreased gluconeogenesis. Clinical case: A 38-year-old female with alcohol use disorder, monoallelic CFTR gene mutation cystic fibrosis, chronic pancreatitis status-post pancreatectomy with failed pancreatic islet cell auto-transplant and subsequent diabetes mellitus on insulin pump presented to the emergency department with severe refractory hypoglycemia 36 hours after starting disulfiram, despite eating and discontinuing insulin administration. On arrival, patient hemodynamically stable but later unresponsive. Labs showed blood glucose (BG) 18 mg/dL (nl 60-99 mg/dL), bicarbonate 21 mmol/L (nl 22-29 mmol/L), ketone 1.1 mmol/L (nl 0-0.6 mmol/L). She responded to infused dextrose 10% with repeat BG 119 mg/dL. Later transitioned to infused dextrose 5%, she became somnolent with BG 30 mg/dL. Prednisone 40 mg PO was then started followed by hydrocortisone 50 mg q12Hr in setting of recurrent severe hypoglycemia (<20 mg/dL) for treatment of adrenal insufficiency. 9AM cortisol 1.7 ug/dL

(nl 3.17-19.4 ug/dL), ACTH 3.9 ng/mL (nl 6-50 pg/mL) and albumin 2.8 g/dL (nl 3.5-5). Hypoglycemic episodes continued despite high-dose steroids and dextrose infusion. Hypoglycemia resolved 48 hours after disulfiram discontinued, allowing for discontinuation of dextrose infusion. Outpatient HPA axis testing showed resolution of adrenal insufficiency with AM cortisol 26 ug/dL; steroids discontinued with no recurrence of hypoglycemia. Pituitary MRI negative for pituitary cause of adrenal insufficiency. We postulate that her severe, refractory hypoglycemia was secondary to disulfiram-induced inhibition of gluconeogenesis in combination with low glycogen stores due to chronic alcohol use. In 2020, research demonstrated previously unknown inhibiting covalent bond between disulfiram and its metabolites and fructose-1,6-bisphosphatase, leading to gluconeogenesis inhibition. Here, history of alcohol use and adrenal insufficiency led to severe refractory hypoglycemia despite glucocorticoids and dextrose infusions. Conclusion: This may be the first reported case of disulfiram-induced hypoglycemia in an adult. Disulfiram should be used in caution in those with labile diabetes.

Drug Index Terms

(Based on full text)

albumin, aldehyde dehydrogenase, bicarbonate, **disulfiram**, fructose bisphosphatase, glucocorticoid, glucose, glycogen, hydrocortisone, ketone, proinsulin

Non-drug Index Terms

(Based on full text)

adrenal insufficiency, adult, adverse drug reaction, alcohol consumption, alcoholism, case report, chronic pancreatitis, clinical article, conference abstract, controlled study, covalent bond, cystic fibrosis, drug administration, drug dose titration, drug therapy, drug withdrawal, emergency ward, female, gene mutation, gluconeogenesis, glucose blood level, human, hypersomnia, **hypoglycemia**, hypophysis, hypothalamus hypophysis adrenal system, insulin dependent diabetes mellitus, insulin pump, male, MRI scanner, nuclear magnetic resonance imaging, pancreas islet cell, pancreatectomy, side effect, unexpected outcome of drug treatment

Comment: valid ICSR

Additional Information

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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A678
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1282
CAS Registry Numbers	aldehyde dehydrogenase (37353-37-0 , 9028-86-8) bicarbonate (144-55-8 , 71-52-3) disulfiram (97-77-8) fructose biphosphatase (9001-52-9) glucose (50-99-7 , 84778-64-3 , 8027-56-3) glycogen (9005-79-2) hydrocortisone (50-23-7) proinsulin (11062-00-3 , 9035-68-1)

68. [Late Diagnosis of Nonclassical Congenital Adrenal Hyperplasia In A Female With Infertility: A Case Report](#)

Moreno C.M.R., Reyes V.F.M., Castillo B.V.P., Medina P.S.D., Mendez E., Shaaban N.H.

J.Endocr. Soc. 2025 9: (A180)

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Abstract

Background: Nonclassical congenital adrenal hyperplasia (NCCAH) is a milder form of congenital adrenal hyperplasia caused by 21-hydroxylase deficiency. It presents later in life with signs of hyperandrogenism, such as irregular menses, hirsutism, and infertility. Due to clinical overlap with polycystic ovary syndrome (PCOS), it is often misdiagnosed, especially when initial laboratory evaluation is limited to ovarian-focused studies without assessing adrenal function. When standard PCOS treatments fail, NCCAH should be considered, as its management and follow-up differ significantly. **Clinical Case:** A 32-year-old woman with a history of prediabetes and oligomenorrhea presented for endocrine evaluation after one year of infertility and elevated androgen levels. She had previously been diagnosed with PCOS based on irregular cycles and mild hirsutism. Initial labs revealed elevated DHEA-S, mildly elevated total and free testosterone, and elevated 17-OHP level (698 ng/dL). A dexamethasone suppression test showed persistent cortisol and DHEA-S elevation. Due to suspicion of NCCAH, a cosyntropin stimulation test was performed, confirming the diagnosis with 17-OHP levels of 1051 ng/dL. She was started on hydrocortisone 15 mg daily, leading to spontaneous conception within weeks. After pregnancy confirmation, her therapy was transitioned to a physiologic dose of prednisone 7.5 mg daily. The plan included monitoring pregnancy progression, testing the newborn for 21-hydroxylase deficiency, and adjusting therapy postpartum. **Conclusion:** This case emphasized the importance of thorough endocrine evaluation in women with infertility and hyperandrogenism. Early differentiation between PCOS and NCCAH is critical, as management strategies and reproductive outcomes differ. Adrenal-focused testing in the initial workup of atypical PCOS presentations can prevent delayed diagnosis and improve fertility outcomes.

Drug Index Terms

(Based on full text)

epinephrine, hydrocortisone, prasterone, prednisone, testosterone, tetracosactide

Non-drug Index Terms

(Based on full text)

adrenal function, **adrenal gland**, adult, case report, cell differentiation, clinical article, conference abstract, congenital adrenal hyperplasia, **delayed diagnosis**, dexamethasone suppression test, diagnosis, drug therapy, female, follow up, hirsutism, human, human tissue, hyperandrogenism, impaired glucose tolerance, **infertility**, **late onset congenital adrenal hyperplasia**, menstruation disorder, newborn, oligomenorrhea, ovary polycystic disease, pregnancy, provocation test, steroid 21 monooxygenase deficiency

Comment: invalid ICSR

Additional Information

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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A180
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.333
CAS Registry Numbers	epinephrine (51-43-4 , 55-31-2 , 6912-68-1) hydrocortisone (50-23-7) prasterone (53-43-0) prednisone (53-03-2) testosterone (58-22-0) tetracosactide (16960-16-0)

69. [Secondary Adrenal Insufficiency due to Inhaled Fluticasone Use Alone](#)

Raja N., Romanovschi M., Hu Y.

J.Endocr. Soc. 2025 9: (A126-A127)

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Abstract

Introduction: Secondary adrenal insufficiency (SAI) is a rare but clinically significant disorder characterized by impaired cortisol production due to decreased stimulation of the adrenal glands by adrenocorticotrophic hormone (ACTH). It has a myriad of pathological causes but has rarely been demonstrated due to suppression of the hormone from inhaled and/or intranasal corticosteroid use alone. Case: This is the case of a 46-year-old female with a past medical history of asthma who presented with concern of adrenal insufficiency. The patient was noted to have low 24-hr urinary cortisol of 1.1 ng/mg (65-200 ng/mg) and low DHEA level of 1.0 ng/mg (20-750 ng/mg), low AM cortisol of <0.1 ng/mg (30-130 ng/mg). Repeat lab work confirmed low ACTH of <5 pg/mL (6-50 pg/mL) and low AM cortisol <0.5 mcg/dL (4.0-22.0 mcg/dL). She endorsed associated symptoms of lightheadedness, weight loss and fatigue. Her life-long history of asthma was controlled on daily fluticasone propionate/salmeterol and albuterol as needed. She also suffered from intermittent asthma flares which required steroid courses with most recent prednisone course a year prior to presentation. She had been on fluticasone propionate/salmeterol for years with recent increase in dose to two puffs a day in the last six months. Her decreased AM cortisol level and symptoms suggested a diagnosis of adrenal insufficiency. Low ACTH suggested a secondary cause. Pituitary MRI was performed confirming no pituitary abnormalities. CT abdomen performed without any acute abnormality noted on adrenal glands. Her secondary AI was likely caused by her fluticasone use. She was recommended to start on hydrocortisone replacement however instead, she slowly tapered off her inhaled steroid over the course of three months. Her HPA axis function slowly recovered along with her fatigue and weight loss. Repeat AM cortisol after three months went back to 10.2 mcg/dL (4.0-22.0 mcg/dL).

Discussion: This case presents secondary adrenal insufficiency arising from chronic inhaled corticosteroid use alone. It is well known that SAI may result from prolonged use of systemic exogenous glucocorticoid use by suppressing the hypothalamic-pituitary-adrenal (HPA) axis, leading to adrenal atrophy. The effects of inhaled or intranasal steroids were thought to be less significant. It has been better recognized that concomitant use of CYP3A4 inhibitor can cause AI such as seen in ritonavir with inhaled fluticasone or itraconazole and inhaled budesonide. It is rare to see inhaled steroids alone cause secondary AI. Through this report, we aim to raise awareness about the potential risk of SAI associated with inhaled steroid use alone. We would like to emphasize the importance of careful clinical evaluation in those receiving long-term corticosteroid therapy regardless of formulation, and prompt recognition and treatment to prevent adrenal crisis and improve patient outcomes.

Drug Index Terms

(Based on full text)

budesonide, corticosteroid, corticotropin, cytochrome P450 3A4, **epinephrine**, **fluticasone**, fluticasone propionate plus salmeterol, glucocorticoid, hydrocortisone, itraconazole, prasterone, prednisone, ritonavir, salbutamol

Non-drug Index Terms

(Based on full text)

adolescent, adrenal cortex atrophy, adrenal cortex insufficiency, **adrenal gland**, **adrenal insufficiency**, adult, asthma, body weight loss, case report, child, clinical article, clinical evaluation, conference abstract, corticosteroid therapy, diagnosis, dizziness, drug administration, drug therapy, fatigue, female, human, hydrocortisone urine level, hypothalamus, hypothalamus hypophysis adrenal system, **inhalation**, inhalational drug administration, intravenous drug administration, low drug dose, MRI scanner, nuclear magnetic resonance imaging, side effect

Comment: valid ICSR

Additional Information

Abbreviated Journal Title J.Endocr. Soc.

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Source Type Journal

Conference Name	ENDO 2025
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A126-A127
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.236
CAS Registry Numbers	budesonide (51333-22-3 , 51372-29-3) corticotropin (11136-52-0 , 9002-60-2 , 9061-27-2) cytochrome P450 3A4 (329736-03-0) epinephrine (51-43-4 , 55-31-2 , 6912-68-1) fluticasone (90566-53-3) fluticasone propionate plus salmeterol (136112-01-1) hydrocortisone (50-23-7) itraconazole (84625-61-6) prasterone (53-43-0) prednisone (53-03-2) ritonavir (155213-67-5) salbutamol (18559-94-9 , 35763-26-9)

70. [Sitagliptin Induced Bullous Pemphigoid-A Rare But Devious Side Effect](#)

Atreya S., Yagnik K., Rios J.P.D., Patel N.K.

J.Endocr. Soc. 2025 9: (A476)

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Abstract

Background: Sitagliptin is a commonly used medication which is used to treat diabetes mellitus type 2 by inhibiting the enzyme dipeptidyl peptidase-4 (DPP-4). Here in, we present an uncommon case of sitagliptin associated bullous pemphigoid (BP). Case presentation: A 75 year old male came with pruritic, fluid filled blisters primarily on his feet, left more than right. He has significant past medical history of diabetes mellitus type 2 for 30 years, asthma, hypertension and hyperlipidemia. Blisters were heralded with a prodrome of generalised itching and were gradually worsening over a period of 6-8 months. Initially, the etiology of blisters was thought to be infectious, however, a short course of doxycycline didn't show any improvement. Over the period of time, blisters extended over his arms, back and chest, but spared the face and mucosal membranes. He started on methylprednisolone initially, later switched to oral prednisone. Sitagliptin was discontinued. A biopsy from the right arm revealed pathology suggestive of bullous pemphigoid. The patient was discharged on steroid taper and topical triamcinolone resulted in full resolution of the bullae in 2 weeks. Discussion: Immunobullous disorders, most commonly bullous pemphigoid, is being recorded as a commonly emerging side effect of DPP-4 inhibitors. Although the onset of drug-induced BP typically occurs within months of medication initiation, delayed presentations even after decades are being increasingly recognized. This case reinforces the need to maintain clinical suspicion for medication-induced BP, even in long-term users of DPP-4 inhibitors. Prompt withdrawal of the offending agent, along with immunosuppressive therapy, most commonly steroids, can result in swift clinical response. Therefore a high suspicion is a prerequisite for rapid diagnosis and prompt discontinuation of the offending agent for best outcomes.

Drug Index Terms

(Based on full text)

dipeptidyl peptidase IV, dipeptidyl peptidase IV inhibitor, doxycycline, methylprednisolone, prednisone, **sitagliptin**, triamcinolone

Non-drug Index Terms

(Based on full text)

aged, asthma, **bullous pemphigoid**, case report, clinical article, conference abstract, diagnosis, drug therapy, drug withdrawal, human, hyperlipidemia, hypertension, immunosuppressive treatment, male, medical history, non insulin dependent diabetes mellitus, oral drug administration, pruritus, side effect, unexpected outcome of drug treatment

Comment: valid ICSR

Additional Information

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Conference Name	ENDO 2025
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A476
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.889
CAS Registry Numbers	dipeptidyl peptidase IV (54249-88-6) doxycycline (10592-13-9 , 17086-28-1 , 564-25-0 , 94088-85-4) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) sitagliptin (486460-32-6 , 654671-78-0 , 654671-77-9 , 2088771-60-0 , 486459-71-6) triamcinolone (124-94-7)

71. [Paradoxical Steroid Use in Ectopic ACTH Syndrome: Navigating Complications of Small Cell Lung Cancer and Chemotherapy-Related Immune Hepatitis](#)

Javaid A., Deyar D., Sabri M.S., Bondarenko D., Rasheed A., O'Sullivan C.

J.Endocr. Soc. 2025 9: (A865-A866)

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Abstract

Introduction: Cushing syndrome from ectopic ACTH production occurs in ~1-5% of SCLC cases, posing diagnostic and therapeutic challenges. This case highlights the complex relationship between ectopic ACTH, hyperglycemia, hepatic dysfunction, and oncologic treatment complications, including the paradoxical use of exogenous steroids. Presentation: A 56-year-old man with poorly controlled type 2 diabetes presented with worsening hyperglycemia, leg swelling, and cough. Initial HbA1c was 11.6% (n <5.7%), reflecting chronic poor glycemic control despite metformin and insulin. Imaging revealed a lung mass confirmed as SCLC. Evaluation showed hypertension, hypokalemia (K⁺ 2.5 mmol/L, n 3.5-5.0 mmol/L), elevated transaminases (ALT 98 U/L, n 7-56 U/L; AST 110 U/L, n 10-40 U/L), and refractory hyperglycemia. Labs demonstrated elevated ACTH (210 pg/mL, n 10-50 pg/mL) and serum cortisol (44 µg/dL, n 6-23 µg/dL), consistent with ectopic ACTH production. Management: Treatment began with ketoconazole, later switched to metyrapone and mifepristone due to poor tolerance and drug interactions. His SCLC was treated with cisplatin/etoposide followed by maintenance atezolizumab. Early improvements in glycemic control and BP were noted, However, persistent transaminitis (ALT: 120 U/L; AST: 130 U/L) raised concerns for chemotherapy-induced immune-mediated hepatitis, a known complication of atezolizumab. Liver biopsy was deferred due to the patient's clinical stability and the high likelihood of drug-induced liver injury (DILI). In a paradoxical therapeutic maneuver, exogenous corticosteroids (prednisone 40 mg daily) were initiated to mitigate the immune-mediated hepatotoxicity. This decision was particularly unconventional given the patient's underlying hypercortisolism. Close

monitoring of cortisol levels, liver enzymes, and glycemic control was maintained throughout this period. Remarkably, the patient's transaminitis improved significantly within two weeks of steroid initiation, without exacerbation of his hypercortisolism symptoms. Conclusion: This case highlights the complexities of managing severe metabolic derangements, including hyperglycemia, hypokalemia, and hepatic dysfunction, in the setting of oncologic treatment. The paradoxical use of exogenous steroids to address chemotherapy-induced liver injury underscores the need for innovative and individualized therapeutic approaches. Further investigation is warranted to establish guidelines for the safe and effective use of corticosteroids in patients with hypercortisolism and concurrent immunemediated complications.

Drug Index Terms

(Based on full text)
atezolizumab, cisplatin, corticosteroid, etoposide, hemoglobin A1c, hydrocortisone, insulin, ketoconazole, liver enzyme, metformin, metyrapone, mifepristone, prednisone, **steroid**

Non-drug Index Terms

(Based on full text)
case report, clinical article, clinical practice guideline, complication, conference abstract, coughing, Cushing syndrome, drug combination, drug interaction, drug therapy, **ectopic corticotropin production**, glycemic control, **hepatitis**, human, hydrocortisone blood level, hypercortisolism, hyperglycemia, hypertension, hypertransaminasemia, hypokalemia, leg edema, leg swelling, liver biopsy, liver dysfunction, liver injury, liver toxicity, male, non insulin dependent diabetes mellitus, **small cell lung cancer**, therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A865-A866
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1638
CAS Registry Numbers	atezolizumab (1380723-44-3) cisplatin (15663-27-1 , 26035-31-4 , 96081-74-2) etoposide (33419-42-0 , 433304-61-1) hemoglobin A1c (62572-11-6) hydrocortisone (50-23-7) insulin (9004-10-8) ketoconazole (65277-42-1 , 83374-59-8) metformin (1115-70-4 , 657-24-9) metyrapone (22752-91-6 , 2405-72-3 , 54-36-4 , 908-35-0) mifepristone (84371-65-3) prednisone (53-03-2)

72. [Hypercalcemia Associated with Cosmetic Oil Injection](#)

Sainz-Mejias A., Vargas J.E.E., Oviedo L.U.

J.Endocr. Soc. 2025 9: (A397)

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Abstract

Background: Hypercalcemia is observed in 0.2-4% of the population, with varying incidence depending on the etiology. The PTH-independent

cases associated with peripheral activation of 1,25-dihydroxyvitamin D should always be considered. Since the 1950s, there has been a global trend toward increasing cosmetic use of inorganic oils, however, hypercalcemia related to cosmetic oil injection is an underreported complication associated with high rates of acute and chronic kidney injury, and even death. Clinical case: A 49-year-old female underwent a cosmetic procedure involving mineral oil injections in both gluteal regions 14 years ago. A year later, she was misdiagnosed with systemic lupus erythematosus and lupus nephritis, treated with immunosuppressive drugs. For the past four years, she had severe hypercalcemia and calcinosis cutis which led to chronic ulcers. She presented with hypercalcemia and hyperphosphatemia (calcium 13.1mg/dL, phosphorus 5.57mg/dL), acute renal injury (creatinine 2.34mg/dL), and low 25-OHD3 levels (24.5ng/mL) with suppressed PTH (1pg/mL), with low PTHrP (0.8pmol/L) and high 1,25-OHD3 levels (120pg/mL). Ultrasound imaging showed nephrocalcinosis and calcifications in soft tissues and arteries. Biopsy of the ulcers showed extensive ulceration and granulation tissue, with fragments of dense fibrous connective tissue containing extensive deposits of inorganic, basophilic granular material with areas of dense calcification, findings consistent with the migration of inorganic substances to the dermis, causing secondary ischemia. The patient was diagnosed with hypercalcemia and calcinosis associated with cosmetic oil migration, received intravenous antibiotics, zoledronic acid and oral prednisone, and then was discharged on ketoconazole and prednisone, achieving continued improvement in her clinical and renal condition. Conclusion: This case report presents a rare instance of hypercalcemia associated with cosmetic oil injection, with peripheral production of 1,25-dihydroxyvitamin D, without granuloma formation, likely due to initial pharmacological immunosuppression. It urges us to consider the importance of recognizing potential complications of such cosmetic procedures and promptly addressing them with targeted therapy.

Drug Index Terms

(Based on full text)

antibiotic agent, calcium, **cosmetic**, creatinine, ketoconazole, mineral oil, phosphorus, prednisone, zoledronic acid

Non-drug Index Terms

(Based on full text)

adult, calcinosis, case report, clinical article, complication, conference abstract, connective tissue, drug therapy, female, granulation tissue, human, human tissue, **hypercalcemia**, hyperphosphatemia, immunosuppressive treatment, incidence, intravenous drug administration, kidney calcification, lupus erythematosus nephritis, skin calcification, soft tissue, systemic lupus erythematosus

Comment: invalid ICSR

Additional Information

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Conference Name	ENDO 2025
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Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A397
Country of Author	Costa Rica
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.742
CAS Registry Numbers	calcium (7440-70-2 , 14092-94-5) creatinine (19230-81-0 , 60-27-5) ketoconazole (65277-42-1 , 83374-59-8) phosphorus (7723-14-0) prednisone (53-03-2) zoledronic acid (118072-93-8 , 131654-46-1 , 165800-06-6 , 165800-07-7)

73. [Lymphocytic Hypophysitis in an Adolescent Female: Diagnostic Challenges and Therapeutic Response](#)

Davuluri H.C., Kaur K., Chaulagain P., Krug E.I.

J.Endocr. Soc. 2025 9: (A763)

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Abstract

A 16-year-old female presented with a history of headaches of 1 year duration and was initially diagnosed with migraine without aura. At the same time, she developed menstrual irregularity and eventual amenorrhea, was prescribed a 3 months course of OCPs. Amenorrhea persisted following discontinuation of OCP regimen. She was seen by a pediatric neurologist for worsening headaches, associated with light sensitivity and worsening of pain with noise, coughing or sneezing. Brain MRI revealed a 1.5 cm pituitary mass with displacement of the optic chiasm. Fortunately, ophthalmologic evaluation was unremarkable. Endocrine evaluation revealed evidence of central hypogonadism and mild hyperprolactinemia with prolactin level of 57.6 ng/ml [4.8-33.4 ng/ml]. ACTH, cortisol, growth hormone, IGF-I, TSH, free T4 levels were within normal limits. Her growth and development were normal for her age. Cabergoline (0.5 mg twice weekly) was started, reducing prolactin to 0.2 ng/ml, with no significant change in headache frequency and no resolution of amenorrhea. Subsequently, she underwent transnasal transsphenoidal biopsy of the suspected pituitary mass. Intraoperative findings revealed a thickened dura with fibrous appearance and a mass-like lesion adherent to the dura. Frozen sections analysis revealed no evidence of a neoplasm; the findings are suggestive of lymphocytic hypophysitis. Histopathology confirmed the diagnosis of lymphocytic hypophysitis. Further underwent a rheumatological workup, including ESR, CRP, ANA, anti-SSA/SSB, anti-dsDNA, and ANCA, all of which were unremarkable. She received glucocorticoid therapy with prednisone, 25 mg daily with prolonged steroid taper over a period of 12 months. Serial brain MRIs revealed a reduction in the size of the pituitary gland to 6 mm after 4 months of treatment. Repeat imaging at 3 and 6, 9 and 12 months showed continued resolution. Her menstrual cycle normalized after 4 months of treatment, and headaches significantly improved. Glucocorticoid therapy has been discontinued after 12 months of

treatment with no evidence of recurrence so far. Discussion: We present a case of histologically confirmed lymphocytic hypophysitis with complete radiologic and functional recovery following prolonged glucocorticoid therapy at a relatively low dose with very slow taper. Affected patients typically present with severe headaches and symptoms of pituitary dysfunction. Optimal treatment is not known at this time. There is a report describing clinical improvement in 4 patients following high dose methylprednisolone pulse therapy and another report describing resolution of symptoms after a short course of glucocorticoids in 2 patients. Additional research is needed to determine best treatment strategy, but it appears that high index of suspicion and early diagnosis may prevent progressive pituitary atrophy and hypopituitarism.

Drug Index Terms

(Based on full text)

C reactive protein, cabergoline, corticotropin, glucocorticoid, growth hormone, hydrocortisone, methylprednisolone, neutrophil cytoplasmic antibody, prednisone, prolactin, steroid, thyrotropin

Non-drug Index Terms

(Based on full text)

adolescent, adult, amenorrhea, **autoimmune hypophysitis**, case report, clinical article, conference abstract, coughing, diagnosis, drug dose reduction, drug therapy, drug withdrawal, dura mater, female, free thyroxine index, frozen section, headache, histopathology, human, human tissue, hyperprolactinemia, hypogonadism, hypophysis disease, hypopituitarism, low drug dose, menstrual cycle, menstrual irregularity, migraine without aura, MRI scanner, nuclear magnetic resonance imaging, optic chiasm, photophobia, sneezing, **treatment response**

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
ISSN	24721972
Source Type	Journal
Conference Name	ENDO 2025
Conference Location	United States, San Francisco, CA
Conference Date	2025-07-12 to 2025-07-15

Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A763
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1447
CAS Registry Numbers	C reactive protein (9007-41-4) cabergoline (81409-90-7) corticotropin (11136-52-0 , 9002-60-2 , 9061-27-2) growth hormone (36992-73-1 , 37267-05-3 , 66419-50-9 , 9002-72-6) hydrocortisone (50-23-7) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) prolactin (12585-34-1 , 50647-00-2 , 9002-62-4) thyrotropin (9002-71-5)

74. [Primary Bilateral Macronodular Adrenal Hyperplasia with co-secretion of Aldosterone and Cortisol in a Patient with Hyperparathyroidism-Jaw Tumor Syndrome due to a Pathogenic CDC73 Mutation](#)

Martel-Duguech L., Doreille A., Latour M., Thibodeau M., Dumas N., Bourdeau I., El Haffaf Z., Lacroix A.

J.Endocr. Soc. 2025 9: (A215-A216)

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Embase Open URL: [redirect to full text](#)

Abstract

Background: Hyperparathyroidism-Jaw Tumor Syndrome (HPT-JT) is secondary to germline inactivating mutations of the tumor suppressor CDC73 gene. Affected patients often present in addition to severe HPT, lesions in kidney, uterus and thyroid. Primary bilateral macronodular adrenal hyperplasia (PBMAH) can be secondary to mutations in ARMC5 and less frequently in KDM1A, MEN1, APC or FH genes. We now report a patient with HPT-JT and germline CDC73 mutation who later developed PBMAH co-secreting cortisol and aldosterone without ARMC5 or MEN1 gene mutations. **Case Presentation:** a 54-y.o female was referred in 2013 for bilateral adrenal nodules with weight gain, fatigue, myopathy, hypertension with sleep apnea. Previous history included recurrent kidney stones since age 19, three parathyroidectomies (26, 46, 53 yo) for severe HPT, nephrocalcinosis, near end-stage renal disease and hysterectomy at age 35 for metrorrhagia. Abdominal CT without contrast showed PBMAH (multiple nodules up to 25 mm, 3 to 33 HU), very atrophic kidneys with extensive nephrocalcinosis and renal cysts. Hormonal dynamic testing confirmed suppressed ACTH with aberrant regulation of excess cortisol (443 nmol/L post-dex) and aldosterone production by vasopressin, upright posture and 5-HT₄R agonists. Initial genetic testing excluded MEN1 pathogenic variants. Left adrenalectomy was performed in 2015 and pathology confirmed PBMAH; recent CYP11B2 IHC showed heterogeneous high expression of aldosterone synthase in wide portions of outer cortex, but it was negative in several central nodules, while CYP11B1 IHC is pending. Late night salivary cortisol and ACTH levels normalized with mild dysregulated cortisol secretion and residual primary aldosteronism was treated with spironolactone. In 2017, breast ductal carcinoma in situ (DCIS) was resected and she underwent kidney transplantation followed by immunosuppression including prednisone. Recently, progressive increase in cortisol secretion and ACTH suppression may require additional therapy. No jaw tumor was found at recent imaging. **Genetic testing:** in 2024, expanded panel genetic testing for HPT and PBMAH identified a pathogenic CDC73 pathogenic variant (c.512del, p.Arg171Serfs*31) without mutations in ARMC5, MEN1, RET, CASR, CDKN1B, GCM2. Family testing is currently under way. **Conclusion:** This is the first case of a patient with HPT-JT with CDC73 gene mutation who developed cortisol- and aldosterone-secreting PBMAH. This finding

suggests the possibility that CDC73 mutations may also predispose to development of PBMAH.

Drug Index Terms

(Based on full text)

aldosterone, aldosterone synthase, corticotropin, cyclin dependent kinase inhibitor 1B, **epinephrine**, **hydrocortisone**, prednisone, spironolactone, vasopressin

Non-drug Index Terms

(Based on full text)

ACTH-independent macronodular adrenal hyperplasia, **adrenal gland**, adrenalectomy, adult, body weight gain, case report, clinical article, conference abstract, ductal breast carcinoma in situ, end stage renal disease, etiology, fatigue, female, gene, gene mutation, genetic screening, human, hydrocortisone release, hyperaldosteronism, **hyperparathyroidism-jaw tumor syndrome**, hypertension, hysterectomy, immunosuppressive treatment, jaw tumor, kidney calcification, kidney colic, kidney cyst, kidney polycystic disease, kidney transplantation, metrorrhagia, middle aged, multiple endocrine neoplasia type 1, myopathy, nephrolithiasis, parathyroidectomy, primary hyperaldosteronism, sleep apnea syndromes, surgery

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
ISSN	24721972
Source Type	Journal
Conference Name	ENDO 2025
Conference Location	United States, San Francisco, CA
Conference Date	2025-07-12 to 2025-07-15
Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A215-A216
Country of Author	Canada

Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.399
CAS Registry Numbers	aldosterone (52-39-1 , 6251-69-0) aldosterone synthase (122933-89-5) corticotropin (11136-52-0 , 9002-60-2 , 9061-27-2) epinephrine (51-43-4 , 55-31-2 , 6912-68-1) hydrocortisone (50-23-7) prednisone (53-03-2) spironolactone (52-01-7) vasopressin (11000-17-2)

75. [Adult Sellar Atypical Teratoid/Rhabdoid Tumor Mimicking Pituitary Adenoma: A Case Presentation](#)

Lin J., Kleinschmidt-DeMasters B., More C.T.

J.Endocr. Soc. 2025 9: (A764)

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Embase Open URL: [redirect to full text](#)

Abstract

Atypical teratoid/rhabdoid tumors (AT/RT) are rare aggressive embryonal central nervous system tumors due to mutations in SMARCB1 or SMARCA4 genes that are well known to occur in children. Far less commonly, they can occur in the sellar region, which is almost exclusively reported in adult women. Both have poor prognosis; timely post-operative chemoradiotherapy has been associated with longer overall survival, mandating expedited diagnosis. Here, we report an example of an adult sellar AT/RT thought preoperatively to be a nonfunctioning pituitary adenoma. A 50-year-old female with history of stage I granulosa cell carcinoma of the right ovary status post resection and etoposide/carboplatin chemotherapy in 2020 in remission who presented with new onset headache with right facial droop. Magnetic resonance imaging (MRI) showed a prominent pituitary gland. Headache

initially improved with course of high dose dexamethasone. A month later, she noted right eye ptosis and was found to have complete right third, fourth, and partial sixth nerve palsy. Repeat MRI pituitary showed diffuse enlargement and heterogeneous enhancement of the pituitary gland with probable invasion of the right cavernous sinus. Biochemical evaluation consistent with secondary hypothyroidism, inappropriately suppressed gonadotropins and hyperprolactinemia likely secondary to stalk effect. Given concern for secondary adrenal insufficiency, she was started on prednisone 5mg daily after completion of dexamethasone course. She underwent endoscopic transsphenoidal resection of the pituitary mass. Pathology returned positive for AT/RT. Post-operatively, patient continued to have right eye ptosis and headaches. She was started on levothyroxine due to persistent secondary hypothyroidism. One-week postoperative MRI with residual disease. She was referred to medical and radiation oncology urgently with plan to start radiation therapy with concurrent cisplatin. To date, about 41 cases of adult sellar AT/RT have been reported. Differentiating between malignant pituitary tumor from benign adenoma is difficult without histological evaluation. Pre-operative hormonal levels are nonspecific. Radiographic features often mimic pituitary macroadenomas. Due to aggressive nature of AT/RT tumors, clinical signs may serve as important clues; current literature reviews have shown that these tumors often have rapid progression of symptoms mostly in less than 2 months. Prognosis is poor with an estimated median overall survival of 23 months. However, longer overall survival has been observed in patients receiving adjuvant chemotherapy post-surgical resection. In conclusion, AT/RT should be included in the differential diagnosis of pituitary tumors especially for those with rapid growth rate and aggressive symptoms to achieve early diagnosis and prompt therapy for optimized survival rate.

Drug Index Terms

(Based on full text)

carboplatin, cisplatin, dexamethasone, etoposide, gonadotropin, levothyroxine, prednisone

Non-drug Index Terms

(Based on full text)

abducens nerve disease, abducens nerve paralysis, adrenal insufficiency, adult, atypical teratoid rhabdoid tumor, case report, cavernous sinus, child, clinical article, conference abstract, diagnosis,

differential diagnosis, drug therapy, facial droop, female, granulosa cell carcinoma, headache, histology, human, human tissue, hyperprolactinemia, **hypophysis**, **hypophysis adenoma**, hypophysis tumor, minimal residual disease, MRI scanner, nonfunctioning pituitary adenoma, nuclear magnetic resonance imaging, overall survival, pituitary macroadenoma, ptosis (eyelid), radiotherapy, remission, secondary hypothyroidism, surgery, survival rate

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
ISSN	24721972
Source Type	Journal
Conference Name	ENDO 2025
Conference Location	United States, San Francisco, CA
Conference Date	2025-07-12 to 2025-07-15
Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A764
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1449
CAS Registry Numbers	carboplatin (41575-94-4) cisplatin (15663-27-1 , 26035 31-4 , 96 081-74-2) dexamethasone (50-02-2) etoposide (33419-42-0 , 433304-61-1) gonadotropin (63231-54-9) levothyroxine (51-48-9) prednisone (53-03-2)

76. [A case of supplement induced Cushing's syndrome resulting in secondary adrenal insufficiency](#)

Kagita N.V.D., Umarji G., Stezzi M., Desai K., Watari J.

J.Endocr. Soc. 2025 9: (A242)

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Abstract

Background: Herbal therapies are readily accessible and frequently considered innocuous. Nevertheless, they may harbor dangerous substances that could result in unanticipated and possibly fatal outcomes. Case Presentation: A 74-year-old female with a history of controlled type 2 diabetes mellitus, Graves' disease, hyperlipidemia and asthma presented for endocrine evaluation for a low morning cortisol level of 1.8 µg/dL and an ACTH level of <1.5 pg/mL. Her electrolytes were within normal ranges, and her vital signs were stable. She weighed 75 kg with a BMI of 30. Physical examination revealed moon facies, facial plethora, a dorsocervical fat pad, and atrophic skin. A cosyntropin stimulation test was performed, showing a cortisol level of 5.1 µg/dL 60 minutes after cosyntropin, confirming adrenal insufficiency. She was initiated on glucocorticoid replacement therapy with prednisone 5 mg daily. The patient denied any recent exogenous glucocorticoid use. Remainder of pituitary labs were normal. MRI pituitary revealed no pituitary adenoma. On further investigation, the patient reported taking Montalin, an herbal supplement used for arthritis. She reported taking this supplement for the last year. On further review, it was discovered that Montalin contains corticosteroids and is associated with central adrenal insufficiency. She was instructed to stop Montalin. She was continued on glucocorticoid replacement therapy with close monitoring of her hypothalamic-pituitary-adrenal (HPA) axis to assess for recovery. Discussion: Exogenous steroid use is the most common etiology of secondary adrenal insufficiency, affecting approximately 2.5% of individuals using steroids for inflammatory or immune-mediated disorders. Patients with a history of glucocorticoid administration exhibiting signs and symptoms of exogenous Cushing's syndrome are presumed to have glucocorticoid-induced adrenal

insufficiency. Montalin, a herbal remedy used for analgesia, was previously found to contain dexamethasone and has been reported to be associated with central adrenal insufficiency. The restoration of the HPA axis is dependent upon the dosage and duration of glucocorticoid exposure. Montalin contains dexamethasone, a potent steroid with an extended duration of action, resulting in HPA axis recovery potentially taking up to one year. Conclusion: Physicians must maintain vigilance and investigate concealed sources of glucocorticoids in patients presenting with secondary adrenal insufficiency.

Drug Index Terms

(Based on full text)

corticosteroid, corticotropin, dexamethasone, electrolyte, **epinephrine**, glucocorticoid, herbal supplement, hydrocortisone, prednisone, tetracosactide

Non-drug Index Terms

(Based on full text)

adrenal gland, **adrenal insufficiency**, adult, analgesia, analgesic activity, asthma, case report, clinical article, conference abstract, **Cushing syndrome**, drug therapy, fat pad, female, Graves disease, human, hyperlipidemia, hypophysis adenoma, hypothalamus, hypothalamus hypophysis adrenal system, moon face, MRI scanner, non insulin dependent diabetes mellitus, nuclear magnetic resonance imaging, provocation test, side effect, substitution therapy, vital sign

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
ISSN	24721972
Source Type	Journal
Conference Name	ENDO 2025
Conference Location	United States, San Francisco, CA
Conference Date	2025-07-12 to 2025-07-15
Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract

Page Range	A242
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.447
CAS Registry Numbers	corticotropin (11136-52-0 , 9002-60-2 , 9061-27-2) dexamethasone (50-02-2) epinephrine (51-43-4 , 55-31-2 , 6912-68-1) hydrocortisone (50-23-7) prednisone (53-03-2) tetracosactide (16960-16-0)

77. [Recovery of HPA Axis With Tapering of Daily Methadone Dose: A Case Report](#)

Delshad J., Hossain S., Marowa S.A., Rosenthal D.S., Kumar S.C., Liao H.

J.Endocr. Soc. 2025 9: (A714)

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Abstract

Introduction: While Opioid-induced adrenal insufficiency (OIAI) is gaining recognition, it is still underdiagnosed, and much remains to be learned about recovery. Time to recovery and the allowable opioid dose in particular remain unclear. We present a case of a 37-year-old female on chronic high-dose methadone diagnosed with OIAI, who regained adrenal function by tapering the daily dose of methadone. Case: A 37-year-old female with a past medical history of polysubstance abuse was admitted to the hospital for dizziness on standing and hypotension. After careful history to exclude other causes, she was diagnosed with OIAI. She reported having been on chronic methadone for over ten years with a maximum dose of 150 mg daily. At the time of diagnosis, she was taking

80 mg methadone daily, and presented with exacerbated hypotension not responding to fluids. Serum cortisol measured at 10:30 AM was 1.89 ug/dL (5.27-22.45 ug/dL) and plasma ACTH was below 5 pg/ml (6-50 pg/ml). She was started on stress dose hydrocortisone which resolved her symptoms and stabilized her blood pressure. Her dose was tapered and ten days later, she was on a physiologic dose of 15 mg at 8 AM and 10 mg at 2 PM. Returning to the clinic three weeks after starting hydrocortisone, she reported that her methadone dose had been rapidly tapered to 60 mg daily. Her morning dose of steroids was held and a standard dose cosyntropin stimulation test was performed at this time with cortisol values <1.00, 2.93, and 2.13 ug/ml respectively at times 0, 30, and 60 minutes after administration of cosyntropin. Additionally, ACTH was <5 pg/ml, and DHEA sulfate was 10 mcg/dL (19-237) and she was switched to prednisone 5 mg daily for ease of administration. Three months after switching to prednisone, she returned to the clinic again reporting that her methadone had been further tapered to 40 mg daily. The morning dose of steroids was again held for a repeat cosyntropin stimulation test which revealed cortisol values of 5.29, 18.17, and 22.75 ug/ml respectively at times 0, 30, and 60 minutes. ACTH was 5 pg/ml and DHEA sulfate 86 mcg/dL and while lab values had improved, they were not sufficient to stop steroids. At her most recent clinic visit, nine months after the original diagnosis and still on 40 mg methadone daily, morning labs were drawn. Withholding prednisone, her HPA axis was considered to be recovered based on cortisol was 17.05 ug/ml and ACTH was 19 pg/ml. Blood pressure has remained stable and the patient was instructed only to take steroids during acute illness. Discussion: While OIAI is gaining recognition, we are still learning about the recovery of adrenal function in this population. Our case demonstrates recovery in a patient while still on 40 mg of methadone daily. Additionally, as DHEA sulfate recovered to normal limits before the cortisol levels, further studies should be performed regarding its prognostic value.

Drug Index Terms

(Based on full text)

biological marker, hydrocortisone, **methadone**, opiate, prasterone sulfate, prednisone, steroid, tetracosactide

Non-drug Index Terms

(Based on full text)

adrenal function, adrenal insufficiency, adult, blood pressure, case

report, clinical article, conference abstract, corticotropin blood level, diagnosis, drug dose, drug dose increase, drug therapy, female, human, hydrocortisone blood level, hypotension, **hypothalamus hypophysis adrenal system**, learning, maximum permissible dose, medical history, morning dosage, orthostatic dizziness, provocation test

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	J.Endocr. Soc.
ISSN	24721972
Source Type	Journal
Conference Name	ENDO 2025
Conference Location	United States, San Francisco, CA
Conference Date	2025-07-12 to 2025-07-15
Source Publication Date	2025-10-01
Entry Date	2025-11-19 (Full record)
Publication Type	Conference Abstract
Page Range	A714
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1210/jendso/bvaf149.1352
CAS Registry Numbers	hydrocortisone (50-23-7) methadone (1095-90-5 , 125-56-4 , 23142-53-2 , 297-88-1 , 76-99-3) opiate (53663-61-9 , 8002-76-4 , 8008-60-4) prasterone sulfate (651-48-9) prednisone (53-03-2) tetracosactide (16960-16-0)

78. [Pancytopenia in the context of hypothyroidism and B12 deficiency: A case report and review literature](#)

Karimpour F., Valizadeh M., Ranjbar H., Vosough Z.

Caspian J. Int. Med. 2025 16:4 (801-806)

Embase, NURSING

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Abstract

Background: This case of pancytopenia due to hypothyroidism and B12 deficiency is rare and severe as a reminder that, upon diagnosis of pancytopenia, endocrine disorders such as secondary thyroid hormone deficiency may need to be considered as a potential cause. **Case Presentation:** A 38-year-old male presented with progressive weakness, lethargy, and anorexia. Initial labs revealed severe pancytopenia, prompting a hematological workup. Bone marrow aspiration ruled out acute leukemia. Further investigations identified severe primary hypothyroidism (TSH: 200 mIU/L) and vitamin B12 deficiency as the underlying causes, after excluding other common etiologies. Treatment with levothyroxine and vitamin B12 was initiated. The patient showed a rapid and significant hematological response, with all cell lines improving and approaching normal ranges on follow-up, confirming the diagnosis. **Conclusion:** In this patient, after the diagnosis of hypothyroidism and receiving levothyroxine, we saw a better stability of the improvement of the patient's symptoms and a further improvement in the number of any blood cell lines chiefly platelet which is one of the regulators of the coagulation pathway, and its deficiency increase the risk of bleeding.

Drug Index Terms

(Based on full text)

copper (endogenous compound), cyanocobalamin (drug therapy), decay accelerating factor (endogenous compound), folic acid (endogenous compound), haptoglobin (endogenous compound), hemoglobin (endogenous compound), hormone (endogenous compound), lactate dehydrogenase (endogenous compound), levothyroxine (drug combination, drug therapy, oral drug administration, special situation for pharmacovigilance), meropenem (drug therapy, intravenous drug

administration), prednisone (drug combination), thyrotropin (endogenous compound)

Non-drug Index Terms

(Based on full text)

acute leukemia, acute lymphoblastic leukemia, acute myeloid leukemia, adolescent, adult, aged, anorexia, article, **B12 deficiency** (drug therapy), bed, biopsy, blood culture, bone marrow aspiration, case report, cell line, cerebral sinus thrombosis, clinical article, computer assisted tomography, copper deficiency, disease severity, drug response, endocarditis, endoscopy, epigastric pain, faintness, female, fever, folic acid deficiency, follow up, hematological procedure, hematuria, hemolytic anemia, hospitalization, human, human tissue, hyperplasia, **hypothyroidism** (drug therapy), incontinence, lethargy, **literature**, lymphoproliferative disease, male, medical history, microscope, myelodysplastic syndrome, **pancytopenia** (diagnosis), paroxysmal nocturnal hemoglobinuria, peripheral venous catheter, pernicious anemia, seizure, septicemia (drug therapy), Staphylococcus aureus, stomach tissue, thrombophlebitis (drug therapy), weakness

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Caspian J. Int. Med.
ISSN	20086172 (electronic), 20086164
Source Type	Journal
Source Publication Date	2025-09-01
Entry Date	2025-11-17 (Full record), 2025-11-11 (Article in Press/In process)
Publication Type	Article
Page Range	801-806
Country of Author	Iran
Country of Source	Iran
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.22088/cjim.16.4.801

Number of References	10
CAS Registry Numbers	copper (15158-11-9 , 7440-50-8) cyanocobalamin (53570-76-6 , 68-19-9 , 8064-09-3) decay accelerating factor (99085-47-9) folic acid (59-30-3 , 6484-89-5) haptoglobin (9087-69-8) hemoglobin (9008-02-0) lactate dehydrogenase (9001-60-9) lactate dehydrogenase A levothyroxine (51-48-9) meropenem (96036-03-2) prednisone (53-03-2) thyrotropin (9002-71-5)

79. [Response of steroid predicts the long-Term clinical and endoscopic prognosis in Cronkhite-Canada syndrome](#)

Xu Q., Ou C., Zhi Y., Zhang R., Liu S., Xu T., Tang H., Ruan G., Yan X., Zhang S., Li J., Wu D., Li J.

QJM 2025 118:8 (567-574)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Background Cronkhite-Canada syndrome (CCS) is a rare gastrointestinal polyposis syndrome characterized by hamartoma polyps and ectodermal abnormalities, with an increased risk of gastrointestinal cancers. Our previous study revealed its relapsing disease behavior and the improved long-Term prognosis following the medications of steroids. However, the response of steroids varies among CCS patients, and there was no consensus about how to deal with steroid-dependent patients. Aim This study aims to update the knowledge of long-Term prognosis of CCS patients and explore the efficacy and safety of low-dose steroid as maintenance therapy. Design Observational cohort study comprising CCS patients hospitalized in Peking Union Medical College Hospital from 1999 until December 2023. Methods All CCS patients were followed up

through telephone interviews and outpatient visits, with clinical data and endoscopic findings sequentially collected. The Cox regression model was used to identify factors associated with clinical outcomes. Results A total of 64 eligible participants were included with a median (interquartile range) follow-up duration of 49.5 (22.75, 101.25) months. The cumulative probability of overall survival (OS) was 90.2% at 3 years, 87.7% at 5 years, 78.6% at 10 years and relapse-free survival (RFS) after steroid treatment was 76.3% at 1 year, 62.9% at 3 years, 54.4% at 5 years, respectively. The poor response to steroids was identified as independent variable related to worse OS in multivariate model. In the subgroup analysis of relapsed patients, low-dose steroid treatment (prednisone ≤ 10 mg/day) for at least one year as maintenance therapy was in association with reduced risk of re-relapse (RFS: 72 (46.3, 78) vs. 12 (9, 15.5) months; $P = 0.02$).

Conclusions The long-Term clinical outcomes of CCS are relatively favorable in patients who responded well to steroid treatment. The low-dose steroid maintenance therapy may prevent re-relapse of disease among CCS patients.

Drug Index Terms

(Based on full text)

albumin (endogenous compound), antibiotic agent (drug therapy), azathioprine (drug therapy), cyclophosphamide (drug therapy), cyclosporine (drug therapy), hemoglobin (endogenous compound), mesalazine (drug therapy), methotrexate (drug therapy), **prednisone** (adverse drug reaction, drug therapy, unexpected outcome of drug treatment), thalidomide (drug therapy)

Non-drug Index Terms

(Based on full text)

abnormal laboratory result, adult, alopecia, article, China, clinical feature, **clinical outcome**, clinical study, cohort analysis, controlled study, **Cronkhite Canada syndrome** (drug therapy), diabetes mellitus (side effect), drug dose reduction, drug efficacy, drug safety, **endoscopy**, female, follow up, fracture (side effect), hospital, hospital patient, human, hyperpigmentation, infection (side effect), laboratory test, liver dysfunction (side effect), low drug dose, maintenance drug dose, maintenance therapy, major clinical study, male, medical school, observational study, osteoporosis (side effect), outpatient, overall survival, patient selection, prediction, **prognosis**, proportional hazards

model, recurrence free survival, relapse, remission, risk reduction, steroid therapy, telephone interview, **treatment response**

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	QJM
ISSN	14602393 (electronic), 14602725
CODEN	QMJPFF
Source Type	Journal
Source Publication Date	2025-08-01
Entry Date	2025-11-20 (Full record), 2025-10-28 (Article in Press/In process)
Publication Type	Article
Page Range	567-574
Country of Author	China, China
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1093/qjmed/hcaf088
MEDLINE PMID	40193473
Number of References	22
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) cyclophosphamide (50-18-0 , 6055-19-2) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) hemoglobin (9008-02-0) mesalazine (89-57-6) methotrexate (15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) prednisone (53-03-2) thalidomide (50-35-1)

80. [A Data-Centric Perspective on Nosocomial Infections Following Surgical Procedures](#)

Staicovici S., Sârbu V., Gheorghe E., Vlase I., Ispas S., Herlo L.F., Nelson-Twakor A., Deacu S.

ARS Medica Tomitana 2025 31:3 (101-111)

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Embase Open URL: [redirect to full text](#)

Abstract

Background: Postoperative nosocomial infections (NIs) increase morbidity, hospitalisation, and mortality. The emergence of multidrug-resistant organisms (MDROs) complicate care and require strict infection control and antibiotic therapy. This study examined postoperative NIs' clinical, microbiological, treatment profiles and identified risk factors. **Methods:** This retrospective observational study included 56 adult surgical patients from two tertiary centers in Constanta, Romania. We analysed demographics, comorbidities, operation type, microbiology, inflammatory indicators, ICU admission, mortality, and antibiotic resistance/sensitivity. **Results:** Patients had a mean age of 53.21 ± 22.61 years, with a slightly male predominance. The ICU had 26.4% admission and 9.4% death. The most common comorbidities were dementia, obesity, diabetes, and atrial fibrillation. *Pseudomonas aeruginosa*, *Acinetobacter baumannii*, and *Staphylococcus epidermidis* dominated the pathogens spectrum. In particular, 40% of cultures were resistant to tetracycline, ceftriaxone, and erythromycin, while vancomycin and linezolid were effective. *Klebsiella pneumoniae*, *Serratia marcescens*, and *Bacteroides fragilis* polymicrobial infections increased ICU utilisation and hospitalisation. **Conclusion:** Host, procedural, and microbiological variables affect postoperative NIs. Antimicrobial medicines must be tailored to multidrug resistance patterns, and surveillance and stewardship programs are essential. Early detection of high-risk infections may enhance results with focused therapy.

Drug Index Terms

(Based on full text)

amikacin (drug therapy), antibiotic agent (drug therapy), C reactive protein (endogenous compound), ceftriaxone, ciprofloxacin (drug therapy), creatinine (endogenous compound), dexamethasone (drug

therapy), erythromycin, fibrinogen (endogenous compound), gentamicin (drug therapy), hydrocortisone (drug therapy), imipenem (drug therapy), linezolid, meropenem (drug therapy), methylprednisolone (drug therapy), piperacillin plus tazobactam (drug therapy), prednisone (drug therapy), tetracycline, urea (endogenous compound), vancomycin

Non-drug Index Terms

(Based on full text)

Acinetobacter baumannii, adult, antibiotic resistance, antibiotic sensitivity, antibiotic therapy, article, atrial fibrillation, Bacteroides fragilis, Bacteroides infection (drug therapy), cholecystectomy, clinical feature, clinical outcome, coinfection (drug therapy), comorbidity, corticosteroid therapy, death, debridement, dementia, demographics, diabetes mellitus, female, fracture fixation, hartmann procedure, health care utilization, hospital admission, **hospital infection** (complication, drug therapy), hospitalization, human, human tissue, hysterectomy, incidence, infection risk, intensive care unit, Klebsiella pneumoniae, leg amputation, lobectomy, major clinical study, male, microbiology, nonhuman, obesity, observational study, postoperative complication, **postoperative infection** (complication, drug therapy), Pseudomonas aeruginosa, quick Sequential Organ Failure Assessment Score, retrospective study, risk factor, Romania, salpingoophorectomy, Serratia infection (drug therapy), Staphylococcus epidermidis, **surgery**, tertiary care center

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	ARS Medica Tomitana
ISSN	18414036 (electronic), 12239666
Source Type	Journal
Source Publication Date	2025-08-01
Entry Date	2025-11-18 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	101-111
Country of Author	Romania
Country of Source	Poland

Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.2478/arsm-2025-0014
Number of References	23
CAS Registry Numbers	amikacin (37517-28-5 , 39831-55-5 , 110660-83-8 , 1257517-67-1) C reactive protein (9007-41-4) ceftriaxone (73384-59-5 , 74578-69-1 , 104376-79-6) ciprofloxacin (85721-33-1 , 86393-32-0 , 128074-72-6 , 128074-76-0 , 192934-52-4 , 93107-08-5 , 86483-48-9 , 96186-80-0) creatinine (19230-81-0 , 60-27-5) dexamethasone (50-02-2) erythromycin (114-07-8 , 70536-18-4) fibrinogen (9001-32-5) gentamicin (1392-48-9 , 1403-66-3 , 1405-41-0) hydrocortisone (50-23-7) imipenem (64221-86-9) linezolid (165800-03-3) meropenem (96036-03-2) methylprednisolone (6923-42-8 , 83-43-2) piperacillin plus tazobactam (157044-21-8) prednisone (53-03-2) tetracycline (23843-90-5 , 60-54-8 , 64-75-5 , 8021-86-1) urea (57-13-6) vancomycin (1404-90-6 , 1404-93-9)

81. [Sjögren's Syndrome and Ankylosing Spondylitis Association: A Case-Based Review](#)

Slouma M., Mehmli T., Hannech E., Dhahri R., Mejri I., Affes M., Gharsallah I.

Curr. Rheumatol. Rev. 2025 21:5 (535-544)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Introduction: Unlike restrictive pulmonary function and apical fibrobullous disease, diffuse interstitial lung disease is scarce in patients with ankylosing spondylitis (AS). We present a systematic review of the association between AS and SS. We also report a new case of SS revealed by interstitial lung disease in AS patients treated with tumor necrosis factor (TNF) inhibitors. **Materials and Methods:** The systematic review was performed according to the Preferred Reporting Items for Systematic Reviews and Meta-Analyses guideline using the MEDLINE and SCOPUS databases and included case reports and case series describing the association between AS and SS. **Results:** There were sixty-three patients, including our case: 16 males and 47 females. The mean age was 49.2 years. The mean SpA duration was 14.1 years. The mean delay between SpA and SS was 12.8 years (0-27). SS was diagnosed after SpA in 62% of cases (n=39). It preceded SpA in 36.5% (n=23) and was concomitant with SpA in 1 case. All patients had sicca symptoms. Minor salivary gland biopsy, showed focal sialadenitis grade III or grade IV in the Chisholm classification in 20 patients. Anti-nuclear antibody was positive in 75.8% of cases. Among them, anti-SSA and anti-SSB were positive in 44.4% and 35.3% of cases. Except for our patient, no patient had interstitial lung disease. SS extra glandular manifestations were reported in 12 cases. **Conclusion:** The occurrence of Sjögren's syndrome is uncommon in patients with ankylosing spondylitis. This association has been reported in the literature, suggesting a pathogenetic link between these two diseases. This association should be considered in ankylosing spondylitis patients with diffuse interstitial lung disease. Knowing this association is necessary for therapeutic adjustment. Our study has some limitations. Publication bias was the major bias in our study. Indeed, we only included case reports and case series describing the association between SpA and SS. We did not search for unpublished work. Moreover, the follow-up was not specified in most included articles.

Drug Index Terms

(Based on full text)

antinuclear antibody (endogenous compound), biological product (drug therapy), cyclic citrullinated peptide antibody (endogenous compound),

HLA B27 antigen (endogenous compound), infliximab (drug therapy), La antibody (endogenous compound), methotrexate (drug therapy, unexpected outcome of drug treatment), nonsteroid antiinflammatory agent (drug therapy), prednisone (drug therapy, unexpected outcome of drug treatment), rheumatoid factor (endogenous compound), Ro antibody (endogenous compound), tumor necrosis factor (endogenous compound), tumor necrosis factor inhibitor

Non-drug Index Terms

(Based on full text)

adult, **ankylosing spondylitis** (diagnosis, drug resistance, drug therapy), Ankylosing Spondylitis Disease Activity Score, ankylosis, Bath ankylosing spondylitis disease activity index, body temperature, breathing rate, case report, clinical article, computer assisted tomography, crackle, delayed diagnosis, **disease association**, disease duration, drug withdrawal, dyspnea, female, follow up, genetic predisposition, human, interstitial lung disease, low back pain, lung fibrosis, lung infiltrate, middle aged, organizing pneumonia, oxygen saturation, pain, plethysmography, polyarthritis, pygalgia, review, sacroiliac joint, salivary gland biopsy, sialoadenitis, **Sjogren syndrome**, spirometry, spondylarthritis, systematic review, thorax radiography, total hip replacement

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Curr. Rheumatol. Rev.
ISSN	18756360 (electronic), 15733971
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Full record), 2025-04-14 (Article in Press/In process)
Publication Type	Review
Page Range	535-544
Country of Author	Tunisia
Country of Source	United Arab Emirates
Language of Article	English
Language of Summary	English

Digital object identifier (DOI) [10.2174/0115733971315532240920065136](https://doi.org/10.2174/0115733971315532240920065136)

MEDLINE PMID [39506443](https://pubmed.ncbi.nlm.nih.gov/39506443/)

Number of References 60

CAS Registry Numbers [infliximab \(170277-31-3\)](#)
[methotrexate \(15475-56-6, 59-05-2, 7413-34-5, 7532-09-4, 6745-93-3, 51865-79-3, 60388-53-6\)](#)
[prednisone \(53-03-2\)](#)
[rheumatoid factor \(9009-79-4\)](#)

82. [Clofarabine monotherapy in refractory multisystem LCH with gastrointestinal involvement](#)

Aronson S., Tang V., Cios K., Pittman M., Çeliker M.Y.

Pediatr. Hematol. Oncol. 2025 42:8 (389-394)

Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Langerhans Cell Histiocytosis (LCH) is a rare histiocytic disorder that involves various organ systems. Recognizing gastrointestinal (GI) tract involvement in LCH is crucial, as GI cases, even without high-risk organ involvement, may be refractory to initial as well as second line therapy. Here, we report two cases of LCH with GI involvement, that were refractory to vinblastine, prednisone and cytarabine, and ultimately required clofarabine to reach clinical remission. We propose that clofarabine should be further studied as a first choice for second line treatment for GI-LCH cases and that GI involvement should be considered as high risk in the LCH risk stratification algorithm.

Drug Index Terms

(Based on full text)

circulating tumor DNA (endogenous compound), **clofarabine** (adverse drug reaction, drug therapy, special situation for pharmacovigilance), **cytarabine** (adverse drug reaction, special situation for pharmacovigilance), **prednisone** (drug combination, oral drug

administration, special situation for pharmacovigilance), protein S 100 (endogenous compound), t6 antigen (endogenous compound), tumor marker (endogenous compound), **vinblastine** (drug combination, special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

abdomen, acute respiratory failure, article, bilious vomiting, biopsy, blood toxicity (side effect), bloody diarrhea, bone marrow, BRAF test kit, cancer prognostic test kit, case report, child, clinical article, colon, colon biopsy, digestive system perforation, DNA determination, endoscopic biopsy, erythema, face, female, fever (side effect), gastrointestinal tract, health, hematochezia, hepatosplenomegaly, hospitalization, human, human tissue, hypertransaminasemia, hypoalbuminemia, immunohistochemistry, infant, **Langerhans cell histiocytosis** (drug therapy), limb, liver, lung biopsy, male, **monotherapy**, multiple cycle treatment, nausea and vomiting (side effect), neutropenia (side effect), pancytopenia, positron emission tomography-computed tomography, proctitis, rash, rectum hemorrhage, recurrent disease, remission, school child, second-line treatment, sigmoidoscopy, skin, skin defect, spleen, splenomegaly, stomach discomfort, stomach lesion, thigh, ulcer

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr. Hematol. Oncol.
ISSN	15210669 (electronic), 08880018
CODEN	PHONE
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Full record), 2025-09-30 (Article in Press/In process)
Publication Type	Article
Page Range	389-394
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English

Language of Summary	English
Digital object identifier(DOI)	10.1080/08880018.2025.2563535
MEDLINE PMID	40988414
Number of References	14
Device Tradenames	Guardant360 CDx (guardant health, United States)
Device Manufacturers	guardant health, guardant health (United States)
CAS Registry Numbers	clofarabine (123318-82-1) cytarabine (147-94-4 , 69-74-9) prednisone (53-03-2) vinblastine (865-21-4)
Clinical Trial Numbers	ClinicalTrials.gov (NCT02670707)

83. [Efficacy and safety of rituximab in rheumatoid arthritis-associated interstitial lung disease: a systematic review and meta-analysis](#)

Ines M., Zeineb Z., Selma B., Olfa S., Leila R., Rawdha T., Aicha B.T., Chadli D., Leila A.

Expert Opin. Biol. Ther. 2025 25:10 (1087-1099)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Objectives: To assess the effectiveness and safety of Rituximab (RTX) treatment among patients diagnosed with rheumatoid arthritis-associated interstitial lung disease (RA-ILD). **Methods:** We performed a systematic review with meta-analysis of studies involving patients with RA-ILD who were treated with RTX. Two investigators independently conducted the research. **Results:** Twenty studies met the criteria for data extraction for the systematic review with a good quality assessment according to an 18-criteria checklist using a modified Delphi method. The total number of patients was 14,523, including 1,619 who received RTX. Stabilization or improvement was observed in more than half of the

patients in all studies. Evaluation based on pulmonary function tests (PFTs) noted disease progression in fewer than 20% of 7 out of 11 studies. Meta-analysis results revealed that, based on PFTs, the proportion of patients showing functional decline was 14.5% (95%CI, 7.6-25.8%) (95%PI, 2-69%); and according to the pulmonary high-resolution computed tomography (HRCT) evaluation, the proportion of worsening was 19.5% (95%CI, 8.1-40%) (95%PI, 1-84%). An overall acceptable safety profile was noted, with respiratory mortality ranging from 4% to 14%. Conclusion: Our review suggests that RTX appears to be an effective therapy in patients with RA-ILD, with a satisfactory safety profile. Protocol registration: <https://www.crd.york.ac.uk/prospero> identifier is CRD420251049957.

Drug Index Terms

(Based on full text)

abatacept (drug comparison, drug therapy), azathioprine (adverse drug reaction, drug combination, drug comparison, drug therapy), disease modifying antirheumatic drug (drug combination, drug comparison, drug therapy), hydroxychloroquine (drug combination, drug comparison, drug therapy), Janus kinase inhibitor (drug comparison, drug therapy), leflunomide (adverse drug reaction, drug combination, drug comparison, drug therapy), methotrexate (drug combination, drug comparison, drug therapy), methylprednisolone (drug combination, drug comparison, drug therapy), mycophenolate mofetil (adverse drug reaction, drug combination, drug therapy), nonsteroid antiinflammatory agent (drug combination, drug comparison, drug therapy), prednisone (adverse drug reaction, drug combination, drug comparison, drug therapy), **rituximab** (adverse drug reaction, drug combination, drug comparison, drug therapy, intravenous drug administration), salazosulfapyridine (drug combination, drug comparison, drug therapy), tocilizumab (drug comparison, drug therapy), tumor necrosis factor inhibitor (drug comparison, drug therapy)

Non-drug Index Terms

(Based on full text)

adult, aged, case control study, clinical outcome, controlled study, cross-sectional study, Delphi study, diffusing capacity for carbon monoxide, **disease association**, disease exacerbation, **drug efficacy**, **drug safety**, drug withdrawal, female, follow up, functional disease, hospitalization, human, infection (side effect), **interstitial lung disease**

(diagnosis, drug therapy), lung function test, male, meta analysis, mortality, prevalence, publication bias, quality control, randomized controlled trial (topic), review, **rheumatoid arthritis** (diagnosis, drug therapy), survival systematic review system

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Expert Opin. Biol. Ther.
ISSN	17447682 (electronic), 14712598
CODEN	EOBTA
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025 11-21 (Full record), 2025-10-13 (Article in Press/In process)
Publication Type	Review
Page Range	1087-1099
Country of Author	Tunisia
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1080/14712598.2025.2567868
MEDLINE PMID	41004196
Number of References	35
CAS Registry Numbers	abatacept (332348-12-6) azathioprine (446-86-6 , 55774-33-9) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) leflunomide (75706-12-6) methotrexate (15475-56-6 , 59 05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6)

prednisone ([53-03-2](#))
rituximab ([174722-31-7](#))
salazosulfapyridine ([599-79-1](#))
tocilizumab ([375823-41-9](#))

84. [Clinical management and burden of cytomegalovirus in D+/R-Kidney transplant recipients in Canada](#)

Gill J., House A.A., Chagla Z., Tchervenkov J., Kim S.J., Vinson A.,
Cervera C., Keown P.A., Sun S.L.W., Khoury C., Ghakis C.

Front. Immunol. 2025 16:

Embase, MEDLINE, NURSING

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Purpose: To document prophylactic practices, infection patterns, and disease burden to inform strategies for CMV management in high-risk kidney transplant recipients. Methods: A retrospective cohort of 311 consecutive CMV D+/R- kidney recipients were enrolled from 7 Canadian programs over 4 years (2018-2021) to provide data on demographic, clinical, therapeutic and health resource use during the 1st year post-transplant. Results: The median age was 58 (46, 67) years, 69% were male, and 53% were White. Diabetes was the principal cause of kidney failure (19%). 208 (69%) received a deceased donor graft; 76 (24%) had ATG induction, and 84% had maintenance therapy with tacrolimus and MMF/MPA ± prednisone. All received antiviral prophylaxis, 90% with valganciclovir, for a median of 180 days. 106 (34%) developed CMV viremia (median peak viral load 14,224 IU/ml) at a median of 218 days, of whom 46 (43%) had CMV disease and 15 (14%) had recurrent infection. Myelotoxicity occurred in 121 (39%) patients at a median of 88 days, lasting a median of 30 days. Opportunistic infections occurred in 119 patients (38%) at a median of 53 days. 141 patients (45%) were hospitalized, 50 (16%) more than once. 20 patients (6%) had biopsy-confirmed rejection, and 293 (94%) were alive with a functioning graft at 1 year. Conclusion: Current prophylaxis strategies fail to prevent CMV infection in 34% of high-risk patients. Myelotoxicity, opportunistic infection, reduced immunosuppression, and hospitalization remain common and serious complications. More effective and less toxic

personalized treatment strategies are required to minimize these risks and burdens.

Drug Index Terms

(Based on full text)

prednisone, tacrolimus

Non-drug Index Terms

(Based on full text)

adult, antiviral therapy, article, **Canada**, cohort analysis, **Cytomegalovirus**, female, follow up, graft recipient, hospitalization, human, immunosuppressive treatment, kidney graft, **kidney transplantation**, major clinical study, male, mortality, nephrotoxicity, neutropenia, prophylaxis, retrospective study, risk factor, statistical analysis, viremia, virus load

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Front. Immunol.
ISSN	16643224 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Full record), 2025-10-10 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Canada
Country of Source	Switzerland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.3389/fimmu.2025.1618748
MEDLINE PMID	41050692
Article Number	1618748
Number of References	65

CAS Registry Numbers

prednisone ([53-03-2](#))

tacrolimus ([104987-11-3](#), [109581-93-3](#))

85. [When and why do children develop hyperglycemia during acute lymphoblastic leukemia therapy?](#)

E Silva L.C.O., Siviero-Miachon A.A., Sousa A.V.L., Spinola-Castro A.M.

Rev. Assoc. Med. Bras. 2025 71:9

Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

OBJECTIVE: To identify risk factors for hyperglycemia and evaluate the impact on outcomes among pediatric patients with acute lymphoblastic leukemia undergoing chemotherapy. **METHODS:** We conducted a retrospective cohort study involving 188 pediatric patients treated for acute lymphoblastic leukemia at a Brazilian cancer referral center between 2004 and 2017. Hyperglycemia was assessed during the induction and consolidation phases of chemotherapy. Associations with patient and disease characteristics were examined through multivariate analysis. Outcomes analyzed included severe infections, relapse, and mortality. **RESULTS:** Hyperglycemia occurred in 43.6% of patients, predominantly during the induction phase. Pubertal status at diagnosis, younger age, and high relapse risk classification were independently associated with increased hyperglycemia risk. Puberty conferred a nearly 8-fold increased risk. However, hyperglycemia was not associated with increased risk of infections, relapse, or mortality. Higher mortality was observed among patients with overweight/obesity and those with pancreatitis or recurrent infections. **CONCLUSION:** Hyperglycemia was a frequent early adverse event in pediatric acute lymphoblastic leukemia treatment, particularly among pubertal patients and those at high relapse risk. Despite its high incidence, hyperglycemia was not associated with worse clinical outcomes in this cohort. These findings underscore the importance of early identification and monitoring, particularly during high-risk treatment phases.

Drug Index Terms

(Based on full text)

dexamethasone, glucocorticoid (endogenous compound), prednisone
(special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

article, body mass, child, clinical outcome, cohort analysis, data analysis software, diabetes mellitus, disease exacerbation, electronic medical record, Escherichia coli, female, glucose blood level, human, **hyperglycemia**, immunophenotyping, leukocyte count, major clinical study, male, mortality, neutrophil count, obesity, overall survival, pediatric patient, Philadelphia 1 chromosome, predictive model, recurrent infection, retrospective study, risk factor, SPSS 20.0, STATA 12 statistical software, treatment response, World Health Organization

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Rev. Assoc. Med. Bras.
ISSN	18069282 (electronic), 01044230
CODEN	RMDBA
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Brazil
Country of Source	Brazil
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1590/1806-9282.20240953
MEDLINE PMID	41124539
Article Number	e20240953
Number of References	24

Device Tradenames	SPSS 20.0, STATA 12 statistical software
CAS Registry Numbers	dexamethasone (50-02-2) prednisone (53-03-2)

86. [Methimazole-Induced Antithyroid Arthritis Syndrome in Graves' Disease Management: A Case Report](#)

Sun K.-W., Wu A.

Am. J. Case Rep. 2025 26:

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Patient: Male, 29-year-old Final Diagnosis: Antithyroid arthritis syndrome

• Graves' disease Symptoms: Systemic migratory pain Clinical

Procedure: — Specialty: Endocrinology and Metabolic Objective:

Background: Case Report: Conclusions: Rare disease Antithyroid arthritis syndrome (AAS) is a rare adverse reaction to antithyroid drugs (ATDs), characterized by fever, rash, myalgia, and migratory polyarthralgia. It mimics rheumatic disorders, complicating diagnosis.

We present a case with unique symptoms presentation of a 29-year-old Chinese man with Graves' disease who presented with fever, rash and myalgia following methimazole initiation, then developed systemic migratory pain in muscle and joints after stopping medication. Initial suspicion of ANCA-associated vasculitis (AAV) or drug-induced lupus arose due to elevated inflammatory markers and negative ANCA/autoimmune serology (except ANA+). Pharmacist-led assessment using the Naranjo scale and the inquiry during daily care confirmed the temporal association with methimazole. After excluding AAV and lupus via clinical and laboratory criteria, AAS was diagnosed. Corticosteroids and lithium carbonate was prescribed for the pain and Graves' disease control at admission, then necessitating dosage adjustment for overall safety. Propylthiouracil was prescribed during a subsequent visit, but he then developed drug-induced liver injury manifested as jaundice, which finally required management with radioiodine therapy, which led to control of symptoms. This case highlights the diagnostic complexity of AAS, particularly in male patients, and underscores the therapeutic role

of lithium carbonate as a transitional agent for thionamide-intolerant Graves' disease. Given the high cross-sensitivity between antithyroid drugs limiting pharmacological alternatives, propylthiouracil also carries a significant risk of adverse reactions. Pharmacist involvement in adverse drug reaction assessment and lithium dosing optimization was critical for successful management. Heightened clinical vigilance and experience are essential when encountering suspected AAS.

Drug Index Terms

(Based on full text)

corticosteroid, desonide (topical drug administration), iodine 131 (oral drug administration), levothyroxine, lithium carbonate (oral drug administration), methylprednisolone (intravenous drug administration), nonsteroid antiinflammatory agent, prednisone (oral drug administration), propranolol, propylthiouracil (adverse drug reaction, oral drug administration), radioactive iodine, technetium 99m, **thiamazole** (adverse drug reaction), tramadol

Non-drug Index Terms

(Based on full text)

adult, **antithyroid arthritis syndrome** (diagnosis, side effect), arthralgia, **arthritis** (diagnosis, side effect), article, case report, clinical article, drug substitution, drug withdrawal, drug-induced liver injury, fever, **Graves disease** (diagnosis), human, hypertension, hypothyroidism, jaundice (side effect), joint tightness, limb tightness, liver dysfunction (side effect), male, muscle tightness, musculoskeletal disease, myalgia, numeric rating scale, pain, pharmacist, pruritus (side effect), rash (side effect), single photon emission computed tomography-computed tomography, swelling (side effect), systemic migratory pain, thyrotoxicosis, urticaria

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	Am. J. Case Rep.
ISSN	19415923 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-19 (Full record), 2025-11-17 (Article in Press/In process)

Publication Type	Article
Page Range	
Country of Author	China
Country of Source	Poland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.12659/AJCR.949509
MEDLINE PMID	41118350
Article Number	e949509
Number of References	27
CAS Registry Numbers	desonide (638-94-8) iodine 131 (10043-66-0 , 15124-39-7) levothyroxine (51-48-9) lithium carbonate (554-13-2) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) propranolol (13013-17-7 , 318-98-9 , 3506-09-0 , 4199-09-1 , 525-66-6) propylthiouracil (51-52-5) technetium 99m (14133-76-7) thiamazole (60-56-0) tramadol (27203-92-5 , 36282-47-0)

87. [Paraneoplastic Dermatomyositis in Patient with Primary Biliary Cirrhosis and Hepatocellular Carcinoma: A Case Report](#)

Neycheva S., Stoyanova D.

Am. J. Case Rep. 2025 26:

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Objective: Rare coexistence of disease or pathology **Background:** Dermatomyositis is an autoimmune disease characterized by distinctive skin changes, muscle involvement, and, in many cases, damage to various organs, such as interstitial lung disease, arthritis, and myocarditis. The association between dermatomyositis and malignancies is well established. The most common cancers that manifest with a dermatomyositis are neoplasms of the breast, lungs, cervix, and gastrointestinal tract. Primary biliary cirrhosis is a slowly progressive autoimmune disease characterized by a triad of chronic cholestasis, positive specific autoantibodies, and characteristic imaging or pathomorphological findings. Regardless of etiology, cirrhosis is considered as a major risk factor for development of hepatocellular carcinoma. This type of liver neoplasm is rarely associated with dermatomyositis. **Case Report:** We report the case of a 64-year-old woman with comorbid primary biliary cirrhosis, hepatocellular carcinoma, and dermatomyositis as a paraneoplastic syndrome. Pathognomonic skin lesions of dermatomyositis, such as heliotrope rash, Gottron papules, Gottron sign, V-sign, and proximal muscle weakness preceded the diagnosis of biliary cholangitis and hepatocellular carcinoma by more than a year but were not initially recognized. The failure to identify the symptoms of dermatomyositis, combined with the lack of pronounced and specific signs of primary biliary cirrhosis and hepatocellular carcinoma, contributed to delayed diagnoses. The patient died due to severe, decompensated heart failure. **Conclusions:** This case report emphasized the importance of precise interpretation of pathogenetic findings by all specialists involved. In patients with multiple comorbidities, close collaboration between specialists and an interdisciplinary approach are essential for timely diagnosis and the selection of appropriate treatment, which can significantly improve prognosis and outcome.

Drug Index Terms

(Based on full text)

alanine aminotransferase (endogenous compound), antinuclear antibody (endogenous compound), aspartate aminotransferase (endogenous compound), creatine kinase (endogenous compound), cyclophosphamide, hydroxychloroquine, methylprednisolone, prednisone, rheumatoid factor (endogenous compound), trimetazidine, ursodeoxycholic acid

Non-drug Index Terms

(Based on full text)

adult, alopecia, arthralgia, article, biliary cirrhosis, case report, Child Pugh score, cholelithiasis, clinical article, computer assisted tomography, dermatomyositis, echography, electromyography, erythema, erythrocyte sedimentation rate, esophagus varices, female, follow up, gastroenterology, gastrointestinal endoscopy, Gottron papule, heart failure, heart left ventricle enddiastolic volume, heart rate, heliotrope rash, hospitalization, human, human tissue, interphalangeal joint, left ventricular end-systolic diameter, **liver cell carcinoma**, liver cirrhosis, liver function, liver hemangioma, liver injury, middle aged, muscle weakness, myopathy, myositis, **paraneoplastic dermatomyositis**, pathology, **primary biliary cirrhosis**, pruritus, radiofrequency ablation, rheumatology, skin defect, virus hepatitis

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Am. J. Case Rep.
ISSN	19415923 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Bulgaria
Country of Source	Poland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.12659/AJCR.949810
MEDLINE PMID	41029813
Article Number	e949810
Number of References	44

CAS Registry Numbers	alanine aminotransferase (9000-86-6 , 9014-30-6) aspartate aminotransferase (9000-97-9) creatine kinase (9001-15-4) cyclophosphamide (50-18-0 , 6055-19-2) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) rheumatoid factor (9009-79-4) trimetazidine (13171-25-0 , 5011-34-7) ursodeoxycholic acid (128-13-2 , 2898-95-5)
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88. [Efficacy of Pola-R-CHP and Autologous Stem Cell Transplantation in Untreated Burkitt Lymphoma: A Case Report](#)

Yang Y., Ren Y., Li J., Liu P.

Am. J. Case Rep. 2025 26:

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Objective: Unusual clinical course **Background:** Burkitt lymphoma is an aggressive B-cell lymphoma curable with highly dose-intensive chemotherapy derived from pediatric leukemia regimens. However, the adverse event profile, including myelosuppression, infection, and multi-organ dysfunction, worsens with increasing age. This report describes the first case of an 56-year-old woman diagnosed with Burkitt lymphoma who was treated with upfront polatuzumab vedotin plus ritux-imab, cyclophosphamide, doxorubicin, and prednisone (Pola-R-CHP) followed by autologous stem cell transplantation consolidation. **Case Report:** An 56-year-old Asian female patient with reduced body weight and severe malnutrition and unfit for standard therapies had an aggravating onset of abdominal pain and constipation. Diagnosis of high-risk Burkitt lymphoma was made by pathological biopsy, with intermediate-sized mature malignant B cells that expressed CD10, bcl-6 but not bcl-2, with high Ki-67 index (90%) and c-MYC translocation. She was treated with Pola-R-CHP. After the first dose of therapy, her abdominal pain was

rapidly relieved. A grade 3 pneumonia after the fourth cycle was treated with antibiotics and best supportive care. After 6 cycles of induction therapy, complete metabolic remission was confirmed by positron emission tomography/computed tomography (PET/CT). To consolidate the response, she underwent autologous stem cell transplantation, and afterward, complete metabolic remission status was maintained. The last CT scan showed a continuous response of 12.9 months after the initiation of chemoimmunotherapy. Follow-up was still ongoing through blood tests and CT examination every 3 months. Conclusions: Upfront Pola-R-CHP had a quick and durable response and was well-tolerated in untreated Burkitt lymphoma. Further research is required to confirm the conclusions.

Drug Index Terms

(Based on full text)

cyclophosphamide, dexamethasone, doxorubicin, epirubicin, lactate dehydrogenase, methotrexate, polatuzumab vedotin (drug combination, intravenous drug administration), prednisone (oral drug administration), rituximab (drug combination)

Non-drug Index Terms

(Based on full text)

abdominal pain, adult, article, **autologous stem cell transplantation**, B cell lymphoma, bone marrow suppression, Burkitt lymphoma, case report, chemoimmunotherapy, clinical article, computer assisted tomography, ECOG Performance Status, female, follow up, Hepatitis B virus, human, leukemia, lymph node, multiple cycle treatment, pneumonia, positron emission tomography-computed tomography, progression free survival

Comment: Invalid ICSR

Abbreviated Journal Title	Am. J. Case Rep.
ISSN	19415923 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
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Language of Article English

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Digital object identifier(DOI) [10.12659/AJCR.948969](https://doi.org/10.12659/AJCR.948969)

MEDLINE PMID [40988325](https://pubmed.ncbi.nlm.nih.gov/40988325/)

Article Number e948969

Number of References 16

CAS Registry Numbers cyclophosphamide ([50-18-0](#), [6055-19-2](#))
dexamethasone ([50-02-2](#))
doxorubicin ([23214-92-8](#), [25316-40-9](#))
epirubicin ([56390-09-1](#), [56420-45-2](#))
lactate dehydrogenase ([9001-60-9](#))
lactate dehydrogenase A
methotrexate ([15475-56-6](#), [59-05-2](#), [7413-34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#), [60388-53-6](#))
polatuzumab vedotin ([1313206-42-6](#))
prednisone ([53-03-2](#))
rituximab ([174722-31-7](#))

89. [Isolated Sensorineural Hearing Loss as the Sole Manifestation of Granulomatosis with Polyangiitis](#)

Arango A., Mumtaz S., Abril A.

Am. J. Case Rep. 2025 26:

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Patient: Final Diagnosis: Symptoms: Clinical Procedure: — Specialty:
Rheumatology Male, 36-year-old Granulomatosis with polyangiitis Acute

unilateral hearing loss • aural fullness and pressure • postural instability • progressive bilateral sensorineural hearing loss • tinnitus Objective: Background: Case Report: Conclusions: Unusual clinical course

Granulomatosis with polyangiitis (GPA) is a rare systemic vasculitis of small vessels associated with anti-neutrophil cytoplasmic antibodies (ANCA), typically affecting the respiratory tract and kidneys. Otologic involvement is relatively common, yet sensorineural hearing loss (SNHL) as the initial and sole clinical manifestation is exceptionally rare. Most reported cases of isolated SNHL in GPA involve unilateral or fluctuating hearing loss. Progressive bilateral SNHL associated with positivity for ANCA targeting proteinase 3 (PR3-ANCA), in the absence of systemic or ocular involvement, is rarely described in the literature. A 36-year-old man with no significant medical history presented with sudden-onset left-sided SNHL, which was partially responsive to oral corticosteroids. Four weeks later, he developed bilateral hearing loss, tinnitus, aural fullness, and non-disabling postural instability. Audiometry revealed moderate-to-profound bilateral SNHL. Infectious and hematologic causes were ruled out. Laboratory tests showed positive PR3-ANCA, negative antinuclear antibodies (ANA), and mildly elevated inflammatory markers, including an erythrocyte sedimentation rate of 31 mm/h and a C-reactive protein level of 9.6 mg/dL. Head MRI was unremarkable, and no ocular, nasal, pulmonary, or renal involvement was found. A diagnosis of GPA with isolated inner ear involvement was made. Treatment with corticosteroids and rituximab led to symptom stabilization. This case highlights an unusual presentation of GPA with PR3-ANCA positivity and progressive bilateral SNHL as the sole manifestation. Early recognition of atypical GPA presentations is essential to prevent irreversible hearing loss. Clinicians should consider ANCA-associated vasculitis in patients with unexplained bilateral SNHL, even in the absence of systemic signs. Early immunosuppressive therapy may preserve auditory function.

Drug Index Terms

(Based on full text)

C reactive protein (endogenous compound), corticosteroid (drug therapy, oral drug administration, unexpected outcome of drug treatment), prednisone (drug therapy, oral drug administration, unexpected outcome of drug treatment), rituximab (drug therapy)

Non-drug Index Terms

(Based on full text)

adult, antibody specificity, article, audiometry, case report, chest computed tomography, clinical article, computer assisted tomography, drug megadose, emergency ward, enzyme linked immunosorbent assay, erythrocyte sedimentation rate, human, male, neurologic examination, otitis, **perception deafness** (drug therapy), tinnitus, unilateral hearing loss, vasculitis, **Wegener granulomatosis** (diagnosis, drug therapy)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Am. J. Case Rep.
ISSN	19415923 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-19 (Full record), 2025-11-17 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Colombia
Country of Source	Poland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.12659/AJCR.948700
MEDLINE PMID	40938809
Article Number	e948700
Number of References	21
CAS Registry Numbers	C reactive protein (9007-41-4) prednisone (53-03-2) rituximab (174722-31-7)

90. [Clinical and out-of-pocket cost outcomes after switching to biosimilar adalimumab-atto in patients with rheumatoid arthritis](#)

Lin A.T.-H., Niu F., Hui R.L., Londa A., Pham C., Teshima S., Delate T.
Expert Opin. Biol. Ther. 2025 25:10 (1113-1119)
Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Background: Limited real-world studies have evaluated outcomes after switching treatment from a reference product (RP) to a biosimilar product. The objective of this real-world study was to assess the outcomes of switching from RP adalimumab to biosimilar adalimumab-atto in patients with rheumatoid arthritis (RA). **Research design and methods:** This was a propensity score matched, non-inferiority study assessing adult patients with RA who switched from RP adalimumab to adalimumab-atto (Switchers) between or received RP adalimumab continuously (Non-Switchers). One-year disease worsening, serious adverse events, and patient out-of-pocket cost outcomes were evaluated. An upper limit of 5% for adalimumab-atto was set as the non-inferiority margin. **Results:** After matching, there were 1,172 patients in each group. 24.8% and 31.0% of patients in the Switcher and Non-Switcher groups, respectively, experienced the disease worsening (non-inferiority $p < 0.01$). The crude rates per 100 patient-years of the safety outcome and mean monthly changes in out-of-pocket costs were 5.6 (95% CI 4.2-7.5) and 7.2 (95% CI 5.2-9.2) ($p = 0.21$) and $-\$51 (\pm \$164)$ and $-\$11 (\pm \$152)$ ($p < 0.01$) in the Switcher and Non-Switcher groups, respectively. **Conclusions:** These results provide evidence that switching from RP adalimumab to biosimilar adalimumab-atto did not result in disease worsening or increased adverse events but did deliver modestly lower patient out-of-pocket costs.

Drug Index Terms

(Based on full text)

abatacept, **adalimumab** (drug therapy, pharmacoeconomics), anakinra, azathioprine, baricitinib, betamethasone, **biosimilar agent** (adverse drug reaction, drug therapy, pharmacoeconomics, unexpected outcome of drug treatment), certolizumab pegol, dexamethasone, disease modifying antirheumatic drug, etanercept, glucocorticoid (drug therapy, intraarticular drug administration), golimumab, hydrocortisone, hydroxychloroquine, infliximab, leflunomide, methotrexate,

methyprednisolone, prednisolone, prednisone, rituximab, salazosulfapyridine, sarilumab, tocilizumab, tofacitinib, triamcinolone, upadacitinib

Non-drug Index Terms

(Based on full text)

adult, aged, aplastic anemia, article, clinical outcome, cohort analysis, congestive heart failure, controlled study, disease exacerbation (drug therapy), drug dose increase, drug efficacy, drug safety, emergency ward, female, health care cost, Hepatitis B virus, hospitalization, human, infection, injection site reaction (side effect), major clinical study, male, malignant neoplasm, middle aged, observational study, **outcome assessment**, rash (side effect), **rheumatoid arthritis** (drug therapy), treatment duration, treatment failure, **treatment switching**, very elderly, virus reactivation

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Expert Opin. Biol. Ther.
ISSN	17447682 (electronic), 14712598
CODEN	EOBTA
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-21 (Full record), 2025-10-21 (Article in Press/In process)
Publication Type	Article
Page Range	1113-1119
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1080/14712598.2025.2574011
MEDLINE PMID	41069165
Number of References	26

CAS Registry Numbers

abatacept ([332348-12-6](#))
adalimumab ([331731-18-1](#), [1446410-95-2](#))
anakinra ([143090-92-0](#))
azathioprine ([446-86-6](#), [55774-33-9](#))
baricitinib ([1187594-09-7](#))
betamethasone ([378-44-9](#))
certolizumab pegol ([428863-50-7](#))
dexamethasone ([50-02-2](#))
etanercept ([185243-69-0](#), [200013-86-1](#),
[2055118-96-0](#))
golimumab ([476181-74-5](#))
hydrocortisone ([50-23-7](#))
hydroxychloroquine ([118-42-3](#), [525-31-5](#),
[137433-23-9](#), [137433-24-0](#))
infliximab ([170277-31-3](#))
leflunomide ([75706-12-6](#))
methotrexate ([15475-56-6](#), [59-05-2](#), [7413-34-5](#),
[7532-09-4](#), [6745-93-3](#), [51865-79-3](#),
[60388-53-6](#))
methylprednisolone ([6923-42-8](#), [83-43-2](#))
prednisolone ([50-24-8](#))
prednisone ([53-03-2](#))
rituximab ([174722-31-7](#))
salazosulfapyridine ([599-79-1](#))
sarilumab ([1189541-98-7](#))
tocilizumab ([375823-41-9](#))
tofacitinib ([477600-75-2](#), [540737-29-9](#))
triamcinolone ([124-94-7](#))
upadacitinib ([1310726-60-3](#), [1607431-21-9](#),
[2095311-41-2](#), [2050057-56-0](#))

91. [Exacerbation and myasthenic crisis in generalized myasthenia gravis: real-world healthcare resource utilization and cost burden in the United States](#)

Jackson L., Liu Z., Pesa J., Campbell A.K., Choudhry Z., Batista A.E., Souayah N.

Curr. Med. Res. Opin. 2025 41:9 (1735-1751)

Embase, MEDLINE

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Abstract

Objectives: Healthcare resource utilization (HCRU) and cost assessments for generalized myasthenia gravis (gMG) rarely include matched controls. HCRU and costs of acute MG-related events (MG exacerbation and myasthenic crisis) have been examined, but aggregation or comparison between studies is limited by heterogeneity. The study aim was to provide comprehensive, real-world data about gMG patients in the United States, including those experiencing MG-related events. A claims database analysis assessed (i) incremental HCRU and cost burden among adults with gMG versus non-gMG controls, (ii) occurrence of MG-related events, (iii) HCRU and cost burden of MG-related events, and (iv) factors associated with MG-related events. **Methods:** In this retrospective cohort study, adults with gMG in the United States were identified from Optum's de-identified Clinformatics® Data Mart Database (1 January 2017–31 March 2023). Rates of MG-related events during follow-up were calculated. Regression models assessed associations between occurrence of MG-related events and baseline demographic/clinical variables using a nested case-control design embedded within the existing gMG cohort. Incremental HCRU and healthcare costs were compared between cohorts with/without gMG using the propensity score method with inverse probability of treatment weighting for confounding controls. Incremental HCRU and healthcare costs were also reported for gMG subgroups with/without MG-related events. **Results:** Analyses included 16,561 gMG patients (12,813 prevalent and 3748 incident) and 105,716 non-gMG controls. Among prevalent and incident gMG patients, respectively, MG exacerbation occurred during follow-up in 12.2 and 22.0% and myasthenic crisis in 6.0 and 12.4%. Patients with prior MG-related events were more likely to experience future events (odds ratio range 1.76–7.05). After weighting, gMG patients (median follow-up 696 days) experienced more frequent all-cause HCRU encounters and >4-fold higher healthcare costs (\$5420 PPM or ~\$65,000 per annum) versus non-gMG controls (\$1298 PPM or ~\$15,500 per annum). Among MG patients, those with MG exacerbation/myasthenic crisis, respectively, experienced means of 3.02/3.35 hospitalizations, 1.91/2.13 emergency department encounters, 12.70/13.39 skilled nursing facility encounters, and 67.86/75.52 outpatient encounters during follow-up; associated costs

were \$17,113/\$18,640 PPPM (or ~\$205,000/\$224,000 per annum). Conclusion: The total healthcare cost of gMG was higher than some previous estimates at \$5420 PPPM, driven primarily by inpatient care. MG exacerbation and myasthenic crisis occurred at a notable rate, could be recurrent, and were associated with high HCRU and costs. Findings can inform treatment decision-making with the aim of improving symptom control and reducing the likelihood of MG-related acute events.

Drug Index Terms

(Based on full text)

azathioprine (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), betamethasone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), budesonide (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), cholinesterase inhibitor (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), corticosteroid (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), cyclophosphamide (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), cyclosporine (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), dexamethasone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), eculizumab (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), efgartigimod alfa (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), fludrocortisone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), fluorometholone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), immunoglobulin (drug therapy, intravenous drug administration, special situation for pharmacovigilance, subcutaneous drug administration), methotrexate (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), methylprednisolone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), mycophenolate mofetil (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), prednisolone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), prednisone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), ravulizumab (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), rituximab (drug therapy, pharmacoeconomics, special situation for pharmacovigilance),

tacrolimus (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), triamcinolone (drug therapy, pharmacoeconomics, special situation for pharmacovigilance), zilucoplan (drug therapy, pharmacoeconomics, special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

adult, aged, all cause mortality, article, case control study, Charlson Comorbidity Index, chronic lung disease, cohort analysis, comorbidity, consumer price index, controlled study, depression, diabetes mellitus, disease burden, **disease exacerbation**, electrolyte disturbance, emergency ward, female, follow up, general practitioner, **health care cost**, health care facility, **health care utilization**, health insurance, hospitalization, human, hypertension, hypothyroidism, ICD-10-CM, major clinical study, male, middle aged, **myasthenia gravis** (disease management, drug therapy, surgery, therapy), obesity, onset age, patient experience, plasma exchange, retrospective study, risk factor, thymectomy, United States, very elderly

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Curr. Med. Res. Opin.
ISSN	14734877 (electronic), 03007995
CODEN	CMROC
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-20 (Full record), 2025-10-30 (Article in Press/In process)
Publication Type	Article
Page Range	1735-1751
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1080/03007995.2025.2576597

MEDLINE PMID	41099443
Number of References	53
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) betamethasone (378-44-9) budesonide (51333-22-3 , 51372-29-3) cyclophosphamide (50-18-0 , 6055-19-2) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) dexamethasone (50-02-2) eculizumab (219685-50-4) efgartigimod alfa (1821402-21-4) fludrocortisone (127-31-1) fluorometholone (426-13-1) immunoglobulin (9007-83-4) methotrexate (15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisolone (50-24-8) prednisone (53-03-2) ravulizumab (1803171-55-2) rituximab (174722-31-7) tacrolimus (104987-11-3 , 109581-93-3) triamcinolone (124-94-7) zilucoplan (1841136-73-9)

92. [Remarkable response to anti-PD-L1 blockade in relapsed and refractory metastatic malignant giant cell tumor of bone: a case report](#)

Wu J.T.,Imamura J.,Nowak E., Leng J.,Reith J.,Kilpatrick S., Schwartz G.K., Burke Z.D.C.

Immunother. 2025 17:15 (1079-1084)

Embase, MEDLINE

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Abstract

Case: A 40-year-old man presented with malignant giant cell tumor of bone of the left distal femur. Despite distal femur resection/replacement and adjuvant chemotherapy, he experienced local recurrence and lung metastases. Salvage radiation therapy was ineffective. Biopsy of the tumor showed a PD-L1 tumor proportion score of 40%. Pembrolizumab was initiated and resulted in a major partial response. However, the patient experienced radiation recall pneumonitis necessitating stoppage of pembrolizumab. The patient then experienced another local recurrence. Conclusion: PD-L1 blockade should be considered for MGCTB expressing PD-L1. Unfortunately, pembrolizumab was stopped, which likely allowed for tumor progression in this case.

Drug Index Terms

(Based on full text)

cisplatin (drug therapy), denosumab (adverse drug reaction, drug therapy), doxorubicin (drug therapy), **pembrolizumab** (drug therapy), prednisone (drug therapy), **programmed death 1 ligand 1** (endogenous compound)

Non-drug Index Terms

(Based on full text)

adult, article, bone lesion (diagnosis), cancer adjuvant therapy, **cancer immunotherapy**, cancer inhibition, cancer radiotherapy, cancer recurrence, case report, clinical article, computer assisted tomography, distal femur fracture (diagnosis), drug safety, drug withdrawal, dyspnea, femoral osteotomy, full time employment, human, human cell, human tissue, knee pain, **lung metastasis** (diagnosis, drug therapy, radiotherapy), lung nodule (diagnosis), lymph vessel metastasis (diagnosis), male, multiple cycle treatment, nuclear magnetic resonance imaging, **osteoclastoma** (diagnosis, drug therapy, radiotherapy, surgery), pain severity, radiation pneumonia (complication, diagnosis, drug therapy), return to work, salvage therapy, tooth disease (side effect), treatment duration, treatment failure, tumor biopsy, tumor recurrence, tumor volume

Comment: valid ICSR

Additional Information

Abbreviated Journal Title Immunother.

ISSN	17507448 (electronic), 1750743X
Source Type	Journal
Source Publication Date	2025-01-01
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Page Range	1079-1084
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1080/1750743X.2025.2582518
MEDLINE PMID	41190726
Number of References	19
CAS Registry Numbers	cisplatin (15663-27-1 , 26035-31-4 , 96081-74-2) denosumab (615258-40-7) doxorubicin (23214-92-8 , 25316-40-9) pembrolizumab (1374853-91-4) prednisone (53-03-2)

93. [Myelin oligodendrocyte glycoprotein antibody-associated isolated aseptic meningitis: a single-center pediatric case series](#)

Liu K., Sun S., Chen F., Wu W., Li X., Pang L., Wang W.

Front. Immunol. 2025 16:

Embase, MEDLINE, NURSING

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Abstract

Background and purpose: Aseptic meningitis represents a rare and underrecognized phenotype of myelin oligodendrocyte glycoprotein (MOG) antibody-associated disease (MOGAD). Despite sporadic case reports, comprehensive series studies remain scarce. This study aimed to describe the clinical characteristics of MOG antibody-associated aseptic meningitis (MOGAM) in pediatric patients without neuroparenchymal lesions. **Methods:** We reviewed the medical records of pediatric patients from January 2019 to July 2025, focusing on cases diagnosed with MOGAM in the absence of neuroparenchymal lesions. Clinical manifestations, brain magnetic resonance imaging (MRI) findings, laboratory results, treatment regimens, and clinical outcomes were retrospectively analyzed. **Results:** Among 159 children with seropositive MOG antibodies, 11 (6.9%) met the inclusion criteria. The mean age at disease onset was 8.3 years (8.3 ± 3.2 years), with a male-to-female ratio of 5:6. The most common symptoms were fever (10/11), lasting from 3 to 50 days (median, 13 days; interquartile range [IQR], 8.0–22.7 days), and headache (10/11). Meningeal irritation signs were positive in 4 patients (4/11). Nine patients had peripheral blood leukocytosis ($21.6 \pm 3.6 \times 10^9/L$; range, 17.2 – $29.1 \times 10^9/L$), and 10 had elevated neutrophil-to-lymphocyte ratio (NLR; median, 4.5; IQR, 4.0–7.9; range, 3.0–14.5) as well as erythrocyte sedimentation rate (ESR; 38.4 ± 10.7 mm/h; range, 25–62 mm/h). All patients had cerebrospinal fluid (CSF) pleocytosis ($117.4 \pm 62.0/\mu L$; range, 50–201/ μL); 2 had elevated CSF pressure (range, 250–350 mmH₂O); 4 had slightly increased CSF protein levels (range, 0.48–0.96 g/L); and none tested positive for infectious pathogens. Six patients showed abnormal electroencephalogram (EEG) results, including focal interictal epileptiform discharges or slow waves (3/6) and slow background activities (5/6). Brain MRI showed linear hyperintense signals along the bilateral cerebral sulci on fluid-attenuated inversion recovery (FLAIR) sequences in 4 patients (4/11), while no abnormalities were observed in 7 patients (7/11). Leptomeningeal enhancement (LME) was detected in 2 of 3 patients who underwent contrast-enhanced brain MRI. All patients received immunotherapy, with 2 administered immunosuppressive therapy after relapse. At the last follow-up, all patients achieved favorable clinical outcomes. **Conclusion:** Myelin oligodendrocyte glycoprotein antibody-associated isolated aseptic meningitis is a novel and underrecognized clinical phenotype. For pediatric patients presenting with prolonged or recurrent fever and headache, particularly those with CSF pleocytosis, leptomeningeal enhancement (LME), and

poor response to anti-infective therapy, early testing for MOG-IgG is strongly recommended. Timely identification of this distinct phenotype may facilitate early diagnosis, initiation of effective immunotherapy, and better clinical outcomes.

Drug Index Terms

(Based on full text)

aciclovir (drug therapy, intravenous drug administration, special situation for pharmacovigilance), antibiotic agent (drug therapy, special situation for pharmacovigilance), aquaporin 4 (endogenous compound), C reactive protein (endogenous compound), glucose (endogenous compound), immunoglobulin (drug therapy, intravenous drug administration, special situation for pharmacovigilance), immunoglobulin G (endogenous compound), methylprednisolone (drug therapy, intravenous drug administration, special situation for pharmacovigilance), myelin oligodendrocyte glycoprotein immunoglobulin G (endogenous compound), oligoclonal band (endogenous compound), prednisone (drug therapy, oral drug administration, special situation for pharmacovigilance), prolactin (endogenous compound), rituximab (drug therapy, special situation for pharmacovigilance), tocilizumab (drug therapy, special situation for pharmacovigilance), unclassified drug

Non-drug Index Terms

(Based on full text)

amplitude modulation, article, **aseptic meningitis** (diagnosis, drug therapy), bacterium culture, blood analysis, blood cell count, blood sampling, cerebral sulci, cerebrospinal fluid analysis, cerebrospinal fluid pressure, child, clinical outcome, cohort analysis, controlled study, drug dose reduction, electroencephalography monitoring, electromyography, electronic medical record, epileptic discharge, erythrocyte sedimentation rate, female, fever, fixed cell based assay, fluid attenuated inversion recovery, focal interictal epileptiform discharge, follow up, headache, herpes simplex encephalitis (drug therapy), high throughput sequencing, hospital discharge, human, immunosuppressive treatment, intracranial infection, leptomeningeal enhancement, leukocyte count, leukocytosis, limb weakness, magnetic field strength, major clinical study, male, metagenomics, **myelin oligodendrocyte glycoprotein antibody associated disease** (diagnosis, drug therapy), neutrophil lymphocyte ratio, nuclear magnetic resonance imaging, **pediatrics**,

phenotype, pleocytosis, polymerase chain reaction, retrospective study, school child, seizure, single center study, T1 weighted imaging, urine retention, visual evoked potential, vomiting

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Front. Immunol.
ISSN	16643224 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	China
Country of Source	Switzerland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.3389/fimmu.2025.1699683
MEDLINE PMID	41208985
Article Number	1699683
Number of References	24
CAS Registry Numbers	aciclovir (59277-89-3 , 69657-51-8) aquaporin 4 (175960-54-0) C reactive protein (9007-41-4) glucose (50-99-7 , 84778-64-3 , 8027-56-3) immunoglobulin (9007-83-4) immunoglobulin G (97794-27-9 , 308067-58-5) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2) procalcitonin (56645-65-9) rituximab (174722-31-7) tocilizumab (375823-41-9)

94. [Chronic Anaphylaxis With Indolent Systemic Mastocytosis: A Case Report](#)

Worth S., George T.I., Shaheen D.J., Roche M., Vachhani P.

Case Rep. Hematol. 2025 2025:1

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Systemic mastocytosis is a rare, clonal mast cell disease neoplasm driven by the KIT D816V mutation in greater than 95% of cases. The complex clinical presentation of systemic mastocytosis can make diagnosis challenging. Treatment strategies often focus on management of symptoms, but many patients' symptoms are not well controlled on these regimens and have poor quality of life. We report the case of a 35-year-old Caucasian female who suffered repeated life-threatening anaphylactic episodes that greatly decreased her quality of life and that the best supportive care measures failed to control. Following extensive diagnostic evaluations including physical examination, clinical labs, hematology, and next-generation genetic screening, indolent systemic mastocytosis was confirmed by bone marrow biopsy according to World Health Organization 2016 criteria. The patient was subsequently treated with avapritinib, a selective tyrosine kinase inhibitor, after which she reported improved quality of life and physical activity. The diagnosis and optimal management of this complex and rare disease require a multidisciplinary approach combined with effective personalized therapeutic strategies. A confirmed diagnosis of indolent systemic mastocytosis led to the patient receiving an approved targeted treatment, with a favorable outcome. Novel precision therapies and refined diagnostic guidelines are critical to meet the high unmet needs of patients with indolent systemic mastocytosis and improve their quality of life. Trial Registration: ClinicalTrials.gov identifier: NCT03731260.

Drug Index Terms

(Based on full text)

acetazolamide (drug therapy), alanine aminotransferase (endogenous compound), alpha interferon (subcutaneous drug administration),

antihistaminic agent (drug combination, drug therapy), aspartate aminotransferase (endogenous compound), avapritinib (drug combination, drug therapy, oral drug administration), CD2 antigen (endogenous compound), cromoglycate disodium (drug combination, drug therapy), diphenhydramine (drug combination, drug therapy), DNA methyltransferase 3A (endogenous compound), doxepin (drug combination, drug therapy), epinephrine (drug combination, drug therapy), famotidine (drug therapy, intravenous drug administration), granisetron (drug therapy), imatinib (drug therapy, unexpected outcome of drug treatment), interleukin 2 receptor alpha (endogenous compound), midostaurin (adverse drug reaction, drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), montelukast (drug combination, drug therapy), omalizumab (drug combination, drug therapy), ondansetron (drug therapy), oxycodone (drug therapy), peppermint, prednisone (drug combination, drug therapy), ranitidine (drug combination, drug therapy), salbutamol (drug combination, drug therapy), stem cell factor receptor (endogenous compound), tryptase (endogenous compound), tumor necrosis factor receptor superfamily member 8 (endogenous compound)

Non-drug Index Terms

(Based on full text)

abdominal pain, adult, alanine aminotransferase level, allele specific polymerase chain reaction, allergy test, **anaphylaxis** (diagnosis, drug therapy, therapy), angioneurotic edema, ant sting, article, aspartate aminotransferase level, bone marrow biopsy, bone pain (drug therapy), case report, cell proliferation, **chronic disease** (diagnosis, drug therapy, therapy), clinical article, clouding of consciousness, convulsion, diarrhea, dizziness, drug dose increase, drug dose titration, drug substitution, drug withdrawal, emergency care, emotional stress, enzyme blood level, esophagogastroduodenoscopy, female, flow cytometry, flushing, food allergy (drug therapy), fragrance allergy (drug therapy), gene mutation, headache (drug therapy, prevention), heart arrest, hematopoiesis, high throughput sequencing, hospital discharge, hot flush, human, human tissue, Hymenoptera venom allergy (drug therapy), idiopathic intracranial hypertension (diagnosis, drug therapy), **indolent systemic mastocytosis** (diagnosis, drug therapy), intensive care, intracranial pressure, leukocyte count, leukoreduction, loose feces (drug therapy), mast cell, mastocytosis, metallic taste, missense mutation, muscle cramp, nausea (drug therapy, side effect), petechia,

physical activity, physical examination, physical stress, postural orthostatic tachycardia syndrome, quality of life, skin defect, strawberry, tachycardia, tomato, unconsciousness, urticaria (drug therapy), venom immunotherapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Case Rep. Hematol.
ISSN	20906579 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-18 (Full record), 2025-11-13 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1155/crh/9562195
Article Number	9562195
Number of References	28
CAS Registry Numbers	acetazolamide (1424-27-7 , 59-66-5) alanine aminotransferase (9000-86-6 , 9014-30-6) aspartate aminotransferase (9000-97-9) avapritinib (1703793-34-3) cromoglycate disodium (15826-37-6 , 16110-51-3 , 93356-79-7 , 93356-84-4) diphenhydramine (147-24-0 , 58-73-1) DNA (cytosine 5) methyltransferase DNA (cytosine 5) methyltransferase 1 DNA methyltransferase 3A DNA methyltransferase 3B doxepin (1229-29-4 , 1668-19-5)

epinephrine ([51-43-4](#), [55-31-2](#), [6912-68-1](#))
famotidine ([76824-35-6](#))
granisetron ([107007-99-8](#), [109889-09-0](#))
imatinib ([152459-95-5](#), [220127-57-1](#))
midostaurin ([120685-11-2](#))
montelukast ([151767-02-1](#), [158966-92-8](#))
omalizumab ([242138-07-4](#))
ondansetron ([103639-04-9](#), [116002-70-1](#),
[99614-01-4](#))
oxycodone ([124-90-3](#), [76-42-6](#))
prednisone ([53-03-2](#))
ranitidine ([66357-35-5](#), [66357-59-3](#))
salbutamol ([18559-94-9](#), [35763-26-9](#))
tryptase ([97501-93-4](#))

Clinical Trial Numbers

ClinicalTrials.gov ([NCT03731260](#))

95. [Case Report: Coexisting cardiac cavernous hemangioma and sarcoidosis in a single lesion: a first reported case confirmed by surgical resection](#)

Xiao C.-Y., Han W.-Y., Wang H.-S., Qian X., Yuan L.-L., Wang M.-F., Liu Y.-Q.

Front. Cardiovasc. Med. 2025 12:

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Cardiac hemangioma (CH) is a rare benign cardiac tumor whose clinical manifestations—including arrhythmias, heart failure, and pericardial effusion—vary by anatomical location. Sarcoidosis is a multisystem disorder of unknown etiology characterized by non-caseating granulomas, commonly involving the lungs, eyes, and skin, with cardiac involvement being relatively uncommon. To date, no cases of concurrent cardiac hemangioma and cardiac sarcoidosis have been reported worldwide. We herein describe a patient with established sarcoidosis who, during follow-up, exhibited progressive enlargement of a cardiac mass. Surgical resection confirmed the co-existence of cardiac

hemangioma and cardiac sarcoidosis within the same anatomical region. Based on these findings, we propose a pathophysiological mechanism wherein cardiac sarcoidosis causes microvascular injury, leading to structural alterations that may promote the development of cardiac hemangioma. The following report details the diagnostic and therapeutic course of this patient.

Drug Index Terms

(Based on full text)
eosin, hematoxylin, methotrexate, methylprednisolone (oral drug administration), prednisone, vasculotropin

Non-drug Index Terms

(Based on full text)
adult, article, benign cardiac tumor, breathing rate, **cardiac hemangioma**, cardiac sarcoidosis, cardiopulmonary bypass, case report, clinical article, computer assisted tomography, coronary flow reserve, Doppler echocardiography, dyspnea, echocardiography, electrocardiography, female, follow up, general anesthesia, granuloma, heart failure, heart palpitation, heart surgery, heart tumor, hemangioma, hilar lymphadenopathy, histopathology, human, human tissue, hypertension, imaging, immunohistochemistry, immunosuppressive treatment, male, mediastinum lymph node, middle aged, mortality, myocarditis, nuclear magnetic resonance imaging, pericardial effusion, periodic acid Schiff stain, physical examination, polymerase chain reaction, positron emission tomography-computed tomography, signal transduction, thorax radiography, transbronchial aspiration, transthoracic echocardiography, visual disorder

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Front. Cardiovasc. Med.
ISSN	2297055X (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-21 (Full record), 2025-11-18 (Article in Press/In process)
Publication Type	Article

Page Range

Country of Author China

Country of Source Switzerland

Language of Article English

Language of Summary English

Digital object identifier(DOI) [10.3389/fcvm.2025.1707426](https://doi.org/10.3389/fcvm.2025.1707426)

Article Number 1707426

Number of References 16

CAS Registry Numbers eosin ([17372-87-1](#), [51395-88-1](#), [548-26-5](#))
hematoxylin ([517-28-2](#))
methotrexate ([15475-56-6](#), [59-05-2](#), [7413-34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#), [60388-53-6](#))
methylprednisolone ([6923-42-8](#), [83-43-2](#))
prednisone ([53-03-2](#))
vasculotropin ([127464-60-2](#))

96. [Clinical characteristics and prognostic analysis of different fusion gene abnormalities in childhood acute lymphoblastic leukaemia](#)

Fan R., Zhao M., Li P., Tian Y., Liu B., Zhu X., Chen X., Wang Y., Ma Y., Li S.

Front. Oncol. 2025 15:

Embase, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

Objective: This study aimed to analyze the clinical features and prognostic significance of different fusion gene subtypes in pediatric patients with acute lymphoblastic leukaemia (ALL). Methods: Clinical data from 132 childhood patients with ALL diagnosed between 2016 and 2025 were retrospectively analyzed. Patients were categorized based on fusion gene status: TEL::AML1, BCR::ABL, E2A::PBX1, MLL::AF4, SIL::TAL1, other, negative and unknown. Clinical characteristics,

laboratory findings, treatment responses, minimal residual disease status and survival outcomes were compared among different fusion gene groups. Survival analyses included overall survival (OS), event-free survival (EFS) and recurrence-free survival using the Kaplan-Meier method and Cox regression models. Results: Among 132 patients, the fusion gene distribution was as follows: negative (48.5%), unknown (32.6%), TEL::AML1 (7.6%), BCR::ABL (3.8%), E2A::PBX1 (3.0%), MLL::AF4 (2.3%), other (1.5%) and SIL::TAL1 (0.8%). B-cell immunophenotype predominated (88.6%). E2A::PBX1-positive patients showed the most favorable outcomes with 100% 5-year OS and EFS. TEL::AML1-positive patients demonstrated good prednisone responses (90%), with 90% 5-year OS. BCR::ABL and MLL::AF4 cases presented with elevated white blood cell counts (median 86.9 and 96.5 $\times 10^9/L$, respectively), higher lactate dehydrogenase levels and inferior treatment responses. In multivariate analysis, poor prednisone response (hazard ratio [HR] = 3.41, $p = 0.005$) and high-risk classification (HR = 4.92, $p < 0.001$) were independent adverse prognostic factors for EFS. Conclusion: Fusion gene abnormalities significantly influence the clinical presentation and prognosis of childhood ALL. E2A::PBX1 and TEL::AML1 demonstrate favorable outcomes, whereas BCR::ABL, MLL::AF4 and SIL::TAL1 are associated with unfavorable prognosis. These findings provide valuable insights for risk stratification and treatment optimization in pediatric ALL.

Drug Index Terms

(Based on full text)

asparaginase (special situation for pharmacovigilance), daunorubicin (special situation for pharmacovigilance), genomic DNA, hemoglobin (endogenous compound), lactate dehydrogenase (endogenous compound), methotrexate (intrathecal drug administration, special situation for pharmacovigilance), prednisone (special situation for pharmacovigilance), vincristine (special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

200 U SuperScript IV Reverse Transcriptase, **acute lymphoblastic leukemia**, adolescent, article, child, controlled study, data analysis software, DNA contamination, event free survival, female, flow cytometry, fluorescence in situ hybridization, gene expression, hematopoietic stem cell transplantation, hepatomegaly, human, IBM

SPSS Statistics version 25.0, immunophenotyping, leukocyte count, lymphadenopathy, major clinical study, male, morphology, multiplex reverse transcription polymerase chain reaction, outcome assessment, overall survival, platelet count, prognosis, prophylaxis, R software version 4.0.3, real time polymerase chain reaction, recurrence free survival, relapse, retrospective study, reverse transcriptase PCR assay kit, sensitivity analysis, splenomegaly, survival analysis, survival outcome, treatment outcome, treatment response

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Front. Oncol.
ISSN	2234943X (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-21 (Full record), 2025-11-18 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	China
Country of Source	Switzerland
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.3389/fonc.2025.1616686
Article Number	1616686
Number of References	30
Device Tradenames	200 U SuperScript IV Reverse Transcriptase (Invitrogen, United States), IBM SPSS Statistics version 25.0 (IBM, United States), R software version 4.0.3 (R Foundation, Austria)
Device Manufacturers	R Foundation (Austria), IBM (United States), Invitrogen (United States)

CAS Registry Numbers	asparaginase (9015-68-3 , 1349719-22-7)
	daunorubicin (12707-28-7 , 20830-81-3 , 23541-50-6 , 371770-68-2)
	hemoglobin (9008-02-0)
	lactate dehydrogenase (9001-60-9)
	lactate dehydrogenase A
	methotrexate (15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6)
	prednisone (53-03-2)
	vincristine (57-22-7)

97. [Prognosis and predictive factors in pediatric IgA nephropathy](#)

Liang W., He Y., Ma X., Shao P., Guo L., Zhou J., Zhang Y., Yuan H., Qiu L.

[Article in Press] [In Process] *Pediatr. Nephrol.* 2025 :
Embase, MEDLINE

Go to publisher for the [full text](#)

Embase Open URL: [redirect to full text](#)

Abstract

Background: IgA nephropathy (IgAN) is a common glomerular disease in children that may progress to chronic kidney disease (CKD) and kidney failure. Identifying reliable prognostic markers is important for guiding clinical management. This study investigated independent predictors of poor prognosis in pediatric IgAN and their impact on eGFR slope, as well as the dynamic patterns of complete kidney remission and relapse, predictive factors associated with remission, and the influence of remission on poor prognosis. Methods: A retrospective cohort study of 224 children (aged 3-18 years) with biopsy-proven IgAN and ≥ 3 years of follow-up was enrolled. Poor prognosis was defined as persistent eGFR < 90 mL/min/1.73 m² for ≥ 3 months. Multivariate logistic regression, receiver operating characteristic (ROC) curve analysis, and linear mixed-effects model were used to identify predictive factors and evaluate eGFR slope. Complete kidney remission was defined as the absence of hematuria and proteinuria with eGFR ≥ 90 mL/min/1.73 m² for ≥ 1 year. Cumulative incidence function and Fine-Gray competing risk regression model were applied to analyze remission dynamic patterns. Results:

Over a median follow-up of 5.41 years, 12.05% reached the poor prognosis. Independent risk factors included male sex, older age at biopsy, Oxford classification E1 and S1 lesions, absence of gross hematuria, and no remission of proteinuria during follow-up, while higher birthweight was protective. eGFR slope was influenced by age at biopsy, gross hematuria and proteinuria remission status during follow-up. Complete kidney remission occurred in 70.98%, with a recurrence rate of 60.38%. Predictors of complete kidney remission included antecedent infection, kidney IgM deposition of 2 +, elevated serum IgM, increased pathological casts count, and prednisone treatment, while prolonged disease duration before biopsy and higher tubular casts count were risk factors. Group comparisons and logistic regression analysis showed no significant associations between kidney remission and poor. Conclusions: Pediatric IgAN shows slow disease progression over 5 years. Despite high complete kidney remission rates, recurrence remains common. These findings support extended follow-up and improved predictive models.

Drug Index Terms

biological marker, immunoglobulin M, prednisone

Non-drug Index Terms

adolescent, adult, article, birth weight, child, chronic kidney failure, cohort analysis, drug combination, estimated glomerular filtration rate, female, follow up, glomerulopathy, hematuria, human, human tissue, **immunoglobulin A nephropathy**, kidney failure, kidney infection, major clinical study, male, observational study, predictive model, **prognosis**, proteinuria, receiver operating characteristic, recurrence risk, recurrent disease, relapse, **remission**, retrospective study, risk factor

Comment: Invalid ICSR

ADDITIONAL INFORMATION

Abbreviated Journal Title	Pediatr. Nephrol.
ISSN	1432198X (electronic), 0931041X
CODEN	PEDNE
Source Type	Journal Source
Publication Date	2025-01-01
Entry Date	2025-11-18 (Article in Press)

Publication Type	Article in Press
Page Range	
Country of Author	China
Country of Source	Germany
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-025-06988-8
MEDLINE PMID	41225194
Number of References	53

98. [Regorafenib pulmonary toxicity: A case of cavitory lung disease and ARDS treated with infliximab](#)

Dong M., Kramer M., Gandhi T., George G.

Respir. Med. Case Rep. 2025 58:

Embase, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

Regorafenib-induced pneumonitis and acute respiratory distress syndrome (ARDS) are rare and poorly characterized toxicities. We describe a 43-year-old male with metastatic colorectal cancer who developed hypoxic respiratory failure after two cycles of regorafenib. Imaging revealed cavitory lung lesions and diffuse ground glass opacities. Despite antibiotics and high-dose corticosteroids, his condition deteriorated, requiring ICU admission and mechanical ventilation. Bronchoalveolar lavage revealed bacterial pathogens, and he was treated for pneumonia. After multidisciplinary discussion, he was assessed to have regorafenib-induced pneumonitis as a major driver of his respiratory failure, and after limited steroid response, infliximab was administered with marked respiratory improvement. However, he later developed fungal sepsis and died from refractory shock. This is the first reported case of regorafenib-induced pneumonitis and ARDS treated with infliximab and corticosteroids. The patient's transient response

suggests overlap with immune-related pneumonitis mechanisms seen in tyrosine kinase and immune checkpoint inhibitor therapies. Further research is needed to clarify regorafenib's immunotoxicity and the role of advanced immunosuppressive therapies in managing pneumonitis.

Drug Index Terms

(Based on full text)
infliximab (drug therapy), methylprednisolone (drug therapy), piperacillin plus tazobactam (drug therapy), prednisone (drug therapy), **regorafenib** (adverse drug reaction, drug therapy), vancomycin (drug therapy)

Non-drug Index Terms

(Based on full text)
acute respiratory distress syndrome (diagnosis, drug therapy, side effect), adult, article, bacterial pneumonia (drug therapy), bacterium detection, case report, **cavitary lung disease** (diagnosis, drug therapy, side effect), clinical article, computer assisted tomography, coughing, crackle, dyspnea, Enterobacter aerogenes, fungemia (complication), human, leukocytosis, **lung disease** (diagnosis, drug therapy, side effect), lung lavage, **lung toxicity** (diagnosis, drug therapy, side effect), male, metastatic colorectal cancer (drug therapy), pneumonia (side effect), shock (complication), Staphylococcus aureus, tachycardia

Comment: valid ICSR

Additional Information

Abbreviated Journal Title	Respir. Med. Case Rep.
ISSN	22130071 (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-18 (Full record), 2025-11-06 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English

Language of Summary	English
Publisher Item Identifier	S2213-0071(25)00138-8
Publisher Item Identifier	S2213-0071(25)00138-8
Digital object identifier(DOI)	10.1016/j.rmcr.2025.102302
Article Number	102302
Number of References	24
CAS Registry Numbers	infliximab (170277-31-3) methylprednisolone (6923-42-8 , 83-43-2) piperacillin plus tazobactam (157044-21-8) prednisone (53-03-2) regorafenib (755037-03-7 , 1019206-88-2) vancomycin (1404-90-6 , 1404-93-9)

99. [Idiopathic Inflammatory Superior Mesenteric Artery Aneurysm: Case Report and Systematic Literature Review](#)

Vallauri C., Kuntz S., Lejay A., Chakfé N.

EJVES. Vas. For 2025 64: (178-182)

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Introduction: Superior mesenteric artery aneurysm (SMAA) is a rare presentation mostly secondary to atherosclerosis, infection, cystic medial dysplasia, collagen vascular disorders, trauma, and arterial dissection. However, inflammatory SMAAs are sometimes idiopathic. The aim of the article is to report the case of an idiopathic inflammatory SMAA and perform a systematic literature review. Case report: A 61 year old man presented with chronic abdominal pain. Computed tomography angiography showed a typical 65 mm diameter inflammatory abdominal aortic aneurysm and a SMAA with similar signs of wall inflammation. Positron emission tomography computed tomography confirmed the inflammation by active fixation. A full workup including all inflammatory markers was negative, and no cause was found. Treatment consisted of fenestrated endograft implantation. At 11 months, follow up confirmed

aneurysm exclusion and patency of the stents and a similar SMAA diameter with some mild remaining inflammation. Literature review: A systematic review of the MEDLINE and PubMed databases extending from 1979 to 2024 by a combined strategy of Medical Subject Headings terms returned only two publications of idiopathic inflammatory SMAA in two patients. In both cases, the symptoms were abdominal or lower back pain and the diagnosis was made by computed tomography angiography. Treatment was open surgery in one case and medical in the other. Median follow up was 18 months. Conclusion: Idiopathic inflammatory SMAA is rarely described in the literature. The aetiological assessment including histological data should be exhaustive in order not to miss a differential diagnosis.

Drug Index Terms

(Based on full text)
methylprednisolone (drug combination), prednisone (drug combination)

Non-drug Index Terms

(Based on full text)
abdominal aortic stent graft (adverse device effect), abdominal pain, adult, aneurysm diameter, article, case report, chronic pain, clinical article, computed tomographic angiography, duplex Doppler ultrasonography, endoleak (complication), fenestrated endovascular aneurysm repair, follow up, human, hypermetabolism, **idiopathic disease**, implantation, **inflammatory disease**, male, **mesenteric artery aneurysm** (surgery), middle aged, positron emission tomography-computed tomography, systematic review

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	EJVES. Vas. For
ISSN	2666688X (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	178-182

Country of Author	France
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S2666-688X(25)00057-7
Publisher Item Identifier	S2666-688X(25)00057-7
Digital object identifier(DOI)	10.1016/j.ejvsf.2025.08.005
Number of References	14
Device Tradenames	Fenestrated Anaconda (Terumo Aortic, United Kingdom)
Device Manufacturers	Terumo Aortic, Terumo Aortic (United Kingdom)
CAS Registry Numbers	methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2)

100. [Clinical, functional, and computed tomographic characterization of idiopathic inflammatory myopathy-associated interstitial lung disease: a retrospective cohort study](#)

de Oliveira Filho J.R.B., Costa A.N., Baldi B.G., Wanderley M., Sawamura M.V.Y., Kairalla R.A.

J.Bras. Pneumol. 2025 51:4

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Objective: To evaluate clinical, functional, and CT characteristics, as well as disease progression, in patients with idiopathic inflammatory myopathy-associated interstitial lung disease (IIM-ILD) treated at a referral center in Brazil. Methods: This was a retrospective cohort study analyzing demographic characteristics, clinical variables, pulmonary function test results, HRCT findings, serological profiles, treatments, and outcomes. Results: Seventy-nine IIM-ILD patients were included in

the present study. The mean follow-up period was 8.7 ± 4.7 years. The most common diagnosis was antisynthetase syndrome, observed in 51 (64.5%) of the 79 patients. The most common symptoms were dyspnea (in 94.9%), arthralgia (in 82.2%), and muscle weakness (in 75.9%). Mean baseline FVC was 2.19 ± 0.75 L, corresponding to 62.5% of the predicted value. During follow-up, FVC showed significant improvement. The most common CT patterns were indeterminate (in 44.4%) and nonspecific interstitial pneumonia (in 35.4%). Treatment most frequently included prednisone (in 98.7%), azathioprine (in 92.3%), or methotrexate (in 57.7%). Overall survival was 84.8%. Mortality was higher among patients who developed pulmonary hypertension and those who required intravenous methylprednisolone pulse therapy. Conclusions: Most patients with IIM-ILD progress well with immunosuppressive therapy. Pulmonary hypertension and the need for methylprednisolone pulse therapy appear to be associated with higher mortality.

Drug Index Terms

(Based on full text)

azathioprine (adverse drug reaction, drug combination, oral drug administration, special situation for pharmacovigilance), corticosteroid (drug combination, special situation for pharmacovigilance), cyclophosphamide (intravenous drug administration, special situation for pharmacovigilance), cyclosporine (oral drug administration, special situation for pharmacovigilance), methotrexate (drug combination, oral drug administration, special situation for pharmacovigilance), methylprednisolone (intravenous drug administration, special situation for pharmacovigilance), mycophenolate mofetil (oral drug administration, special situation for pharmacovigilance), prednisone (oral drug administration, special situation for pharmacovigilance), rituximab (special situation for pharmacovigilance)

Non-drug Index Terms

(Based on full text)

adult, aged, amyopathic dermatomyositis, antisynthetase syndrome, arthralgia, article, body weight loss, cohort analysis, **computer assisted tomography**, coughing, **CT scanner**, cytopenia (side effect), dysphagia, dyspnea, echocardiography, female, fever, follow up, gastroesophageal reflux, human, **idiopathic inflammatory myopathy**, immunosuppressive treatment, **interstitial lung disease**, interstitial

pneumonia, leukemia, lung cancer, lung function test, lymphoma, major clinical study, melanoma, mortality, muscle weakness, myalgia, myositis, nausea side effect), observational study, polymyositis, prostate cancer, pulmonary hypertension, Raynaud phenomenon, retrospective study, salivary gland cancer, stomach cancer, uterine cervix cancer

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Bras. Pneumol.
ISSN	18063756 (electronic), 18063713
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-19 (Full record), 2025-11-14 (Article in Press/In process)
Publication Type	Article
Page Range	
Country of Author	Brazil
Country of Source	Brazil
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.36416/1806-3756/e20250123
MEDLINE PMID	41172412
Article Number	e20250123
Number of References	40

CAS Registry Numbers

azathioprine ([446-86-6](#), [55774-33-9](#))
cyclophosphamide ([50-18-0](#), [6055-19-2](#))
cyclosporine ([59865-13-3](#), [63798-73-2](#),
[79217-60-0](#))
methotrexate([15475-56-6](#), [59-05-2](#), [7413-34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#),
[60388-53-6](#))
methylprednisolone ([6923-42-8](#), [83-43-2](#))
mycophenolate mofetil ([116680-01-4](#),
[128794-94-5](#), [115007-34-6](#))
prednisone ([53-03-2](#))
rituximab ([174722-31-7](#))

101. [When two truths collide: overlapping herpes zoster and leukocytoclastic vasculitis in an atypical presentation](#)

Cohn H., Hennessy J., Cotell S., Fabbro S.

[Article in Press] Diagnosis 2025 :

Embase, MEDLINE, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

Objectives: Diagnostic reasoning often favors simplicity, with clinicians seeking a single unifying explanation for a patient's presentation. However, in certain cases, a narrow approach can lead to diagnostic oversight and delay. In this case, an early attribution of symptoms to leukocytoclastic vasculitis (LCV) postponed the recognition and treatment of an underlying varicella-zoster virus (VZV) infection. Case presentation: An 88-year-old woman presented with palpable purpuric macules in a linear distribution along the right pretibial region, dorsum of the foot, and toes, accompanied by severe unilateral pain radiating from the foot to the buttock. The pain preceded the rash by one day, and no vesicles were noted by the patient or clinician. Clinically, the morphology was consistent with leukocytoclastic vasculitis, but the pain followed a dermatomal distribution, raising concern for varicella-zoster virus (VZV) infection. A biopsy ultimately confirmed both leukocytoclastic vasculitis and viral cytopathic changes. The patient improved with a combination of valacyclovir, prednisone, and gabapentin. Conclusions: This case illustrates a diagnostically challenging presentation in which two distinct pathologies mimicked and masked one another. Anchoring on a unifying diagnosis led to premature closure, delaying antiviral therapy. Herpes zoster without vesicles is uncommon but documented, and VZV-induced vasculitis is rare. Together, they created a deceptive clinical picture. The case reinforces several key teaching points: the value of maintaining diagnostic flexibility, the importance of empiric antiviral treatment when herpes zoster is suspected, and the need to recognize cognitive biases

that can distort clinical judgment. When findings are discordant or incomplete, clinicians should resist diagnostic simplification and remain open to diagnosing coexisting conditions.

Drug Index Terms

gabapentin, prednisone, valaciclovir

Non-drug Index Terms

adult, antiviral therapy, article, buttock, case report, chickenpox, **cognitive bias**, diagnosis, **diagnostic reasoning**, drug therapy, female, **herpes zoster**, human, **leukocytoclastic vasculitis**, oral drug administration, **prematurity**, purpura, rash, Varicella zoster virus, vasculitis

Comment: Invalid ICSR

ADDITIONAL INFORMATION

Abbreviated Journal Title	Diagnosis
ISSN	2194802X (electronic)
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-17 (Article in Press)
Publication Type	Article in Press
Page Range	
Country of Author	United States
Country of Source	Germany
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1515/dx-2025-0134
MEDLINE PMID	41204619
Number of References	6

102. [A Rare Case of Hepatosplenic \$\gamma\delta\$ T-Cell Lymphoma with Hemophagocytic Syndrome](#)

Jia W., Xu L.

Clin. Lab. 2025 71:11 (2124-2127)

Embase, MEDLINE, NURSING

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Embase Open URL: [redirect to full text](#)

Abstract

Background: Hepatosplenic $\gamma\delta$ T-cell lymphoma is a rare subtype of peripheral T-cell lymphomas. It is clinically marked by significant involvement of the liver and spleen, minimal to no lymphadenopathy, and typically follows an aggressive progression. This condition primarily affects adolescents and young adults, with a higher prevalence in males. There are only a limited number of reported cases of this kind of lymphoma associated with hemophagocytic syndrome. Methods: We report a rare case of a 35-year-old man, who was suffering from Hepatosplenic $\gamma\delta$ T-cell lymphoma combined with hemophagocytic syndrome. Results: The patient was ultimately diagnosed with Hepatosplenic $\gamma\delta$ T-cell lymphoma accompanied by hemophagocytic syndrome following a thorough diagnostic evaluation. Conclusions: In cases where patients exhibit atypical clinical features, the diagnosis of HSTL can solely be established by the relevant morphological, immunohistochemical, molecular, and cytogenetic information.

Drug Index Terms

(Based on full text)

cyclophosphamide (drug combination, drug therapy), doxorubicin (drug combination, drug therapy), etoposide (drug combination, drug therapy), prednisone (drug combination, drug therapy), vincristine (drug combination, drug therapy)

Non-drug Index Terms

(Based on full text)

adult, article, bone marrow suppression, case report, clinical article, decreased appetite, disease duration, disease severity, erythrophagocytosis, **hemophagocytic syndrome** (diagnosis, drug therapy), hepatography, hepatomegaly, **hepatosplenic gamma delta T cell lymphoma** (diagnosis, drug therapy), hepatosplenomegaly, hospital admission, hospital discharge, human, leukocytosis, male,

physical examination, **rare disease** (diagnosis, drug therapy), splenomegaly, thrombocytopenia, treatment outcome

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Clin. Lab.
ISSN	14336510
CODEN	CLLAF
Source Type	Journal
Source Publication Date	2025-01-01
Entry Date	2025-11-19 (Full record), 2025-11-17 (Article in Press/In process)
Publication Type	Article
Page Range	2124-2127
Country of Author	China
Country of Source	Germany
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.7754/Clin.Lab.2025.250357
MEDLINE PMID	41221762
Number of References	5
CAS Registry Numbers	cyclophosphamide (50-18-0 , 6055-19-2) doxorubicin (23214-92-8 , 25316-40-9) etoposide (33419-42-0 , 433304-61-1) prednisone (53-03-2) vincristine (57-22-7)

103. [**Bullous Pemphigoid as an Adverse Event of Checkpoint Inhibitor Therapy - Case Report**](#)

Radosavljević J., Salatić I., Mijušković Ž., Petrov N., Dinić M., Kandolf L.

Serb. J.Dermatol. Venerol. 2024 13:2 (97-103)

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Checkpoint inhibitors, such as the anti-PD-1 antibody nivolumab and the anti-CTLA-4 antibody ipilimumab, have revolutionized cancer treatment over the past decade. By releasing immune checkpoints, they enhance anti-tumor cellular immunity and have shown durable responses in several malignancies, including metastatic melanoma, renal cell carcinoma, and both squamous and non-squamous non-small-cell lung cancer. However, the non-specific activation of the immune system frequently leads to immune-related adverse events, which may affect any organ system, most commonly the skin. The most frequent cutaneous adverse events are maculopapular rash and pruritus, while rare but severe life-threatening toxicities, such as blistering diseases like bullous pemphigoid, may also occur. We present a case of bullous pemphigoid in a patient treated with combination of nivolumab and ipilimumab for metastatic melanoma. Early recognition and prompt management of immune-related adverse effects are critical to minimizing their severity and duration, ensuring treatment adherence, and preserving the therapeutic efficacy of immune checkpoint inhibitors.

Original (Non-English) Title

Bulozni Pemfigoid Kao Neželjeni Efekat Terapije Nivolumabom - Prikaz Slučaja

Drug Index Terms

(Based on full text)

C reactive protein (endogenous compound), complement component C3 (endogenous compound), immunoglobulin G (endogenous compound), **ipilimumab** (adverse drug reaction, drug combination, drug therapy, special situation for pharmacovigilance, unexpected outcome of drug treatment), methylprednisolone (intravenous drug administration, special situation for pharmacovigilance), **nivolumab** (adverse drug reaction, drug combination, drug therapy, special situation for pharmacovigilance, unexpected outcome of drug

treatment), prednisone (drug combination, oral drug administration), triamcinolone acetonide (special situation for pharmacovigilance, topical drug administration)

Non-drug Index Terms

(Based on full text)

aged, anemia, article, axillary lymph node, basement membrane, blister, bone metastasis, brain metastasis, **bullous pemphigoid** (diagnosis, side effect), cancer staging, case report, **checkpoint inhibitor therapy**, clinical article, clinical examination, drug dose reduction, drug withdrawal, eosinophil, eosinophilia, erythrocyte sedimentation rate, excision, excoriation (injury), histiocyte, histopathology, hospital discharge, hospitalization, human, human cell, human tissue, immunofluorescence, inflammatory infiltrate, lower limb, lung metastasis, lymph node dissection, lymph node metastasis, lymphocytic infiltration, male, mediastinum lymph node, metastatic melanoma (diagnosis, drug therapy), monotherapy, mortality, multiple cycle treatment, parietal bone, protein blood level, rash, remission, scalp, skin biopsy, skin bleeding, skin pruritus, trunk, upper limb, whole body CT

Comment: Valid ICSR

Additional Information

Abbreviated Journal Title	Serb. J.Dermatol. Venerol.
ISSN	24060631 (electronic), 18210902
Source Type	Journal
Source Publication Date	2024-12-01
Entry Date	2025-11-18 (Full record), 2025-11-12 (Article in Press/In process)
Publication Type	Article
Page Range	97-103
Country of Author	Serbia
Country of Source	Poland
Language of Article	English, Bosnian
Language of Summary	English, Bosnian
Digital object identifier (DOI)	10.2478/sjdv-2024-0009

Number of References	20
CAS Registry Numbers	C reactive protein (9007-41-4) complement component C3 (80295-41-6) immunoglobulin G (97794-27-9 , 308067-58-5) ipilimumab (477202-00-9) methylprednisolone (6923-42-8 , 83-43-2) nivolumab (946414-94-4) prednisone (53-03-2) triamcinolone acetonide (76-25-5)

104. [Polyarteritis nodosa \(PAN\) of renal blood vessels in adolescent - case report](#)

Majce A.S., Saraga M., Srsen S., Jelusić M., Kovačević T., Ardalić T.C., Markić J., Polić B., Durdov M.G., Tot A., Ljubanović D.G., Arapović A.

Pediatr. Nephrol. 2024 39: (S59)

Embase

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Abstract

Aims/Purpose: Polyarteritis nodosa (PAN) is a necrotizing vasculitis of small- and medium-sized arteries that most often affects the kidneys, heart, and liver but can affect any organ system. The disease is rare in children, and its epidemiology probably imprecise due to poorly defined diagnostic criteria of the illness in the young, and the considerable degree of overlap in the clinical features of the vasculitis in childhood. **Methods:** We presented a seventeen-year-old adolescent diagnosed with autism and epilepsy. He was admitted to our clinic after an epileptic seizure with severe impairment of consciousness. Urgent diagnostic workup confirmed a hypertensive emergency (HE) (arterial pressure 200/140 mmHg) and acute renal failure (urea 43.7 mmol/L, creatinine 1862 umol/L, anuria). **Results:** Radiological diagnostics of the brain imaging revealed acute changes consistent with Posterior Reversible Encephalopathy Syndrome (PRES) in the context of HE. Renal ultrasound showed diffusely altered cortex of both kidneys with

very poor perfusion, leading to kidney biopsy. Light microscopy findings suggested acute tubulointerstitial nephritis and acute post-infectious granulomatous glomerulonephritis. Ophthalmological fundus examination described bilateral optic nerve papilla with indistinct borders and abundant peripapillary retinal haemorrhages. Cardiac ultrasound diagnosed mild concentric left ventricular hypertrophy, diastolic dysfunction, and grade I aortic valve insufficiency. Treatment was initiated in the intensive care unit where he was monitored, sedated, mechanically ventilated, and underwent continuous veno-venous hemodiafiltration (CVVHDF) and antihypertensive therapy (urapidil). Three pulse doses of methylprednisolone were administered, followed by prednisone therapy. Doppler ultrasound of blood vessels revealed thickening and narrowing of radial artery walls. Subsequent electron microscopy findings described necrotizing vasculitis affecting muscular arteries without involvement of glomeruli, consistent with medium-sized vessel vasculitis - polyarteritis nodosa. PET-CT scan didn't show active vasculitis (it was done after one month of steroid therapy). Treatment with cyclophosphamide was initiated after definitive diagnosis. Following the first dose of cyclophosphamide, he developed a grand mal epileptic seizure, which was terminated with diazepam, a similar event occurred after the sixth cycle of cyclophosphamide. In 2023, he was referred to an adult rheumatologist, and treatment with mycophenolate mofetil was initiated. Estimated glomerular filtration rate (Bedside Schwartz formula) was 20 ml/min/1,7m². Conclusion: Here is described seventeen-year-old adolescent with autism and epilepsy whom is diagnosed very rare disease in children - polyarteritis nodosa.

Drug Index Terms

(Based on full text)

creatinine, cyclophosphamide, diazepam, methylprednisolone, mycophenolate mofetil, prednisone, urapidil, urea

Non-drug Index Terms

(Based on full text)

acute kidney failure, adolescent, adult, antihypertensive therapy, anuria, aortic regurgitation, arterial pressure, autism, case report, child, clinical article, clinical feature, conference abstract, continuous hemodiafiltration, diagnosis, diastolic dysfunction, Doppler ultrasonography, drug therapy, electron microscopy, epilepsy,

estimated glomerular filtration rate, eye fundus, glomerulonephritis, grand mal epilepsy, human, intensive care unit, interstitial nephritis, kidney biopsy, **kidney blood vessel**, left ventricular hypertrophy, light microscopy, male, necrotizing arteritis, nephritis, neuroimaging, **polyarteritis nodosa**, positron emission tomography-computed tomography, posterior reversible encephalopathy syndrome, radial artery, retina hemorrhage, seizure, special situation for pharmacovigilance, steroid therapy, vasculitis

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Nephrol.
ISSN	0931041X
Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
Entry Date	2025-11-20 (Full record)
Publication Type	Conference Abstract
Page Range	S59
Country of Author	Croatia
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	creatinine (19230-81-0 , 60-27-5) cyclophosphamide (50-18-0 , 6055-19-2) diazepam (439-14-5 , 11100-37-1 , 53320-84-6) methylprednisolone (6923-42-8 , 83-43-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2)

urapidil ([34661-75-1](#))

urea ([57-13-6](#))

105. [Pegcetacoplan for the Treatment of Pediatric Complement 3 Glomerulonephritis: A Case Report](#)

Guzman G.L.

Pediatr. Nephrol. 2024 39: (S157-S158)

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Abstract

Aims/Purpose: Complement 3 glomerulopathy (C3G) is an ultrarare glomerular disease that includes two subgroups, C3 glomerulonephritis (C3GN) and dense deposit disease (DDD), both of which involve dysregulation of the complement system. Amplification and propagation of the complement cascade results in downstream assembly of the membrane attack complex (C5b-9) and ultimately culminates in glomerular deposition of C3 and C5 breakdown products and subsequent renal parenchymal damage. Without targeted disease-modifying medications, the prognosis for C3G is poor. Pegcetacoplan binds to C3 and its activation fragment C3b, thus controlling the cleavage of C3 and generation of downstream effectors. This case report describes our experience using pegcetacoplan in a pediatric patient for treatment resistant C3GN. **Methods:** Narrative report of a pediatric case from a US referral hospital. **Results:** A previously healthy 9-year-old boy was referred to our center for a second opinion and evaluation of refractory membranoproliferative glomerulonephritis following several inpatient admissions for acute kidney injury and edema. Despite treatment with high dose of prednisone 40 mg/day and mycophenolate mofetil 500 mg twice daily for immunosuppression and amlodipine, lisinopril, hydrochlorothiazide, and fluid restriction for difficult-to-manage edema, he continued to have nephrotic range proteinuria, with a urine protein-to-creatinine ratio (uPCR) of 10 g/g, a serum creatinine of 0.4 mg/dL, a low serum C3 level (35 mg/dL), and anasarca. Given the concern for refractory C3GN following a steroid taper and tacrolimus trial with modest response (reduced proteinuria), we initiated pegcetacoplan 540 mg twice weekly for 1 week followed by

648 mg twice weekly. Laboratory values before pegcetacoplan initiation included a uPCR of 1.1 g/g, serum creatinine of 0.87 mg/dL, serum albumin of 4.7 g/dL, and serum C3 level of 30 mg/dL. Clinically significant improvements in serum C3 levels (142 mg/dL) and uPCR (422 mg/g) were observed within 1 week (Figure 1). Within 3 months of starting pegcetacoplan, all immunosuppressive and antihypertensive medications were discontinued completely. No adverse effects of pegcetacoplan were reported. A kidney biopsy after 6 months of pegcetacoplan treatment showed mesangial and focal endocapillary proliferative glomerulonephritis with isolated C3c deposition by immunofluorescence, consistent with previous C3GN diagnosis. However, this biopsy showed a reduction in endocapillary hypercellularity, capillary wall double contour formation, and glomerular C3c deposition by immunofluorescence. Conclusion: In this pediatric patient, compassionate use of pegcetacoplan was associated with rapid clinical improvement without adverse effects, and clinical effectiveness was confirmed by laboratory and histologic results within 6 months of treatment initiation.

Drug Index Terms

(Based on full text)

amlodipine, **complement component C3**, complement component C3b, complement component C5b, complement membrane attack complex, hydrochlorothiazide plus lisinopril, mycophenolate mofetil, **pegcetacoplan**, prednisone, serum albumin, steroid, tacrolimus

Non-drug Index Terms

(Based on full text)

acute kidney failure, anasarca, antihypertensive therapy, case report, clinical article, clinical effectiveness, compassionate use, complement system, conference abstract, creatinine blood level, diagnosis, drug megadose, drug therapy, edema, **glomerulonephritis**, glomerulopathy, glomerulus, histology, human, human tissue, hyperplasia, immune deficiency, immunofluorescence, immunohistochemistry, immunosuppressive treatment, kidney biopsy, kidney parenchyma, membranoproliferative glomerulonephritis, patient referral, pediatric patient, proliferative glomerulonephritis, protein creatinine ratio, proteinuria, special situation for pharmacovigilance, therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr. Nephrol.
ISSN	0931041X
Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
Entry Date	2025-11-20 (Full record)
Publication Type	Conference Abstract
Page Range	S157-S158
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	amlodipine (88150-42-9) complement component C3 (80295-41-6) complement component C5b (80295-55-2) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) pegcetacoplan (2019171-69-6) prednisone (53-03-2) serum albumin (9048-46-8) tacrolimus (104987-11-3 , 109581-93-3)

106. [Can We Predict Which Children with Idiopathic Nephrotic Syndrome Will Be Steroid Dependent](#)

Lazar A., Zion E., Gurevich E., Dagan A.
Pediatr. Nephrol. 2024 39: (S163)
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Abstract

Aims/Purpose: Around half of children with idiopathic nephrotic syndrome will develop steroid dependent or frequent relapsing disease. Guidelines recommend switching these patients to immunosuppressive therapy to minimize side effects of corticosteroids and achieve remission. Clinicians often wait to diagnose steroid dependence until the patient relapses more than once while on steroids. Finding an early, reliable predictor of steroid dependent nephrotic syndrome can help clinicians to confidently initiate immunosuppressive therapy earlier in these patients, thus avoiding additional steroid courses. We hypothesize that a single relapse on a steroid dose of at least 0.5 mg/kg every other day predicts future relapses on this dose or higher. **Methods:** Children diagnosed with nephrotic syndrome in a single center were enrolled prospectively. We performed a post hoc analysis comparing the outcomes of patients with relapses on prednisone doses of 0.5 mg/kg/every other day or higher to those that relapsed while on lower doses or no steroids. **Results:** Of the 122 enrolled patients with nephrotic syndrome, 51/122 (42%) relapsed on doses of at least 0.5 mg/kg/every other day with 29/51 (57%) of these patients relapsing for the first time during induction. Fourteen of these patients were switched to alternative therapy before they experienced another relapse. The other 36 patients that relapsed on doses beyond this threshold were not switched immediately to steroid alternatives. The vast majority, 33/36 (92%) went on to have at least one more relapse above this threshold and were then switched to alternate therapy. **Conclusions:** Patients that relapsed once on steroid doses equal to or higher than 0.5 mg/kg/alternate days are very likely to continue to relapse on similar or higher doses of steroids. It might thus be reasonable to consider initiation of immunosuppression following the first relapse on steroid doses beyond this cut-off.

Drug Index Terms

(Based on full text)

corticosteroid, prednisone, **steroid**

Non-drug Index Terms

(Based on full text)

child, conference abstract, diagnosis, drug megadose, drug therapy, female, human, immune deficiency, immunosuppressive treatment, low drug dose, major clinical study, male, **nephrotic syndrome**, post hoc analysis, recurrent disease, relapse, remission, side effect, special situation for pharmacovigilance

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Nephrol.
ISSN	0931041X
Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
Entry Date	2025-11-20 (Full record)
Publication Type	Conference Abstract
Page Range	S163
Country of Author	Israel
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472
CAS Registry Numbers	prednisone (53-03-2) -9

107. [Case report: IgA Vasculitis Nephritis Associated with Nephrotic Syndrome and Rapidly Progressive Glomerulonephritis](#)

Libinson-Zebegret I., Toder M.P., Tibi R., Yakubov R., Pollack S., Magen D.

Pediatr. Nephrol. 2024 39: (S164-S165)

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Abstract

Background: IgA Vasculitis (Henoch Schonlein Purpura) is the most common small vessel vasculitis during childhood. It is characterized by a purpuric rash, arthritis/arthralgia and abdominal pain. Approximately 50% of patients develop renal manifestations termed as IgA Vasculitis Nephritis (IgAVN). These most commonly including isolated microscopic hematuria or non-nephrotic proteinuria, while fullblown nephrotic syndrome affect ~20% of cases. Rapidly Progressive Glomerulonephritis (RPGN) is the rarest manifestaion of IgAVN.

Methods: A 5-years-old, previously healthy male, developed nephrotic range proteinuria two weeks after presenting with a typical purpuric rash, arthritis of the ankles, abdominal pain, and normal urinalysis. Within two weeks from initial presentation, proteinuria up to 5 gr/day, hypoalbuminemia up to 2.9 mg/dL, hypertriglyceridemia, and hypercholesterolemia were abruptly followed by worsening of kidney functions with increase of creatinine up to 1.66 mg/dL. An urgent kidney biopsy showed more than 50% cellular crescents, extensive endocapillary proliferation, fibrinoid necrosis, and acute tubular injury, with massive mesangial deposition of C3C IgA. In accordance with KDIGO-2021 and SHARE-2019 guidelines for the treatment of IgAVN associated with RPGN, the patient was managed with intra-venous (IV) pulse methylprednisolone followed by IV Cyclophosphamide every two weeks and oral prednisone.

Results: Within two weeks under this therapeutic combination, there was a dramatic improvement of kidney function, with a decrease of serum creatinine to 0.48 mg/dL, but yet with clinical and laboratory signs of ongoing nephrotic syndrome.

Conclusion: RPGN is rare presentation of IgAVN. Our case presents rapid improvement of kidney function in response to combination therapy of IV high dose glucocorticoid and cyclophosphamide therapy. The long-term effect of this regimen on resolution of nephrotic syndrome and remains to be seen.

Drug Index Terms

(Based on full text)

creatinine, cyclophosphamide, glucocorticoid, methylprednisolone, prednisone

Non-drug Index Terms

(Based on full text)

abdominal pain, **anaphylactoid purpura**, arthralgia, case report, clinical article, conference abstract, creatinine blood level, drug therapy, fibrinoid necrosis, hematuria, human, hypercholesterolemia, hypertriglyceridemia, hypoalbuminemia, intravenous drug administration, kidney biopsy, kidney function, male, **nephritis**, nephrosis, **nephrotic syndrome**, proteinuria, purpuric rash, **rapidly progressive glomerulonephritis**, small vessel vasculitis, therapy, urinalysis

Comment: Invalid ICSR

Additional Information

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Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
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Publication Type	Conference Abstract
Page Range	S164-S165
Country of Author	Israel
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	creatinine (19230-81-0 , 60-27-5) cyclophosphamide (50-18-0 , 6055-19-2) methylprednisolone (6923-42-8 , 83-43-2) prednisone (53-03-2)

108. [Updates in ANCA - associated vasculitis](#)

Petruzzelli L.A., Rizzo S., Romano P., Carucci M., Serio V., De Luca A., Luongo I., Giannattasio P., Malgieri G.

Pediatr. Nephrol. 2024 39: (S257)

Embase

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Embase Open URL: [redirect to full text](#)

Abstract

Background: Antineutrophil Cytoplasmic Antibodies (ANCA) associated vasculitis (AAV) is a rare entity in childhood. However, renal involvement is frequent in up to 90% of cases but is not well discussed in literature. **Objectives:** The aim of this retrospective study is to describe cases of AAV among the population of Pediatric Nephrology Santobono children's Hospital. **Methods:** We analyzed ten patients diagnosed as AAV, in accordance with EULAR/PReS criteria. We evaluated renal function at onset, ANCA antibody titer, renal biopsy findings, proteinuria and hematuria at onset, induction and maintenance therapy performed, need for hemodialysis, need for plasmapheresis, outcome. **Results:** All patients presented with acute renal failure at onset with average value of Schwartz eGFR of 32 mL/min. They showed proteinuria in non-nephrotic range with average value of 1800 mg/day. Glomerular hematuria was found in all patients. Renal ultrasound showed cortical hyper echogenicity in 70% of patients in absence of other anomalies. Three patients required at least one hemodialysis session at onset. All patients required renal biopsy with the finding of ANCA glomerulonephritis associated with extracapillary proliferation and glomerular sclerosis in more than 50% of the glomeruli in 70% of patients. All patients analyzed were ANCA-positive: 5 were affected by microscopic polyangiitis, 5 by granulomatosis with polyangiitis. As induction therapy, all patients underwent steroid pulses; in addition, Cyclophosphamide was administered in 6 patients, while Rituximab in 2 others. In a single patient, plasmapheresis was performed in association with steroid pulses and Rituximab. 70% of patients underwent in complete remission. The median follow-up of the population was 115 months instead of 70 months of those treated with Rituximab +/- plasmapheresis. At the last follow-up only 3 patients had chronic kidney disease (CKD) (one patient went on hemodialysis replacement therapy, one patient had renal transplant and one was

classified as stage V CKD). Conclusions: Renal involvement was highlighted in all the patients. Patients treated with Rituximab and/or plasmapheresis, in comparison to those treated with Prednisone plus Cyclophosphamide, did not show relapses after remission. However, the number of cases is too small, and the followup of patients treated with Rituximab and/or plasmapheresis is shorter than those treated with Cyclophosphamide and Prednisone. The renal outcome shows a recovery rate of 70% and this data appears to be more favorable compared with adult patients. 70% of patients do not require renal replacement therapy, showing that AAV if diagnosed in time, can be controlled using current therapeutic approaches.

Drug Index Terms

(Based on full text)

cyclophosphamide, **neutrophil cytoplasmic antibody**, prednisone, rituximab

Non-drug Index Terms

(Based on full text)

acute kidney failure, adult, aged, **ANCA associated vasculitis**, antibody titer, child, chronic kidney failure, clinical article, conference abstract, drug therapy, female, focal glomerulosclerosis, follow up, glomerulonephritis, glomerulosclerosis, glomerulus, hematuria, hemodialysis, human, kidney biopsy, kidney function, kidney graft, maintenance therapy, major clinical study, male, microscopic polyangiitis, middle aged, nephrosis, plasmapheresis, proteinuria, remission, renal replacement therapy, retrospective study, special situation for pharmacovigilance, substitution therapy, therapy, Wegener granulomatosis

Comment: Invalid ICSR

Additional Information

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Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27

Source Publication Date	2024-09-01
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Publication Type	Conference Abstract
Page Range	S257
Country of Author	Italy
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	cyclophosphamide (50-18-0 , 6055-19-2) prednisone (53-03-2) rituximab (174722-31-7)

109. [Initial treatment of idiopathic nephrotic syndrome in children with mycophenolate mofetil vs. prednisone: A prospective, randomized, controlled, multicenter, open, phase III, non-inferiority study \(INTENT-Study\) - basis for amending the standard?](#)

Benz M.

Pediatr. Nephrol. 2024 39: (S166-S167)

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Embase Open URL: [redirect to full text](#)

Abstract

Aims/Purpose: Initial treatment of idiopathic nephrotic syndrome in children requires sufficient immunosuppressive therapy to induce and sustain remission and consists of a prolonged course with glucocorticoids. Even though being effective, this treatment is associated with pronounced glucocorticoid associated toxicity. Mycophenolate mofetil (MMF) is effective in sustaining remission in patients with frequently relapsing or glucocorticoid dependent nephrotic syndrome. The hypothesis of the INTENT-Study was that MMF is not inferior to standard therapy with glucocorticoids (GC) in the initial

treatment of steroid-sensitive nephrotic syndrome (SSNS) in children with regard to maintenance of remission and subsequent recurrence rate. Methods: 272 children (mean age at onset 4.1 ± 2.3 years; 64.3% males) with a first episode of SSNS were randomized to either standard treatment (12 weeks of GC) or experimental treatment (MMF only after induction of remission with GC for a total treatment period of 12 weeks). Primary endpoint was occurrence of treated relapse within follow-up of 24 months after completion of initial treatment. Secondary endpoints included i.a. course of the disease and drug toxicities. Results: MMF was not inferior to GC treatment in terms of the primary end-point (imputed mITT set: relapse rate 79.1% (MMF group) vs. 74.8% (GC group), difference 4.3% [-4.2%;12.7%], $p=0.019$; imputed per protocol set: relapse rate 79.2% (MMF group) vs. 77.7% (GC group), difference 1.5% [-7.7%;10.8%], $p=0.008$). In the MMF arm, there were fewer glucocorticoid-related side effects, such as lower blood pressure and body mass index as well as less frequent psychological abnormalities and cushingoid appearance. Cytopenias were more frequent in the MMF group but overall rare and mild. The rate of frequently relapsing nephrotic syndrome in the follow-up was comparable between the groups (MMF: 47.2%, GC: 45.2%). Conclusion: The presented results of the INTENT-Study show non-inferiority of the MMF arm to the standard GC arm with no safety concerns and fewer glucocorticoid-related side effects, providing an excellent extension of the evidence base for future patient-centered shared therapeutic decision-making.

Drug Index Terms

(Based on full text)

glucocorticoid, mycophenolate mofetil, prednisone

Non-drug Index Terms

(Based on full text)

adult, adverse drug reaction, body mass, child, conference abstract, controlled study, cushingoid habitus, cytopenia, drug therapy, drug toxicity, female, follow up, human, hypotension, immunosuppressive treatment, major clinical study, male, multicenter study, **nephrotic syndrome**, onset age, randomized controlled trial, recurrence risk, relapse, remission, serum group Gc system, side effect, therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr. Nephrol.
ISSN	0931041X
Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
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Publication Type	Conference Abstract
Page Range	S166-S167
Country of Author	Germany
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2)

110. [Therapeutic drug monitoring improves the efficacy of mycophenolate mofetil in children with steroid dependent and frequently relapsing nephrotic syndrome](#)

Bernardi S., Puccio G., Nittoli T., Castelli P., Prevot M., Boistault M., Grapin M., Boyer O., Montini G., Morello W.

Pediatr. Nephrol. 2024 39: (S229-S230)

Embase

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Abstract

Aims/Purpose: The aim of this retrospective and observational study was to assess if mycophenolic acid (MPA) therapeutic drug monitoring (TDM), performed by trough levels, improves the efficacy of Mycophenolate Mofetil (MMF) in maintaining remission at 6 months in children with frequently relapsing or steroid-dependent nephrotic syndrome (FRNS/SDNS). **Methods:** We included FRNS or SDNS patients, aged between 1-18 years, and treated with MMF as the first steroid-sparing agent with a follow-up >6 months. Patients were enrolled in the Pediatric Nephrology Units of Ospedale Maggiore Policlinico of Milan (Italy) and Necker-Enfants Malades Hospital in Paris (France) from January 2013 to December 2022. Children were divided in two groups: TDM, if MPA trough levels were monitored and NO-TDM, if trough levels were not performed. In the TDM group, MPA levels were monitored 1 month after the introduction and on a 3-months basis thereafter. MMF doses were modified to maintain MPA trough levels >3 mcg/ml, unless toxicity occurred. **Results:** 167 patients were included, 90 in the TDM and 77 in the NO-TDM group. Relapse-free survival was significantly different in the 2 groups (TDM 73.3% vs no-TDM 55.5%; $p=0.018$) (Figure 1). After correcting for confounders (numbers of relapses 1 year before MMF, age at MMF introduction, ethnicity and mean daily dose of prednisone [PDN] before MMF), the relationship between TDM and relapse-free survival remained statistically significant ($p=0.002$). Accordingly, TDM patients received lower doses of PDN after the introduction of MMF (median PDN/ year, TDM 463 mg/sqm vs NO-TDM 814 mg/sqm; $p=0.02$). In the TDM group, 72.2% of patients increased the initial MMF doses according to trough levels, while only 11.7% of children in the NO-TDM group increased the initial dose ($p<0.00001$). Reported side effects were similar in both groups (20% of patients in group TDM, 22% in group NO TDM). Finally, in the TDM group, children who did not relapse within the first six months of treatment had higher mean MPA trough levels (Figure 2) and higher maximum trough levels than those who relapsed ($p=0.002$).

Conclusion: MPA trough levels improved by around 20% the efficacy of MMF in maintaining remission at 6 months in children with FRNS or SDNS. It is safe and effective to use personalized and increased doses of MMF, even when exceeding 1200 mg/sqm/day in order to maintain MPA levels above 3 mcg/ml. We suggest starting with an initial dose of MMF of 1200 mg/sqm/day and then using MPA trough levels to adjust the dosage. If TDM by trough levels is not available, MMF doses >1200 mg/sqm/day should be tried, when relapses occur. Future studies

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Entry Date	2025-11-20 (Full record)
Publication Type	Conference Abstract
Page Range	S229-S230
Country of Author	Italy
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472

CAS Registry Numbers	mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) mycophenolic acid (23047-11-2 , 24280-93-1 , 37415-62-6) prednisone (53-03-2)
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111. [Ifosfamide-induced Fanconi Syndrome in Paediatrics: A Review of cases in a tertiary hospital](#)

Alves M.E.C., Saraiva B.M., Sousa A., Costa M.S.-M., Borges M.A.,
Baptista R., Neto G., Telma C.F.

Pediatr. Nephrol. 2024 39: (S106-S107)

Embase

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Abstract

Purpose: Ifosfamide (IFO)-induced renal toxicity often presents as Fanconi syndrome (FS) and/or reduction in glomerular filtration rate (GFR). While most cases resolve after completion of chemotherapy (CTX), FS may be detected several months after exposure. If left untreated, metabolic bone disease may develop. We present 3 cases of FS secondary to IFO. Methods: We collected data from patients over the last ten years who presented with ifosfamide-induced Fanconi Syndrome at our hospital. Results: Case 1: A 2-year-old boy with Burkitt lymphoma and GFR of 60 mL/min/1.73m² was treated with vincristine (VCR), cyclophosphamide, doxorubicin, and prednisone. He relapsed 3 months later, which prompted a salvage regimen with rituximab, IFO, carboplatin and etoposide, followed by allogeneic stem cell transplantation. During relapse treatment, downward crossing of weight and height centiles, metabolic acidosis and electrolyte disturbances were noted. FS was diagnosed and adequate supplementation was initiated with improvement of the patient's condition. Treatment was stopped 5 years later. Case 2: A 17-monthold boy with bladder rhabdomyosarcoma causing bilateral obstructive kidney disease underwent decompressive percutaneous nephrostomy and 4 cycles of IFO, VCR, actinomycin and doxorubicin. GFR before CTX initiation was 75 mL/min/1.73m². Following treatment, tumor enlargement occurred. Biochemical disturbances consistent with FS were noted and the

patient started oral supplementation. A second CTX protocol with VCR, irinotectan and temozolamide was performed. To date, the patient maintains osteopenia, supplemented with vitamin D and phosphorus. Case 3: A 6-year-old boy with anaplastic nephroblastoma and preserved renal function underwent nephroureterectomy and CTX protocol with VCR, dactinomycin, and doxorubicin. After one year, he relapsed and was treated with 6 rounds of IFO, carboplatin and etoposide. One year after finishing CTX he reported general malaise, and biochemical workup was consistent with FS, with plasma phosphate of $<0,7$ mg/ dL. However, the patient lost follow up. At 15 years of age, he presented multiple pathologic fractures and weight and height at -6,2 and -7,6 SDS, respectively. Rickets secondary to previous FS was assumed, currently with normal plasma phosphate. The patient was started on vitamin D and zoledronic acid. Conclusions: These cases illustrate the importance of close monitoring to identify renal toxicity of IFO as early as possible to allow adequate supplementation. Follow up must include detailed CTX protocols, growth, blood pressure evaluation, and laboratorial surveillance of kidney function and signs of tubulopathy.

Drug Index Terms

(Based on full text)

carboplatin, cyclophosphamide, dactinomycin, doxorubicin, etoposide, **ifosfamide**, phosphorus, prednisone, rituximab, temozolamide, vincristine, vitamin D, zoledronic acid

Non-drug Index Terms

(Based on full text)

adverse drug reaction, allogeneic stem cell transplantation, bladder sarcoma, Burkitt lymphoma, case report, child, clinical article, conference abstract, **cystinosis**, diagnosis, drug combination, drug dose, drug therapy, electrolyte disturbance, **Fanconi renotubular syndrome**, follow up, glomerulus filtration rate, human, kidney function, kidney tubule disorder, malaise, male, metabolic acidosis, metabolic bone disease, nephroblastoma, nephrotoxicity, nephroureterectomy, oral drug administration, osteopenia, pathologic fracture, percutaneous nephrostomy, phosphate blood level, preschool child, relapse, special situation for pharmacovigilance, **tertiary care center**, therapy

Comment: Valid ICSR

Additional Information

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Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
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Publication Type	Conference Abstract
Page Range	S106-S107
Country of Author	Portugal
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	carboplatin (41575-94-4) cyclophosphamide (50-18-0 , 6055-19-2) dactinomycin (1402-38-6 , 1402-58-0 , 50-76-0) doxorubicin (23214-92-8 , 25316-40-9) etoposide (33419-42-0 , 433304-61-1) ifosfamide (3778-73-2) phosphorus (7723-14-0) prednisone (53-03-2) rituximab (174722-31-7) temozolomide (85622-93-1) vincristine (57-22-7) zoledronic acid (118072-93-8 , 131654-46-1 , 165800-06-6 , 165800-07-7)

112. [Acute kidney injury upon return from the tropics](#)

Gomez B., Pastor S.G., De la Torre Sanchez N., Lopez-Baez V.,
Martinez P.R., Fanjul J.R., Abad C.C., Rodrigo C., De Liria G.
Pediatr. Nephrol. 2024 39: (S143)

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Abstract

Introduction: Multiple tropical infectious diseases can cause acute kidney injury, with malaria being the most common. Different mechanisms have been described, including acute tubular necrosis due to established hypoperfusion or direct tubular damage, thrombotic microangiopathy, and immune dysfunction with acute tubulointerstitial nephritis (ATIN) or glomerulonephritis. Due to limited resources in developing countries, diagnosis can be delayed, leading to worsened renal prognosis. Case Presentation: We present the case of a 15-year-old girl who presented to our pediatric emergency department. Two weeks prior, in Equatorial Guinea, she experienced fever, diarrhea, and vomiting. Diagnosed with typhoid fever and malaria, she required hospital admission and received ibuprofen, ciprofloxacin, antimalarials, and ceftriaxone. During hospitalization, she underwent blood tests showing severe anemia and a renal ultrasound revealing bilateral renal hyperechogenicity. Renal function was not evaluated. The family requested discharge due to progressive clinical deterioration and traveled to Spain, where they sought care at our center. She presented with malaise, nausea, and abdominal pain. Blood tests revealed anemia without evidence of hemolysis, acute kidney injury with creatinine of 21.6 mg/dL, urea of 325 mg/dL, metabolic acidosis, and hyperphosphatemia with no other electrolyte abnormalities. Urine analysis showed mixed proteinuria (protein/creatinine ratio of 1.3 mg/mg) with mild microscopic hematuria and 50 leukocytes/field. Antigen detection for plasmodium falciparum was found positive but with negative blood smear. She was admitted to the pediatric ICU and started on continuous hemodiafiltration. A renal biopsy confirmed ATIN with eosinophil predominance. Treatment with prednisone 1 mg/kg/day was initiated with an excellent response, leading to discontinuation of extrarenal clearance within 72 hours and normalization of renal function within a week. Comments: ATIN is an immune-mediated cause

of acute kidney injury. Multiple etiologies have been described, with drugs being the main cause. Prognosis is generally favorable when the causative agent is withdrawn early. The use of corticosteroids is controversial but appears more effective if initiated within the first few days. Our patient had received multiple treatments known to cause ATIN. The presence of eosinophils is not specific but given the absence of active malaria (negative thick smear and no signs of hemolysis), drug-induced ATIN was suspected. Suspected treatments were excluded until lymphocyte activation testing could be performed.

Drug Index Terms

(Based on full text)

ceftriaxone, ciprofloxacin, corticosteroid, creatinine, ibuprofen, prednisone, urea

Non-drug Index Terms

(Based on full text)

abdominal pain, **acute kidney failure**, acute kidney tubule necrosis, adolescent, anemia, antigen detection, blood smear, case report, clinical article, conference abstract, continuous hemodiafiltration, diagnosis, diarrhea, drug therapy, emergency ward, Equatorial Guinea, female, fever, glomerulonephritis, hematuria, hemolysis, hospital admission, hospitalization, human, hyperphosphatemia, interstitial nephritis, kidney biopsy, kidney function, lymphocyte activation, malaise, malaria, metabolic acidosis, nausea, nephritis, oral drug administration, pediatric intensive care unit, protein creatinine ratio, proteinuria, special situation for pharmacovigilance, therapy, thrombotic microangiopathy, typhoid fever, urinalysis, vomiting

Comment: Invalid ICSR

Additional Information

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Conference Location	Spain, Valencia
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Publication Type	Conference Abstract
Page Range	S143
Country of Author	Spain
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	ceftriaxone (73384-59-5 , 74578-69-1 , 104376-79-6) ciprofloxacin (85721-33-1 , 86393-32-0 , 128074-72-6 , 128074-76-0 , 192934-52-4 , 93107-08-5 , 86483-48-9 , 96186-80-0) creatinine (19230-81-0 , 60-27-5) ibuprofen (15687-27-1 , 79261-49-7 , 31121-93-4 , 527688-20-6) prednisone (53-03-2) urea (57-13-6)

113. [Parvovirus B19 Infection related anemia in the early period of kidney transplantation: a case report](#)

Yildirim P.A., Basaran C., Soyaltin E., Camlar S.A., Kasap-Demir B.
 Pediatr. Nephrol. 2024 39: (S386)
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Abstract

Introduction: After kidney transplantation (KTx), anemia may occur in recipients for many reasons, including allograft dysfunction, dietary deficiencies, infections. However, Parvovirus B19 infection is less common than other causes. Here, we present a case of Parvovirus B19 infection that developed in the early period after KTx. Case: An 11-year-old female patient diagnosed with end stage renal failure due to

Stromme syndrome underwent peritoneal dialysis for 2 years and hemodialysis for the last 1 year. She underwent a living KTx from his mother. HLA mismatching showed no mismatches at A locus, 1 mismatches at B locus, and 1 mismatches at DR locus. The patient received induction with antithymocyte globulin (ATG) with a total dose of 5 mg/kg. The patient was discharged on postoperative 15th day with maintenance immunosuppression consisting of tacrolimus, mycophenolate sodium, and prednisone. During the post-transplant second month, the patient developed resistant anemia, even though her graft functions were normal. During this time, the hemoglobin level decreased to 6.7 g/dl from baseline 11.5 g/dl. In other laboratory studies; hematocrit %21.2, platelets 313000 mm³; reticulocyte relative 0.19% (0.5-2.0), LDH 159 µ/l, ferritin 557 ng/ml, B12 231 pg/ml and folate 3.7 ng/ml. B12 and folic acid replacements were initiated. As a result of the worsening anemia, Parvovirus B19 serology and a qualitative RT-polymerase chain reaction (PCR) for parvovirus B19 was studied. Parvovirus IgM was 102.6 U/mL positive and Parvovirus B19 IgG was negative. Peripheral blood Parvovirus B19 PCR resulted as 835,717,434 copies/ mL. The patient was started on intravenous immunoglobulin at a dose of 500 mg/kg for 4 doses. After treatment Hb was seen as 11,3 g/dL on the second week. Conclusions: Parvovirus B19 should be considered in the differential diagnosis of anemia, even in the early period after kidney transplantation. This emphasizes the importance of early screening and treatment of this potentially life-threatening complication.

Drug Index Terms

(Based on full text)

cyanocobalamin, ferritin, folic acid, hemoglobin, immunoglobulin G, immunoglobulin M, lactate dehydrogenase, mycophenolic acid, prednisone, tacrolimus, thymocyte antibody

Non-drug Index Terms

(Based on full text)

allograft, **anemia**, case report, chronic kidney failure, clinical article, conference abstract, diagnosis, differential diagnosis, drug administration, drug combination, drug dose, drug therapy, end stage renal disease, **erythema infectiosum**, female, hematocrit, hemodialysis, hemoglobin blood level, human, **Human parvovirus B1G**, immune deficiency, immunosuppressive treatment, intravenous drug

administration, kidney transplantation, male, oral drug administration, peritoneal dialysis, polymerase chain reaction, reticulocyte, serology, therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Nephrol.
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Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
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Publication Type	Conference Abstract
Page Range	S386
Country of Author	Turkey
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	cyanocobalamin (53570-76-6 , 68-19-9 , 8064-09-3) ferritin (9007-73-2) folic acid (59-30-3 , 6484-89-5) hemoglobin (9008-02-0) - immunoglobulin G (97794-27-9 , 308067-58-5) immunoglobulin M (9007-85-6) lactate dehydrogenase (9001-60-9) lactate dehydrogenase A mycophenolic acid (23047-11-2 , 24280-93-1 , 37415-62-6)

prednisone ([53-03-2](#))

tacrolimus ([104987-11-3](#), [109581-93-3](#))

114. [Nephrotic syndrome as a paraneoplastic syndrome](#)

Xhango O., Shtiza D., Kapllanaj M., Saraci B.

Pediatr. Nephrol. 2024 39: (S150)

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Abstract

Aims/Purpose: The association of nephrotic syndrome (NS) and different malignancies is well recognized, but few cases find a specific link between Hodgkin lymphoma(HL) and NS. It has been demonstrated that glomerular injury can be found as a complication in these malignancies. **Methods:** We report a very rare case of a child that NS preceded the diagnosis of HL by 12 months. **Case:** 7 years old girl diagnosed with NS, which responded very well to supportive care and steroids. There were no adenopathy or other manifestation of HL. No relapses during 12 months. When she was on minimal dose of prednisone, just 5 mg every other day, in a routine control, a right cervical lymphadenopathy was observed. The child was referred to oncohematologist. The diagnose of Hodgkin lymphoma was made by biopsy. **Results:** Actually the child is under treatment according to the protocol for HL. Her condition is stable in both terms (nephrologic and oncologic). **Conclusion:** This report raise awareness of a potential association between HL and NS in pediatric patients, for a careful evaluation and follow-up.

Drug Index Terms

(Based on full text)

prednisone

Non-drug Index Terms

(Based on full text)

adolescent, case report, cervical lymphadenopathy, child, clinical article, complication, conference abstract, diagnosis, drug combination, drug therapy, evaluation and follow up, female,

glomerulus, Hodgkin disease, human, human tissue, lymphadenopathy, **nephrotic syndrome**, **paraneoplastic syndrome**, pediatric patient, protocol

Comment: Invalid ICSR

Additional Information

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Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
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Source Publication Date	2024-09-01
Entry Date	2025-11-20 (Full record)
Publication Type	Conference Abstract
Page Range	S150
Country of Author	Albania
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	prednisone (53-03-2)

115. [Non lupus full house nephropathy: a case report](#)

Herrera R., Echeverria A., Gutierrez E.B., Marin A.A., Ponce C.O.
Pediatr. Nephrol. 2024 39: (S178)
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Abstract

Aims/Purpose: To describe the clinical presentation of a rare pathology for which there is no therapeutic consensus

Methods: Full house nephropathy refers to the detection of deposits of IgA, IgG, IgM, C3, and C1q in the renal biopsy with immunofluorescence (IF), almost pathognomonic of lupus nephritis. However, in patients with negative serology for autoantibodies, it is called non-lupus full house nephropathy. It is believed that their physiological bases may be similar, but more studies are required for prognosis and treatment purposes.

Results: A 12-year-old male began experiencing fever, nausea, vomiting, fainting, and dysarthria. A doctor took a capillary blood glucose level, which was reported as 170 mg/dl, and ordered additional tests to rule out diabetes mellitus. Hematuria (164cel) and nephrotic proteinuria (191.6mg/m²/h) were detected, without nephrotic syndrome or abnormalities in kidney function. Hypertension and glucosuria were ruled out. There is no family history or medication use reported. A renal biopsy was performed, which reported membranoproliferative glomerulonephritis mediated by immune complexes with full house immunofluorescence. Negative ANA, no complement consumption, 18 negative immunospecificities. Rheumatologic and infectious pathologies are ruled out. Renal ultrasound shows loss of corticomedullary relationship and left renal hypoplasia. Cranial MRI is normal. The prednisone was tapered off, and cyclophosphamide was started at a dose of 750 mg/m² (4 doses), along with mycophenolate acid at a dose of 750 mg/m² subcutaneously, and rituximab at a dose of 375 mg/m² subcutaneously (2 doses), with no response. An inhibitor of the angiotensin-converting enzyme (IECA) was started, and there was a decrease in massive proteinuria to nephrotic levels. Later, Tacrolimus was started at a dose of 0.1 mg/kg/day, and there was a significant decrease in nephrotic proteinuria (from 47 to 26.3 mg/m²/h).

Conclusion: Full house non-lupus nephropathy, treated with immunosuppressants as for SLE, few case series report favorable results; but improvement was observed in the corticosteroid-resistant nephrotic syndrome treatment algorithm. This report is made because it is infrequent, for prognostic and treatment purposes

1-3. HCE: Hypercellularity in the endocapillary region, leukostasis, and subendothelial deposits. Minimal tubular atrophy and interstitial fibrosis.

4. Masson: Hypercellularity in the endocapillary region, leukostasis, and subendothelial deposits.

5-7. Jones: Hypercellularity in the endocapillary region, leukostasis, and subendothelial deposits. Irregularities in the basement membranes with duplication. Minimal

tubular atrophy and interstitial fibrosis. 10. Immunofluorescence: Mesangial and subendothelial immunocomplex deposits.

Drug Index Terms

(Based on full text)

autoantibody, biological marker, complement component C1q, corticosteroid, cyclophosphamide, immunoglobulin G, immunoglobulin M, mycophenolate mofetil, mycophenolic acid, prednisone, rituximab, tacrolimus

Non-drug Index Terms

(Based on full text)

adult, antigen antibody complex, basement membrane, capillary blood, case report, clinical article, conference abstract, drug therapy, dysarthria, faintness, family history, female, fever, glucose blood level, glucosuria, hematuria, human, hyperplasia, hypertension, immunofluorescence, immunohistochemistry, immunosuppressive treatment, kidney biopsy, **kidney disease**, kidney function, kidney hypoplasia, leukostasis, lupus erythematosus nephritis, male, membranoproliferative glomerulonephritis, MRI scanner, nausea and vomiting, nephrosis, nephrotic syndrome, nuclear magnetic resonance imaging, proteinuria, serology, **systemic lupus erythematosus**

Comment: Invalid ICSR

Additional Information

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Publication Type	Conference Abstract
Page Range	S178
Country of Author	Mexico

Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	complement component C1q (80295-33-6) cyclophosphamide (50-18-0, 6055-19-2) immunoglobulin G (97794-27-9, 308067-58-5) immunoglobulin M (9007-85-6) mycophenolate mofetil (116680-01-4, 128794-94-5, 115007-34-6) mycophenolic acid (23047-11-2, 24280-93-1, 37415-62-6) prednisone (53-03-2) rituximab (174722-31-7) tacrolimus (104987-11-3, 109581-93-3)

116. [A severe hypocomplementemic glomerulonephritis requiring dialysis: when it rains, it pours!](#)

Manieri E., Scola C.L., Biscardi L., Anile L., Nardini B., Colombo C., d'Alanno G., Alberici I., Pillon R., Pasini A.

Pediatr. Nephrol. 2024 39: (S242)

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Abstract

Aims: We report a pediatric case of acute hypocomplementemic glomerulonephritis concurrent with pneumococcal pneumonia and nonsteroidal anti-inflammatory drug (NSAID) intake, presenting with renal failure and requiring dialysis. **Case report:** In February 2024, a 7-year-old boy with an unremarkable personal and family medical history suffered an episode of vomiting, abdominal pain, and fever which was treated with two doses of ketoprofen; an episode of pharyngotonsillitis with fever had occurred a few weeks before symptom onset. On

admission to the emergency department, blood and urine tests were performed which revealed renal failure (urea 114 mg/dl, creatinine 3.87 mg/dl) and complement consumption, negative antinuclear antibodies, and macrohematuria, indicative of acute hypocomplementemic glomerulonephritis. Chest X-ray showed signs of pneumonia, and urinalysis was positive for pneumococcal antigen. The association between pneumococcal pneumonia and concurrent hypocomplementemic glomerulonephritis has been described in the literature. He was hospitalized and experienced worsening renal function and decreased urine output, leading to anuria despite diuretic therapy. Peritoneal dialysis was initiated, which was discontinued after a week as his condition improved. Due to acute renal failure associated with acute glomerulonephritis, he received three doses of intravenous methylprednisolone for three days, followed by oral prednisone for seven days with rapid tapering. Kidney biopsy revealed an overall picture of diffuse acute glomerulonephritis, endocapillary proliferation (mainly neutrophilic granulocytes), with an 'exudative' pattern. Extensive areas of glomerular necrosis were noted, along with diffuse acute tubular necrosis and tubular hemoglobin casts. Immunofluorescence showed diffuse granular glomerular deposits, predominantly C3-dominant. Electronic microscopy showed a mesangial-proliferative/endocapillary necrotizing pattern and markedly electron-dense deposits. Renal function was significantly improved at discharge; complement levels normalized, however macrohematuria with nephrotic-range proteinuria persisted. Conclusion: Our patient presented with acute hypocomplementemic glomerulonephritis, renal failure, and anuria, requiring renal replacement therapy for seven days. Given the concurrent pneumococcal pneumonia, it is likely that the glomerulonephritis was secondary to the lung infection, as described in the literature. An alternative hypothesis is poststreptococcal glomerulonephritis, given a previous episode of febrile pharyngitis and the raised Antistreptolysin O titer, which could also be secondary to the inflammatory response to pneumonia. The presence of tubular damage and the serious kidney damage was probably caused by the concurrent use of NSAIDs and their associated risk of nephrotoxicity.

Drug Index Terms

(Based on full text)

antinuclear antibody, creatinine, hemoglobin, ketoprofen,

methylprednisolone, nonsteroid antiinflammatory agent, prednisone, urea

Non-drug Index Terms

(Based on full text)

abdominal pain, acute glomerulonephritis, acute kidney failure, acute kidney tubule necrosis, alternative hypothesis, anuria, case report, clinical article, conference abstract, controlled study, diagnosis, **dialysis**, diuretic therapy, drug therapy, emergency ward, family history, fever, glomerulonephritis, glomerulus, hematuria, human, human tissue, immunofluorescence, immunohistochemistry, intravenous drug administration, kidney biopsy, kidney cortex necrosis, kidney failure, kidney function, lung infection, male, **membranoproliferative glomerulonephritis**, microscopy, nephrotoxicity, neutrophil, oliguria, oral drug administration, peritoneal dialysis, pharyngitis, pneumococcal pneumonia, pneumonia, poststreptococcal glomerulonephritis, proteinuria, **rain**, renal replacement therapy, streptolysin titer, thorax radiography, urinalysis, vomiting

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Nephrol.
ISSN	0931041X
Source Type	Journal
Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
Entry Date	2025-11-20 (Full record)
Publication Type	Conference Abstract
Page Range	S242
Country of Author	Italy
Country of Source	Netherlands
Language of Article	English
Language of Summary	English

Digital object identifier (DOI) [10.1007/s00467-024-06472-9](https://doi.org/10.1007/s00467-024-06472-9)

CAS Registry Numbers creatinine ([19230-81-0](#), [60-27-5](#))
 hemoglobin ([9008-02-0](#))
 ketoprofen ([22071-15-4](#), [57495-14-4](#))
 methylprednisolone ([6923-42-8](#), [83-43-2](#))
 prednisone ([53-03-2](#))
 urea ([57-13-6](#))

117. [Atypical Hemolytic Uremic Syndrome triggered by Shiga toxin-producing Escherichia coli infection](#)

Alconcher L., Bronfen S., Lucarelli L.

Pediatr. Nephrol. 2024 39: (S152-S153)

Embase

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Abstract

Aim: Argentina has the highest incidence of Shiga-toxin producing E. coli Hemolytic Uremic Syndrome (STEC-HUS) in the world; therefore, in our country STEC is always the first etiology suspected faced with a patient with HUS. The aim of this case report is to raise awareness about the unusual situation in which STEC triggered an atypical HUS. Case Report: A 11- year-old boy was admitted to the hospital due to vomiting without diarrhea. Laboratory tests showed: hematocrit 28% hemoglobin 9g/dl, LDH 1415 IU/l, platelets 48000/mm³, schistocytes +, Urea 182 mg/ dl, creatinine 3.4 mg/dl, C3 50 mg/dl (RV: 90-120mg/dl). He became anuric and hemodialysis was initiated. PCR multiple for Shiga-toxin in stools in the local laboratory was negative. After 14 days without neither hematologic nor kidney improvement a dose of monoclonal anti C5 antibodies was prescribed. After that, we received from the National Reference Laboratory PCR Real Time in stools positive for Stx 2 and Lipopolysaccharide antibodies 0145 IgM- /IgG +. With these results eculizumab was withdrawn assuming that the patient had an STEC-HUS. The patient continued without neither hematologic nor kidney improvement and 2 weeks later further laboratory results showed very high titers of anti FH antibodies by ELISA (8500UA/ ml, RV < 100 UA/ml). Eculizumab was re-initiated, adding

prednisone and mofetil mycophenolate. Genetic test performed by the next generation sequencing and multiplex ligation-dependent probe amplification revealed a homozygous deletion in the CFHR1 and CFHR3 and a heterozygous variant of uncertain significance in CFI. During the following 2 weeks he normalized hematological parameters. He withdrew hemodialysis after the second dose of Eculizumab and normalized the renal function and the titers of anti FH antibodies 2 months later. After 6 months, with the patient in total remission, eculizumab was discontinued, whereas continued receiving prednisone during the first year and mofetil mycophenolate for 2 years without relapses. Currently he is in remission and free of treatment. Conclusion: Atypical HUS is a diagnosis of exclusion and STEC-HUS is the most important diagnose to rule out. Exceptionally, both situations can coexist and STEC can be the trigger of the disease in children with a genetic predisposition.

Drug Index Terms

(Based on full text)

creatinine, eculizumab, hemoglobin, immunoglobulin G, immunoglobulin M, lactate dehydrogenase, lipopolysaccharide, mycophenolate mofetil, mycophenolic acid, prednisone, **Shiga toxin**, urea

Non-drug Index Terms

(Based on full text)

anuria, **atypical hemolytic uraemic syndrome**, case report, child, clinical article, conference abstract, diagnosis, diarrhea, ELISA kit, enzyme linked immunosorbent assay, **Escherichia coli infection**, genetic predisposition, hematocrit, hematological parameters, hemodialysis, hemolytic uremic syndrome, high throughput sequencing, human, kidney function, laboratory test, male, multiplex ligation dependent probe amplification, multiplex polymerase chain reaction, remission, schistocyte, **Shiga toxin producing Escherichia coli**, vomiting

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Nephrol.
ISSN	0931041X
Source Type	Journal

Conference Name	56th ESPN Annual Meeting
Conference Location	Spain, Valencia
Conference Date	2024-09-24 to 2024-09-27
Source Publication Date	2024-09-01
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Publication Type	Conference Abstract
Page Range	S152-S153
Country of Author	Argentina
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00467-024-06472-9
CAS Registry Numbers	creatinine (19230-81-0 , 60-27-5) eculizumab (219685-50-4) hemoglobin (9008-02-0) immunoglobulin G (97794-27-9 , 308067-58-5) immunoglobulin M (9007-85-6) lactate dehydrogenase (9001-60-9) lactate dehydrogenase A mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) mycophenolic acid (23047-11-2 , 24280-93-1 , 37415-62-6) prednisone (53-03-2) Shiga toxin (75757-64-1) urea (57-13-6)

118. [Digital Poster Contouring practices and educational priorities in Southeast Asia: Results from a SEAROG survey](#)

Anderson A.C., Flores J., Obciana D., Thiagarajan M., Tangco E.C., Hlaing M., Calaguas M.J., Giselvania A., Chitapanarux I., Li B.

Radiother. Oncol. 2024 197: (S118-S120)

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Embase Open URL: [redirect to full text](#)

Abstract

Purpose/Objective: When using conformal treatment techniques (including 3D, IMRT, VMAT, SBRT, and SRS), additional training and time are required for accurate contouring of target and normal tissue volumes compared to 2-dimensional radiotherapy. Many radiation oncology centers in Southeast Asian Low- and Middle-Income Countries (LMICs) have implemented conformal planning techniques, but the patterns and priorities for contouring practices and education between consultants and trainees in this region are unclear. We report a cross-sectional survey on contouring practices and education priorities. **Material/Methods:** Members of the Southeast Asian Radiation Oncology Group and Nepalese radiation oncologists were invited to complete an English-language online survey designed by Rayos Contra Cancer, a non-profit organization focused on radiation oncology education in limited-resource settings. Questions were developed to assess needs and priorities for online remote contouring and planning education courses in the region. Questions included demographics; treatment techniques; time contouring target and normal volumes, and the number of separate normal organs contoured, across disease sites; as well as perceived needs and priorities for further training. Responses were summarized with descriptive statistics. **Results:** Of 127 initiated surveys, 121 completed surveys were analyzed. Respondents were from Malaysia (46; 38%), Indonesia (39; 32%), Thailand (22; 18%), Myanmar (12; 9.9%), and Nepal (2, 1.7%). Consultants (n = 79, 65%) were from Indonesia (38; 48%), Thailand (17; 22%), Malaysia (12; 15%), Myanmar (10; 13%), and Nepal (2; 2.5%); 92% practiced in the public sector. The three most common disease sites treated overall were Head/Neck (for 36% of consultants), Breast (for 30%), and Gynecologic (for 23%); most used conformal techniques (3D 95%; IMRT 71%; VMAT 71%; SRS 25%; SBRT 29%). Trainees (n = 42, 35%) were largely from Malaysia (81% of trainees); almost all had training in conformal treatment techniques (3D 93%; IMRT 79%; VMAT 50%; SRS 40%; SBRT 48%). For 3D, IMRT, and VMAT treatments, target and normal volumes were typically contoured by physicians. Almost all trainee respondents

reported contouring target (95%) and normal tissue volumes (98%). Targets were also contoured by medical physicists (in Malaysia and Indonesia) while normal volumes were also contoured by medical physicists (in Malaysia, Indonesia, and Myanmar), and therapists (in Malaysia and Indonesia). For consultants, the median time spent contouring targets ranged from 15-29 minutes (Breast, Gynecologic, Palliative) to more than 60 minutes (Head/Neck) (Figure 1). The median time for contouring normal volumes (Figure 2) ranged from 0-14 minutes (Palliative) and between 0-14 and 15-29 minutes (Breast) to 30-59 minutes (Head/Neck, Pediatric). Trainees spent more time contouring target and normal volumes for all sites except Breast and Palliative. Both consultants and trainees contoured median 6-10 normal organs for all sites except Gynecologic (median 1-5 for consultants; median between 1-5 and 6-10 for trainees), and Head/Neck (median 11-15 for both). Interest in contouring education and training was 100%. The top three areas of interest for contouring education overall were Head/Neck (86%), Thoracic/Lung (40%) and CNS disease sites (39%), similar between consultants and trainees. The highest priority for all was Head/Neck. Interest in education and training for plan evaluation was 98% overall, and 100% among trainees. The top three interests for plan evaluation education overall were Head/Neck (85%), CNS (42%) and Thoracic/Lung disease sites (36%), while Breast was the second most common interest for consultants (41%). Conclusion: Conformal techniques have been implemented at many centers in Southeast Asian LMICs, and the use of SRS/SBRT is more limited. Contouring is time-intensive for physicians and physicists, particularly for Head/Neck treatments. There is high interest in contouring and planning education for physicians. Keywords: contouring, conformal treatment, virtual training

Drug Index Terms

(Based on full text)
prednisone

Non-drug Index Terms

(Based on full text)
central nervous system, central nervous system disease, conference abstract, demographics, English (language), female, human, Indonesia, intensity modulated radiation therapy, Ireland, lung disease, Malaysia, medical physicist, middle income country, Myanmar, Nepal, Nepalese,

physician, physicist, public sector, radiation oncologist, radiation oncology, radiosurgery, radiotherapy, **Southeast Asia**, stereotactic body radiation therapy, Thailand

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Radiother. Oncol.
ISSN	18790887 (electronic), 01678140
Source Type	Journal
Conference Name	ESTRO meets Asia 2024
Conference Location	Malaysia, Kuala Lumpur
Conference Date	2024-08-23 to 2024-08-25
Source Publication Date	2024-08-01
Entry Date	2025-11-18 (Full record)
Publication Type	Conference Abstract
Page Range	S118-S120
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S0167814024042464
Publisher Item Identifier	S0167814024042464
Digital object identifier (DOI)	10.1016/S0167-8140(24)04246-4
CAS Registry Numbers	prednisone (53-03-2)

119. [Advanced-fractured osteoradionecrosis in long-survivor ENT cancer patients: a medical challenge?](#)

Delanian S.

Radiother. Oncol. 2024 194: (S1190-S1192)

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Abstract

Purpose/Objective: Mandibular osteoradionecrosis (ORN), limited in frequency today, is described in the literature on series of 22 to 87 patients, of which 60% of advanced cases including mucosal defect > 2.5 cm (or length > 2 teeth) and radiological depth of bone lysis beyond the inferior alveolar canal, with severe complications such as fracture, osteomyelitis, fistula, lower dental neuropathy. The treatment of these patients suffering with multiple co-morbidities (diabetes, neutropenia, HIV, .) is always complex. Radical surgery as hemi-mandibulectomy - osteocutaneous free flap reconstruction is an option when the risk of serial postoperative complications is not too high. During the last decade, based on pathophysiological concepts and PENTOCLO favorable therapeutic results [1], a retrospective study was realized on all inoperable patients with advanced-fractured ORN, treated at Hôpital Saint-Louis, single-center setting. **Material/Methods:** Eighty patients, 64 ± 9 years old, presented an advanced ORN within 7 ± 10 years after RT, bilateral in 31 (39%). Mandibular bone was exposed (mean 22 ± 13 mm) with mandibular fracture (Epstein stage IIIB) without healing for 15 ± 16 months, despite usual care with short antibiotherapy, local curettage and/or hyperbaric oxygen. The mandibular fracture site was the body and angle in 71% (57/80) of cases. It occurred after mechanical trauma (large sequestration, forced trismus, repeated surgeries), tooth extraction, acute step on chronic infection (cellulitis, abscess, purulence) and/or hypo-vascularization (carotid). All patients received a 6-week "DISINFILTRATING treatment" (phase 1) combining prednisone 20mg amoxicillin/ clavulanate 2g - ciprofloxacin 1g - fluconazole 50mg. A PENTOCLO combination (Pentoxifylline 800mg- Tocopherol 1000mg- Clodronate 1600mg) was further introduced (phase 2) until mucosal healing, helped until purulence control, if any, by a continuous clindamycin (300-600mg/day) prescription. Then, a maintenance TRICO treatment (clodronate 1600mg - colchicine 1mg) with cholecalciferol supplementation, was administered (phase 3), as a relay, in order to obtain superficial (avoid rebound effect by insufficient duration of treatment) and bone (strength of the callus) consolidation. **Results:** Treatments were tolerated. Symptomatic improvement was observed in all patients after phase 1. A complete mucosal healing was progressively achieved using PENTOCLO in a median time of 15 ± 7 months: (a) most patients underwent "spontaneous" removal of a bulky sequestra (mean 19 ± 11 mm) during phase 2 followup; (b) objective response combined a reduction then cessation of analgesics,

discontinuation of antibiotic therapy (mean 7 ±4 months), and closed skin fistulae. Then, phase 3 TRICO helped to improve or achieve bone healing : a progressive radiological consolidation of the fracture in 18-42 months for 90% of patients with unexpected delayed and solid bone callus (except for surgical catch-up for mobility, carcinologic evolution, or death). Conclusion: Advanced-fracture mandibular ORN may be conservatively and successfully managed in a curative intent. This cohort of 80 fragile patients, the largest ever described, has made it possible to define an innovative and effective therapeutic sequence, in real life, using “DESINFILTRANT- PENTOCLO -TRICO” combinations, whenever radical surgery was not an option. Keywords: radiation-induced toxicity, osteoradionecrosis References: Delanian: Complete restoration of refractory mandibular osteoradionecrosis by prolonged treatment with PENTOCLO. Int JRadiat Oncol Biol Phys 2011; 80: 832-9

Drug Index Terms

(Based on full text)

ciprofloxacin, clavulanic acid, clindamycin, clodronic acid, colchicine, colecalciferol, fluconazole, pentoxifylline, prednisone

Non-drug Index Terms

(Based on full text)

abscess, adult, aged, antibiotic therapy, callus, **cancer patient**, cellulitis, cohort analysis, comorbidity, complication, conference abstract, curettage, diabetes mellitus, drug therapy, **ear nose throat cancer**, female, fibrin glue, follow up, fracture healing, free tissue graft, hemimandibulectomy, human, hyperbaric oxygen therapy, major clinical study, male, mandible fracture, mandibular canal, neutropenia, osteomyelitis, **osteoradionecrosis**, postoperative complication, prescription, reoperation, retrospective study, sequestrum, skin fistula, suppuration, surgery, **survivor**, therapy, tooth extraction, treatment duration, trismus

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Radiother. Oncol.
ISSN	18790887 (electronic), 01678140
Source Type	Journal
Conference Name	ESTRO 2024

Conference Location	United Kingdom, Glasgow
Conference Date	2024-05-03 to 2024-05-07
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Entry Date	2025-11-18 (Full record)
Publication Type	Conference Abstract
Page Range	S1190-S1192
Country of Source	Netherlands
Language of Article	English
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Publisher Item Identifier	S0167814024010041
Publisher Item Identifier	S0167814024010041
Digital object identifier (DOI)	10.1016/S0167-8140(24)01004-1
CAS Registry Numbers	ciprofloxacin (85721-33-1 , 86393-32-0 , 128074-72-6 , 128074-76-0 , 192934-52-4 , 93107-08-5 , 86483-48-9 , 96186-80-0) clavulanic acid (58001-44-8) clindamycin (18323-44-9 , 21462-39-5) clodronic acid (10596-23-3 , 22560-50-5) colchicine (64-86-8) colecalciferol (1406-16-2 , 67-97-0) fluconazole (86386-73-4 , 123631-92-5) pentoxifylline (6493-05-6) prednisone (53-03-2)

120. [SBRT after progression during abiraterone plus prednisone. A subgroup analysis from ARTO trial](#)

Francolini G., Cataldo V.D., Garlatti P., Detti B., Caini S., Bruni A.,
 Ingrosso G., D'angelillo R.M., Alitto A.R., Augugliaro M., Triggiani L.,
 Parisi S., Facchini G., Simontacchi G., Desideri I., Meattini I.,
 Marvaso G., Jereczek-Fossa B.A., Alongi F., Arcidiacono F., Lancia A.,
 Genovesi D., Franzese C., Scorsetti M., Livi L.

Radiother. Oncol. 2024 194: (S2321-S2323)

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Abstract

Purpose/Objective: ARTO (NCT03449719) is a multicentre, randomized phase II trial testing the benefit of upfront stereotactic body radiation therapy (SBRT) in addition to abiraterone acetate plus prednisone (AAP) in oligometastatic Castration Resistant Prostate Cancer (CRPC) patients. Benefit was measured in terms of biochemical response and Progression Free Survival (PFS) in the experimental arm [1]. However, oligometastatic status persisted in a significant number of patients after progression under first line abiraterone plus prednisone treatment. Considering that patients may still benefit from SBRT in this setting, we performed a secondary analysis focusing on impact of SBRT received after progression in these cases, in order to explore if metastasis directed therapy maintained its impact in a later stage of disease.

Material/Methods: Patients affected by oligometastatic CRPC (≤ 3 non-visceral metastatic lesions) were enrolled in the trial and randomized 1:1 to receive either AAP alone (control arm) or associated with concomitant SBRT on all sites of disease (treatment arm). PFS1 and PFS2 were defined as time between AAP start and first progression event and time between first and second progression event, death or last follow up respectively. Cox regression analysis was performed to compare PFS1+PFS2 in patients treated with SBRT on oligo-progressive metastatic sites vs patients undergoing second line systemic therapy (primarily taxane chemotherapy). **Results:** One hundred fifty-seven patients were enrolled in ARTO trial. At last follow up (August 2023), 77 patients progressed after AAP treatment (23 vs 54 in the experimental vs control arm, respectively). Of these, 14 received best supportive care and were excluded from the analysis, 23 underwent SBRT on oligo-progressive metastatic sites (17 and 6 originally randomized to the experimental vs control arm, respectively), 40 were treated with second line systemic treatment. A second progression event was detected in 14 patients. Median PFS1+PFS 2 was 28 months (95% CI 25-34) and 23 months (95% CI 18-32) for patients undergoing SBRT vs second line treatment, respectively (HR 1.09, 95% CI 0.29-4.12, p-value 0.898).

Conclusion: Benefit of SBRT after progression was significantly reduced

if compared to ARTO trial results testing upfront SBRT at castration resistance occurrence. Indeed, no significant impact was detected for patients undergoing SBRT after further progression if compared to patients shifting to following systemic treatment. Considering the positive results of ARTO trial, these data suggest that the best integrated treatment strategy relies on upfront concomitant SBRT at systemic treatment start. Waiting for further progression after AAP start may decrease benefit of local approach in oligometastatic CRPC patients. Keywords: ARTO trial, Castrate resistant prostate cancer References: 1) Francolini G, Gaetano Allegra A, Detti B, Di Cataldo V, Caini S, Bruni A, Ingrosso G, D'Angelillo RM, Alitto AR, Augugliaro M, Triggiani L, Parisi S, Facchini G, Banini M, Simontacchi G, Desideri I, Meattini I, Valicenti RK, Livi L; ARTO Working Group members. Stereotactic Body Radiation Therapy and Abiraterone Acetate for Patients Affected by Oligometastatic Castrate-Resistant Prostate Cancer: A Randomized Phase II Trial (ARTO). JClin Oncol. 2023 Sep 21:JCO2300985.

Drug Index Terms

(Based on full text)

abiraterone, abiraterone acetate, **prednisone**

Non-drug Index Terms

(Based on full text)

adult, castration, castration resistant prostate cancer, conference abstract, controlled study, drug therapy, follow up, human, major clinical study, male, metastasis site, multicenter study, phase 2 clinical trial, progression free survival, **radiosurgery**, randomized controlled trial, second-line treatment, secondary analysis, special situation for pharmacovigilance, **stereotactic body radiation therapy**, systemic therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Radiother. Oncol.
ISSN	18790887 (electronic), 01678140
Source Type	Journal
Conference Name	ESTRO 2024
Conference Location	United Kingdom, Glasgow
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Publication Type	Conference Abstract
Page Range	S2321-S2323
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S016781402401048X
Publisher Item Identifier	S016781402401048X
Digital object identifier (DOI)	10.1016/S0167-8140(24)01048-X
CAS Registry Numbers	abiraterone (154229-19-3) abiraterone acetate (154229-18-2) prednisone (53-03-2)

121. [Secukinumab in acne conglobata: A treatment alternative](#)

Capdevila P.M., Morel L.N., Fernández A.T.

Pediatr. Dermatol. 2024 41: (27)

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Abstract

Objectives: N/A Method: N/A Introduction: Acne conglobata is a severe form of nodular acne that tends to be more severe on the back, chest and buttocks, but can also appears on the face and other sites. It may results in drainage, sinus tracts and severe scarring. Oral isotretinoin is first line treatment, but in those patients in whom isotretinoin is not well tolerated or have considerable side effects, the treatment can be difficult. Results: Case Report: A 16-years-old boy was referred to our Department for assessment and treatment of severe nodulocystic acne, compatible with acne conglobata on the face since he was 13 years. We started with isotretinoin 5 mg/day with oral prednisone. When increasing doses of isotretinoin up to 10 mg, our patient started with

joint pains, so our patient is referred to Rheumatology in order to rule out chronic inflammatory process. Body MRI is performed, without objectifying musculoskeletal alterations. FR, HLA-B27 and acute phase reactants are also negative. Given the appearance of side effects when increasing the dose of isotretinoin, it is decreased to the initial dose. After a few months with isotretinoin and prednisone, no improvement of the lesions is observed, so it is decided to start adalimumab, seeing an initial improvement, but reappearing the initial lesions in a few weeks. Given the minimal effectiveness of the treatment done so far, it was decided to start Secukinumab 300 mg. Currently, our patient, after 3 doses of Secukinumab, shows a considerable improvement in his acne, expecting a progressive substantial improvement in the following months. Discussion: Conclusion: In those patients with acne conglobata who do not respond adequately to isotretinoin, Secukinumab could be a good alternative as a second-line treatment, but studies are still lacking in this regard to consider it as such.

Drug Index Terms

(Based on full text)

acute phase protein, adalimumab, HLA B27 antigen, isotretinoin, paint, prednisone, **secukinumab**

Non-drug Index Terms

(Based on full text)

acne conglobata, **alternative medicine**, buttock, case report, clinical article, conference abstract, drug therapy, first-line treatment, human, major clinical study, male, MRI scanner, nuclear magnetic resonance imaging, scar tissue, second-line treatment, side effect, therapy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Dermatol.
ISSN	07368046
Source Type	Journal
Conference Name	23rd ESPD Congress
Conference Location	Slovakia, Kosice
Conference Date	2024-05-02 to 2024-05-04
Source Publication Date	2024-04-01

Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	27
Country of Author	Spain
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1111/pde.15585
CAS Registry Numbers	adalimumab (331731-18-1 , 1446410-95-2) isotretinoin (4759-48-2) prednisone (53-03-2) secukinumab (875356-43-7 , 875356-44-8 , 1229022-83-6)

122. [Unusual Imaging Presentation of Miliary Meningeal Tuberculosis](#)

Venkatakrishna S.S.B., Vasileiadi E., Siu Navarro Y.J., Villavicencio K.L.H., Schaeubinger M.M., Schoeman S., Otero H.J., Andronikou S. *Pediatr. Radiol.* 2024 54:1 (S95-S96)

Embase

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Abstract

Purpose or Case Report: Tuberculous meningitis (TBM) causes significant morbidity and mortality in young children. Early treatment can be initiated with MR imaging diagnosis. We present MR detectable miliary meningeal TB in two patients presenting in a developed world setting. Case 1: A 9 y/o girl of Haitian descent, developed fevers, cough, lethargy and seizures. Brain MRI demonstrated multiple, small, T2-dark, rim-enhancing lesions, associated with cranial nerve and leptomeningeal enhancement. Chest CT showed mediastinal lymphadenopathy, multiple small interstitial lung nodules and a splenic hypo enhancing lesion. CSF showed pleocytosis, low glucose, and high protein. She was started on 4-drug treatment for TBM and

dexamethasone. Serial bronchoalveolar lavage was Xpert MTB/RIF and acid-fast negative. Contact tracing revealed a remote positive contact with pulmonary tuberculosis from Haiti. Endobronchial US-guided biopsy of a subcarinal lymph node was positive for Xpert MTB PCR. Case 2: A 17 y/o female with Crohn's disease on adalimumab developed refractory ear infections despite multiple courses of antibiotics and underwent myringotomy, with negative aerobic ear fluid culture. She had two chronic skin lesions. Brain MRI, obtained due to persistent otorrhea, showed multiple, small, round, T2-dark lesions. CSF studies were unremarkable. CT chest, abdomen and pelvis showed left upper lobe tree-in-bud nodules, hypoattenuating splenic lesions and a left obturator internus abscess with adjacent osteomyelitis. She underwent CT guided aspiration of the obturator muscle collection, bronchoscopy with Bronchoalveolar Lavage (BAL), biopsy of her skin lesions and ear fluid aspiration. QuantiFERON Gold was positive. Ear fluid was Xpert MTB/RIF assay and acid-fast stain positive. She was started on 4-drug therapy and prednisone. Cultures from the ear fluid, skin tissue, muscle tissue and BAL showed growth of acid-fast bacilli. TB meningitis is most often due to blood borne spread of the disease from the lungs which may result in miliary nodules in the leptomeninges and brain. The two cases presented illustrate that although TBM is unusual in a developed world, we should suspect it in children of migrants as well as in patients with immunosuppression, where atypical imaging patterns including lack of typical basal enhancement and presence of miliary meningeal nodules may be seen.

Drug Index Terms

(Based on full text)

adalimumab, dexamethasone, glucose, prednisone

Non-drug Index Terms

(Based on full text)

adult, bronchoalveolar lavage fluid, bronchoscopy, case report, child, clinical article, conference abstract, contact examination, coughing, cranial nerve, Crohn disease, diagnosis, drug therapy, female, fever, Haiti, Haitian, human, human tissue, immunosuppressive treatment, intravenous drug administration, leptomeninges, lethargy, lung lavage, lung nodule, lung tuberculosis, mediastinum lymphadenopathy, migrant, MRI scanner, Mycobacterium tuberculosis test kit, myringotomy, nuclear magnetic resonance imaging, osteomyelitis,

otorrhea, pleocytosis, secondary prevention, seizure, skin defect, spleen injury, **tuberculous meningitis**, ultrasound guided biopsy

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Pediatr.Radiol.
ISSN	03010449
Source Type	Journal
Conference Name	67th Annual Meeting of the Society for Pediatric Radiology, SPR 2024
Conference Location	United States, Miami, FL
Conference Date	2024-04-13 to 2024-04-15
Source Publication Date	2024-04-01
Entry Date	2025-11-18 (Full record)
Publication Type	Conference Abstract
Page Range	S95-S96
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1007/s00247-024-05904-w
CAS Registry Numbers	adalimumab (331731-18-1 , 1446410-95-2) dexamethasone (50-02-2) glucose (50-99-7 , 84778-64-3 , 8027-56-3) prednisone (53-03-2)

123. [MANAGEMENT OF ISOTRETINOIN-INDUCED PSEUDO-ACNE FULMINANS](#)

Secrest A.M.

Australas. J.Dermatol. 2024 62:2 (227)

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Abstract

Introduction: Pseudo-acne fulminans is a rare side effect of isotretinoin. Less than 200 cases have been reported, commonly in males and most pronounced on the trunk. **Case:** An 11-year-old prepubertal female presented with severe comedonal acne. Topical tretinoin showed with minimal improvement. Her older brother and father had severe cystic acne requiring isotretinoin. She failed benzoyl peroxide, topical tazarotene, topical clindamycin, and oral doxycycline. Due to the significant psychologic distress her acne caused both her and her mother (her father had severe facial acne scarring), isotretinoin was started at 20 mg daily (0.4 mg/kg/d) and quickly increased to 40 mg daily (0.8 mg/kg/d). Within 4 weeks, she developed several, painful, sterile nodules and cysts on the cheeks, ears, and forehead. **Outcome:** Isotretinoin was stopped, and a 4-week prednisone taper from 40 mg daily was started. Intralesional triamcinolone 2.5 mg/ml was used with good response. Isotretinoin was restarted at 10 mg daily (0.2 mg/kg/d), and the pseudo-acne fulminans recurred, so another 6-week prednisone taper was started, causing a 10 kg weight gain. Ciclosporin was started at 3.3 mg/kg/d, and isotretinoin was again restarted at 10 mg daily and slowly increased to 40 mg over a year resulting in complete resolution of her acne. **Discussion:** Immunosuppressive agents are necessary for early and continued control of pseudo-acne fulminans following isotretinoin initiation. While prednisone offers rapid response, its side effects often impact quality of life in adolescent patients. Ciclosporin is an affordable, rapid-acting, and well-tolerated alternative for concomitant use with isotretinoin to manage pseudo-acne fulminans. Aaron Secrest is a US dermatologist working at Christchurch Hospital (NZ vocational registration pending).

Drug Index Terms

(Based on full text)

benzoyl peroxide, clindamycin, cyclosporine, doxycycline, **isotretinoin**, prednisone, retinoic acid, tazarotene, triamcinolone

Non-drug Index Terms

(Based on full text)

acne conglobata, acne cystica, **acne fulminans**, acne vulgaris,

adolescent, adverse drug reaction, body weight gain, case report, cheek, clinical article, complication, conference abstract, diagnosis, distress syndrome, drug therapy, female, forehead, human, male, quality of life, scar formation, scar tissue, side effect, topical drug administration

Comment: Valid ICSR

Additional Information

Abbreviated Journal Title	Australas. J.Dermatol.
ISSN	00048380
Source Type	Journal
Conference Name	New Zealand Dermatological Society (NZDSI) 2023 Annual Conference
Conference Location	New Zealand, Queenstown
Conference Date	2023-08-02 to 2023-08-05
Source Publication Date	2024-03-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	227
Country of Author	New Zealand
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.1111/ajd.14217
CAS Registry Numbers	benzoyl peroxide (94-36-0) clindamycin (18323-44-9 , 21462-39-5) cyclosporine (59865-13-3 , 63798-73-2 , 79217-60-0) doxycycline (10592-13-9 , 17086 28-1 , 564-25-0 , 94088-85-4) isotretinoin (4759-48-2) prednisone (53-03-2) retinoic acid (302-79-4)

tazarotene ([118292-40-3](#))

triamcinolone ([124-94-7](#))

124. [Delirium, Catatonia or Both? A Case of VZV Encephalitis Induced Delirium and Catatonia in an Older Adult with Polypharmacy](#)

Caetano C., Gleason-Vergados J., Cai H., Ouellet J.

J.Am. Geriatr. Soc. 2024 72: (S125)

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Abstract

Background: Catatonia and delirium are distinct neuropsychiatric syndromes with multiple potential etiologies; recent consideration has been given to their possible co-existence. We present a case of likely coexisting delirium and catatonia in an older patient with bipolar disorder with multiple underlying infectious etiologies. This case presentation reflects the challenge of differentiating hypoactive delirium and catatonia and selecting an appropriate therapy strategy. Case Report: A 75-year-old woman, previously dependent in IADLs and assisted in ADLs, presented with several days of altered mental status. Medical history included bipolar disorder, complicated by prior catatonia, rheumatoid arthritis on chronic prednisone and lumbar stenosis on methadone. She was febrile and found to have a positive urine culture. Despite appropriate antibiotics, her mental status worsened. She developed a dermatomal vesicular rash concerning for varicella zoster virus (VZV) and was started on oral antivirals. Due to concern that antipsychotics were contributing to hypoactive delirium, they were tapered. Lumbar puncture was then performed, showing varicella encephalitis, and she was treated with IV acyclovir. Poor mental status persisted after completion of therapy. At this point, catatonia was considered and was treated on several pharmacologic agents. After partial response, she was treated with electroconvulsive therapy (ECT), with full recovery to her baseline cognitive status. Conclusions: This report describes a woman presenting with delirium caused by a urinary tract infection who subsequently developed catatonia in the setting of change in psychoactive medications and varicella encephalitis. There are several important learning points in this

case. First, VZV encephalitis should be included on the differential for a patient with delirium and zoster rash, particularly older adults with impaired Tcell immunity (e.g. those with chronic steroids). Additionally, delirium and catatonia may present as independent or co-morbid entities that are difficult to differentiate, particularly in a vulnerable older adult. As such, health professionals should apply a nuanced approach to treatment including therapies for catatonia when appropriate, despite the risk of potentiating delirium with psychoactive medications (ie. Benzodiazepines, ambien, etc.).

Drug Index Terms

(Based on full text)

aciclovir, antibiotic agent, methadone, prednisone, steroid, zolpidem tartrate

Non-drug Index Terms

(Based on full text)

adult, adverse drug reaction, aged, altered state of consciousness, antiviral activity, bipolar disorder, case report, **catatonia**, chickenpox, clinical article, cognition, complication, conference abstract, daily life activity, **delirium**, diagnosis, drug therapy, drug withdrawal, electroconvulsive therapy, female, herpes zoster, **herpes zoster encephalitis**, human, hypoactive delirium, intravenous drug administration, lumbar puncture, medical history, **polypharmacy**, rash, rheumatoid arthritis, side effect, special situation for pharmacovigilance, therapy, unexpected outcome of drug treatment, urinary tract infection, urine culture, **Varicella zoster virus**, vesicular rash

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Am. Geriatr. Soc.
ISSN	00028614
Source Type	Journal
Conference Name	The American Geriatrics Society (AGS) 2024 Annual Scientific Meeting
Conference Location	Vitrual/Online
Conference Date	2024-05-09 to 2024-05-11

Source Publication Date	2024-01-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	S125
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
CAS Registry Numbers	aciclovir (59277-89-3 , 69657-51-8) methadone (1095-90-5 , 125-56-4 , 23142-53-2 , 297-88-1 , 76-99-3) prednisone (53-03-2) zolpidem tartrate (99294-93-6)

125. [Anifrolumab in Two Elderly Patients with Refractory Discoid Lupus Erythematosus](#)

Karagenova R., Timlin H.

J.Am. Geriatr. Soc. 2024 72: (S184)

Embase

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Abstract

Background: Discoid lupus erythematosus (DLE) accounts for 50-85% of cases of cutaneous lupus erythematosus (CLE).¹ Anifrolumab, a human monoclonal antibody against IFNAR1, received FDA approval for systemic lupus erythematosus (SLE) in 2021. We present the response and tolerability of anifrolumab in two elderly patients with DLE.

Methods: Case 1. A 69-year-old African American woman presented with refractory DLE characterized by scarring alopecia. She failed to improve on hydroxychloroquine, mycophenolate mofetil, methotrexate, azathioprine, betamethasone cream, and prednisone. She required intralesional triamcinolone injections every 2-3 months. The patient started on anifrolumab infusions once monthly. Case 2. A 65-year-old

African American woman with DLE characterized by scarring alopecia failed treatment with hydroxychloroquine, prednisone, mycophenolate mofetil, and clobetasol cream. The patient started on anifrolumab infusions once monthly. Results: Case 1. By the third month of infusions with anifrolumab, the patient's symptoms improved without further hair loss. She had one UTI that resolved with antibiotics. Triamcinolone injections were halted, and she was tapered off prednisone. Case 2. Seven months after initiating anifrolumab, the patient is improving well and is tapering off prednisone. Both patients tolerated anifrolumab without severe infections. No adverse effects were reported by either. Conclusions: The combination of older age² and SLE³ increases the risk of infections and therapy-related adverse events. We present two cases of DLE patients who failed multiple immunosuppressants but responded to anifrolumab. These patients had no serious infections. Further clinical trials should formally evaluate the response and tolerability of anifrolumab in older DLE patients.

Drug Index Terms

(Based on full text)

anifrolumab, antibiotic agent, azathioprine, betamethasone, clobetasol, human monoclonal antibody, hydroxychloroquine, methotrexate, mycophenolate mofetil, prednisone, triamcinolone

Non-drug Index Terms

(Based on full text)

adverse drug reaction, African American, aged, case report, clinical article, conference abstract, **discoïd lupus erythematosus**, drug administration, drug therapy, drug withdrawal, evening dosage, female, Food and Drug Administration, hair loss, human, immunosuppressive treatment, intravenous drug administration, multiple cycle treatment, oral drug administration, scarring alopecia, side effect, skin lupus erythematosus, special situation for pharmacovigilance, systemic lupus erythematosus, topical drug administration, unexpected outcome of drug treatment

Comment: Invalid ICSR

ADDITIONAL INFORMATION

Abbreviated Journal Title J.Am. Geriatr. Soc.

ISSN 00028614

Source Type Journal

Conference Name	The American Geriatrics Society (AGS) 2024 Annual Scientific Meeting
Conference Location	Vitrual/Online
Conference Date	2024-05-09 to 2024-05-11
Source Publication Date	2024-01-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	S184
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
CAS Registry Numbers	anifrolumab (1326232-46-5) azathioprine (446-86-6 , 55774-33-9) betamethasone (378-44-9) clobetasol (25122-41-2) hydroxychloroquine (118-42-3 , 525-31-5 , 137433-23-9 , 137433-24-0) methotrexate (15475-56-6 , 59-05-2 , 7413-34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) mycophenolate mofetil (116680-01-4 , 128794-94-5 , 115007-34-6) prednisone (53-03-2) triamcinolone (124-94-7)

126. [Polymyalgia rheumatica symptoms mimic late-onset rheumatoid arthritis](#)

Shu S., Takahashi P.

J.Am. Geriatr. Soc. 2024 72: (S115-S116)

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Abstract

Background: Polymyalgia rheumatica (PMR) is a common rheumatic disease with peak incidence between ages 70 and 80. The classical proximal muscle symptoms may mimic other disease processes, such as late-onset rheumatoid arthritis and solid malignancies, and must not be overlooked. Here we present a case of an older adult presenting with proximal myalgias refractory to corticosteroid therapy. **Case**

Description: An 86-year-old woman with a medical history significant for hypertension and chronic kidney disease presented to the geriatrics clinic for evaluation of bilateral shoulder myalgia in absence of trauma nor injury. Physical exam did not reveal swelling or synovitis in the hands or wrists. Laboratory testing revealed elevated C-reactive protein (CRP) to 28.2 and erythrocyte sedimentation rate (ESR) to 45. Her primary physician started prednisone 15mg daily for one month with significant improvement of her symptoms. However, her pain persisted through multiple attempts at prednisone taper. Interval blood tests showed persistent elevations of CRP (43.5) and ESR (45) despite moderate prednisone use. She developed extensive right lower extremity DVT, prompting investigation into malignant causes. CT abdomen pelvis revealed no suspicious solid renal nor hepatic masses. She subsequently developed boutonniere deformity in her bilateral digits, favoring a presentation of late-onset rheumatoid arthritis (LORA). Blood tests revealed positive rheumatoid factor and CCP antibodies. The rheumatology team initiated oral methotrexate 15mg weekly and daily folic acid supplementation. **Discussion:** Providers often diagnose PMR; however, this diagnosis can be revised in up to 20% of cases. Bilateral shoulder pain is present in up to 95% of cases of PMR. Similarly, shoulder arthritis has been observed in the early disease course of LORA in up to 23% of cases. LORA will present with more large and proximal joint involvement, rather than classical peripheral polyarthritis of the hands and feet. It is prudent to have a broadened differential with individuals presenting with PMR symptoms, and even more crucial to consider alternative diagnoses in persistent symptoms refractory to treatment. This case highlights the importance of familiarizing the PMR-like phenotype of LORA in order for early recognition, and the need for further consideration of adverse safety profiles of treatment options for this population.

Drug Index Terms

(Based on full text)

C reactive protein, folic acid, methotrexate, prednisone, rheumatoid factor

Non-drug Index Terms

(Based on full text)

adult, aged, boutonniere deformity, case report, chronic kidney failure, clinical article, conference abstract, corticosteroid therapy, diagnosis, drug therapy, erythrocyte sedimentation rate, female, human, hypertension, laboratory test, medical history, myalgia, phenotype, physical examination, polyarthritis, rheumatic disease, **rheumatic polymyalgia**, **rheumatoid arthritis**, shoulder arthritis, shoulder pain, solid malignant neoplasm, synovitis

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	J.Am. Geriatr. Soc.
ISSN	00028614
Source Type	Journal
Conference Name	The American Geriatrics Society (AGS) 2024 Annual Scientific Meeting
Conference Location	Vitrual/Online
Conference Date	2024-05-09 to 2024-05-11
Source Publication Date	2024-01-01
Entry Date	2025-11-17 (Full record)
Publication Type	Conference Abstract
Page Range	S115-S116
Country of Author	United States
Country of Source	Netherlands
Language of Article	English
Language of Summary	English
CAS Registry Numbers	C reactive protein (9007-41-4) folic acid (59-30-3 , 6484-89-5)

methotrexate([15475-56-6](#), [59-05-2](#), [7413-34-5](#), [7532-09-4](#), [6745-93-3](#), [51865-79-3](#), [60388-53-6](#))
prednisone ([53-03-2](#))
rheumatoid factor ([9009-79-4](#))

127. [POEMS syndrome: diagnostic delay and successful treatment with lenalidomide, cyclophosphamide and prednisone followed by autologous peripheral stem cell transplantation](#)

Ordway S., Gilbert L., Wanko S.

[In Process] BMJ Case Rep. 2018 2018:
Embase, MEDLINE, NURSING

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Abstract

Polyneuropathy, organomegaly, endocrinopathy, monoclonal gammopathy and skin changes (POEMS) syndrome is a rare systemic disease, often unrecognised in the primary care setting. POEMS syndrome is associated with plasma cell dyscrasias and upregulation of vascular endothelial growth factor leading to systemic oedema, papilloedema and pulmonary hypertension. A wide constellation of presenting symptoms often leads to late diagnosis. Unrecognised and untreated disease rapidly leads to death from neuropathic exhaustion or cardiopulmonary failure. Treatment is extrapolated from other plasma cell dyscrasias such as multiple myeloma. Autologous peripheral blood stem cell transplantation (PBSCT) is often an important component of treatment. There is no established standard of care for POEMS syndrome. Therapies include lenalidomide, bortezomib and targeted monoclonal antibodies. We present a patient with POEMS syndrome who achieved rapid complete response to triple therapy consisting of lenalidomide, cyclophosphamide and prednisone, followed by high-dose chemotherapy and PBSCT.

Drug Index Terms

bortezomib, cyclophosphamide, lenalidomide, monoclonal antibody, prednisone, vasculotropin

Non-drug Index Terms

adult, aged, article, **autologous peripheral blood stem cell transplantation**, case report, clinical article, controlled study, delayed diagnosis, diagnosis, drug combination, drug dose, drug megadose, drug therapy, human, human cell, human tissue, low drug dose, male, **monoclonal gammopathy of undetermined significance**, multiple myeloma, **neuropathy**, papilledema, **peripheral neuropathy**, plasma cell dyscrasia, **POEMS syndrome**, pulmonary hypertension, special situation for pharmacovigilance, therapy, upregulation

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	BMJ Case Rep.
ISSN	1757790X (electronic)
Source Type	Journal
Source Publication Date	2018-01-01
Entry Date	2025-11-21 (In process)
Publication Type	Article
Page Range	
Country of Author	United States
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier (DOI)	10.1136/bcr-2017-223168 ⁶⁸
MEDLINE PMID	30054320
Article Number	bcr-2017-223168
Number of References	19

128. [Inhibition of selective adhesion molecules in treatment of inflammatory bowel disease](#)

Ghosh N., Chaki R., Mandal S.C.

Int. Rev. Immunol. 2012 31:5 (410-427)

Embase, MEDLINE

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Abstract

Lymphocyte infiltration into the intestinal tract in inflammatory bowel disease (IBD) is mediated by interaction between $\alpha 4$ integrin and its specific ligands. Development of monoclonal antibodies against $\alpha 4$ integrin allowed targeting of lymphocyte trafficking into the intestine as a novel therapeutic intervention. Natalizumab, vedolizumab, alicaforsen AJM300, rhuMAb B7, CCX282-B, and PF-00547,659 are few of monoclonal antibodies that have shown high promise in trials with the potential for more attractive benefit:risk ratio than currently available therapies. In this review, an attempt is made to underline the therapeutic potential and the safety of anti-adhesion molecule treatment in IBD. © 2012 Informa Healthcare USA, Inc.

Drug Index Terms

(Based on full text)

agents used in inflammatory bowel disease (clinical trial, drug therapy, pharmacology), ajm 300 (adverse drug reaction, clinical trial, drug therapy, oral drug administration, pharmacology), alicaforsen (drug therapy, intravenous drug administration, pharmacology), ccx 282b, **cell adhesion molecule** (endogenous compound), endothelial leukocyte adhesion molecule 1 (endogenous compound), infliximab (clinical trial, drug therapy), integrin (endogenous compound), intercellular adhesion molecule 1 (endogenous compound), L selectin (endogenous compound), monoclonal antibody (adverse drug reaction, clinical trial, drug therapy, oral drug administration, pharmacology), natalizumab (clinical trial, drug comparison, drug therapy, intravenous drug administration, pharmacology), PADGEM protein (endogenous compound), pf 00547 659, ontamalimab (drug therapy, intravenous drug administration, pharmacology, subcutaneous drug administration), prednisone (clinical trial, drug therapy), unclassified drug, vascular cell adhesion molecule 1 (endogenous compound), vedolizumab (clinical trial, drug comparison, drug development, drug

therapy, pharmacology), vercirnon (adverse drug reaction, drug therapy, pharmacology)

Non-drug Index Terms

(Based on full text)

article, cell homing, cell migration, corticosteroid therapy, Crohn disease (drug therapy), drug efficacy, drug mechanism, drug response, drug safety, drug tolerability, **enteritis** (disease management, drug therapy), human, intestine mucosa, intracellular signaling, leukocyte, leukocyte rolling, nonhuman, priority journal, protein binding, protein expression, protein function, protein protein interaction, quality of life, randomized controlled trial (topic), remission, Short Form 36, ulcerative colitis (drug therapy), unspecified side effect (side effect)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Int. Rev. Immunol.
ISSN	15635244 (electronic), 08830185
CODEN	IRIME
Source Type	Journal
Source Publication Date	October 2012
Entry Date	2025-11-19 (Full record), 2012-10-31 (Article in Press/In process)
Publication Type	Tombstone
Page Range	410-427
Country of Author	India
Country of Source	United Kingdom
Language of Article	English
Language of Summary	English
Digital object identifier(DOI)	10.3109/08830185.2012.690794
MEDLINE PMID	23083349
Number of References	90
Drug Tradenames	ajm 300, ccx 282b, isis 2302, mln 02, pf 00547 659

CAS Registry Numbers	alicaforfen (142442-63-5 , 185229-68-9 , 331257-52-4) endothelial leukocyte adhesion molecule 1 (128875-25-2) infliximab (170277-31-3) intercellular adhesion molecule 1 (126547-89-5) L selectin (126880-86-2) natalizumab (189261-10-7) prednisone (53-03-2) vedolizumab (943609-66-3) verciron (698394-73-9 , 886214-18-2)
Clinical Trial Numbers	ClinicalTrials.gov (NCT00783692 , NCT00790933)

129. [Immune therapy in autism: Historical experience and future directions with immunomodulatory therapy](#)

Chez M.G., Guido-Estrada N.

Neurotherapeutics 2010 7:3 (293-301)

Embase, MEDLINE

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Abstract

Autism affects 1 in 110 new births, and it has no single etiology with uniform agreement. This has a significant impact on the quality of life for individuals who have been diagnosed with autism. Although autism has a spectrum quality with a shared diagnosis, it presents a uniquely different clinical appearance in each individual. Recent research of suspected immunological factors have provided more support for a probable immunological process or for processes that may play a role in the acquisition of an autistic condition. These factors include prenatal, genetic, and postnatal findings, as well as the discovery of a dysfunctional chronic pro-inflammatory state in brain tissue and cerebrospinal fluid in subsets of autistic patients. These findings offer new theories that may lead to the development of disease modification or preventative therapeutic options in the near future. This article

reviews prenatal, genetic, and observed immune aspects of the autism condition that may be risk factors in the presentation of the autistic clinical phenotype. Historical immune interventions in autism are reviewed and potential new therapies and interventions are discussed.
© 2010 The American Society for Experimental NeuroTherapeutics, Inc.

Drug Index Terms

(Based on full text)

acetylcysteine (pharmacology), amantadine (drug therapy, pharmacology), autoantibody (endogenous compound), carnosine (drug therapy, pharmacology), corticotropin (drug therapy, parenteral drug administration), cycloserine (drug therapy, pharmacology), etanercept (drug therapy, parenteral drug administration), glutamic acid (endogenous compound), immunoglobulin (drug therapy, intravenous drug administration, pharmacology), immunoglobulin G (endogenous compound), interleukin 1alpha (endogenous compound), interleukin 1beta (endogenous compound), lenalidomide (drug therapy), maternal antibody (endogenous compound), memantine (drug therapy, pharmacology), metronidazole (drug combination, drug therapy), minocycline (drug therapy), n methyl dextro aspartic acid receptor (endogenous compound), phosphatidylinositol 3,4,5 trisphosphate 3 phosphatase (endogenous compound), pioglitazone (oral drug administration), prednisolone (drug therapy), prednisone (adverse drug reaction, drug combination, drug therapy, drug toxicity), tumor necrosis factor (endogenous compound), valproic acid (drug therapy, pharmacology), vancomycin (pharmacology)

Non-drug Index Terms

(Based on full text)

antibody response, article, **autism** (congenital disorder, disease management, drug therapy, etiology), colitis (drug therapy), comorbidity, corticosteroid therapy, Cushingoid syndrome (side effect), cytokine production, drug mechanism, drug pulse therapy, EEG abnormality, epilepsy (drug therapy), gene mutation, genetic risk, human, immunoglobulin blood level, **immunotherapy**, inflammation, lymphoproliferative disease (drug therapy), nonhuman, priority journal, quality of life, risk factor, treatment duration, treatment response

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title Neurotherapeutics

ISSN	19337213
Source Type	Journal
Source Publication Date	July 2010
Entry Date	2025-11-21 (Full record), 2010-08-03 (Article in Press/In process)
Publication Type	Article
Page Range	293-301
Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S1878747923002258
Publisher Item Identifier	S1878747923002258
Digital object identifier (DOI)	10.1016/j.nurt.2010.05.008
MEDLINE PMID	20643382
Number of References	91
Drug Tradenames	revlimid (Celgene, United States)
Drug Manufacturers	Celgene (United States)
CAS Registry Numbers	acetylcysteine (616-91-1 , 89344-48-9) amantadine (665-66-7 , 768-94-5) carnosine (305-84-0) corticotropin (11136-52-0 , 9002-60-2 , 9061-27-2) cycloserine (339-72-0 , 68-39-3 , 68-41-7) etanercept (185243-69-0 , 200013-86-1 , 2055118-96-0) glutamic acid (11070-68-1 , 138-15-8 , 56-86-0 , 6899-05-4) immunoglobulin (9007-83-4) immunoglobulin G (97794-27-9 , 308067-58-5) lenalidomide (191732-72-6 , 1243329-97-

[6](#), [202271-91-8](#), [874946-00-6](#))
memantine ([19982-08-2](#), [41100-52-1](#),
[51052-62-1](#))
metronidazole ([39322-38-8](#), [443-48-1](#),
[69198-10-3](#))
minocycline ([10118-90-8](#), [11006-27-2](#),
[13614-98-7](#))
phosphatidylinositol 3,4,5 trisphosphate 3
phosphatase ([210488-47-4](#))
pioglitazone ([105355-27-9](#), [111025-46-8](#),
[1266523-09-4](#), [105390-47-4](#), [112529-15-4](#))
prednisolone ([50-24-8](#))
prednisone ([53-03-2](#))
valproic acid ([1069-66-5](#), [99-66-1](#))
vancomycin ([1404-90-6](#), [1404-93-9](#))

130. [Collagenous sprue is not always associated with dismal outcomes:
A clinicopathological study of 1Gpatients](#)

Vakiani E., Arguelles-Grande C., Mansukhani M.M., Lewis S.K.,
Rotterdam H., Green P.H., Bhagat G.

Mod. Pathol. 2010 23:1 (12-26)

Embase, MEDLINE

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Abstract

Collagenous sprue is associated with high morbidity; however, the etiology of this disorder is unclear. Data regarding the pathological and clinical manifestations of patients with collagenous sprue are also limited. We, thus, undertook this study to gain insight into the etiology, disease manifestations and outcomes of collagenous sprue. We searched our departmental database (1999-2008) to identify cases of collagenous sprue and to obtain clinical and laboratory data. Small bowel histology, including thickness of subepithelial collagen, intra-epithelial lymphocyte phenotype and results of T-cell clonality assays were evaluated. Nineteen patients (15 women, 4 men, age 22-80 years, mean 57 years) were identified. Seventeen (89%) had celiac disease and two had unclassified sprue; 9 of 17 (53%) celiac disease patients

had refractory disease; 5 of 15 (33%) lacked diarrhea (atypical presentation), including 2 of 6 (33%) with active (untreated) celiac disease and 3 of 9 (33%) with refractory celiac disease. Autoimmune disorders were seen in 12 of 19 (63%) patients and microscopic colitis (n7), lymphocytic gastritis (n2) or collagenous gastritis (n2) were seen in nine patients. Subepithelial collagen thickness was mildly (n6), moderately (n10), or markedly (n3) increased and villous atrophy was total (n13) or subtotal (n6). Phenotypically aberrant intraepithelial lymphocytes were not detected in any case. Polymerase chain reaction analysis showed a dominant T-cell clone in the only patient with refractory celiac disease type II. Histological improvement occurred in 7 of 11 (64%) patients. Overall, 8 of 19 (42%) responded to gluten-free diet, including 2 of 9 (22%) with refractory celiac disease and 10 responded to immunomodulatory therapy, including 6 of 9 (67%) with refractory celiac disease. Only one patient died from complications of refractory celiac disease. No patient developed lymphoma. The vast majority of our patients with collagenous sprue had celiac disease. Although, many patients required immunomodulatory therapy for symptom control, a subset responded to gluten-free diet alone. In our experience, collagenous sprue patients had relatively good clinical outcomes. © 2010 USCAP, Inc. All rights reserved.

Drug Index Terms

(Based on full text)

azathioprine (drug therapy), budesonide (drug therapy), collagen (endogenous compound), cyclosporine (drug therapy), prednisone (drug therapy)

Non-drug Index Terms

(Based on full text)

adult, aged, anemia, arthritis, article, autoimmune hepatitis, celiac disease, cellular distribution, clinical article, collagenous colitis, **collagenous sprue** (drug therapy), dermatitis herpetiformis, diarrhea, disease duration, female, gastritis, gluten free diet, histopathology, human, human tissue, immunoglobulin A nephropathy, immunoglobulin deficiency, immunomodulation, insulin dependent diabetes mellitus, intestine villus atrophy, lymphocyte clone, lymphocytic colitis, male, microscopic colitis, mortality, neutrophil, outcome assessment, peripheral neuropathy, pernicious anemia, phenotype, polymerase chain reaction, priority journal, psoriasis, **small**

intestine disease (drug therapy), small intestine mucosa, T lymphocyte, treatment response

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Mod. Pathol.
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CODEN	MODPE
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Source Publication Date	January 2010
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Country of Author	United States
Country of Source	United States
Language of Article	English
Language of Summary	English
Publisher Item Identifier	S0893395222027764
Publisher Item Identifier	S0893395222027764
Digital object identifier(DOI)	10.1038/modpathol.2009.151
MEDLINE PMID	19855376
Number of References	47
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) budesonide (51333-22-3 , 51372-29-3) collagen (9007-34-5) prednisone (53-03-2)

131. [Comparative Study Between Oral Azathioprine and Intravenous Cyclophosphamide Pulses in the Treatment of Idiopathic Pulmonary Fibrosis](#)

Roig V., Herrero A., Arroyo-Cózar M., Vielba D., Juarros S., Macías E.

Arch. Bronconeumol. 2010 46:1 (15-19)

Embase, MEDLINE

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Embase Open URL: [redirect to full text](#)

Abstract

Introduction: The purpose of the study was to establish the efficacy of treatment with intravenous cyclophosphamide pulses and oral corticoids in idiopathic pulmonary fibrosis, compared with the standard treatment with corticoids and azathioprine. **Patients and method:** A

prospective, non-controlled study with 2 parallel groups. One group received prednisone plus oral azathioprine for 24 months (AZA group). The second one (CIC group) received prednisone plus intravenous cyclophosphamide pulses (6 monthly and 6 three-monthly). The primary outcome was survival or period without need for a transplant at 36 months. The secondary outcomes were the forced vital capacity, the carbon monoxide diffusing capacity, and baseline arterial oxygen pressure at 3, 6, 12, 18, 24, 30, and 36 months. **Results:** A total of 46 patients were included in the study, of whom 25 were assigned to the AZA group and 21 to the CIC group. The survival or absence of lung transplant at 36 months was 44% in the AZA group and 76% in the CIC group ($P=0.028$). The forced vital capacity was worse in 8.8% of the AZA group compared to 6.7% in the CIC group ($P=0.16$). The carbon monoxide diffusing capacity worsened in 11.81% of patients of the AZA group and in 4.6% of those in the CIC group ($P=0.0569$). No significant differences were observed in the arterial oxygen pressure. There was one dropout of treatment with prednisone. **Conclusions:** Treatment with intravenous cyclophosphamide pulses produced a significant improvement in survival. There were no significant differences in the lung function parameters or gas exchange. Neither of the cytostatics had serious side effects. © 2009 SEPAR.

Original (Non-English) Title

Estudio comparativo entre azatioprina oral y pulsos intravenosos de ciclofosfamida en el tratamiento de la fibrosis pulmonar

idiopática

Drug Index Terms

(Based on full text)

azathioprine (adverse drug reaction, clinical trial, drug combination, drug comparison, drug dose, drug therapy, oral drug administration), carbon monoxide, **cyclophosphamide** (adverse drug reaction, clinical trial, drug combination, drug comparison, drug dose, drug therapy, intravenous drug administration), prednisone (clinical trial, drug combination, drug therapy, oral drug administration)

Non-drug Index Terms

(Based on full text)

adult, aged, arterial oxygen tension, article, clinical article, clinical trial, comparative study, controlled clinical trial, controlled study, dose response, drug dose reduction, drug efficacy, drug withdrawal, female, **fibrosing alveolitis** (drug therapy), forced expiratory volume, human, leukopenia (side effect), liver toxicity (side effect), lung diffusion capacity, lung function test, lung gas exchange, lung toxicity (side effect), lung transplantation, male, survival rate, treatment duration, treatment outcome, urinary tract disease (side effect)

Comment: Invalid ICSR

Additional Information

Abbreviated Journal Title	Arch. Bronconeumol.
ISSN	15792129 (electronic), 03002896
CODEN	ARBRD
Source Type	Journal
Source Publication Date	January 2010
Entry Date	2025-11-21 (Full record), 2009-10-30 (Article in Press/In process)
Publication Type	Article
Page Range	15-19
Country of Author	Spain
Country of Source	Spain
Language of Article	Spanish
Language of Summary	English, Spanish
Publisher Item Identifier	S1579212910700045

Publisher Item Identifier	S1579212910700045
Digital object identifier(DOI)	10.1016/j.arbres.2009.09.015
MEDLINE PMID	19879035
Number of References	21
CAS Registry Numbers	azathioprine (446-86-6 , 55774-33-9) carbon monoxide (630-08-0) cyclophosphamide (50-18-0 , 6055-19-2) prednisone (53-03-2)

132. [Subscapular fibrohistiocyoma: Casual finding in a ¹⁸F-FDG-PET/CT scan due to solitary pulmonary nodule](#)

Banzo J., Ubieto M.A., Velilla J., Prats E., Razola P., Tardín L., Andrés A., Santapau A., Parra A.

Rev. Esp. Med. Nucl. 2010 29:2 (93-94)

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Original (Non-English) Title

Fibrohistiocitoma subescapular: hallazgo casual en una exploración PET/TAC con ¹⁸F-FDG por nódulo pulmonar solitario

Drug Index Terms

(Based on full text)

acenocoumarol (drug therapy), CD68 antigen (endogenous compound), fluorodeoxyglucose f 18, glucose 6 phosphate dehydrogenase (endogenous compound), methotrexate (drug combination, drug therapy), phospholipid antibody (endogenous compound), prednisone (drug combination, drug therapy)

Non-drug Index Terms

(Based on full text)

adult, antiphospholipid syndrome, article, **benign fibrous histiocytoma** (diagnosis), case report, **computer assisted emission tomography**, dermatofibroma, endometriosis (surgery), female, fibrosarcoma (drug therapy), glucose 6 phosphate dehydrogenase

deficiency, hemangioma, histopathology, human, hysterectomy, lung nodule, oophorectomy, ovary tumor (surgery), systemic lupus erythematosus, tumor thrombus (complication, drug therapy, prevention)

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Additional Information

Abbreviated Journal Title	Rev. Esp. Med. Nucl.
ISSN	1578200X (electronic), 02126982
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Source Type	Journal
Source Publication Date	March 2010/April 2010
Entry Date	2025-11-21 (Full record), 2009-12-04 (Article in Press/In process)
Publication Type	Article
Page Range	93-94
Country of Author	Spain
Country of Source	Spain
Language of Article	Spanish
Publisher Item Identifier	S1578200X10700152
Publisher Item Identifier	S1578200X10700152
Digital object identifier(DOI)	10.1016/j.remne.2009.10.002
MEDLINE PMID	19962216
Number of References	2
CAS Registry Numbers	acenocoumarol (152-72-7) fluorodeoxyglucose f 18 (63503-12-8) glucose 6 phosphate dehydrogenase (37259-83-9 , 9001-40-5) methotrexate(15475-56-6 , 59-05-2 , 7413 - 34-5 , 7532-09-4 , 6745-93-3 , 51865-79-3 , 60388-53-6) prednisone (53-03-2)